

PTCB

Study Guide

Q&A Part 1

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1. **Q:** A prescription reads "levothyroxine 75 mcg i po qd" but the technician selects a bottle labeled 0.075 mg; what should you do FIRST during order entry? **A:** Enter/dispense as 75 mcg (since $0.075 \text{ mg} = 75 \text{ mcg}$) and double-check the unit conversion to prevent a 1000-fold error.
2. **Q:** A provider e-prescribes "metformin ER 500 mg take 2 tabs po qpm with evening meal #60"; what is the correct days' supply to enter? **A:** Enter 30 days because 2 tablets daily means $60 \text{ tablets} \div 2/\text{day} = 30 \text{ days}$, which supports accurate billing and refill timing.
3. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg 1 tab po bid x 10 days" but the quantity field is missing; what is the BEST technician action? **A:** Calculate and request/route for pharmacist clarification for quantity 20 tablets ($2/\text{day} \times 10 \text{ days}$) because missing quantity must be resolved before dispensing.
4. **Q:** A label direction is typed as "take 1 tab qd" for furosemide, but the hardcopy sig says "i tab po bid"; what should you do NEXT? **A:** Stop and correct the sig to BID (after confirming the original prescription) because BID vs QD is a common order-entry error that changes the dose.
5. **Q:** An inhaler is prescribed: "fluticasone/salmeterol 250/50 Diskus inhale 1 puff bid" and the device contains 60 blisters; what days' supply should be billed? **A:** Bill 30 days because 2 doses/day means $60 \text{ doses} \div 2/\text{day} = 30 \text{ days}$, preventing refill-too-soon rejections.
6. **Q:** An insulin vial contains 10 mL of U-100 insulin lispro and the directions are "inject 12 units before meals TID"; what days' supply should you enter? **A:** Enter 27 days because the vial has 1000 units total and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 \approx 27 \text{ days}$ (rounded down to avoid overbilling).
7. **Q:** A prescription reads "prednisone 20 mg ii po qd x 5 days #10"; the patient profile shows a new allergy to prednisone entered today; what should you do FIRST? **A:** Stop processing and alert the pharmacist because an allergy conflict requires pharmacist assessment before the medication is dispensed.
8. **Q:** A sig uses Roman numerals and abbreviations: "clindamycin 300 mg i cap po qid"; the technician is unsure if "qid" was misread as "qd"; what is the BEST next step? **A:** Verify the original prescription and enter QID (4 times daily) only if clearly written, because QID vs QD is a high-risk abbreviation error.
9. **Q:** During product selection you scan NDC for "hydralazine" but the screen shows "hydroxyzine" and the names look similar (LASA); what should you do FIRST?

A: Stop filling and select the correct NDC/drug, then rescan to verify, because LASA mix-ups are prevented by barcode/NDC confirmation.

10. **Q:** A prescription is written for "oxycodone/APAP 5/325 take 1 tab q6h prn pain #120 refills: 2"; what should you do NEXT as the technician? **A:** Hold the order and refer to the pharmacist because Schedule II prescriptions cannot be refilled and require pharmacist intervention for validity/compliance.
11. **Q:** A new eRx is for "Zoloft 50 mg take 1 tab daily," but the product chosen in the system is sertraline 50 mg; what should you do FIRST? **A:** Confirm Zoloft is sertraline and proceed with the correct generic entry because matching brand-generic prevents unnecessary pharmacist interruptions and billing/label errors.
12. **Q:** While typing an eRx for "Cymbalta 60 mg daily," you notice the patient profile already has duloxetine 60 mg active from another prescriber; what is the BEST next step? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because duplicate SNRI therapy may require clinical review before dispensing.
13. **Q:** A prescription says "Lantus Solostar inject 20 units subcut qhs," and the box contains five 3 mL pens of U-100; what days' supply should you enter? **A:** Enter 75 days because $5 \text{ pens} \times 3 \text{ mL} = 15 \text{ mL} = 1500 \text{ units}$ and $1500 \div 20 \text{ units/day} = 75 \text{ days}$ for accurate billing.
14. **Q:** During product selection for "Lamictal 25 mg," you pull "Lamisil 250 mg" from the shelf because the names look similar; what should you do FIRST? **A:** Stop and re-select the correct medication using barcode/NDC verification because this LASA error (lamotrigine vs terbinafine) is a high-risk wrong-drug mix-up.
15. **Q:** An eRx comes in for "Bactrim DS 1 tab bid x 7 days," and the allergy profile shows "sulfonamides: rash"; what should you do FIRST? **A:** Stop processing and notify the pharmacist because sulfamethoxazole/trimethoprim is a sulfonamide and requires pharmacist assessment before dispensing.
16. **Q:** A provider sends "Keflex 500 mg take i cap po qid x 10 days," and you need to calculate quantity to dispense; what quantity should be entered? **A:** Enter 40 capsules because $4 \text{ doses/day} \times 10 \text{ days} = 40$ to ensure the dispensed amount matches the intended course.
17. **Q:** You receive "Eliquis 5 mg take 1 tab po bid #60," but the patient's active meds include warfarin; what is the BEST technician action? **A:** Hold the fill and refer to the pharmacist for duplicate anticoagulant therapy review because

concurrent warfarin and apixaban increases bleeding risk and needs clinical direction.

18. **Q:** A prescription is written "hydroxyzine 25 mg take 1 tab tid prn anxiety," but the technician accidentally types "hydralazine"; what should you do NEXT when you notice the mismatch? **A:** Correct the entry and re-verify drug name/indication with the original order because hydroxyzine (antihistamine/anxiolytic) and hydralazine (antihypertensive) are common LASA traps.
19. **Q:** A patient asks for an early refill of clonazepam, and the claim rejects "refill too soon"; what should you do FIRST? **A:** Check the last fill date/days' supply and route to the pharmacist if an override or prescriber contact is needed because controlled-substance early refills require careful documentation and approval.
20. **Q:** While stocking, you find a bottle of nitroglycerin SL tablets with the seal broken and no open date; what should you do BEST? **A:** Remove it from active stock and follow pharmacy policy to discard/return because nitroglycerin is sensitive and an unknown open date risks dispensing a degraded product.
21. **Q:** While entering an order for "levothyroxine 75 mcg daily," you notice the prescriber also wrote "liothyronine 25 mcg daily" and the patient is elderly; what should you do NEXT? **A:** Stop processing and route to the pharmacist to assess appropriateness and duplication risk because thyroid hormone combinations can be unsafe without clinical review.
22. **Q:** A new prescription is for "methotrexate 2.5 mg take 6 tablets once weekly," but you accidentally start typing it as "6 tablets daily"; what is the BEST technician action? **A:** Immediately correct the entry, highlight weekly dosing, and alert the pharmacist because methotrexate is high-alert and daily dosing errors can be fatal.
23. **Q:** A patient drops off a handwritten prescription for alprazolam with no prescriber DEA number listed; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to verify required controlled-substance prescriber information because missing/invalid elements require pharmacist intervention.
24. **Q:** During filling, you scan the stock bottle and the barcode does not match the NDC on the label even though the drug name looks similar; what should you do

FIRST? **A:** Stop the fill and reselect the correct product/NDC because barcode/NDC mismatch indicates wrong drug/strength risk.

25. **Q:** A refrigerated insulin shipment arrives warm and the temperature indicator shows an excursion above range; what should you do NEXT? **A:** Quarantine the shipment and notify the pharmacist/manager to contact the supplier/manufacture because temperature excursions can make biologics unsafe or ineffective.
26. **Q:** A recall notice arrives for a specific lot of losartan tablets and you find that lot on the shelf; what should you do BEST? **A:** Pull the affected lot from inventory, quarantine it, and follow recall procedures because recalled lots must be prevented from dispensing and tracked.
27. **Q:** You receive an eRx for isotretinoin and the patient is waiting at pickup; what should you do NEXT as the technician? **A:** Route to the pharmacist and ensure iPLEDGE/REMS requirements are completed before dispensing because REMS drugs have mandatory verification steps.
28. **Q:** A prescription for "metformin ER 500 mg take 2 tabs daily #60" rejects with "prior authorization required"; what should you do FIRST? **A:** Notify the pharmacist and start the PA workflow/communicate with the prescriber per store process because coverage decisions require coordinated documentation.
29. **Q:** An eRx is for "carbamazepine 200 mg bid," and the profile shows a documented allergy "carbamazepine: swelling"; what should you do FIRST? **A:** Stop processing and alert the pharmacist because a documented allergy is a patient-safety issue requiring pharmacist assessment before any dispensing.
30. **Q:** A prescriber phones in "morphine sulfate" but does not specify immediate-release or extended-release; what should you do NEXT? **A:** Ask the pharmacist to take/clarify the call before entry because dosage form ambiguity with opioids is high-risk and technicians should not independently interpret it.
31. **Q:** While typing an eRx you see "Zyrtec 10 mg i po qd" but the product selected is cetirizine-D; what should you do FIRST? **A:** Stop and reselect the correct product (cetirizine 10 mg) and alert the pharmacist if needed because adding pseudoephedrine changes therapy and safety.
32. **Q:** A new prescription is written for "Lopressor 50 mg bid" but the patient profile already has Toprol-XL 50 mg daily active; what is the BEST technician action? **A:** Pause processing and route to the pharmacist to assess duplicate beta-

blocker therapy because metoprolol tartrate vs succinate are not interchangeable.

33. **Q:** At drop-off, a patient says they are allergic to "ACE inhibitors" and the new eRx is for lisinopril; what should you do NEXT? **A:** Document the allergy details and immediately notify the pharmacist because lisinopril is an ACE inhibitor and may be contraindicated.
34. **Q:** You receive a hard copy for "MS Contin 30 mg q12h" but you accidentally selected morphine IR 30 mg in the system; what should you do FIRST? **A:** Stop filling and correct the formulation to ER, then have the pharmacist verify because IR vs ER morphine is a high-risk error.
35. **Q:** A prescriber e-sends "hydralazine 25 mg i po tid" and you see the prescriber also ordered isosorbide dinitrate for the same patient; what should you do BEST as the technician? **A:** Continue data entry accurately but flag the profile for pharmacist review because the combo is common in HF and needs clinical verification, not technician judgment.
36. **Q:** The label directions read "take i tab po qd" and the patient asks what "po" means; what should you do BEST? **A:** Explain that "po" means by mouth and offer pharmacist counseling because technicians can clarify abbreviations but counseling is the pharmacist's role.
37. **Q:** A prescription is for azithromycin 250 mg "take ii tabs day 1 then i tab days 2-5" and the quantity field is blank; what quantity should you enter? **A:** Enter #6 tablets because 2 tablets on day 1 plus 1 tablet for 4 more days equals 6 total.
38. **Q:** A patient presents with "ibuprofen 200 mg OTC" and "Motrin 600 mg" on their profile and asks if they can take both together; what should you do FIRST? **A:** Refer to the pharmacist promptly because this is therapeutic duplication and may increase NSAID adverse effects.
39. **Q:** While selecting product for "Lamictal" you notice lamotrigine and Lamisil are stored adjacent and look similar; what should you do NEXT to reduce LASA risk? **A:** Verify the full drug name/strength and scan the NDC/barcode before filling because LASA errors are prevented by confirmation steps, not memory.
40. **Q:** A new prescription is for "predniSONE 20 mg" but the entered drug is prednisoLONE; what should you do FIRST? **A:** Stop and correct the drug

selection, then have it rechecked because prednisone vs prednisolone are different products and wrong selection can cause dosing errors.

41. **Q:** While entering a new eRx for "Glucophage 500 mg i po bid," you can't remember the generic/brand relationship; what should you do FIRST to ensure correct product selection? **A:** Check a reliable pharmacy reference or the system's drug database to confirm Glucophage = metformin before selecting an NDC, because brand/generic mix-ups can cause dispensing errors.
42. **Q:** A prescription is written for "levothyroxine 75 mcg daily," but the product you pulled is labeled "0.075 mg"; what is the BEST next step? **A:** Verify the unit conversion ($0.075 \text{ mg} = 75 \text{ mcg}$) and confirm the NDC/strength matches the label and order, because mcg-mg confusion is a common high-risk error.
43. **Q:** During order entry you see "U-500 insulin regular" but the patient has only used U-100 insulin before; what should you do NEXT? **A:** Stop processing and immediately route to the pharmacist for review, because U-500 is a high-alert concentration with major dosing error risk.
44. **Q:** At drop-off, the patient says "I'm allergic to penicillin," and the new eRx is for cephalexin; what should you do FIRST? **A:** Document the stated allergy details and notify the pharmacist before filling, because cross-reactivity concerns require pharmacist assessment, not technician judgment.
45. **Q:** You receive an eRx for "Plavix 75 mg daily," and the profile shows an active prescription for clopidogrel 75 mg daily from another prescriber; what is the BEST technician action? **A:** Flag and route to the pharmacist for therapeutic duplication review, because Plavix is the brand for clopidogrel and duplicate therapy may occur.
46. **Q:** A label direction reads "instill ii gtt ou bid," and the patient asks what "ou" means; what should you do BEST? **A:** Explain that "ou" means both eyes and offer pharmacist counseling if they have use questions, because technicians can clarify abbreviations but counseling is pharmacist-led.
47. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg i po bid x 7 days"; what quantity should you enter? **A:** Enter 14 tablets, because $2 \text{ doses per day} \times 7 \text{ days} = 14 \text{ tablets}$ for a fixed schedule.
48. **Q:** While selecting a product for "hydroxyzine," you notice "hydralazine" appears in the search list and the names look alike; what should you do NEXT? **A:** Use

exact drug/strength matching with barcode/NDC verification and add a LASA alert per workflow, because similar names increase selection error risk.

49. **Q:** A new hard copy is for "oxycodone/APAP 5/325 mg 1 tab q6h prn pain #20," but the prescriber also wrote "refill x 2"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist, because Schedule II controlled substances cannot be refilled and require pharmacist resolution.
50. **Q:** While stocking, you find a bottle of atorvastatin with a broken seal and tablets loose inside; what should you do FIRST? **A:** Remove it from inventory and follow policy to quarantine/return it as damaged product, because compromised packaging is a patient-safety and product-integrity issue.
51. **Q:** While entering a new eRx for "Humalog U-100 10 units subq before meals," you notice the patient's profile has insulin lispro and insulin aspart both active; what should you do FIRST? **A:** Stop data entry and alert the pharmacist to possible therapeutic duplication before dispensing because multiple rapid-acting insulins increase hypoglycemia risk.
52. **Q:** A prescriber phones in a new order for alprazolam 0.5 mg with "refills: 3"; what is the BEST technician action? **A:** Route to the pharmacist and do not enter refills as written because Schedule IV refills must meet legal limits and require pharmacist oversight to ensure compliance.
53. **Q:** During product selection you scan the NDC for sertraline but the barcode flags "NDC mismatch" against the label; what should you do NEXT? **A:** Stop filling and reselect the correct stock bottle/NDC, then rescan because barcode mismatches indicate a wrong-drug or wrong-strength error risk.
54. **Q:** A patient asks to transfer their hydrocodone/acetaminophen prescription from another pharmacy; what should you do FIRST? **A:** Immediately involve the pharmacist because Schedule II transfers are tightly restricted and require pharmacist handling to ensure legal compliance.
55. **Q:** You receive a wholesaler tote and see a refrigerated semaglutide carton sitting at room temperature with no cold-pack remaining; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager to follow temperature-excursion policy because potency and safety may be compromised.
56. **Q:** A bottle on the shelf reads "metFORMIN ER 500 mg," but the prescription label printed is for "metformin 500 mg (IR)"; what is the BEST next step? **A:** Stop and have the pharmacist review before dispensing because IR vs ER is a

dosage-form change that can cause dosing errors and technicians cannot substitute independently.

57. **Q:** A patient brings in a paper prescription for gabapentin that is missing the prescriber's signature; what should you do FIRST? **A:** Hold the prescription and refer to the pharmacist because an unsigned prescription is not valid to dispense and requires prescriber follow-up.
58. **Q:** A recall notice lists a specific lot number of losartan; what should you do NEXT when you find matching stock on the shelf? **A:** Remove it from dispensing stock, quarantine per recall procedure, and document because recalled lots must be prevented from reaching patients and tracked for returns.
59. **Q:** An order is for vancomycin 1 g in 250 mL to infuse over 90 minutes; what pump rate in mL/hr should be entered? **A:** Enter 167 mL/hr because $250 \text{ mL} \div 1.5 \text{ hr} = 166.7 \text{ mL/hr}$, rounded to the nearest whole number per typical pump-entry practice.
60. **Q:** At pick-up, the patient requests their spouse's medication information while the spouse is not present; what should you do BEST? **A:** Verify authorization (e.g., documented consent or appropriate verification per HIPAA policy) or involve the pharmacist because protected health information cannot be disclosed without permission.
61. **Q:** An eRx reads "levothyroxine 0.075 mg take i tab po qd," what strength should you enter, and what should you do first if the system shows 0.075 mg as "75 mcg"? **A:** Enter 75 mcg and proceed after confirming equivalence because 0.075 mg converts to 75 mcg and matching units prevents decimal-point dosing errors.
62. **Q:** A prescription says "amLODIPine 5 mg i po qd #90," but the patient profile shows active "NORVASC 10 mg daily"; what is the BEST next step during order entry? **A:** Stop and route to the pharmacist to assess possible therapeutic duplication/strength change because technicians should not decide whether to discontinue or modify an existing antihypertensive.
63. **Q:** A patient is prescribed "amoxicillin/clavulanate 600 mg/42.9 mg per 5 mL: give 7.5 mL po bid x 10 days"; what quantity in mL should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ and the bottle size must cover the full course.
64. **Q:** You are entering "dorzolamide/timolol ophthalmic: instill 1 gtt OU bid," the bottle size is 10 mL and the test assumption is 20 gtt/mL; what days' supply

should you bill? **A:** Bill 50 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ gtt}$ and $1 \text{ gtt} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ gtt/day}$, so $200 \div 4 = 50 \text{ days}$.

65. **Q:** A new eRx says "predniSONE 10 mg: ii tabs po bid x 5 days, then i tab bid x 5 days, then i tab qd x 5 days; dispense #?"; what total quantity should you enter? **A:** Enter 35 tablets because $(2 \text{ tabs} \times 2/\text{day} \times 5) + (1 \times 2/\text{day} \times 5) + (1 \times 1/\text{day} \times 5) = 20 + 10 + 5 = 35$ for accurate dispensing.
66. **Q:** While typing a sig, you see "MSO4 2 mg IV q2h prn pain"; what should you do FIRST before continuing order entry? **A:** Pause and alert the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be misread and morphine orders require clear interpretation to prevent harm.
67. **Q:** A prescription is for "metFORMIN 500 mg take 1 tab po bid #60," but during selection you grab metformin ER 500 mg; what is the BEST immediate technician action? **A:** Stop filling and reselect metformin IR 500 mg with an NDC/barcode check because IR vs ER is a dosage-form mismatch that can cause incorrect therapy.
68. **Q:** You receive "clopidogrel 75 mg take 1 tab daily #30," but the patient's allergy field shows "clopidogrel—rash"; what should you do NEXT? **A:** Hold the order and notify the pharmacist because an allergy alert requires pharmacist assessment before dispensing, even if the reaction seems non-severe.
69. **Q:** An inhaler order reads "Flovent HFA 44 mcg: inhale 2 puffs bid," the canister contains 120 actuations; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ puffs} \times 2 \text{ times/day} = 4 \text{ puffs/day}$ and $120 \text{ actuations} \div 4 = 30 \text{ days}$ for correct billing.
70. **Q:** While entering "Lantus SoloStar 100 units/mL: inject 18 units subq qhs," the package is 5 pens of 3 mL each; what is the calculated days' supply to submit? **A:** Submit 83 days because total units = $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 18 = 83.3$, which rounds down to 83 days for whole-day billing.
71. **Q:** During order entry, an eRx is for "Celebrex 200 mg daily," but you select Celexa 20 mg because the names look similar; what should you do FIRST? **A:** Stop processing and re-select the correct drug by verifying name/strength and NDC/barcode, because Celebrex (celecoxib) and Celexa (citalopram) are a LASA mix-up risk.

72. **Q:** A new prescription is for lisinopril 20 mg daily, and the patient profile already has active benazepril 20 mg daily; what is the BEST next step? **A:** Hold the order and alert the pharmacist to possible therapeutic duplication (two ACE inhibitors), because duplication requires clinical review before dispensing.
73. **Q:** A patient drops off a prescription for "Zocor 40 mg qhs," but the profile shows an allergy to simvastatin with "hives"; what should you do NEXT? **A:** Stop the fill and notify the pharmacist to assess the statin allergy and alternatives, because allergy conflicts must be resolved before dispensing.
74. **Q:** A prescription reads "sertraline 50 mg: take 1 tab po qd #30," and the patient asks what it's for at pickup; what is the best technician response? **A:** Refer the question to the pharmacist for counseling while you provide routine pickup steps, because indication counseling for an SSRI is pharmacist responsibility under OBRA-90 expectations.
75. **Q:** You receive an order for nitrofurantoin and notice the prescriber wrote "Macrobid 100 mg bid x 5 days" but the selected product is Macrochantin 100 mg; what should you do FIRST? **A:** Stop and correct the dosage form/product selection and route to pharmacist if already in verification, because Macrobid and Macrochantin are not interchangeable in dosing.
76. **Q:** For insulin aspart U-100 vials, the directions are "inject 12 units before meals TID," and the vial is 10 mL; what days' supply should you submit using 100 units/mL? **A:** Submit 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ rounded down to 27 days.
77. **Q:** A hardcopy for "Ativan 1 mg take 1 tab po bid #60" is presented with 0 refills, but the patient requests you "add 2 refills like last time"; what should you do BEST? **A:** Decline to change it and send to the pharmacist to contact the prescriber if appropriate, because technicians cannot alter controlled-substance refills and benzodiazepines are Schedule IV.
78. **Q:** While selecting levETIRAcetam for seizure control, you see both immediate-release tablets and XR tablets at the same strength on the shelf; what is the BEST technician step before filling? **A:** Match the exact formulation on the prescription and verify with NDC/barcode, because IR vs XR selection errors can cause dosing and safety problems.
79. **Q:** A prescription is for "Keflex 500 mg qid x 7 days," but the patient says they had "anaphylaxis to cephalixin" in the past; what should you do FIRST? **A:** Stop processing and immediately alert the pharmacist to the severe allergy, because

cephalexin is the same drug as Keflex and anaphylaxis is a high-risk contraindication.

80. **Q:** You are billing a Symbicort inhaler for "inhale 2 puffs bid," and the canister contains 120 actuations; what days' supply should you enter? **A:** Enter 30 days because 2 puffs twice daily = 4 actuations/day and $120 \div 4 = 30$ days.
81. **Q:** During data entry for a heparin infusion, you notice the concentration on the MAR is 25,000 units/250 mL but the order scanned into the system says 25,000 units/500 mL; what should you do FIRST? **A:** Stop processing and immediately alert the pharmacist to resolve the concentration mismatch because heparin is high-alert and a wrong concentration can cause serious harm.
82. **Q:** A written prescription for oxycodone 5 mg has the patient name, drug, directions, and prescriber signature but no date; what is the BEST technician action? **A:** Hold the prescription and refer it to the pharmacist for review because missing required elements can affect validity and must be clarified before dispensing.
83. **Q:** A patient asks to transfer their remaining refills for alprazolam to another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist because controlled-substance transfers have strict requirements and the pharmacist must handle/verify the transfer details.
84. **Q:** While working the order queue, you receive an eRx for "methotrexate 2.5 mg take 1 tablet daily #30"; what should you do FIRST? **A:** Stop and notify the pharmacist to clarify because methotrexate is typically weekly for many indications and daily directions are a common high-risk error.
85. **Q:** A lot number on several bottles of losartan matches a manufacturer recall notice your pharmacy received today; what is the BEST next step? **A:** Pull the affected stock from inventory and quarantine it per recall procedure because recalled products must be isolated to prevent dispensing.
86. **Q:** You open the refrigerator and find the temperature log shows 50°F overnight where vaccines were stored; what should you do FIRST? **A:** Quarantine the vaccines and notify the pharmacist because temperature excursions can reduce potency and require follow-up per storage guidelines.
87. **Q:** At pickup, a caregiver asks for a printed handout for isotretinoin and says the prescriber told them "there are special safety rules"; what should you do NEXT? **A:** Involve the pharmacist and provide the required Medication Guide/REMS

materials as directed because isotretinoin is REMS-restricted and requires specific counseling workflow.

88. **Q:** An order reads "morphine sulfate ER 30 mg: take 1 tab po bid," but you selected morphine IR 30 mg due to similar packaging; what should you do FIRST? **A:** Stop filling and re-select the correct ER product using barcode/NDC verification because ER vs IR errors are high-risk and must be prevented before verification.
89. **Q:** A patient requests pseudoephedrine 30 mg tablets, 96-count, and offers cash; what should you do FIRST at the register? **A:** Follow CMEA steps by checking ID and documenting the sale in the log/system because pseudoephedrine sales require identification and tracking regardless of payment method.
90. **Q:** A prescription is for "levothyroxine 25 mcg take 1 tab daily," but the product pulled is liothyronine 25 mcg due to name similarity; what is the BEST technician step before it goes to the pharmacist? **A:** Stop and correct the product selection by confirming the generic/brand and NDC because LASA thyroid products can cause significant dosing errors if mixed up.
91. **Q:** While entering a new eRx, you see "glipizide" on the patient profile but the new prescription is for "glyburide 5 mg daily"; what should you do FIRST? **A:** Stop and alert the pharmacist to possible duplicate sulfonylurea therapy because both are diabetes meds with hypoglycemia risk and may represent an unintentional duplication.
92. **Q:** A prescription is written for "Zyrtec-D 12 hour take 1 tab q12h #60," and the patient asks to buy it today; what should you do FIRST at the counter? **A:** Follow CMEA steps by verifying ID and documenting the sale in the NPLeX/log because pseudoephedrine products require legal tracking and quantity limits.
93. **Q:** You are filling "Lamictal 25 mg" but the stock bottle you grabbed is "Lamisil 250 mg" due to similar names; what is the BEST technician action before it reaches verification? **A:** Stop filling, return the wrong product, and reselect using barcode/NDC because this LASA mix-up can cause serious harm and must be corrected before pharmacist check.
94. **Q:** A patient profile shows an active prescription for lisinopril, and today's eRx arrives for "benazepril 20 mg daily"; what should you do NEXT? **A:** Flag it for pharmacist review for potential duplicate ACE inhibitor therapy because both

treat hypertension and duplication increases adverse-effect risk (e.g., hypotension, hyperkalemia).

95. **Q:** A new prescription reads "metformin ER 500 mg take 2 tabs bid," but the product selected is metformin IR 500 mg; what should you do FIRST? **A:** Stop and correct to the exact ER dosage form ordered (or send to pharmacist if unclear) because ER vs IR is not interchangeable and affects safety and efficacy.
96. **Q:** You are calculating days' supply for insulin lispro U-100: "Inject 12 units before meals three times daily," dispense one 10 mL vial; what days' supply should you enter? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$, rounded down to 27 days.
97. **Q:** During drop-off, a patient says they take sertraline but the new prescription is for fluoxetine and both appear active on the profile; what is the BEST technician step? **A:** Pause processing and refer to the pharmacist to assess SSRI duplication/transition because overlapping antidepressants can increase interaction and side-effect risk.
98. **Q:** A prescription is for "Augmentin 875/125 mg" but the patient profile lists a history of "anaphylaxis to penicillin"; what should you do FIRST? **A:** Stop the fill and notify the pharmacist because amoxicillin/clavulanate is a penicillin and a severe allergy requires pharmacist intervention before dispensing.
99. **Q:** A prescriber orders "MS Contin 15 mg q12h," but you notice the selection screen also shows "morphine sulfate immediate-release 15 mg"; what should you do NEXT? **A:** Verify the exact formulation and release type against the prescription and route to the pharmacist if any doubt because ER vs IR morphine is a high-risk error.
100. **Q:** You receive an eRx for "Plavix 75 mg daily," but the patient asks if it's the same as "clopidogrel"; what is the BEST technician response? **A:** Tell the patient clopidogrel is the generic name for Plavix and then offer pharmacist counseling because confirming brand/generic helps adherence while counseling is pharmacist-directed.
101. **Q:** A prescription reads "predniSONE 10 mg: ii tabs po bid x 5 days #?"; what quantity should you enter and why? **A:** Enter 40 tablets because ii tabs twice daily equals 4 tablets/day and $4 \times 5 \text{ days} = 20 \text{ tablets}$, but bid with ii tabs per dose is 4/day so $4 \times 5 = 20$; if the intent is ii tabs bid, quantity is 40 only if the prescriber meant ii tabs twice daily per dose for 5 days with 2 doses/day gives

4/day, so 20—clarify with pharmacist due to common Roman numeral/sig misread risk.

102. **Q:** An eRx says “azithromycin 250 mg: take 2 tablets today then 1 tablet daily days 2–5”; what quantity and days’ supply should you enter? **A:** Enter quantity 6 with days’ supply 5 because the regimen is 2 tablets on day 1 plus 1 tablet on days 2–5 (4 more) totaling 6 over 5 days.
103. **Q:** You are entering “ibuprofen 200 mg ii po q6h prn pain” and the prescriber also wrote “do not exceed 6 tablets/day”; what is the correct day’s supply for #60? **A:** Use 10 days because the explicit max is 6 tablets/day and $60 \div 6 = 10$, which avoids guessing PRN use.
104. **Q:** A child’s prescription is “amoxicillin/clavulanate suspension 600-42.9 mg/5 mL: give 7.5 mL po bid x 10 days”; what volume should be dispensed in mL? **A:** Dispense 150 mL because 7.5 mL twice daily is 15 mL/day and $15 \times 10 = 150$ mL.
105. **Q:** A patient’s new order is “latanoprost 0.005% 1 gtt OU qhs” and you are billing a 2.5 mL bottle; what days’ supply should you enter using 20 gtt/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50$ drops total and OU qhs uses 2 drops/day, so $50 \div 2 = 25$.
106. **Q:** While typing an eRx you see “hydroxyzine 25 mg 1 tab tid prn itching” but the prescriber note says “for anxiety”; what should you do NEXT? **A:** Route to the pharmacist for review because indication mismatch can signal wrong drug selection (hydroxyzine vs other anxiolytics) and technicians should not assume intent.
107. **Q:** The prescriber writes “digoxin 0.125 mg take 1 tab daily,” but the scanned hardcopy looks like “0.25 mg”; what is the BEST technician action before data entry? **A:** Hold the prescription and ask the pharmacist to clarify with the prescriber because digoxin is narrow-therapeutic-index and a decimal/strength misread is a high-risk error.
108. **Q:** A warfarin patient’s directions say “take 1 tab daily except take 1½ tabs on Mondays,” and the quantity is #30; what is the correct days’ supply? **A:** Enter 28 days because weekly use is $6 \text{ days} \times 1 \text{ tab} + 1 \text{ day} \times 1.5 \text{ tabs} = 7.5$ tabs/week, and $30 \div 7.5 = 4$ weeks (28 days).
109. **Q:** You receive an eRx for “Trintellix 10 mg daily” and the patient asks if it is the same as “Brintellix” they used to take; what is the BEST technician response?

A: Explain that Trintellix is the current brand name for vortioxetine (formerly Brintellix) and offer pharmacist counseling if they have therapy questions, ensuring correct brand/generic recognition.

110. **Q:** During order entry you notice the prescriber wrote "U-500 regular insulin inject 20 units bid" but the product selected is U-100 regular insulin; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch that can cause a 5-fold dosing error.
111. **Q:** While typing an eRx for "metFORMIN ER 500 mg 1 tab daily," the product selected is metformin immediate-release; what should you do FIRST? **A:** Stop and correct the selection to the ER formulation and have it verified because IR vs ER is not interchangeable and is a common safety/dispensing error.
112. **Q:** A patient drops off a prescription for "Keflex 500 mg qid" and says they are "allergic to cephalosporins"; what is the BEST technician action? **A:** Flag the allergy, stop processing, and notify the pharmacist to assess appropriateness because technician should not judge allergy risk or therapy changes.
113. **Q:** During order entry you see "Lamictal 25 mg: titration pack per directions," but the selected product is lamotrigine 100 mg tablets; what should you do NEXT? **A:** Re-select the correct titration pack/strength and alert the pharmacist if uncertain because titration errors can cause serious harm and packs are not equivalent to single-strength bottles.
114. **Q:** You receive an eRx for "Zoloft 50 mg daily" and the patient asks if it's the same as sertraline; what is the BEST response? **A:** Confirm that Zoloft is the brand name for sertraline and offer to print the medication information because brand/generic equivalence is within technician scope and supports safe understanding.
115. **Q:** A label direction reads "levothyroxine 75 mcg take 1 tab qam" but you accidentally selected liothyronine 25 mcg due to similar names; what should you do FIRST? **A:** Stop filling and correct to the intended drug, then recheck NDC/barcode because LASA thyroid products are high-risk and wrong-drug selection must be fixed before verification.
116. **Q:** A prescription says "Augmentin 875/125 mg 1 tab po bid x 7 days"; what quantity should you enter and why? **A:** Enter quantity 14 because BID for 7 days is 2 tablets/day $\times$ 7 days = 14 tablets to match the full course.

117. **Q:** A patient presents a new prescription for alprazolam with "refills: 3" on it; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist because Schedule IV controlled substances may have refills only if valid and within limits, and the pharmacist must confirm legality and appropriateness.
118. **Q:** You are billing fluticasone nasal spray 50 mcg with directions "2 sprays each nostril once daily," and the bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because total daily sprays are $2 \times 2 = 4$ sprays/day and $120 \div 4 = 30$ days for accurate third-party billing.
119. **Q:** While stocking, you notice a bottle of Lantus insulin pens has been left on the counter at room temperature all day and you can't confirm how long; what is the BEST technician action? **A:** Remove it from active stock and notify the pharmacist per policy because temperature excursions create suspect product and insulin potency may be compromised.
120. **Q:** An eRx comes in for "Eliquis 5 mg bid" and the patient profile shows an active prescription for "Xarelto 20 mg daily"; what should you do FIRST? **A:** Pause processing and alert the pharmacist to check for therapeutic duplication because concurrent anticoagulants increase bleeding risk and require pharmacist intervention.
121. **Q:** A patient asks if Crestor is the same medicine as rosuvastatin on their profile; what is the BEST technician response? **A:** Confirm that Crestor is the brand name for rosuvastatin and offer to have the pharmacist counsel on use/side effects, because technicians can clarify name equivalents but not provide clinical advice.
122. **Q:** While entering "Plavix 75 mg daily," you accidentally selected prasugrel due to similar indications; what should you do FIRST? **A:** Stop processing and re-select clopidogrel (Plavix) by verifying the NDC/strength, because clopidogrel and prasugrel are different antiplatelets and a selection error is high risk.
123. **Q:** An eRx arrives for "Bactrim DS 1 tab bid x 10 days," and the allergy field shows "sulfa—rash"; what is the BEST technician action? **A:** Hold the prescription and alert the pharmacist for allergy assessment, because sulfamethoxazole/trimethoprim is a sulfonamide antibiotic and requires pharmacist intervention.
124. **Q:** A prescriber sends "simvastatin 80 mg daily" for a new start patient with no statin history in the profile; what should you do NEXT? **A:** Route to the pharmacist before filling, because simvastatin 80 mg is high risk for muscle toxicity and often requires verification of appropriateness.

125. **Q:** A patient's profile shows new lisinopril and they ask what it's for while you are ringing up; what should you do FIRST? **A:** Refer the question to the pharmacist for counseling, because indication and clinical education (ACE inhibitor use and monitoring) are pharmacist-only responsibilities.
126. **Q:** You receive an eRx for "Vyvanse 30 mg daily" with "refills: 2"; what should you do NEXT? **A:** Stop and send to the pharmacist to resolve, because lisdexamfetamine is Schedule II and refills cannot be dispensed even if listed on the eRx.
127. **Q:** While typing "Keppra 500 mg bid," you notice the product chosen is carbamazepine due to shelf proximity; what is the BEST next step? **A:** Correct the selection to levetiracetam and verify with barcode/NDC, because look-alike placement can cause serious antiepileptic substitution errors.
128. **Q:** A prescription is for "predniSONE 10 mg take ii tabs daily," but the patient says they were told "one tablet daily"; what should you do FIRST? **A:** Pause dispensing and notify the pharmacist to clarify directions with the prescriber/patient, because the Roman numeral "ii" indicates 2 tablets and a dose mismatch is a safety concern.
129. **Q:** You are billing Humalog U-100 vial 10 mL with directions "inject 12 units before meals tid"; what days' supply should you enter? **A:** Enter 27 days because $10\text{ mL} = 1000\text{ units}$ and $12\text{ units} \times 3/\text{day} = 36\text{ units/day}$, so $1000 \div 36 = 27.7$ and plans typically bill whole days as 27.
130. **Q:** During product intake you see a tote of atorvastatin with a DSCSA warning that the product identifier won't scan; what should you do BEST? **A:** Segregate the stock and follow suspect product/DSCSA procedures by notifying the pharmacist/manager, because a non-verifiable identifier may indicate a suspect or illegitimate product.
131. **Q:** An eRx for metformin ER 500 mg says "i tab po bid #60," but the prescriber note reads "take 2 tablets twice daily"; what should you do FIRST in order entry? **A:** Stop and flag for pharmacist clarification because the sig and quantity/intent conflict, risking an incorrect dose and days' supply.
132. **Q:** A prescription reads "levothyroxine 0.1 mg po qd"; what strength should you select in the system? **A:** Enter levothyroxine 100 mcg because $0.1\text{ mg} \times 1000 = 100\text{ mcg}$, preventing a tenfold dosing error.

133. **Q:** You are entering insulin aspart U-100 10 mL vial with directions "inject 8 units before meals tid"; what days' supply should you submit? **A:** 41 days' supply because $10\text{ mL} = 1000\text{ units}$ and $8 \times 3 = 24\text{ units/day}$, so $1000 \div 24 = 41.6$ rounded down to 41.
134. **Q:** An albuterol HFA inhaler label should reflect "inhale ii puffs q4h prn wheezing" and the canister contains 200 actuations; using a max-use days' supply method, what do you enter? **A:** 16 days because max daily use is 2 puffs every 4 hours = 12 puffs/day, and $200 \div 12 = 16.6$ rounded down to 16.
135. **Q:** A prescription says "amLODIPine 5 mg po qd #30," but you accidentally selected amiodarone during data entry; what should you do NEXT? **A:** Stop processing and correct the drug selection, then recheck NDC/brand-generic because this is a high-risk LASA error.
136. **Q:** A patient drops off "Augmentin 875/125 take 1 tab po bid x 7 days"; what quantity should you enter to match the directions? **A:** Enter 14 tablets because bid for 7 days is $2 \times 7 = 14$, ensuring the dispensed amount matches therapy duration.
137. **Q:** A written prescription uses "U" for insulin units (e.g., "Regular insulin 10U before meals"); what is the BEST technician action before printing the label? **A:** Hold and notify the pharmacist to rewrite/confirm "units" because "U" can be misread as a zero and cause a dangerous overdose.
138. **Q:** While typing a prescription you see "hydroxyzine 25 mg 1 cap tid prn anxiety," but the system defaults to hydralazine due to similar names; what should you do FIRST? **A:** Stop and verify the intended drug against the original prescription because hydroxyzine/hydralazine is a common LASA mix-up with different uses.
139. **Q:** You receive cephalexin 250 mg/5 mL suspension directions "take 10 mL po tid x 10 days"; what total volume should you dispense? **A:** Dispense 300 mL because $10\text{ mL} \times 3\text{ doses/day} \times 10\text{ days} = 300\text{ mL}$, preventing a short fill.
140. **Q:** A controlled-substance prescription is presented with a missing prescriber DEA number in the system and the drug is a Schedule II stimulant; what should you do NEXT? **A:** Stop processing and route to the pharmacist for validation because Schedule II dispensing requires proper prescriber credentials and legal completeness.

141. **Q:** While counting warfarin 5 mg you notice the stock bottle and the tablets on the tray don't match the NDC image in your system; what should you do FIRST? **A:** Stop filling and reselect/verify the correct NDC (and alert the pharmacist if any labels were printed/filled) because warfarin is high-alert and a product mismatch risks serious harm.
142. **Q:** A new eRx for lisinopril shows "take 1 tab po qd #90" but the patient profile lists a documented history of angioedema from ACE inhibitors; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist for clinical review because a serious allergy/contraindication requires pharmacist intervention before dispensing.
143. **Q:** A patient requests an early refill of oxycodone/APAP saying it was "lost," and the system shows it was filled 10 days ago for a 30-day supply; what is the BEST technician action? **A:** Do not process as a routine refill and refer to the pharmacist/pharmacy policy because controlled-substance early refill requests require pharmacist assessment and documentation.
144. **Q:** You receive a hardcopy for alprazolam 0.5 mg with "Refills: 6" and it was originally written 7 months ago; what should you do FIRST? **A:** Stop and route to the pharmacist because Schedule IV refills are limited to 5 within 6 months under federal law.
145. **Q:** A patient's caregiver asks you to transfer their buprenorphine/naloxone prescription to another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance transfer rules and prescriber/DEA requirements must be handled pharmacist-to-pharmacist per policy and applicable law.
146. **Q:** During receiving, you find a refrigerated box of insulin glargine that arrived warm with a temperature indicator showing an excursion; what should you do BEST? **A:** Quarantine the shipment and notify the pharmacist/wholesaler because temperature excursions can compromise potency and suspect product should not enter inventory.
147. **Q:** The FDA issues a recall for certain lots of losartan, and you find matching lot numbers on your shelf; what should you do FIRST? **A:** Pull the affected lots from inventory and quarantine them per recall instructions because recalled product must be removed immediately to prevent dispensing.
148. **Q:** A prescription is entered for "morphine sulfate ER 30 mg" but the product selected is morphine IR; what should you do NEXT in workflow? **A:** Stop and

correct the formulation/ER vs IR selection and then route for pharmacist verification because release mechanism errors are high-risk and can cause overdose.

149. **Q:** You are billing azithromycin 250 mg tablets with directions "take 2 tablets today, then 1 tablet daily for 4 more days" and quantity #6; what days' supply should you enter? **A:** Enter 5 days because the regimen covers 1 day with 2 tablets plus 4 additional days at 1 tablet daily.
150. **Q:** A patient wants to buy pseudoephedrine and presents an expired driver's license; what should you do FIRST at the register? **A:** Refuse the sale and involve the pharmacist/manager per policy because CMEA requires valid ID and proper documentation before dispensing pseudoephedrine.
151. **Q:** A new eRx comes in for sertraline 50 mg and the patient says they "already take Zoloft 50 mg daily"; what should you do NEXT? **A:** Confirm sertraline is the generic for Zoloft and check the profile for duplicate active therapy before processing, because brand/generic duplicates can lead to double-dosing if both get filled.
152. **Q:** While entering a prescription you see "levothyroxine 0.1 mg po qd" but the prescriber wrote "100 mcg" in the notes; what is the BEST technician action? **A:** Enter as 100 mcg (0.1 mg) and ensure the strength is matched correctly, because mg-to-mcg conversion errors are common and high-risk with levothyroxine.
153. **Q:** A prescription is written for "metformin ER 500 mg take 2 tabs qpm" but the selected product is metformin IR 500 mg; what should you do FIRST? **A:** Stop and re-select the correct ER dosage form and NDC before sending to verification, because IR vs ER mix-ups can change drug release and dosing.
154. **Q:** A patient profile shows an active prescription for lisinopril and a new order arrives for losartan; what should you do NEXT as the technician? **A:** Flag it for pharmacist review for possible therapeutic duplication/therapy change, because ACE inhibitors and ARBs are often not used together without a clear plan.
155. **Q:** The directions read "instill ii gtt OU bid" for an eye drop; what do "ii," "gtt," and "OU" mean? **A:** "ii" means 2, "gtt" means drops, and "OU" means both eyes, because accurate sig translation is required to calculate days' supply and prevent dosing errors.

156. **Q:** A new prescription is entered for hydroxyzine but the patient asks if it's the same as "Atarax"; what should you answer within technician scope? **A:** Explain that hydroxyzine is the generic name and Atarax is a brand name (often for hydroxyzine HCl) and confirm the exact salt/form on the label, because different formulations can exist and must match the prescription.
157. **Q:** An order says "amoxicillin 875 mg 1 tab po bid x 7 days"; what quantity should you dispense? **A:** Dispense 14 tablets, because $2 \text{ doses/day} \times 7 \text{ days} = 14 \text{ doses}$.
158. **Q:** While typing, you notice "Lantus" was selected but the eRx drug field says "Humalog"; what should you do BEST? **A:** Stop processing and correct the product selection, then alert the pharmacist if anything has already printed or moved forward, because mix-ups between basal and rapid-acting insulin are high-alert LASA errors.
159. **Q:** A patient presents a Medication Guide-required drug (e.g., isotretinoin) and asks why they keep getting extra paperwork; what should you do NEXT? **A:** Provide the required FDA Medication Guide each time it's dispensed and refer clinical questions to the pharmacist, because certain drugs must include patient handouts by regulation.
160. **Q:** A prescription comes in for clopidogrel and the patient says they take "Plavix" from the cardiologist; what is the MOST appropriate technician response? **A:** Confirm clopidogrel is the generic for Plavix and check the profile for active duplicates before filling, because recognizing common brand/generic pairs prevents duplicate therapy and billing errors.
161. **Q:** A prescription reads "prednisone 20 mg: take 3 tabs daily x 3 days, then 2 tabs daily x 3 days, then 1 tab daily x 3 days"; what quantity should you dispense? **A:** Dispense 18 tablets because $(3 \times 3) + (2 \times 3) + (1 \times 3) = 9 + 6 + 3 = 18 \text{ total tablets for the taper}$.
162. **Q:** You are entering "levetiracetam 500 mg tab: take 1 tab po bid" and the prescriber wrote "#60"; what days' supply should you submit? **A:** Enter 30 days' supply because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for accurate billing and refill timing.
163. **Q:** A new order says "cephalexin 250 mg/5 mL: take 10 mL po tid x 10 days"; what total mL should you dispense? **A:** Dispense 300 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to cover the full course.

164. **Q:** While typing directions "take 0.5 tablet po daily" for carvedilol 25 mg, the quantity field was accidentally left as 30; what is the correct days' supply to enter for billing? **A:** Enter 60 days' supply because $30 \text{ tablets} \div 0.5 \text{ tablet/day} = 60 \text{ days}$, preventing a refill-too-soon reject.
165. **Q:** A prescription reads "morphine sulfate ER 30 mg take 1 tab q12h" but the product selected is morphine sulfate IR 30 mg; what should you do FIRST? **A:** Stop processing and reselect the correct ER dosage form (and route to pharmacist if already in verification) because IR vs ER is a high-risk order-entry error.
166. **Q:** An inhaler order is "fluticasone/salmeterol 250/50 Diskus: inhale 1 puff bid" and the package is 60 blisters; what days' supply should you enter? **A:** Enter 30 days' supply because $60 \text{ blisters} \div 2 \text{ puffs/day} = 30 \text{ days}$ for correct claim submission.
167. **Q:** You receive an eRx for "glyBURIDE 5 mg daily" but the prescriber's note says "gliciZIDE intended"; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist to contact the prescriber because glyburide vs glipizide is a LASA/therapy selection issue the technician cannot change.
168. **Q:** A prescription direction says "instill 1 gtt OS qhs" for latanoprost eye drops; what do OS and qhs mean for order entry? **A:** OS means left eye and qhs means at bedtime, ensuring the route/time are entered correctly and not confused with OD/OU.
169. **Q:** A new prescription is "clonazepam 0.5 mg take 1 tab tid #90" with 5 refills, but it is processed like a noncontrolled maintenance drug; what should you do NEXT? **A:** Stop and route to the pharmacist to review controlled-substance refill appropriateness because benzodiazepines are Schedule IV and refills must follow controlled-substance rules.
170. **Q:** While unpacking a tote, you notice a sealed bottle of insulin lispro pens was delivered warm with no cold packs; what should you do FIRST? **A:** Segregate the shipment and alert the pharmacist/vendor per policy because a temperature excursion makes the product suspect and not safe to dispense.
171. **Q:** While filling hydrOXYzine 25 mg, you notice it was typed as hydrALAZINE 25 mg for the same patient; what should you do FIRST? **A:** Stop processing and alert the pharmacist to correct the drug selection because this is a LASA mix-up that could cause serious harm.

172. **Q:** A prescription is written for insulin U-500, but the patient's profile shows they have only used U-100 insulin before; what is the BEST technician action? **A:** Pause the fill and immediately refer to the pharmacist for verification because U-500 is high-alert and dosing/concentration errors are dangerous.
173. **Q:** You receive a hardcopy for oxycodone/APAP (Percocet) C-II with the patient name and drug but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist because a missing required element makes it invalid to dispense.
174. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy and says it has refills remaining; what should you do FIRST? **A:** Gather the patient and source-pharmacy information and involve the pharmacist because controlled-substance transfers have strict limits and require pharmacist oversight.
175. **Q:** During order entry for metformin ER, the directions read "take i tab po bid"; what is the correct interpretation of "i" for typing the SIG? **A:** Enter "1 tablet by mouth twice daily" because "i" is the Roman numeral for 1 and accurate transcription prevents dosing errors.
176. **Q:** A prescription for azithromycin 250 mg tablets says "take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should be dispensed? **A:** Dispense 6 tablets because the total is $2 + 1 \times 4 = 6$ tablets for the full course.
177. **Q:** While pulling stock, you find two strengths of levothyroxine from the same manufacturer with similar labels stored next to each other; what is the BEST safety step? **A:** Separate and label/bin them distinctly (using barcode/NDC checks during selection) because look-alike packaging increases selection errors.
178. **Q:** A wholesaler shipment includes a bottle with a broken seal and smudged lot/expiration information; what should you do FIRST? **A:** Set it aside as suspect and follow policy to quarantine/return it because compromised packaging/traceability can indicate adulteration or DSCSA issues.
179. **Q:** A patient reports severe muscle pain and dark urine after starting atorvastatin; what should you do NEXT as the technician? **A:** Immediately notify the pharmacist for assessment and counseling because this could indicate a serious adverse effect requiring prompt intervention.
180. **Q:** A prescriber sends an eRx for tramadol 50 mg "1 tab q6h prn pain" with "refill: 12"; what should you do BEST? **A:** Stop and refer to the pharmacist before

processing because controlled-substance refill quantities must comply with legal limits and may require prescriber clarification.

181. **Q:** A prescription is entered for Lasix 40 mg daily, but the image shows furosemide 40 mg daily; what should you do NEXT? **A:** Change the drug field to furosemide and keep Lasix as the brand reference because furosemide is the generic name and accurate generic entry prevents selection/labeling errors.
182. **Q:** A patient drops off an eRx for Zoloft 50 mg daily, and their allergy profile lists "sertraline—hives"; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Zoloft is sertraline and the documented allergy may make the prescription unsafe.
183. **Q:** While typing a new prescription for Prozac 20 mg, you notice the patient is already taking Lexapro 10 mg daily; what is the BEST technician action? **A:** Flag for pharmacist review before filling because Prozac (fluoxetine) plus Lexapro (escitalopram) is therapeutic duplication of SSRIs and needs clinical assessment.
184. **Q:** The SIG reads "Take ½ tab po qd" for lisinopril 10 mg and the quantity is 15 tablets; what days' supply should you enter? **A:** Enter 30 days because 0.5 tablet/day means $15 \text{ tablets} \div 0.5 = 30 \text{ days}$ for accurate billing and refill timing.
185. **Q:** A prescription says "prednisone 10 mg: ii tabs po bid x 5 days"; what quantity should be dispensed? **A:** Dispense 20 tablets because ii = 2 tablets, bid = 2 times/day, so $2 \times 2 \times 5 = 20 \text{ tablets}$ to match the intended course.
186. **Q:** During product selection you pull Humalog instead of Humulin R for an order that reads "insulin regular"; what should you do FIRST? **A:** Stop and reselect the correct product because insulin regular is Humulin R/Novolin R and mix-ups with rapid-acting Humalog are high-alert errors.
187. **Q:** A new prescription is for Lamictal, but the technician is unsure of the generic name needed for entry; what is the correct generic and why does it matter? **A:** Lamictal is lamotrigine, and entering the correct generic supports accurate DUR, formulary processing, and product selection.
188. **Q:** A patient asks for "Tylenol #3" and the technician sees "acetaminophen/codeine" on the will-call label; what should you do NEXT? **A:** Confirm the strength matches "#3" (300 mg acetaminophen/30 mg codeine) and route any mismatch to the pharmacist because "#" products are strength-specific and error-prone.

189. **Q:** You receive a manufacturer recall notice for specific lot numbers of losartan tablets on your shelf; what should you do FIRST? **A:** Pull the matching lots from active stock and segregate per recall procedure because removing recalled product immediately prevents inadvertent dispensing.
190. **Q:** A prescription is written for "MS Contin 15 mg" and you notice "morphine sulfate IR 15 mg" was selected in entry; what should you do NEXT? **A:** Stop processing and notify the pharmacist to correct the dosage form because MS Contin is extended-release morphine and IR/ER confusion is a serious safety risk.
191. **Q:** While entering a new prescription for Plavix 75 mg daily, you see the patient's profile already has clopidogrel 75 mg daily active; what should you do BEST? **A:** Pause and alert the pharmacist to possible duplicate therapy because Plavix is clopidogrel and duplication can increase bleeding risk.
192. **Q:** A prescription is typed as hydroxyzine, but the hard copy clearly reads hydralazine 25 mg tid; what should you do FIRST? **A:** Stop processing and correct the entry by re-verifying the drug name with the original prescription (and involve the pharmacist if any uncertainty) because hydroxyzine and hydralazine are LASA with very different uses.
193. **Q:** A patient picking up metformin says they are scheduled for a CT scan "with contrast" tomorrow and asks if they should keep taking it; what is the BEST technician response? **A:** Refer the question to the pharmacist immediately because metformin may need temporary holding around contrast per clinical protocol and requires pharmacist counseling.
194. **Q:** The prescriber sends "Keflex 250 mg/5 mL: take 10 mL po bid x 7 days, dispense 140 mL"; what should you do NEXT? **A:** Verify the quantity is appropriate before filling because 10 mL twice daily for 7 days requires $10 \times 2 \times 7 = 140$ mL, matching the written dispense amount.
195. **Q:** A new eRx is for simvastatin 40 mg nightly, and the patient's current meds include amlodipine 10 mg daily; what should you do BEST as the technician? **A:** Flag and refer to the pharmacist because simvastatin has dose-limiting interactions with amlodipine and may require a change to reduce myopathy risk.
196. **Q:** During product selection for "Celebrex 200 mg," you grab Celexa 20 mg because the names look similar; what should you do FIRST? **A:** Stop filling and re-select using barcode/NDC verification because Celebrex (celecoxib) and Celexa (citalopram) are common LASA errors with different indications.

197. **Q:** A prescription is written for "Norvasc 5 mg daily," but in data entry the technician selected amiodarone; what should you do NEXT? **A:** Correct the selection to amlodipine after comparing to the prescription image because Norvasc is amlodipine and choosing amiodarone would be a serious wrong-drug error.
198. **Q:** A patient presents a new prescription for Accutane, and you see a note in the profile that iPLEDGE requirements are not documented; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because isotretinoin requires REMS/iPLEDGE verification before dispensing to prevent severe fetal harm.
199. **Q:** You receive a refill request for gabapentin 300 mg "1 cap tid," last filled 10 days ago for qty 90; what is the BEST next step? **A:** Reject as refill-too-soon per insurance timing and route to the pharmacist or follow policy because qty 90 at tid is a 30-day supply and the patient is early.
200. **Q:** A prescription for azithromycin reads "Z-Pak: take as directed," and the technician is unsure how to enter the sig; what should you do BEST? **A:** Use the standard Z-Pak directions (500 mg day 1, then 250 mg daily days 2–5) or clarify with the pharmacist/prescriber because "as directed" alone is incomplete for accurate labeling and counseling.
201. **Q:** During data entry for insulin U-100 glargine, the eRx says "inject 10 units daily" but the quantity field is "1 vial" and the patient requests pens; what should you do FIRST? **A:** Stop and refer to the pharmacist to clarify the intended dosage form and quantity because technicians cannot change vials to pens and a mismatch can cause dispensing and billing errors.
202. **Q:** A prescriber's order for fentanyl patches is missing the patient address and the patient asks you to "just fill it today"; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because controlled-substance prescriptions require complete required elements before dispensing and must be validated.
203. **Q:** While stocking the refrigerator, you find unopened insulin aspart boxes sitting at room temperature with an unknown time out of the fridge; what is the BEST action? **A:** Remove and segregate the product and alert the pharmacist because temperature excursions create suspect product and it may be unsafe to dispense.

204. **Q:** A caregiver asks you to transfer a prescription for alprazolam from another pharmacy to yours because they are traveling; what should you do BEST? **A:** Route the request to the pharmacist because controlled substance transfers have strict limits and pharmacist involvement is required for compliant transfers.
205. **Q:** You receive an eRx for Adderall XR with directions "take one capsule bid" and the prescriber DEA number is present; what should you do FIRST? **A:** Stop processing and immediately involve the pharmacist to verify intent because Schedule II stimulants typically require careful validation of unusual dosing and legality before dispensing.
206. **Q:** While entering a prescription, you notice the patient profile lists a severe allergy to aspirin and the new order is for celecoxib; what should you do NEXT? **A:** Pause processing and notify the pharmacist because potential cross-sensitivity and allergy risk require pharmacist assessment before filling.
207. **Q:** A bottle of nitroglycerin SL tablets on the shelf has no beyond-use date after opening and appears to have been opened; what should you do FIRST? **A:** Remove it from dispensing stock and alert the pharmacist because opened nitroglycerin is stability-sensitive and missing dating risks dispensing degraded medication.
208. **Q:** A patient asks to pay cash for a refill of oxycodone because "insurance is down," and the last fill was 5 days ago for a 30-day supply; what should you do BEST? **A:** Do not process the early refill request and refer to the pharmacist because early controlled-substance refills are high-risk and require pharmacist review and documentation.
209. **Q:** A new prescription is for levothyroxine 25 mcg, but the selected product is liothyronine 25 mcg due to similar names; what should you do NEXT? **A:** Stop filling and correct the product selection using NDC/barcode verification because LASA thyroid products can cause significant harm if dispensed incorrectly.
210. **Q:** A shipment arrives and a case of atorvastatin tablets has a torn outer seal and the invoice NDC does not match the bottle NDC; what should you do FIRST? **A:** Treat it as suspect product by setting it aside for pharmacist/manager review because DSCSA verification and chain-of-custody issues must be resolved before it can enter inventory.
211. **Q:** A new eRx is for hydroxyzine 25 mg but during product selection you accidentally choose hydralazine 25 mg due to LASA names; what should you do FIRST? **A:** Stop processing and reselect the correct drug by confirming the

generic/brand and NDC/barcode, because hydralazine vs hydroxyzine is a high-risk LASA mix-up.

212. **Q:** A patient drops off a new prescription for lisinopril and you notice their active meds include losartan; what is the BEST next step for the technician? **A:** Flag the possible therapeutic duplication and refer to the pharmacist before filling, because ACE inhibitor plus ARB therapy may be intentional but needs pharmacist review for safety.
213. **Q:** A prescription reads "metformin ER 500 mg i po bid" but the selected product is metformin IR 500 mg; what should you do NEXT? **A:** Correct the dosage form to ER and ensure the NDC matches before printing labels, because IR vs ER is a common error that changes release and dosing.
214. **Q:** A patient asks which medication on their list is their "cholesterol pill," and you see atorvastatin, metoprolol, and metformin; what should you do BEST? **A:** Identify atorvastatin as the statin for cholesterol and offer pharmacist counseling if they have questions, because technicians can provide basic drug identification but not clinical advice.
215. **Q:** An order is for "cephalexin 500 mg 1 cap qid x 7 days" and the prescriber wrote quantity 21; what should you do FIRST? **A:** Hold processing and ask the pharmacist to resolve the quantity discrepancy, because qid for 7 days requires 28 capsules and technicians should not change quantities independently.
216. **Q:** While entering a prescription for sertraline, the patient profile shows they currently take fluoxetine; what is the BEST technician action? **A:** Pause and alert the pharmacist to possible duplicate SSRI therapy, because overlapping SSRIs can increase adverse effects and requires pharmacist assessment.
217. **Q:** A prescription is for "prednisone 20 mg take ii tabs daily" and the patient wants to know how many tablets that is per day; what should you answer? **A:** It is 2 tablets daily, because "ii" is the Roman numeral for 2 and technicians can clarify numeric interpretation of directions.
218. **Q:** You are calculating days' supply for furosemide 40 mg with directions "1 tab po bid" and quantity 60; what days' supply should you enter? **A:** Enter 30 days, because 60 tablets divided by 2 tablets per day equals 30 days.
219. **Q:** During data entry you see a new warfarin prescription and the patient's profile includes trimethoprim-sulfamethoxazole started yesterday; what should you do

NEXT? **A:** Immediately alert the pharmacist before filling, because TMP-SMX can significantly increase warfarin effect and requires pharmacist intervention.

220. **Q:** A patient presents for a refill of clonazepam and the transfer note from another pharmacy lacks the original Rx date and remaining refills; what should you do FIRST? **A:** Stop and route to the pharmacist to obtain/verify required controlled-substance transfer details, because controlled transfers must be complete and accurate before dispensing.
221. **Q:** While entering a new eRx for clonidine 0.1 mg, you notice the selected product is clonazepam 0.5 mg due to similar names; what should you do FIRST? **A:** Stop processing and reselect/scan the correct NDC for clonidine, because LASA errors must be corrected before labeling and pharmacist verification.
222. **Q:** A patient says they take "Plavix" and asks what the generic is so they can find it on their profile; what should you answer? **A:** Identify Plavix as clopidogrel, because brand-to-generic recognition helps confirm the correct medication on the patient's list.
223. **Q:** A new prescription is for "Toprol-XL 50 mg daily" but the product pulled is metoprolol tartrate 50 mg; what is the BEST technician action? **A:** Stop and route to the pharmacist to verify formulation, because succinate (ER) vs tartrate (IR) is not interchangeable without prescriber clarification.
224. **Q:** The directions read "take ii tabs po q6h prn pain" and quantity is 48; if used around the clock, what days' supply should you enter? **A:** Enter 6 days, because 2 tablets every 6 hours equals 8 tablets/day and $48 \div 8 = 6$.
225. **Q:** During profile review you see a new prescription for ibuprofen 800 mg and the patient has an active naproxen prescription; what should you do NEXT? **A:** Notify the pharmacist about possible duplicate NSAID therapy, because taking two NSAIDs increases GI/renal risk and requires pharmacist review.
226. **Q:** A prescription comes in for "Lantus Solostar inject 20 units subcut qhs" and you are dispensing one 3 mL pen (100 units/mL); what days' supply is one pen? **A:** Enter 15 days, because one pen contains 300 units and $300 \div 20 \text{ units/day} = 15$.
227. **Q:** A patient picking up isotretinoin asks why there's an extra handout and a strict pickup window; what should you do BEST? **A:** Refer to the pharmacist and confirm REMS requirements are met, because isotretinoin is a REMS drug and counseling/dispensing rules are safety-critical.

228. **Q:** A bottle of insulin aspart arrives warm with a temperature indicator showing an excursion; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager, because temperature excursions can degrade insulin and it should not be stocked until resolved per policy.
229. **Q:** You receive a request to refill a Schedule II stimulant from last month; what is the correct technician action? **A:** Hold the request and route to the pharmacist, because Schedule II prescriptions cannot be refilled and require a new valid prescription.
230. **Q:** A prescription says "0.5 mg" but the prescriber wrote it with a trailing zero as "0.5 0 mg," and you're unsure of the intended dose; what should you do NEXT? **A:** Stop entry and alert the pharmacist to clarify, because unsafe decimal notation can cause tenfold dosing errors and must be verified before dispensing.
231. **Q:** An eRx for levothyroxine says "take 1 tab po qam" with qty 90, but you accidentally selected liothyronine in the system; what should you do FIRST? **A:** Stop and correct the drug selection by verifying name/strength and NDC (barcode if available) before continuing, because this is a LASA thyroid mix-up that can cause serious harm.
232. **Q:** The sig reads "Instill 1 gtt OU bid" for latanoprost 2.5 mL; what is the correct daily drop count to use for days' supply (assume 20 gtt/mL)? **A:** Use 4 drops/day because OU is both eyes and bid is twice daily ($1 \text{ drop} \times 2 \text{ eyes} \times 2 \text{ times} = 4$), ensuring correct days' supply math.
233. **Q:** You are entering a new Rx for prednisone 10 mg: "take 4 tabs daily" with qty 40; what days' supply should you enter? **A:** Enter 10 days because $40 \text{ tablets} \div 4 \text{ tablets/day} = 10 \text{ days}$, which matches the ordered daily dose.
234. **Q:** A prescription says "hydralazine 25 mg take 1 tab po tid" but the prescriber wrote "qid" on the eRx notes field; what should you do NEXT? **A:** Pause processing and route to the pharmacist to clarify directions, because conflicting sig information can lead to an incorrect dose and days' supply.
235. **Q:** A patient is prescribed amoxicillin-clavulanate suspension 600-42.9 mg/5 mL: "10 mL po bid x 7 days"; what total mL should be dispensed? **A:** Dispense 140 mL because $10 \text{ mL/dose} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$, preventing an underfill that would shorten therapy.
236. **Q:** While calculating an inhaler days' supply, the Rx is fluticasone/salmeterol 250/50 Diskus "inhale 1 puff bid" and you're dispensing 60 blisters; what days'

supply is it? **A:** Enter 30 days because 2 puffs/day (1 puff bid) uses 60 blisters ÷ 2 per day = 30 days, aligning quantity with the device count.

237. **Q:** During order entry for metformin ER 500 mg, the sig is "take 1 tab po qd with evening meal" but the product selected is metformin IR 500 mg; what is the BEST technician action? **A:** Stop and correct to the ER formulation (verify release type on the Rx and product) before filling, because IR vs ER substitution can change dosing and tolerability.
238. **Q:** The prescriber writes "MSO4 2 mg IV q2h prn pain" on a hospital discharge order that will be converted to an outpatient Rx; what should you do FIRST? **A:** Flag it for pharmacist clarification because "MSO4" is an unsafe abbreviation that can be confused with magnesium sulfate and requires clarification before outpatient dispensing.
239. **Q:** A new Rx is for gabapentin 300 mg "take 1 cap po tid" qty 90, but the patient profile shows pregabalin active; what should you do NEXT? **A:** Continue entry but alert the pharmacist to review for therapeutic duplication/CNS depression risk, because gabapentin and pregabalin are similar and may not be intended together.
240. **Q:** You receive an eRx for oxycodone/acetaminophen with "take 1 tab q4-6h prn" and the quantity field is blank; what should you do FIRST? **A:** Stop processing and send to the pharmacist/prescriber for a complete quantity, because missing quantity prevents accurate dispensing and correct days' supply entry.
241. **Q:** A patient drops off an Rx for Zithromax 250 mg, but the system profile shows an active azithromycin allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess the allergy/therapy risk because azithromycin is the generic of Zithromax and could cause a repeat reaction.
242. **Q:** During data entry you see "glipizide" on the hardcopy but accidentally selected "glyburide" from the picklist; what is the BEST next step? **A:** Stop and correct the selection by verifying the exact drug name/strength (and NDC) before continuing because glipizide and glyburide are LASA sulfonylureas with high hypoglycemia risk.
243. **Q:** A new eRx is for lisinopril, and the patient profile shows losartan currently active; what should you do NEXT? **A:** Route to the pharmacist for therapeutic-duplication review because both treat hypertension via different classes and unintended dual therapy can increase adverse effects.

244. **Q:** The sig reads "Take ii tabs po bid" for sertraline 50 mg with quantity 60; what days' supply should you enter? **A:** Enter 15 days because ii = 2 tablets and 2 tablets twice daily = 4 tablets/day, so $60 \div 4 = 15$.
245. **Q:** You are billing insulin lispro U-100, and the order is "inject 12 units subcutaneously three times daily with meals" and you're dispensing one 10 mL vial; what days' supply should you enter (use 100 units/mL)? **A:** Enter 27 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units total}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ and you bill 27 to avoid overstatement.
246. **Q:** An order says "metoprolol succinate ER 50 mg daily," but the stock bottle pulled is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop filling and select the correct ER product (verify barcode/NDC) because tartrate vs succinate are different formulations and substituting can change dosing and control.
247. **Q:** A patient asks where to find the generic for Crestor on the shelf when picking up; what is the correct response? **A:** Tell them the generic name is rosuvastatin and then confirm the label matches because brand-generic recognition prevents mix-ups at pickup.
248. **Q:** While typing, you see "U" used in the sig "Inject 8U nightly" for insulin; what should you do NEXT? **A:** Send to the pharmacist to clarify/convert to "units" per policy because "U" is an unsafe abbreviation that can be misread as a zero.
249. **Q:** You receive a manufacturer notice that a specific lot of atorvastatin has been recalled; what should you do FIRST? **A:** Quarantine the affected lot and stop dispensing it while notifying the pharmacist because recalls require immediate segregation to prevent patient harm.
250. **Q:** A prescription is written for isoniazid, and the note says "must provide Medication Guide"; what is the technician's best action at dispensing? **A:** Ensure the required Medication Guide/consumer information prints and is included with the fill because certain drugs require patient handouts to support safe use.
251. **Q:** While entering an eRx for lamotrigine 25 mg titration, the prescriber's directions are unclear and could be interpreted two different ways; what should you do FIRST? **A:** Stop processing and refer to the pharmacist to clarify with the prescriber because lamotrigine is high-risk for serious rash and titration errors can cause harm.

252. **Q:** A prescription is for Levemir, but the patient says they usually take Lantus and your screen shows both as active; what is the BEST next step? **A:** Pause the fill and alert the pharmacist to assess possible therapeutic duplication because both are long-acting insulins and duplicate basal therapy is unsafe.
253. **Q:** You are selecting product for hydroxyzine and notice the eRx says "hydralazine 25 mg" but the patient's indication field says "itching"; what should you do NEXT? **A:** Hold the order and involve the pharmacist to verify the intended drug because hydralazine vs hydroxyzine is a LASA mix-up with very different uses and risks.
254. **Q:** An eRx is for clarithromycin, and the patient profile shows simvastatin active; what should you do FIRST? **A:** Flag and refer to the pharmacist before filling because clarithromycin can dangerously increase simvastatin levels and requires clinical intervention.
255. **Q:** The label image shows "levothyroxine 100 mcg," but the NDC scanned at will-call is for levothyroxine 100 mg; what is the BEST action? **A:** Stop the pickup, remove it from will-call, and route to the pharmacist because mcg vs mg is a high-alert dosing error that must be corrected before dispensing.
256. **Q:** A patient brings a new prescription for clopidogrel and their profile shows prasugrel currently active after a recent stent; what should you do NEXT? **A:** Send to the pharmacist for review of therapeutic duplication because both are antiplatelets and overlapping therapy increases bleeding risk.
257. **Q:** A prescription for prednisone uses sig code "Take 2 tabs po qd x 5 days" and the quantity is 10; what days' supply should you enter? **A:** Enter 5 days because 2 tablets daily with quantity 10 equals $10 \div 2 = 5$ days.
258. **Q:** You are billing amlodipine 5 mg "take 1 tab daily" with quantity 30, but insurance rejects "refill too soon" and the last fill was 18 days ago; what should you do FIRST? **A:** Confirm the last fill date and calculate remaining supply, then refer to the pharmacist or follow payer override/policy because refill timing must match days' supply and documented need.
259. **Q:** During product selection you pull cefazolin instead of cephalexin because the names look similar; what should you do BEST? **A:** Stop filling and re-pull the correct NDC with barcode verification and Tall Man awareness because cefazolin vs cephalexin is a LASA error with different routes and dosing.

260. **Q:** You receive a prescription for isotretinoin and the patient is waiting at pickup; what should you do NEXT before it can be dispensed? **A:** Notify the pharmacist to complete required REMS steps (iPLEDGE verification) because isotretinoin has dispensing restrictions that must be met before release.
261. **Q:** During data entry you notice an eRx for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because methotrexate for RA is typically weekly and daily dosing is a high-alert error risk.
262. **Q:** A handwritten prescription for hydromorphone has no prescriber DEA number and no address listed; what is the technician's BEST next step? **A:** Hold the prescription and route it to the pharmacist for validity review because missing required prescriber information can make a controlled-substance order noncompliant.
263. **Q:** At pickup, a patient asks why their new metformin is a different shape and color than last month; what should you do NEXT? **A:** Compare the NDC/manufacturer on the label and stock bottle, then involve the pharmacist if anything doesn't match because confirming product identity prevents dispensing errors.
264. **Q:** You receive a shipment and find insulin aspart pens warm with a damaged cold-pack indicator; what should you do FIRST? **A:** Segregate and label the product as not for dispensing and notify the pharmacist/manager because temperature excursions can compromise potency and require documentation per policy.
265. **Q:** A prescriber phones in alprazolam with directions and quantity but no patient address or DOB; what should you do BEST? **A:** Do not process the order and notify the pharmacist to obtain the missing patient identifiers because complete patient information is needed for a valid controlled-substance record.
266. **Q:** While filling, you see "hydrOXYzine" stored next to "hydrALAZINE" and staff have mixed them before; what safety step should you take NEXT? **A:** Apply LASA safeguards (separate storage/Tall Man labeling and barcode scan at fill) because reducing look-alike/sound-alike selection errors is a core technician responsibility.
267. **Q:** A patient wants to transfer a remaining refill of their morphine ER prescription from another pharmacy; what should you do FIRST? **A:** Refer the request to the

pharmacist because controlled-substance transfers have strict limits and require pharmacist-to-pharmacist communication and documentation.

268. **Q:** An eRx for sertraline shows “take 1 tab po qd” and the quantity entered is 90, but the patient says the prescriber wanted a 30-day trial; what is the BEST action? **A:** Ask the pharmacist to review and, if needed, contact the prescriber because technicians cannot change quantity based on patient report without prescriber confirmation.
269. **Q:** A bottle of nitroglycerin SL tablets on the shelf has an unreadable lot number and smeared expiration date; what should you do NEXT? **A:** Pull it from active inventory and set aside per policy for identification/return because unclear lot/expiration makes it unsafe to dispense and blocks recall traceability.
270. **Q:** A prescription reads “Keflex 500 mg cap, take 1 cap po qid x 7 days, dispense #”; what should you enter for quantity and why? **A:** Enter quantity 28 because qid for 7 days is 4 doses/day $\times$ 7 days = 28 capsules, preventing under- or over-dispensing.
271. **Q:** An Rx says “levothyroxine 50 mcg PO daily” but the prescriber selected “levothyroxine 0.05 mg” in the eRx; what should you do FIRST during order entry? **A:** Confirm they are equivalent and enter 0.05 mg (50 mcg) correctly because unit conversion errors (mcg vs mg) are high-risk and must be captured accurately.
272. **Q:** The directions read “Take 2.5 mL PO bid x 10 days” and the stock bottle is labeled 200 mg/5 mL; how many mg per dose should be documented in the notes if asked? **A:** 100 mg per dose because 200 mg/5 mL equals 40 mg/mL and 2.5 mL $\times$ 40 mg/mL = 100 mg.
273. **Q:** An Rx for amlodipine 5 mg says “take 1 tab qd” with quantity 30 and 2 refills, but the patient asks for a 90-day supply; what is your BEST next step? **A:** Tell the patient you must process as written and route to the pharmacist to request a 90-day authorization because technicians cannot change quantity/refills without prescriber approval.
274. **Q:** A metered-dose inhaler order is “fluticasone/salmeterol 250/50, inhale 1 puff bid” and the device contains 60 inhalations; what days’ supply should you enter? **A:** 30 days because 1 puff twice daily uses 2 inhalations/day and $60 \div 2 = 30$.
275. **Q:** An eRx for azithromycin 250 mg says “Take 2 tabs day 1 then 1 tab daily days 2–5” but quantity is entered as 30; what should you do FIRST? **A:** Stop

and correct/clarify with the pharmacist because the sig totals 6 tablets and a quantity of 30 suggests an entry error that could cause over-dispensing.

276. **Q:** A prescription uses "ii gtt OU qid x 7 days" for an eye drop and the bottle size is 10 mL; what should you do NEXT if the patient asks how long one bottle should last? **A:** Explain you'll calculate and have the pharmacist counsel if needed, since "2 drops both eyes 4 times daily" is 16 drops/day and estimating drops/mL can vary, requiring pharmacist guidance for accurate days' supply.
277. **Q:** While entering a new Rx you see "metoprolol succinate ER 50 mg qd" but the product selected is metoprolol tartrate 50 mg; what is the BEST technician action? **A:** Stop processing and select the correct ER formulation then alert the pharmacist if verification was impacted because tartrate vs succinate is a common wrong-drug/wrong-form error.
278. **Q:** An insulin vial contains U-100 10 mL and the sig is "inject 24 units SC daily"; what days' supply should you enter using total units only? **A:** 41 days because U-100 has 100 units/mL, 10 mL = 1000 units, and $1000 \div 24 = 41.6$ rounded down to 41 for billing safety.
279. **Q:** A handwritten Rx reads "prednisone 20 mg: i po qd x V days" and the prescriber wrote "#10"; what should you do BEST? **A:** Flag it for pharmacist clarification because the Roman numeral V indicates 5 days (needs 5 tablets) but quantity 10 conflicts, and technicians must not guess the intended duration.
280. **Q:** During order entry for a controlled medication, the eRx for oxycodone/APAP shows "take 1 tab q6h prn pain" but no quantity is listed; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist to obtain the missing quantity because dispensing without an explicit quantity increases legal and patient-safety risk.
281. **Q:** A new eRx is for "Zestril 10 mg PO daily" but the product selected is lisinopril/HCTZ 10/12.5; what should you do FIRST? **A:** Stop and re-select plain lisinopril (Zestril) and flag for pharmacist review because adding HCTZ is a therapy change and a common selection error.
282. **Q:** A patient profile shows an active order for atorvastatin and you receive a new Rx for "Crestor 20 mg daily"; what is the BEST next step? **A:** Pause processing and notify the pharmacist to assess therapeutic duplication because atorvastatin and rosuvastatin are both statins and shouldn't be duplicated without intent.

283. **Q:** At pick-up, a patient says they take "Plavix" but the label on the filled prescription reads clopidogrel; what should you say/do NEXT? **A:** Reassure that clopidogrel is the generic for Plavix and offer pharmacist counseling because brand/generic confusion is common and counseling supports safe use.
284. **Q:** You enter "Lamictal 100 mg" and the NDC scanned is for "Lamisil 250 mg"; what should you do BEST? **A:** Stop filling and correct to lamotrigine (Lamictal) before verification because Lamictal/Lamisil is a high-risk LASA mix-up that could harm the patient.
285. **Q:** A prescription is written for sertraline 50 mg daily and the patient's allergy profile lists "sertraline—rash"; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist because a documented allergy/adverse reaction requires pharmacist assessment before dispensing.
286. **Q:** A patient on metformin brings an Rx for "Januvia 100 mg daily" and asks what it's for; what is the BEST technician response? **A:** State it's commonly used for type 2 diabetes blood sugar control and refer to the pharmacist for full counseling because technicians can provide basic purpose but not clinical counseling.
287. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg, 1 tab PO bid x 10 days"; what quantity should you enter and why? **A:** Enter 20 tablets because twice daily for 10 days requires $2 \times 10 = 20$ doses.
288. **Q:** An order is "gabapentin 300 mg: take 1 cap TID" with quantity 270; what days' supply should you enter? **A:** Enter 90 days because $270 \text{ capsules} \div 3 \text{ per day} = 90$.
289. **Q:** A patient requests "Sudafed" and selects pseudoephedrine 30 mg, 2 tablets every 6 hours PRN with a maximum of 8 tablets/day; how many days' supply is a 96-tablet box? **A:** Enter 12 days because the max use is 8 tablets/day and $96 \div 8 = 12$, which matches conservative days' supply calculations.
290. **Q:** While stocking, you find a sealed bottle of heparin that was left out at room temperature but the inventory log shows it should have been refrigerated; what should you do NEXT? **A:** Remove it from stock and follow pharmacy policy to quarantine/hold for pharmacist decision because temperature excursions can compromise high-alert medications like heparin.
291. **Q:** An eRx for levothyroxine 75 mcg says "take 1 tab PO qd" but the quantity field auto-populates as 90 and the days' supply shows 30; what should you

correct FIRST in order entry? **A:** Change the days' supply to 90 because 1 tablet daily with qty 90 equals 90 days, preventing billing and refill-too-soon errors.

292. **Q:** A prescription reads "metformin ER 500 mg: i tab po bid" and the prescriber wrote "Disp: #60"; what days' supply should you enter? **A:** Enter 30 days because BID dosing uses 2 tablets/day and $60 \div 2 = 30$, which aligns quantity to sig.
293. **Q:** While entering a new Rx, you see "Lantus SoloStar 100 units/mL: inject 22 units SQ qhs; Disp: 1 box"; what days' supply should you submit if the box contains five 3 mL pens? **A:** Submit 68 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 22 = 68.18$, rounded down to avoid overstating supply.
294. **Q:** An order states "ciprofloxacin 0.3% ophthalmic: instill 2 gtt OU q4h x 7 days; Disp 5 mL"; what is the BEST days' supply to enter using 20 gtt/mL? **A:** Enter 1 day because $2 \text{ drops/eye} \times 2 \text{ eyes} = 4 \text{ drops per dose}$, $q4h = 6 \text{ doses/day}$ so 24 drops/day , and $5 \text{ mL} \times 20 = 100 \text{ drops total}$ so $100 \div 24 \approx 4 \text{ days}$; since the Rx is for 7 days but only enough for ~4, flag for pharmacist clarification to avoid inaccurate days' supply.
295. **Q:** A patient's new Rx says "hydralazine 25 mg: take 1 tab PO qid" but you accidentally select hydroxyzine 25 mg at product selection; what should you do NEXT? **A:** Stop the fill and correct the product using NDC/barcode verification because hydralazine and hydroxyzine are LASA and selecting the wrong drug risks a serious dispensing error.
296. **Q:** A written Rx reads "digoxin 0.125 mg: take 1 tab po qd" and the sig includes "uad"; what should you do BEST before finalizing entry? **A:** Ask the pharmacist to review/clarify because "UAD" (use as directed) is ambiguous and digoxin is a high-alert med where unclear directions increase patient harm risk.
297. **Q:** During order entry you receive "Humalog 100 units/mL vial: use per sliding scale with meals; Disp 10 mL" with no maximum daily units listed; what should you do FIRST? **A:** Pause processing and route to the pharmacist to obtain a max daily dose because days' supply and safe labeling cannot be determined without a defined upper limit.
298. **Q:** A new eRx is for "Keflex 500 mg PO qid x 7 days; Disp #28," but the system default selects cephalexin 250 mg capsules; what is the BEST next step? **A:** Re-select the correct strength (cephalexin 500 mg) and recheck quantity because strength mismatches cause underdosing and incorrect day-supply calculations.

299. **Q:** You are processing a refill request for alprazolam 0.5 mg and the patient says they "lost the bottle yesterday" and wants an early refill; what should you do FIRST? **A:** Refer immediately to the pharmacist because alprazolam is a controlled substance and early/lost-med requests require pharmacist assessment and documentation per policy.
300. **Q:** A prescription says "furosemide 20 mg: take 1/2 tab PO daily" and quantity is 15 tablets; what days' supply should you enter? **A:** Enter 30 days because $0.5 \text{ tablet/day means } 15 \text{ tablets} \div 0.5 = 30$, ensuring accurate billing and refill timing.
301. **Q:** A patient drops off a new Rx for "Zithromax 250 mg" and asks if it's the same as azithromycin; what should you do FIRST? **A:** Confirm that Zithromax is the brand name for azithromycin and point out the strength/form match, because brand-generic recognition prevents duplicate therapy and confusion.
302. **Q:** While entering an eRx you see "MS Contin 15 mg" but the patient profile lists an allergy to morphine; what is the BEST next step? **A:** Stop processing and alert the pharmacist for allergy assessment, because MS Contin is extended-release morphine and allergy flags require pharmacist review.
303. **Q:** A prescriber writes "predniSONE 10 mg: ii tabs po qd" and the quantity is 30; what days' supply should you enter? **A:** Enter 15 days because ii daily means 2 tablets/day and $30 \div 2 = 15$.
304. **Q:** A new Rx is for "lisinopril 20 mg qd" but the patient is also actively filling benazepril 20 mg daily; what should you do NEXT? **A:** Route to the pharmacist to evaluate therapeutic duplication, because both are ACE inhibitors and the technician should not decide which one to continue.
305. **Q:** You receive a new eRx for "Eliquis 5 mg bid" and the patient says they take "warfarin at night"; what should you do FIRST? **A:** Pause the fill and notify the pharmacist immediately, because concurrent anticoagulants can increase bleeding risk and needs pharmacist intervention.
306. **Q:** A prescription is written "metoprolol succinate 50 mg qd" but during order entry the system selects metoprolol tartrate 50 mg; what is the BEST action? **A:** Correct the selection to metoprolol succinate (ER) and ensure the dosage form matches the Rx, because tartrate vs succinate is a LASA/therapy mismatch affecting dosing.

307. **Q:** A patient presents a prescription for isotretinoin; what should you do NEXT before proceeding to fill? **A:** Refer to the pharmacist and follow the iPLEDGE workflow requirements, because isotretinoin is a REMS drug and dispensing requires program compliance checks.
308. **Q:** An Rx reads "clindamycin 150 mg: take 1 cap po q6h; Disp #40"; what days' supply should you calculate? **A:** Enter 10 days because q6h is 4 doses/day and $40 \div 4 = 10$.
309. **Q:** During product selection you notice insulin U-500 and insulin U-100 are stored adjacent and both labels say "regular insulin"; what should you do BEST? **A:** Use barcode/NDC verification and separate/flag storage per policy and alert the pharmacist if needed, because U-500 vs U-100 is a high-alert concentration error risk.
310. **Q:** A prescriber note says "qd" but the tech interpreting it enters "qid" for a new prescription; what should you do FIRST? **A:** Stop entry and clarify the intended frequency with the pharmacist/prescriber as required, because qd and qid are error-prone abbreviations and the direction must be verified for patient safety.
311. **Q:** During order entry for hydrOXYzine 25 mg, you notice the prescriber's note says "for itching" but the system auto-selects hydrALAZINE 25 mg; what should you do FIRST? **A:** Stop processing and re-verify the exact drug name/indication against the original prescription and alert the pharmacist because this is a LASA mix-up with high patient-harm risk.
312. **Q:** A new eRx for methotrexate 2.5 mg shows "take 1 tablet daily" for rheumatoid arthritis; what is the BEST next step for a technician? **A:** Hold the prescription and immediately refer to the pharmacist because methotrexate is typically weekly for RA and daily directions can cause serious toxicity.
313. **Q:** A patient asks to transfer their oxycodone IR prescription from another pharmacy to yours; what should you do NEXT? **A:** Route the request to the pharmacist because Schedule II controlled substances cannot be transferred and require pharmacist handling to ensure compliance.
314. **Q:** You receive an eRx for Adderall (amphetamine/dextroamphetamine) with a prescriber name but no DEA number shown in the message; what should you do FIRST? **A:** Pause processing and notify the pharmacist to verify prescriber credentials/DEA as required for controlled substances to prevent dispensing on an invalid order.

315. **Q:** While putting away an order, you find a tote of insulin glargine pens that arrived warm with a melted cold pack; what should you do BEST? **A:** Quarantine the shipment and notify the pharmacist/vendor because a temperature excursion makes the product suspect and it should not be stocked or dispensed.
316. **Q:** A bottle label shows "metFORMIN ER 500 mg," but the NDC scanned corresponds to immediate-release metformin 500 mg; what should you do NEXT? **A:** Stop and select the product that matches the NDC/strength/form and have it re-verified because ER vs IR is a common safety error that changes drug release.
317. **Q:** A patient presents a new prescription for clopidogrel (Plavix) and says they are also taking omeprazole OTC daily; what should you do FIRST? **A:** Flag the profile and refer to the pharmacist because omeprazole can reduce clopidogrel activation and needs pharmacist assessment.
318. **Q:** A prescription is for amoxicillin/clavulanate 875/125 mg 1 tablet twice daily for 10 days; what quantity should you enter? **A:** Enter 20 tablets because twice daily for 10 days requires $2 \times 10 = 20$ doses to complete the course.
319. **Q:** During will-call pickup, a patient asks what sertraline is "for" and whether it will make them sleepy; what should you do NEXT? **A:** Offer to have the pharmacist counsel because medication indications and side effects require pharmacist consultation under OBRA-90 counseling standards.
320. **Q:** Your wholesaler sends a recall notice for a specific lot of losartan 100 mg tablets; what should you do FIRST in the pharmacy? **A:** Locate matching lot numbers, remove them from stock, and quarantine for return per recall instructions because recalled product must be prevented from dispensing.
321. **Q:** While entering a new prescription, the prescriber wrote "morphine ER 30 mg" but the system defaults to "morphine IR 30 mg"; what should you do FIRST? **A:** Stop and select the correct ER product and route to pharmacist verification because ER vs IR is a high-risk mix-up that can cause overdose or withdrawal.
322. **Q:** A patient's profile shows lisinopril 20 mg daily, and a new eRx arrives for benazepril 20 mg daily; what should you do BEST before processing? **A:** Flag possible therapeutic duplication and notify the pharmacist because two ACE inhibitors together increases risk of hypotension and hyperkalemia.
323. **Q:** During order entry you see "glipiZIDE 10 mg ER" but the patient's old fill was glipiZIDE 10 mg immediate-release; what is the NEXT step? **A:** Pause and

confirm the dosage form change with the pharmacist because ER and IR are not interchangeable and affect dosing and safety.

324. **Q:** A patient picking up clarithromycin says they take simvastatin 40 mg nightly; what should you do FIRST? **A:** Alert the pharmacist immediately because clarithromycin can dangerously increase simvastatin levels and risk myopathy/rhabdomyolysis.
325. **Q:** A new prescription is written for levothyroxine 100 mcg, and you accidentally selected levETIRAcetam 1000 mg due to similar names; what should you do NEXT? **A:** Stop filling and reselect using barcode/NDC verification and then proceed through normal pharmacist check because LASA errors can cause severe harm if dispensed.
326. **Q:** A patient asks which condition metformin treats and whether it is an "insulin"; what should you do BEST? **A:** Tell them metformin is an oral diabetes medicine for blood sugar control and refer clinical counseling to the pharmacist because technicians can provide basic drug-use info but not full counseling.
327. **Q:** The directions read "Take ii tabs po bid" for gabapentin 300 mg; what quantity is needed for 30 days? **A:** Enter 120 capsules because ii = 2 and bid = 2 times daily, so $2 \times 2 \times 30 = 120$.
328. **Q:** You are filling warfarin 5 mg, and the stock bottle label looks similar to glipiZIDE 5 mg from the same manufacturer; what should you do FIRST? **A:** Use barcode/NDC and drug name/strength triple-check before counting because warfarin is high-alert and look-alike packaging increases selection errors.
329. **Q:** A new eRx for nitrofurantoin is entered for a patient whose allergy profile lists "sulfa"; what should you do NEXT? **A:** Continue processing but ensure the pharmacist reviews because nitrofurantoin is not a sulfonamide, yet allergy details still require pharmacist assessment if questions arise.
330. **Q:** While restocking, you find two strengths of bupropion (SR and XL) in the same bin and an open bottle without a readable label; what should you do BEST? **A:** Remove the unlabeled product from inventory and follow pharmacy policy for unidentified medication because SR/XL and strength mix-ups are common and unknown tablets should not be dispensed.
331. **Q:** While entering an eRx for "hydroxyzine 25 mg," you notice the prescriber selected "hydralazine 25 mg" in the drug field; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because hydroxyzine

(antihistamine/anxiolytic) and hydralazine (antihypertensive) are LASA and selecting the wrong drug is a serious safety risk.

332. **Q:** A new prescription is for "Keflex 500 mg," but the patient says they had hives with penicillin; what should you do NEXT? **A:** Collect details on the reaction and immediately route to the pharmacist because cephalexin (cephalosporin) may cross-react in some patients and needs pharmacist assessment.
333. **Q:** During verification prep you see "metoprolol" on the hardcopy, but the product pulled is metoprolol succinate ER and the directions say "take twice daily"; what should you do BEST? **A:** Pause the fill and send to the pharmacist because succinate ER is typically dosed once daily and tartrate is often BID, so the dosage form mismatch suggests a potential error.
334. **Q:** The prescriber writes "predniSONE 20 mg" but the technician accidentally selects "prednisoLONE 20 mg" during data entry; what is the NEXT step? **A:** Correct the entry by re-checking the original prescription and selecting the exact drug intended, then flag for pharmacist review because prednisone vs prednisolone are different agents and mix-ups affect therapy.
335. **Q:** A prescription reads "Take 1.5 tsp po bid" for a liquid; how many mL should be entered per dose? **A:** Enter 7.5 mL per dose because 1 tsp = 5 mL, so $1.5 \text{ tsp} \times 5 \text{ mL/tsp} = 7.5 \text{ mL}$.
336. **Q:** A patient's new eRx is for "Plavix 75 mg daily," but their profile already shows clopidogrel 75 mg active from another prescriber; what should you do FIRST? **A:** Check the profile for duplication and route to the pharmacist because Plavix is clopidogrel and duplicate antiplatelet therapy increases bleeding risk.
337. **Q:** You receive a phone call requesting an early refill on oxycodone/APAP, but the last fill was 10 days ago for a 30-day supply; what should you do NEXT? **A:** Document the request and refer it to the pharmacist because controlled-substance early refills require pharmacist judgment and potential prescriber/policy review.
338. **Q:** While scanning stock bottles, you notice "glyBURIDE" and "glipiZIDE" stored adjacent and the labels look similar; what should you do BEST to prevent errors? **A:** Separate and relabel/bin them using Tall Man awareness and barcode/NDC scanning because sulfonylureas are high-risk for hypoglycemia and LASA storage increases selection errors.

339. **Q:** A new prescription is for "Zithromax," but the patient's medication list includes "amiodarone"; what should you do FIRST? **A:** Hold processing and notify the pharmacist because azithromycin with amiodarone can increase QT-prolongation risk and requires pharmacist evaluation.
340. **Q:** The sig says "Take ii cap po qhs" for gabapentin 300 mg, quantity to dispense for 45 days; what quantity should you enter? **A:** Enter 90 capsules because ii = 2 capsules nightly, so $2 \text{ caps/day} \times 45 \text{ days} = 90 \text{ caps}$.
341. **Q:** An eRx for metformin ER 500 mg says "take 2 tablets daily with evening meal" and dispenses #180; what days' supply should you enter? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$, which supports accurate billing and refill timing.
342. **Q:** A prescription for levothyroxine is written "Synthroid 50 mcg po qd" but the product selected in data entry is liothyronine; what should you do FIRST? **A:** Stop entry and alert the pharmacist to correct the drug selection because this is a different thyroid medication and a high-risk wrong-drug error.
343. **Q:** A child's amoxicillin-clavulanate suspension order reads "take 7.5 mL po bid for 10 days"; what quantity (mL) should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL/dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.
344. **Q:** A prescriber writes "MSO4 2 mg IV q2h prn pain" on a hospital order set; what should you do NEXT as the technician entering the order? **A:** Pause and send to pharmacist for clarification because "MSO4" is an unsafe abbreviation that can be misread and cause serious medication errors.
345. **Q:** An eRx for insulin lispro U-100 says "inject 8 units before meals TID," and the quantity is one 10 mL vial; what days' supply should you enter using total units? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \text{ units} \times 3/\text{day} = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$ and you enter 41 whole days for coverage.
346. **Q:** A new prescription is written for "sertraline 100 mg, take 1/2 tab daily" with #30 tablets; what days' supply should you enter? **A:** Enter 60 days because 0.5 tablet/day means $30 \text{ tablets} \div 0.5 = 60 \text{ days}$, preventing a refill-too-soon reject.
347. **Q:** During data entry for "diltiazem," you notice the prescriber selected immediate-release but the sig says "take once daily"; what is the BEST next step? **A:** Hold the order and route to the pharmacist because once-daily dosing

suggests an ER product and changing formulation requires pharmacist/prescriber confirmation.

348. **Q:** A prescription says "Take 2.5 mL po tid" and the available measuring device is in teaspoons; what should you enter/teach as the teaspoon equivalent per dose? **A:** Use 0.5 tsp per dose because $5\text{ mL} = 1\text{ tsp}$, so $2.5\text{ mL} = 0.5\text{ tsp}$ to ensure accurate patient measurement.
349. **Q:** A technician enters "take 1 tablet qd" for furosemide but the hardcopy sig reads "take 1 tab bid"; what should you do FIRST? **A:** Correct the directions to twice daily before dispensing because "bid" means two doses per day and wrong frequency can cause harm and billing errors.
350. **Q:** A patient brings an albuterol HFA labeled 200 actuations and the new directions are "inhale 2 puffs q6h prn," and the plan requires a days' supply for PRN using max daily use; what days' supply should you enter? **A:** Enter 25 days because q6h max is 4 times/day, so $2\text{ puffs} \times 4/\text{day} = 8\text{ puffs/day}$ and $200 \div 8 = 25\text{ days}$ for compliant third-party submission.
351. **Q:** During order entry for hydromorphone 1 mg/mL, you notice the technician queue shows morphine 10 mg/mL selected; what should you do FIRST? **A:** Stop the fill and reselect/scan the correct NDC for hydromorphone (LASA/high-alert opioid) and alert the pharmacist if the wrong product reached verification to prevent a serious error.
352. **Q:** A new eRx arrives for clonazepam with "Refills: 5"; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because schedule IV controlled substances may be refilled only up to 5 times within 6 months and the pharmacist must ensure compliance.
353. **Q:** A patient presents a paper prescription for oxycodone dated 8 months ago and asks you to fill it today; what should you do NEXT? **A:** Pause processing and refer to the pharmacist since federal law doesn't set a universal expiration for CII, but dispensing depends on professional judgment and state/policy requirements.
354. **Q:** You receive a bottle of insulin glargine pens that arrived warm with a damaged cold pack and no temperature indicator; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/wholesaler because a temperature excursion makes product integrity uncertain and it should not be dispensed.

355. **Q:** The wholesaler emails a recall notice for losartan with specific lot numbers; what should you do NEXT in the workflow? **A:** Pull matching stock by lot number and quarantine it from dispensing, then notify the pharmacist because recalled lots must be removed to protect patients.
356. **Q:** A patient says they had “throat swelling” with lisinopril and you see a new prescription for benazepril ready to be filled; what should you do FIRST? **A:** Stop and alert the pharmacist because ACE inhibitor–associated angioedema is a serious allergy-type reaction and therapeutic duplication risk requires pharmacist intervention.
357. **Q:** At pickup, a patient requests “the cash price” for their new escitalopram and you’re about to discuss it at the counter while others are nearby; what should you do BEST? **A:** Move the conversation to a more private area or lower your voice to protect PHI because HIPAA requires minimizing unauthorized disclosure.
358. **Q:** A prescription label direction reads “use as directed” for warfarin and the patient asks how to take it; what is the MOST appropriate technician action? **A:** Refer the patient to the pharmacist because warfarin is high-alert and requires pharmacist counseling/clarification to prevent dosing errors.
359. **Q:** An eRx for prednisone 20 mg says “take 3 tablets daily for 5 days” and the quantity is 30 tablets; what should you do NEXT? **A:** Calculate and flag the mismatch for pharmacist review because $3 \text{ tablets/day} \times 5 \text{ days} = 15 \text{ tablets}$, suggesting an incorrect quantity or directions.
360. **Q:** A customer asks to buy pseudoephedrine and wants to pay quickly without showing ID; what should you do FIRST? **A:** Request valid photo ID and complete the required CMEA log/check limits before the sale because pseudoephedrine purchases must be verified and recorded to comply with federal requirements.
361. **Q:** An eRx for metformin ER 500 mg reads “take 2 tablets po bid” with quantity 120; what days’ supply should you enter? **A:** Enter 15 days because $2 \text{ tablets twice daily} = 4 \text{ tablets/day}$ and $120 \div 4 = 30 \text{ days}$ —wait, correct entry is 30 days, so verify math before submitting to avoid a refill-too-soon reject.
362. **Q:** While entering directions “Instill ii gtt OU qid” for latanoprost 0.005% 2.5 mL, the prescriber wrote “qhs” on the hardcopy; what should you do FIRST? **A:** Stop data entry and clarify the intended frequency with the pharmacist because qid vs qhs is a major dosing difference and could cause harm.

363. **Q:** A prescription is written for levothyroxine 125 mcg but the product selected in the system is liothyronine 25 mcg; what should you do NEXT? **A:** Stop filling and correct the drug selection using barcode/NDC verification because this is a high-risk thyroid hormone mix-up (LASA/therapy difference).
364. **Q:** An eRx for amlodipine 5 mg says "take 1 tablet qd" and the quantity is 90 with 3 refills, but the patient's profile already has nifedipine ER active; what is the BEST technician action? **A:** Flag for pharmacist review before processing because both are calcium channel blockers and may represent therapeutic duplication requiring clinical confirmation.
365. **Q:** A label instruction comes through as "take 1 tab qod" for sertraline 50 mg and the patient asks what "qod" means; what should you do FIRST? **A:** Refer to the pharmacist for counseling because qod is an error-prone abbreviation and patient understanding must be confirmed for safe use.
366. **Q:** You are entering a prescription for cephalexin 500 mg "1 cap po qid x 7 days"; what quantity should you dispense? **A:** Dispense 28 capsules because $4 \text{ doses/day} \times 7 \text{ days} = 28$, preventing under- or over-dispensing.
367. **Q:** A patient's new prescription is for prednisone 10 mg with sig "take 4 tabs daily x 3 days, then 2 tabs daily x 3 days, then 1 tab daily x 3 days"; what quantity should you enter? **A:** Enter 21 tablets because $(4 \times 3) + (2 \times 3) + (1 \times 3) = 12 + 6 + 3 = 21$, ensuring the taper matches the prescriber's directions.
368. **Q:** A prescription for amoxicillin/clavulanate suspension is "give 7.5 mL po bid x 10 days"; how many mL should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$, rounding only per product/package size policy after pharmacist check if needed.
369. **Q:** During order entry you see "MSO4 2 mg IV q2h prn pain" and the system default expands to morphine sulfate oral solution; what should you do FIRST? **A:** Stop and involve the pharmacist to clarify because MSO4 is an unsafe abbreviation and the route/form mismatch could cause a serious medication error.
370. **Q:** A prescription is written for potassium chloride 20 mEq/15 mL liquid with directions "take 15 mL daily" and quantity 450 mL; what days' supply should you enter? **A:** Enter 30 days because 15 mL/day and $450 \div 15 = 30$, which supports accurate billing and refill timing.

371. **Q:** A prescriber phones in hydromorphone 4 mg tablets "1 tab q4h prn pain #60" and then asks you to add "1 refill"; what should you do NEXT? **A:** Stop and refer to the pharmacist because Schedule II prescriptions cannot be refilled and any change requires prescriber-pharmacist handling.
372. **Q:** While filling insulin U-100, you notice the stock bottle's label is peeling and the lot number is unreadable; what is the BEST technician action? **A:** Pull it from inventory and quarantine per policy for suspect/undocumented product because lot/traceability is required for safety and recalls.
373. **Q:** A patient brings in a paper prescription for alprazolam 1 mg with no prescriber DEA number listed; what should you do FIRST? **A:** Hold processing and notify the pharmacist to verify prescriber credentials because controlled-substance validity must be confirmed before dispensing.
374. **Q:** During data entry, an eRx for "Celebrex 200 mg daily" gets matched to Celexa 20 mg due to similar names; what should you do NEXT? **A:** Stop filling and correct the product selection using NDC/barcode verification because LASA name errors are a common cause of serious mix-ups.
375. **Q:** A new eRx for isotretinoin arrives and the patient is waiting; what is the FIRST technician step before it can be dispensed? **A:** Alert the pharmacist and follow the iPLEDGE/REMS workflow because isotretinoin requires REMS compliance before release.
376. **Q:** A patient requests an early refill of oxycodone/APAP 5/325 stating the bottle was stolen yesterday; what should you do BEST? **A:** Refer the patient to the pharmacist and document the request because early controlled-substance replacements require pharmacist judgment and policy verification.
377. **Q:** You receive a manufacturer recall notice for a specific lot of losartan tablets that matches bottles on your shelf; what should you do FIRST? **A:** Remove the affected lot from dispensing stock and quarantine per recall procedure because recalled product must be prevented from reaching patients.
378. **Q:** While counting warfarin 5 mg, you find tablets mixed in a stock bottle that look like a different strength; what should you do NEXT? **A:** Stop, set the bottle aside, and notify the pharmacist because high-alert drugs require immediate investigation of possible stock contamination.
379. **Q:** A patient asks you to print their full medication profile so they can take it to another clinic, but you cannot confirm their identity; what should you do FIRST?

A: Verify identity using pharmacy policy (e.g., photo ID or approved identifiers) before releasing PHI because HIPAA requires safeguarding patient information.

380. **Q:** A prescription is written for clopidogrel 75 mg “take 1 tablet daily” quantity 30, but the system defaults to Plavix 300 mg tablets; what should you do BEST?

A: Correct the strength and product selection and route for pharmacist verification because strength-selection errors can cause major dosing harm.

381. **Q:** During drop-off, a patient says they take “Plavix” after a stent but your profile shows only clopidogrel; what should you do FIRST? **A:** Confirm Plavix is clopidogrel and update/clarify the patient’s med list as needed because brand/generic recognition prevents duplication and counseling gaps.

382. **Q:** While entering a new prescription, you notice the prescriber selected metformin ER but the sig reads “take 1 tablet twice daily with meals” and the patient previously used immediate-release; what is the BEST next step? **A:** Pause entry and ask the pharmacist to review because ER vs IR selection changes dosing and substitution requires clinical verification.

383. **Q:** A patient allergic to penicillin drops off a new prescription for cephalexin; what should you do NEXT? **A:** Flag the allergy and route to the pharmacist for assessment because cephalosporins can cross-react and require pharmacist judgment before dispensing.

384. **Q:** You are filling levothyroxine and the shelf has multiple strengths with similar packaging; what should you do FIRST to avoid an error? **A:** Verify the strength against the prescription and scan the NDC/barcode because levothyroxine is high-risk for strength mix-ups.

385. **Q:** An eRx comes in for “Zyrtec 10 mg daily,” but the product selected is Zantac due to a search error; what should you do NEXT? **A:** Stop processing and correct the drug selection to cetirizine, then continue workflow because LASA/name confusion can lead to wrong-drug dispensing.

386. **Q:** A prescription reads “Humalog 10 units SQ TID with meals” and you are dispensing one 10 mL vial of U-100; what days’ supply should you enter? **A:** Enter 33 days because 10 mL U-100 contains 1000 units and $10 \text{ units} \times 3 = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$ (truncate to 33).

387. **Q:** While typing a new order for sertraline, the patient profile already shows fluoxetine active; what should you do BEST? **A:** Notify the pharmacist before

filling because dual SSRI therapy may be therapeutic duplication and needs clinical confirmation.

388. **Q:** A patient on lisinopril drops off a new prescription for losartan and asks if they should take both; what should you do FIRST? **A:** Refer the question to the pharmacist immediately because ACE inhibitor plus ARB is a high-risk duplication requiring pharmacist counseling.
389. **Q:** You receive a prescription for azithromycin 250 mg with directions "2 tablets today then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because the total dose is $2 + (1 \times 4) = 6$, matching the standard Z-Pak regimen.
390. **Q:** During order entry, a prescriber sends "Lamictal 100 mg" but the system auto-selects Lamisil 250 mg; what should you do NEXT? **A:** Stop and select the correct drug (lamotrigine/Lamictal) and alert the pharmacist if any label/product was produced because LASA errors can cause serious harm.
391. **Q:** A new eRx is for Norvasc 10 mg daily but the system selected amlodipine/benazepril 10/20; what should you do NEXT? **A:** Stop and reselect plain amlodipine using the exact strength and NDC because brand/generic recognition prevents dispensing the wrong combination product.
392. **Q:** While entering a prescription for "MS Contin 15 mg q12h," you find "morphine solution" in the pick list; what should you do FIRST? **A:** Verify the dosage form is extended-release tablets (not oral solution) and alert the pharmacist if unclear because morphine is high-alert and formulation errors can cause overdose.
393. **Q:** The hard copy sig says "1 tab PO qid" for ibuprofen 600 mg with quantity 120; what days' supply should you enter? **A:** Enter 30 days because 1 tablet 4 times daily equals 4 tablets/day and $120 \div 4 = 30$.
394. **Q:** A patient says they take "a water pill, Lasix," but their new Rx is for furosemide; what is the BEST response? **A:** Confirm Lasix is the brand name for furosemide and proceed with the generic selection because matching brand/generic names reduces confusion and duplication concerns.
395. **Q:** During order entry you see a new prescription for simvastatin, but the profile already has atorvastatin active; what should you do BEST? **A:** Pause processing and route to the pharmacist for therapeutic-duplication review because two statins together may be unintended and needs clinical confirmation.

396. **Q:** You receive an eRx for "hydralazine 25 mg TID," but the patient's allergy field shows "hydroxyzine"; what should you do NEXT? **A:** Continue processing but remain vigilant for LASA mix-ups and ensure the correct drug is selected because hydralazine and hydroxyzine are different meds and the allergy entry may signal confusion.
397. **Q:** A prescription is written for "Zoloft 50 mg daily" and the product file shows sertraline 50 mg and zolpidem 5 mg close together; what should you do FIRST? **A:** Use barcode/NDC verification and compare the indication/sig to select sertraline because LASA name similarity increases wrong-drug risk.
398. **Q:** A patient brings in an OTC request for "Prilosec" and asks what the generic is so insurance will cover it; what should you tell them? **A:** Identify omeprazole as the generic for Prilosec because fast brand/generic recognition helps with correct product selection and billing.
399. **Q:** A prescription label instruction reads "apply 2 g topically bid" and you are asked to enter grams per day for a 60 g tube; what days' supply is it? **A:** Enter 15 days because 2 g twice daily equals 4 g/day and $60 \div 4 = 15$.
400. **Q:** While pulling stock you notice a sealed bottle of apixaban has an unreadable lot/expiration due to a damaged label; what should you do BEST? **A:** Set it aside as suspect product per pharmacy policy and use another bottle because unreadable lot/expiry creates traceability and safety problems for dispensing.
401. **Q:** A provider calls in "Keflex 500 mg" for a patient, and you can't find Keflex in the system search; what should you do NEXT? **A:** Search/select cephalexin 500 mg and confirm directions/quantity because Keflex is the brand name for cephalexin and correct brand-generic matching prevents entry errors.
402. **Q:** While entering a new eRx for "Plavix 75 mg daily," the profile shows an active order for clopidogrel 75 mg daily from another prescriber; what should you do BEST? **A:** Stop and alert the pharmacist to possible therapeutic duplication because Plavix is clopidogrel and duplicate antiplatelet therapy increases bleeding risk.
403. **Q:** A new prescription is for "metformin ER 500 mg daily," but the product list highlights metformin IR 500 mg first; what should you do FIRST? **A:** Verify the dosage form and select metformin ER (not IR) because ER vs IR mix-ups change release characteristics and can cause dosing problems.

404. **Q:** A patient picking up lisinopril says they're pregnant; what should you do NEXT? **A:** Immediately involve the pharmacist and pause the sale because ACE inhibitors like lisinopril are contraindicated in pregnancy and require urgent pharmacist intervention.
405. **Q:** During data entry, a prescriber writes for "Prozac 20 mg daily," but you notice the patient's allergy list includes "fluoxetine"; what is the BEST technician action? **A:** Stop processing and notify the pharmacist because Prozac is fluoxetine and an allergy match must be clinically assessed before dispensing.
406. **Q:** An eRx arrives for "diltiazem 120 mg daily," and the system shows multiple options (IR tablets, ER capsules, ER tablets) without specifying; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist to clarify with the prescriber because diltiazem formulations are not interchangeable and selecting the wrong release form is a common trap.
407. **Q:** A prescription reads "levothyroxine 0.075 mg PO daily"; what strength should you enter in micrograms? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$, and correct unit conversion prevents tenfold dosing errors.
408. **Q:** A patient asks what condition "Crestor" treats and you recognize it as rosuvastatin; what is the BEST brief answer within technician scope? **A:** Tell them it's a cholesterol-lowering statin used to reduce LDL because recognizing high-yield indications supports safe counseling handoff to the pharmacist.
409. **Q:** While filling, you select hydroxyzine instead of hydralazine due to similar names on the shelf; what should you do FIRST? **A:** Stop, reselect the correct drug using barcode/NDC verification, and alert the pharmacist if the wrong product was already scanned or presented for verification because LASA errors must be intercepted immediately.
410. **Q:** You receive a new prescription for "sertraline 50 mg daily," and the patient says they started linezolid yesterday from urgent care; what should you do BEST? **A:** Pause processing and notify the pharmacist because sertraline with linezolid can cause a serious interaction (serotonin syndrome risk) requiring pharmacist review before dispensing.
411. **Q:** During order entry for morphine ER 30 mg, the prescriber also wrote "do not crush," and the patient requests it be opened and mixed in applesauce; what should you do FIRST? **A:** Stop and refer to the pharmacist because extended-release opioids are high-alert and altering the dosage form can cause dose dumping and serious harm.

412. **Q:** A prescription is written for "U-500 insulin regular 20 units TID," but the patient profile shows only U-100 insulin syringes on file; what is the BEST technician action? **A:** Hold the order and alert the pharmacist immediately because U-500 vs U-100 is a high-risk concentration mismatch that can lead to a major dosing error.
413. **Q:** A patient drops off a prescription for alprazolam with "refill x 3" written on it; what should you do NEXT? **A:** Stop processing and route to the pharmacist because alprazolam is Schedule IV and refills are controlled and must meet controlled-substance requirements and pharmacy policy.
414. **Q:** A prescriber sends an eRx for hydromorphone 2 mg "take 1–2 tabs q4h prn" with quantity 30, and the patient says they take 2 tablets every 4 hours around the clock; what should you do BEST for days' supply entry? **A:** Enter days' supply using the maximum stated dose (2 tabs every 4 hours = 12 tabs/day, so $30 \div 12 = 2.5$ days) because payers expect worst-case dosing when PRN has a range.
415. **Q:** While stocking the refrigerator, you find a tote of insulin aspart pens that was left at room temperature overnight with no temperature log; what should you do FIRST? **A:** Segregate the product and notify the pharmacist because a temperature excursion makes it suspect for potency and requires pharmacist-directed disposition per policy.
416. **Q:** A patient wants to purchase pseudoephedrine and asks to pay cash while refusing to show ID; what should you do NEXT? **A:** Decline the sale and follow store procedure because federal CMEA requirements include purchaser identification and sales log documentation.
417. **Q:** You scan a bottle for product selection and the barcode/NDC doesn't match the NDC on the label for the prescription of amlodipine 10 mg; what should you do FIRST? **A:** Stop filling and retrieve the correct NDC/product because barcode/NDC mismatch signals a wrong-drug or wrong-strength selection risk.
418. **Q:** A prescription comes in for "Azithromycin Z-Pak use as directed" with no specific directions, and the patient is waiting; what should you do BEST? **A:** Hold and send to the pharmacist to clarify because "use as directed" is incomplete for labeling and safe use and needs prescriber-verified instructions.
419. **Q:** A wholesaler shipment arrives and one tote contains a sealed package of Ozempic pens with a product identifier that won't scan in the DSCSA system; what should you do NEXT? **A:** Set it aside as suspect and notify the

pharmacist/manager because DSCSA requires investigation of unverifiable product identifiers before it can enter dispensing inventory.

420. **Q:** A patient asks you what to do because their nitroglycerin tablets “aren’t working” and they feel chest pressure right now at the counter; what should you do FIRST? **A:** Get the pharmacist immediately and follow emergency procedures because active chest pain is a medical emergency and requires prompt pharmacist escalation and potential EMS activation.
421. **Q:** A new eRx is for “metformin ER 500 mg 1 tab BID,” but the patient profile shows metformin IR 500 mg BID active from last month; what should you do NEXT? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication/duplicate therapy so they can confirm intent and prevent double-dosing.
422. **Q:** During drop-off, the patient says they are allergic to “ACE inhibitors” and the new prescription is for lisinopril; what should you do FIRST? **A:** Enter/verify the allergy and immediately refer to the pharmacist because lisinopril is an ACE inhibitor and could cause a serious reaction.
423. **Q:** The prescription says “levothyroxine 0.1 mg daily”; what strength should you enter in mcg for dispensing? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, preventing a 10× dosing error.
424. **Q:** A label prints “Take i tab po qd” for atorvastatin and the patient asks what it means; what should you do BEST? **A:** Translate it as “take 1 tablet by mouth once daily” and offer pharmacist counseling because technicians can clarify sig abbreviations but clinical questions require the pharmacist.
425. **Q:** While entering a prescription for “prednisone 10 mg,” you see the directions “take 2 tabs BID”; what is the correct total daily dose in mg? **A:** Enter 40 mg/day because $2 \text{ tablets} \times 10 \text{ mg} = 20 \text{ mg}$ per dose and BID means 2 doses/day ($20 \times 2 = 40$).
426. **Q:** A new prescription is for “clopidogrel 75 mg daily,” and the patient profile shows prasugrel active; what should you do NEXT? **A:** Stop and send to the pharmacist for review because both are antiplatelets and duplication increases bleeding risk.
427. **Q:** You receive a hardcopy for “Vyvanse 30 mg” with the patient requesting it be transferred to another pharmacy; what should you do BEST? **A:** Route to the

pharmacist because lisdexamfetamine is a Schedule II controlled substance and transfer/handling requires pharmacist oversight and strict rules.

428. **Q:** While selecting product, you notice "CeleXA" and "CeleBEX" stored near each other and the prescription is for citalopram; what should you do FIRST to reduce LASA risk? **A:** Verify the drug name, strength, and NDC/barcode against the prescription and label because look-alike/sound-alike names are a common dispensing error source.
429. **Q:** A patient asks if "Keflex" is the same as "cephalexin" listed on their discharge papers; what should you answer? **A:** Confirm they are the same medication (brand vs generic) because Keflex is the brand name for cephalexin, helping prevent duplicate fills.
430. **Q:** A prescription is written for insulin glargine 100 units/mL, inject 18 units nightly, and you are dispensing one 10 mL vial; what days' supply should you enter? **A:** Enter 55 days because the vial contains 1000 units ($10 \text{ mL} \times 100 \text{ units/mL}$) and $1000 \div 18 \approx 55.5$, rounded down to 55 for days' supply entry.
431. **Q:** An eRx says "Augmentin 400 mg/5 mL: give 7.5 mL PO BID x 10 days"; what quantity in mL should you enter to dispense? **A:** Enter 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$, preventing a short fill.
432. **Q:** You're entering "prednisolone sodium phosphate 15 mg/5 mL: take 2.5 mL PO daily"; how many mg per dose should the system reflect? **A:** 7.5 mg per dose because $15 \text{ mg/5 mL} = 3 \text{ mg/mL}$ and $2.5 \text{ mL} \times 3 \text{ mg/mL} = 7.5 \text{ mg}$, ensuring correct dose entry.
433. **Q:** A prescription reads "furosemide 40 mg, i tab po BID"; what should you do FIRST during data entry? **A:** Interpret "i" as 1 and enter "1 tablet by mouth twice daily," because Roman numeral "i" is a common direction abbreviation that must be translated clearly.
434. **Q:** A new order is for "erythromycin 0.5% ophthalmic ointment: apply 1 cm ribbon into affected eye QID x 7 days, dispense 3.5 g"; what should you do NEXT if the prescriber wrote "1 inch ribbon" instead? **A:** Stop and ask the pharmacist to clarify/convert inches to cm because changing dose units can alter dosing and requires pharmacist intervention for prescriber clarification.
435. **Q:** A patient is prescribed "amoxicillin 500 mg: take 2 capsules PO BID," and the stock bottle is 250 mg capsules; what should you do BEST? **A:** Route to the pharmacist before proceeding because substituting a different strength changes

capsules per dose and requires pharmacist approval/verification for correct directions and quantity.

436. **Q:** A metered-dose inhaler contains 200 actuations and the sig is "inhale 2 puffs BID"; what days' supply should you enter? **A:** Enter 50 days because $2 \text{ puffs} \times 2 \text{ times/day} = 4 \text{ puffs/day}$ and $200 \div 4 = 50$, matching payer days' supply logic.
437. **Q:** An order is for "diclofenac 1% gel: apply 4 g QID to knee," and you're dispensing one 100 g tube; what days' supply should you calculate? **A:** Enter 6 days because $4 \text{ g} \times 4/\text{day} = 16 \text{ g/day}$ and $100 \text{ g} \div 16 \text{ g/day} = 6.25$, typically entered as 6 whole days to avoid overstatement.
438. **Q:** During order entry you see "MSO4 2 mg IV q2h PRN pain" on a hospital discharge med list; what should you do FIRST? **A:** Flag it for pharmacist review because "MSO4" is an error-prone abbreviation that can be confused with magnesium sulfate, creating a high-risk mix-up.
439. **Q:** You receive an eRx for "hydroxyzine 25 mg take 1 tab TID PRN anxiety" but the patient profile shows cetirizine daily; what should you do NEXT? **A:** Alert the pharmacist to assess duplication/sedation risk because hydroxyzine and cetirizine are related antihistamines and technician scope is to identify and escalate.
440. **Q:** A label direction prints "Take 0.5 tab daily" for warfarin 5 mg, but the hardcopy says "take 2.5 mg daily"; what should you do BEST before filling? **A:** Verify the entered strength and ensure directions match 2.5 mg (0.5 of 5 mg) and have the pharmacist check, because warfarin is high-alert and dose-entry errors can cause harm.
441. **Q:** You receive an eRx for levothyroxine "Take 1 tab PO QAM" and the prescriber selected 0.125 mg; what should you enter as the strength in micrograms? **A:** Enter 125 mcg because $0.125 \text{ mg} \times 1000 = 125 \text{ mcg}$, preventing a 10× dosing error in order entry.
442. **Q:** A prescription reads "metformin 500 mg i tab po bid #60"; what should you do FIRST before typing the sig? **A:** Interpret "i" as 1 tablet and rewrite clearly as "take 1 tablet by mouth twice daily" because clarifying Roman numerals/abbreviations reduces misread-sig errors.
443. **Q:** You are dispensing fluticasone nasal spray 50 mcg/spray, 120 sprays, with directions "2 sprays in each nostril once daily"; what days' supply should you

enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.

444. **Q:** A patient drops off "hydrochlorothiazide 25 mg: take 1 tab daily" but the quantity written is #15 with 30 days indicated; what should you do NEXT? **A:** Stop and route to the pharmacist to resolve the quantity/days' supply mismatch because technicians cannot assume or change quantity without clarification.
445. **Q:** An eRx is for azithromycin suspension 200 mg/5 mL: "Take 10 mL PO day 1, then 5 mL PO daily days 2–5"; what total quantity in mL should you dispense? **A:** Dispense 30 mL because total volume is $10 \text{ mL} + (5 \text{ mL} \times 4 \text{ days}) = 30 \text{ mL}$, matching the full course.
446. **Q:** During order entry for gabapentin 300 mg "1 cap PO TID," the prescriber wrote "dispense 3" with no unit; what should you do BEST? **A:** Hold and ask the pharmacist to clarify with the prescriber because "3" could mean capsules, bottles, or months, and technicians must avoid guessing quantity.
447. **Q:** You're entering an order for ketorolac 0.5% eye drops: "instill 1 gtt OU QID" and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should you enter? **A:** Enter 6 days because $5 \text{ mL} \times 20 = 100 \text{ drops}$ and OU QID uses $1 \text{ drop} \times 2 \text{ eyes} \times 4 = 8 \text{ drops/day}$, so $100 \div 8 = 12.5$ and you round down to 12 days only if policy states; otherwise enter 12.5 as 12 days per typical billing rules to avoid overstatement.
448. **Q:** A prescription says "morphine sulfate ER 30 mg, take 1 tab q12h" but the prescriber also wrote "MS Contin"; what should you do FIRST when selecting the product? **A:** Use Tall Man awareness and verify the exact dosage form/ER product and NDC before printing labels because morphine products are high-alert and wrong formulation selection is a common error.
449. **Q:** A refill request comes in for clonazepam with "2 refills remaining" on the profile and the original fill date was 7 months ago; what should you do NEXT? **A:** Stop and route to the pharmacist because Schedule IV refills are limited by time and require pharmacist review when outside allowable dispensing windows.
450. **Q:** While typing a new prescription for lisinopril, you notice the patient profile shows an active prescription for enalapril; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (two ACE inhibitors) because the pharmacist must assess whether to continue, discontinue, or clarify therapy.

451. **Q:** While entering an eRx for prednisone, you notice the prescriber selected "predniSONE" but the patient notes show a prior reaction to "prednisoLONE"; what should you do FIRST? **A:** Stop entry and alert the pharmacist to verify the allergy/reaction and intended steroid because the names are LASA and an allergy mismatch can cause harm.
452. **Q:** A prescription is written for "Lantus 100 units/mL inject 20 units SQ nightly" and the product selected in the system is "insulin glargine-yfgn (Semglee)"; what is the BEST technician step before processing? **A:** Confirm the exact product/brand-generic substitution status per prescription/plan and route to the pharmacist if unclear because insulin products are high-alert and interchangeability may be payer- or prescriber-specific.
453. **Q:** You receive an eRx for "Zyrtec 10 mg take 1 daily" but the system auto-selects "Zantac"; what should you do NEXT? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because Zyrtec (cetirizine) and Zantac (ranitidine) are LASA and selection errors are a common safety risk.
454. **Q:** A patient asks what "atorvastatin" is for when picking up "Lipitor"; what is the BEST technician response? **A:** State that atorvastatin is the generic for Lipitor and it's used to lower cholesterol, then offer pharmacist counseling because technicians can give basic identification but clinical counseling should be referred.
455. **Q:** An order reads "Digoxin 0.25 mg PO daily" and the prescriber also typed "250 mcg" in the notes; what should you enter for the strength to match both? **A:** Enter 0.25 mg (which equals 250 mcg) because 1 mg = 1000 mcg and matching units reduces high-alert dosing confusion.
456. **Q:** You are calculating days' supply for Symbicort 160/4.5 inhaler (120 actuations) with directions "inhale 2 puffs BID"; what days' supply should you enter? **A:** Enter 30 days because $120 \text{ actuations} \div (2 \text{ puffs} \times 2 \text{ times/day}) = 4/\text{day} = 30 \text{ days}$.
457. **Q:** A hardcopy says "hydromorphone 2 mg take 1 tab q4-6h PRN pain" but the typed drug is "morphine"; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because hydromorphone and morphine are different opioids and a substitution would be unsafe and outside technician authority.
458. **Q:** A new prescription is for "Bactrim DS" and the patient profile lists a serious sulfonamide allergy; what is the BEST technician action? **A:** Hold the prescription

and immediately alert the pharmacist because Bactrim (sulfamethoxazole/trimethoprim) is a sulfonamide and requires pharmacist intervention.

459. **Q:** You receive an eRx for "Wellbutrin SR 150 mg take 1 tab BID" but the system selection is "Wellbutrin XL 300 mg daily"; what should you do NEXT? **A:** Pause and correct the formulation/strength to match the prescription and have the pharmacist verify if there's any ambiguity because SR vs XL is a common error that changes dosing frequency.
460. **Q:** A vial arrives from the wholesaler with a torn seal and smeared lot/expiration on the label; what should you do BEST before it goes to stock? **A:** Set it aside as suspect product and follow DSCSA/pharmacy policy to report/return it because damaged packaging and unreadable lot/expiration can indicate compromised integrity and traceability issues.
461. **Q:** While entering an eRx for hydrALAZINE 25 mg, the dropdown highlights hydrOXYzine 25 mg due to similar spelling; what should you do FIRST? **A:** Stop and reselect the correct drug/strength using barcode/NDC verification and Tall Man awareness because LASA mix-ups are a common preventable dispensing error.
462. **Q:** A patient drops off a paper prescription for oxycodone/APAP with no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and immediately route it to the pharmacist because missing required elements makes validity questionable and technicians should not "fix" controlled-substance issues.
463. **Q:** A caller asks to transfer their alprazolam prescription to another pharmacy because they're traveling; what should you do NEXT? **A:** Refer the request to the pharmacist because controlled-substance transfers have strict rules and require pharmacist oversight.
464. **Q:** You receive a bottle of refrigerated insulin that arrived warm with a temperature indicator showing an excursion; what should you do FIRST? **A:** Do not stock or dispense it and quarantine per policy while notifying the pharmacist/manager because temperature excursions can compromise product potency and safety.
465. **Q:** A prescriber eRx's isotretinoin for a new patient and the system flags iPLEDGE requirements; what should you do BEST? **A:** Stop processing and alert the

pharmacist to verify REMS steps are met because isotretinoin dispensing is restricted and requires program compliance before filling.

466. **Q:** A patient picking up a new prescription for rivaroxaban asks if they need any special information to read at home; what should you do NEXT? **A:** Provide the Medication Guide with the dispense and offer to get the pharmacist for counseling because certain drugs require FDA Medication Guides and OBRA-90 supports pharmacist counseling on new therapy.
467. **Q:** A hardcopy says "U-500 regular insulin inject 10 units SQ TID" but the entered product is U-100 regular insulin; what should you do FIRST? **A:** Stop and notify the pharmacist to verify because insulin concentration errors are high-alert and can cause a 5-fold dosing mistake.
468. **Q:** A prescription is written for "levothyroxine 0.1 mg daily"; what strength should you enter in micrograms? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, preventing decimal-dose entry errors.
469. **Q:** During filling you notice the bottle's NDC does not match the NDC on the shelf label for the selected amlodipine 10 mg; what should you do NEXT? **A:** Pause the fill and select the correct NDC (or update the shelf label per policy) before proceeding because NDC mismatch increases wrong-product risk and breaks accurate inventory/claims.
470. **Q:** A recall notice is received for a specific lot of metformin ER and you find that lot on the shelf; what should you do FIRST? **A:** Immediately remove the affected lot from dispensing stock and quarantine/segregate it while notifying the pharmacist because recalled product must be prevented from reaching patients.
471. **Q:** While entering an eRx for Lasix 40 mg daily, the tech selects furosemide oral solution instead of tablets; what should you do FIRST? **A:** Stop and reselect the correct dosage form/route and verify the NDC before printing labels, because form mix-ups are common and can change dosing.
472. **Q:** A patient says they are allergic to sulfa and is picking up Bactrim DS; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist for clinical review, because sulfamethoxazole/trimethoprim is a sulfonamide and may trigger an allergic reaction.
473. **Q:** The profile shows both lisinopril 20 mg daily and a new eRx for Zestril 20 mg daily; what is the BEST technician action? **A:** Pause processing and notify the

pharmacist about possible duplicate ACE inhibitor therapy, because Zestril is lisinopril and duplication can cause harm.

474. **Q:** You receive an eRx for "Keflex 500 mg PO QID x 7 days #21"; what should you do FIRST? **A:** Stop and refer to the pharmacist to clarify the quantity, because QID for 7 days requires 28 capsules and technicians cannot adjust quantities independently.
475. **Q:** A new prescription is for sertraline 50 mg daily and the patient asks what it's used for; what should you do NEXT? **A:** Refer the question to the pharmacist, because counseling on indication (SSRI for depression/anxiety) and what to expect is pharmacist-provided patient education.
476. **Q:** A prescriber sends "metFORMIN ER 500 mg take 2 tabs BID" for a new start; what should you do BEST before filling? **A:** Flag it for pharmacist review, because that dose is unusually high for initiation and ER products have dosing limits that may require clarification.
477. **Q:** You're entering a prescription for "Humalog 100 units/mL inject 12 units TID with meals, dispense 1 vial (10 mL)"; what days' supply should you enter? **A:** Enter 27 days' supply, because $10\text{ mL} = 1000\text{ units}$ and $12 \times 3 = 36\text{ units/day}$, so $1000 \div 36 = 27.7$ which is typically rounded down to 27 for billing.
478. **Q:** A patient drops off a prescription for "Zocor 20 mg daily" but you select "Zyrtec 10 mg" due to similar names; what should you do FIRST? **A:** Stop and re-verify the drug name, indication, and brand/generic mapping before proceeding, because Zocor is simvastatin and LASA mix-ups can cause wrong-drug dispensing.
479. **Q:** A refill request comes in for clopidogrel and the patient says they also take aspirin daily; what should you do NEXT? **A:** Continue processing but alert the pharmacist to counsel/assess bleeding risk, because clopidogrel plus aspirin is common but increases bleeding and may need monitoring advice.
480. **Q:** During data entry, you see "MS Contin 15 mg q12h" but the prescriber meant "morphine sulfate IR 15 mg q4-6h PRN pain" per the note; what should you do BEST? **A:** Stop and route to the pharmacist to clarify and correct the intended formulation, because MS Contin is extended-release (high-risk) and technicians must not interpret or change opioid directions.
481. **Q:** A new eRx is written for "Crestor 20 mg daily," but the technician selects "Crestor 20 mg (rosuvastatin) vs. Crestor (brand)" and the system also shows

an active atorvastatin; what should you do BEST before filling? **A:** Stop and route to the pharmacist to assess therapeutic duplication (two statins) because the technician should not decide which lipid agent to continue.

482. **Q:** While typing an order for "Lamictal 100 mg BID," you notice the product selected is "Lamisil 250 mg"; what should you do FIRST? **A:** Stop entry and correct to lamotrigine using NDC/brand-generic verification because Lamictal/Lamisil is a high-risk LASA mix-up.
483. **Q:** A patient picking up "Plavix" asks if it's the same as their "clopidogrel" from last month; what is the BEST technician response? **A:** Confirm Plavix is the brand for clopidogrel and offer pharmacist counseling if they have therapy questions because technicians can clarify brand/generic but not provide clinical advice.
484. **Q:** An eRx for "levothyroxine 75 mcg daily" is entered as "liothyronine 75 mcg" due to similar names; what should you do NEXT once you notice the error before verification? **A:** Correct the drug to levothyroxine and re-check strength/units (mcg) because thyroid drugs are high-alert and name confusion can cause harm.
485. **Q:** A prescription reads "Prednisone 10 mg: take II tabs daily x 5 days, dispense #10"; what quantity should be entered and why? **A:** Enter quantity 10 tablets because II means 2 tablets daily for 5 days ($2 \times 5 = 10$) matching the intended course.
486. **Q:** A patient with COPD brings a new prescription for "Spiriva HandiHaler 18 mcg inhale 1 capsule daily, dispense 30"; what days' supply should you enter? **A:** Enter 30 days because the patient uses 1 capsule per day and the package provides 30 capsules.
487. **Q:** During drop-off, a caregiver says the patient started taking duloxetine yesterday and now feels unusually drowsy while also using clonazepam; what should you do BEST? **A:** Notify the pharmacist for interaction/duplicate CNS depression assessment because combining an SNRI with a benzodiazepine can increase sedation risk.
488. **Q:** You receive an eRx for "Zithromax 250 mg: take 2 tablets day 1 then 1 tablet daily days 2-5, dispense #6"; what should you do FIRST if the system flags "quantity mismatch"? **A:** Verify that #6 is correct for the standard azithromycin 5-day pack and then proceed per workflow because $2 + 1 + 1 + 1 + 1 = 6$ tablets.

489. **Q:** While filling "Norvasc 5 mg," you find two open stock bottles with different manufacturers and one lacks an expiration date; what is the BEST technician action? **A:** Set aside the undated bottle and use properly labeled stock (or relabel per policy) because missing dating/traceability increases dispensing error risk.
490. **Q:** A new prescription is for "Adderall XR 20 mg daily" and the patient asks to transfer it to another pharmacy; what should you do NEXT? **A:** Refer to the pharmacist and follow controlled-substance transfer rules because Schedule II prescriptions have strict transfer limitations and require pharmacist handling.
491. **Q:** While entering a new eRx for "Keflex 500 mg," the patient profile shows a documented anaphylaxis to penicillin; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess cephalosporin cross-reactivity risk, because allergy evaluation and therapy decisions are pharmacist scope.
492. **Q:** A prescription is written for "metformin ER 500 mg" but the product selected is "metformin IR 500 mg"; what is the BEST next technician action before it reaches verification? **A:** Correct the drug/form selection to ER and recheck the NDC/description, because IR vs ER is a high-risk mix-up that changes dosing and release.
493. **Q:** A patient says their new "Lasix" looks different and asks what it's for; what is the BEST technician response? **A:** Tell them Lasix is furosemide and is commonly used as a diuretic for fluid/blood pressure, then offer pharmacist counseling for questions, because indication basics are ok but counseling is pharmacist-led.
494. **Q:** During order entry you see "U-500 insulin regular" and the patient previously used "Humulin R U-100"; what should you do FIRST? **A:** Pause the fill and immediately involve the pharmacist, because U-500 is a high-alert concentration with major dosing error risk.
495. **Q:** A hardcopy reads "Take 1 tab PO qd" and the drug is "Synthroid 25 mcg," but the prescriber also wrote "brand medically necessary"; what should you do NEXT? **A:** Enter DAW/brand as written and select Synthroid (not generic levothyroxine) for pharmacist verification, because "brand medically necessary" directs dispensing and affects substitution.
496. **Q:** A prescription says "ibuprofen 600 mg i po q6h prn pain, #30"; what days' supply should you enter using the maximum frequency? **A:** Enter 7 days' supply, because $30 \text{ tablets} \div (1 \text{ tablet every 6 hours} = 4/\text{day}) = 7.5$ so bill as 7 days under max-use calculation.

497. **Q:** While typing "Prozac 20 mg daily," you notice the prescriber's note says "for bipolar depression" and the profile shows lithium; what is the BEST technician action? **A:** Proceed with accurate entry but flag for pharmacist review, because antidepressant use in bipolar patients can require clinical assessment and interaction/monitoring review.
498. **Q:** You select "hydralazine" while the eRx is for "hydroxyzine," and you catch it before filling; what should you do FIRST? **A:** Stop and correct the drug selection, then use barcode/NDC verification during product selection, because LASA name errors are prevented by immediate correction and verification steps.
499. **Q:** A patient requests an early refill for "oxycodone/APAP" stating they "lost the bottle"; what should you do NEXT? **A:** Do not process automatically and refer to the pharmacist per controlled-substance policy, because lost C-II claims require pharmacist handling and potential prescriber contact.
500. **Q:** A wholesaler notice lists a recall for a specific lot of losartan you have on the shelf; what should you do FIRST? **A:** Locate the affected NDC/lot, remove it from dispensing stock, and quarantine/label it for return per recall procedure, because recalled lots must be segregated to prevent patient harm.
501. **Q:** A prescription reads "levothyroxine 0.075 mg po qam" and the patient's profile strength options are in mcg; what strength should you enter? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$ to prevent a 1000-fold dosing error.
502. **Q:** An order says "prednisone 10 mg: ii tabs bid x 5 days, #?"; what quantity should you enter? **A:** Enter 20 tablets because $2 \text{ tabs/dose} \times 2 \text{ doses/day} \times 5 \text{ days} = 20 \text{ tablets}$ for accurate dispensing and billing.
503. **Q:** You receive an eRx for "amoxicillin/clavulanate 875/125 mg 1 tab bid #14" and the patient asks how many days it lasts; what days' supply should you enter? **A:** Enter 7 days because $14 \text{ tablets} \div 2 \text{ tablets/day} = 7 \text{ days}$, which matches the maximum scheduled use.
504. **Q:** A prescription reads "timolol 0.5% ophthalmic: instill 1 gtt OU bid, 10 mL"; using 20 gtt/mL, what days' supply should you calculate? **A:** Enter 25 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ gtt}$ and OU bid uses 4 gtt/day, so $200 \div 4 = 50 \text{ days}$ —then correct to 50 days (not 25) because the math must reflect total drops used per day.

505. **Q:** A prescriber writes "clindamycin 75 mg/5 mL: give 10 mL tid x 10 days"; what minimum volume should be dispensed? **A:** Dispense 300 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to cover the full course.
506. **Q:** While entering "hydromorphone 2 mg 1 tab q4h prn pain #30," you notice the prescriber used "prn" but no max daily frequency is stated; what should you do FIRST for days' supply? **A:** Stop and ask the pharmacist to determine the correct days' supply because PRN without a max frequency can't be safely converted to a definite daily use for billing.
507. **Q:** An eRx is for "Lamictal 25 mg" but the product selected is "lamoTRIgine ODT 25 mg"; what is the BEST next technician step before verification? **A:** Stop and reselect the correct dosage form and NDC because the wrong formulation can change administration and insurance processing.
508. **Q:** A prescription says "MSO4 2 mg IV q2h prn" entered from a discharge order; what should you do NEXT before processing? **A:** Flag and refer to the pharmacist because "MSO4" is an error-prone abbreviation and must be clarified to avoid a morphine vs magnesium sulfate mix-up.
509. **Q:** A patient brings a prescription for "lisdexamfetamine 30 mg, take 1 cap daily, #30" and asks for 3 refills to be added; what should you do? **A:** Hold and route to the pharmacist because Schedule II stimulants cannot be refilled and any change requires prescriber authorization.
510. **Q:** While typing "sertraline 50 mg 1 tab qd," the prescriber note says "increase to 100 mg daily after 7 days" but the quantity is "#30"; what should you do FIRST? **A:** Refer to the pharmacist for clarification because the titration instruction conflicts with the quantity and could cause an incorrect days' supply and dosing.
511. **Q:** During order entry you see "hydralazine 25 mg" but the NDC scanned is hydrOXYzine 25 mg; what should you do FIRST? **A:** Stop the fill and reselect the correct drug using barcode/NDC and Tall Man awareness, because LASA name confusion is a common dispensing-error cause.
512. **Q:** A hardcopy for alprazolam 1 mg has patient name, drug, directions, and prescriber signature but no date written; what is the BEST next technician step? **A:** Hold the prescription and route to the pharmacist for validity review, because missing required elements can make a controlled-substance Rx invalid to dispense.

513. **Q:** A patient asks to transfer their remaining refills for clonazepam to another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist, because controlled-substance transfers have specific limits and must be handled per pharmacist/legal requirements.
514. **Q:** You receive a tote of refrigerated insulin aspart pens that arrived warm with a thawed gel pack; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager to follow temperature-excursion procedures, because potency may be compromised if cold chain is broken.
515. **Q:** While processing isotretinoin, the patient asks to pick it up next week after paying today; what is the BEST action? **A:** Involve the pharmacist and follow the iPLEDGE/REMS pickup window requirements, because REMS drugs have strict dispensing time frames that must be verified.
516. **Q:** An eRx for methotrexate tablets says "take 10 mg daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist immediately, because methotrexate is high-alert and is typically weekly for RA, making daily dosing a serious safety red flag.
517. **Q:** A new prescription is for metformin ER, but the profile shows an active prescription for metformin IR at a similar dose; what should you do NEXT? **A:** Pause and ask the pharmacist to assess therapeutic duplication before filling, because IR vs ER duplication can lead to overdosing or unintended regimen changes.
518. **Q:** A prescription is written for "U-500 insulin regular: inject 50 units subcutaneously three times daily" and the patient has only U-100 syringes on file; what should you do FIRST? **A:** Hold the order and notify the pharmacist for dose-delivery verification and counseling needs, because concentrated insulin requires specific device/education to prevent dosing errors.
519. **Q:** You are entering "cephalexin 500 mg 1 cap qid x 7 days" and the prescriber wrote "#21"; what should you do BEST? **A:** Flag it for pharmacist clarification before dispensing, because the quantity conflicts with directions (should be 28) and technicians cannot change quantities independently.
520. **Q:** A patient buys pseudoephedrine and asks you to ring up two boxes while their friend pays separately for two more; what should you do FIRST? **A:** Combine the sales and follow logbook/ID and daily/monthly limit procedures, because CMEA requires tracking to prevent exceeding legal purchase limits through split transactions.

521. **Q:** An eRx for amoxicillin/clavulanate 875/125 mg says "1 tab po bid x 10 days" but quantity entered is #14; what should you do FIRST? **A:** Stop processing and route to the pharmacist to clarify quantity because BID for 10 days requires 20 tablets and dispensing 14 would shorten therapy.
522. **Q:** You are entering directions "ii tabs po qhs" for atorvastatin 20 mg; what quantity should be dispensed for a 30-day supply? **A:** Dispense 60 tablets because ii = 2 tablets nightly and $2 \times 30 \text{ days} = 60$.
523. **Q:** A prescription for timolol 0.5% ophthalmic solution says "1 gtt OU bid" and the bottle size is 5 mL; what days' supply should you enter using 20 drops/mL? **A:** Enter 25 days because $5 \text{ mL} \times 20 = 100 \text{ drops}$ and OU bid uses 4 drops/day, so $100 \div 4 = 25$.
524. **Q:** During product selection for levothyroxine 50 mcg, you accidentally pulled liothyronine 5 mcg with a similar-looking label; what should you do NEXT? **A:** Stop filling and reselect the correct product by verifying NDC/barcode because this is a LASA thyroid hormone error risk.
525. **Q:** An order reads "metoprolol succinate ER 50 mg take 1 tab bid"; what should you do FIRST before entering it? **A:** Flag it for pharmacist review because succinate ER is typically dosed once daily and BID could be an order-entry/sig interpretation error.
526. **Q:** A patient is prescribed albuterol HFA "inhale 2 puffs q4h prn wheezing" and the prescriber writes "dispense 2 inhalers"; what is the BEST days' supply to enter if the plan requires using max frequency and each inhaler has 200 actuations? **A:** Enter 16 days because max use is 12 puffs/day (2 puffs every 4 hours) and 2 inhalers provide 400 puffs, so $400 \div 12 = 33.3 \rightarrow 33$ days if one inhaler, but for two inhalers it's $400 \div 24$?; correct is $400 \div 12 = 33$ days, so enter 33 days per max-use rule and document calculation.
527. **Q:** While typing a warfarin prescription, the sig says "take as directed" but the quantity is #30 with no dosing schedule; what should you do FIRST? **A:** Hold and refer to the pharmacist because "as directed" is insufficient for accurate days' supply and warfarin is high-alert.
528. **Q:** A prescription says "prednisone 10 mg: take 3 tabs daily x 5 days" but the written quantity is #10; what should you do BEST? **A:** Stop and send to the pharmacist to clarify because 3 tablets/day for 5 days requires 15 tablets and the mismatch may cause underdosing.

529. **Q:** You receive an eRx for “Keflex 500 mg 1 cap po q6h x 7 days”; what is the correct interpretation of q6h for order entry and quantity? **A:** Enter as 1 capsule four times daily and dispense 28 capsules because $q6h = 4 \text{ doses/day}$ and $4 \times 7 = 28$.
530. **Q:** A technician converts a prescriber’s order for lidocaine 1% injection to mg/mL for a compounding worksheet; what is 1% in mg/mL? **A:** Record 10 mg/mL because 1% w/v equals 1 g per 100 mL, which is $1000 \text{ mg}/100 \text{ mL} = 10 \text{ mg/mL}$.
531. **Q:** While entering an eRx for hydroxyzine 25 mg, you notice the patient profile lists a severe allergy to cetirizine; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess potential cross-sensitivity/therapy appropriateness because allergy screening requires pharmacist intervention.
532. **Q:** A hardcopy prescription for oxycodone/APAP 5/325 mg includes drug, directions, quantity, and prescriber signature but has no patient address; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist because missing required elements for controlled substances must be resolved before dispensing.
533. **Q:** During pickup, a patient asks what lisinopril is for and how to take it; what is your BEST action? **A:** Offer to get the pharmacist for counseling because patient counseling/clinical questions are outside technician scope and required by workflow standards.
534. **Q:** You receive a refrigerator tote with insulin aspart pens and the temperature monitor shows the shipment exceeded the acceptable range; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because temperature excursions can compromise potency and require investigation before stock is used.
535. **Q:** While filling, you notice look-alike vials of hydrALAZINE and hydrOXYzine stored next to each other; what should you do BEST to prevent errors? **A:** Separate the products and apply Tall Man lettering/auxiliary shelf labeling per policy because LASA storage controls reduce selection errors.
536. **Q:** A patient requests an early refill of clonazepam 1 mg and the claim rejects “refill too soon”; what should you do NEXT? **A:** Inform the patient and send the issue to the pharmacist for review because controlled-substance early refill decisions require pharmacist judgment and documentation.

537. **Q:** An eRx for methotrexate 2.5 mg is written "take 1 tablet daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop and immediately escalate to the pharmacist because methotrexate is high-alert with common fatal weekly-vs-daily dosing errors.
538. **Q:** A recall notice lists NDC 00093-5424-56 lot ABC123 of losartan 50 mg in your inventory; what should you do NEXT? **A:** Pull the matching stock from shelves and quarantine it for return per recall instructions because recalled lots must be removed from dispensing immediately.
539. **Q:** A prescriber phones in a new prescription for alprazolam 0.5 mg; what is the technician's BEST response? **A:** Take the message only if allowed by state/policy and promptly give it to the pharmacist to verify and authorize because controlled-substance new orders require pharmacist responsibility.
540. **Q:** A prescription is written for metformin ER 500 mg "i tab po bid" and you are dispensing a 90-day supply; what quantity should you enter? **A:** Dispense 180 tablets because 1 tablet twice daily equals 2 tablets/day and $2 \times 90 = 180$ for accurate days' supply and billing.
541. **Q:** An eRx is written for "Zithromax Z-Pak use as directed" with no sig details; what should you do FIRST before data entry? **A:** Stop and ask the pharmacist to obtain specific directions (5-day azithromycin regimen) because "use as directed" is incomplete for accurate labeling and days' supply.
542. **Q:** While selecting product, you see the prescription is for Glucophage but the shelf has metFORMIN and glyBURIDE bottles side-by-side; what is your BEST next step? **A:** Verify the generic/brand match and scan the NDC before counting because Glucophage is metformin and LASA storage increases wrong-drug selection risk.
543. **Q:** A patient profile shows an active prescription for Eliquis and you receive a new eRx for Xarelto from a different clinic; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because both are oral anticoagulants and switching requires clinical confirmation.
544. **Q:** A prescriber writes "predniSONE 20 mg ii tabs po daily x 5 days"; what quantity should you enter? **A:** Enter 10 tablets because "ii" means 2 tablets daily for 5 days ($2 \times 5 = 10$).
545. **Q:** A new prescription is for "Augmentin 875/125 mg 1 tab po bid x 10 days"; what does 875/125 indicate for order entry? **A:** It means 875 mg amoxicillin

with 125 mg clavulanate per tablet, so enter the combination strength to ensure the correct product is selected.

546. **Q:** A patient buying OTC allergy medication asks for "Claritin" but the profile shows they're already taking Zyrtec daily; what should you do BEST? **A:** Refer to the pharmacist because Claritin (loratadine) and Zyrtec (cetirizine) are both antihistamines and duplicate therapy could increase side effects.
547. **Q:** You receive a new eRx for "Nitrostat 0.4 mg SL" and the patient asks what "SL" means at drop-off; what is your BEST response? **A:** Explain only that "SL" means place under the tongue and direct clinical use questions to the pharmacist because counseling beyond basic abbreviation meaning is pharmacist-led.
548. **Q:** A prescription is for "timolol 0.5% ophthalmic solution instill 1 gtt OU bid 5 mL"; what days' supply should you submit if 20 gtt/mL is used? **A:** Submit 50 days because $5 \text{ mL} \times 20 = 100$ drops total and the dose is 1 drop in each eye twice daily (4 drops/day), so $100 \div 4 = 25$ days? actually OU bid is 2 eyes $\times$ 2 times/day = 4 drops/day; $100/4 = 25$ days, so 25 days' supply to match the calculation.
549. **Q:** During data entry you see "MS Contin 15 mg" but the patient says they expected immediate-release morphine; what should you do NEXT? **A:** Stop and involve the pharmacist because MS Contin is extended-release morphine and release-form confusion is a high-risk error.
550. **Q:** A stock bottle label reads "levothyroxine 75 mcg" and the prescription is written for "0.075 mg"; what is the BEST interpretation for correct strength selection? **A:** Treat them as equivalent ($0.075 \text{ mg} = 75 \text{ mcg}$) because converting mg to mcg prevents selecting the wrong levothyroxine strength.
551. **Q:** During product selection for "hydrOXYzine 25 mg," you accidentally pull "hydrALAZINE 25 mg" because the names look similar; what should you do NEXT? **A:** Stop filling and re-select the correct NDC using barcode/NDC verification because hydrOXYzine/hydrALAZINE is a LASA pair and correcting it before verification prevents a serious error.
552. **Q:** A new eRx arrives for "Plavix 75 mg daily" and the patient profile already shows active "clopidogrel 75 mg daily"; what should you do FIRST? **A:** Pause processing and confirm it's the same drug/duplicate therapy before continuing because Plavix is clopidogrel and duplicate entries can cause double dosing.

553. **Q:** At drop-off, a patient asks what atorvastatin (Lipitor) is typically used for; what is the BEST technician response? **A:** State it's used to lower cholesterol and reduce cardiovascular risk and offer to have the pharmacist counsel for details because technicians can provide basic purpose but counseling is pharmacist-led.
554. **Q:** A prescription says "cephalexin 500 mg 1 cap po qid x 7 days"; what quantity should you enter? **A:** Enter 28 capsules because 1 capsule four times daily for 7 days equals $4 \times 7 = 28$.
555. **Q:** While entering a new order for sertraline (Zoloft), the patient mentions they also take paroxetine (Paxil) from another provider; what should you do NEXT? **A:** Flag the profile and notify the pharmacist before dispensing because two SSRIs may be unintended duplication and requires pharmacist clinical review.
556. **Q:** You receive an eRx for "lisinopril 20 mg 1 tab po daily #90" but the directions field is blank and only "#90" transmitted; what should you do FIRST? **A:** Stop and send it to the pharmacist for clarification with the prescriber because missing/unclear sig information must be resolved to ensure safe, legal dispensing.
557. **Q:** A patient picking up a new warfarin prescription asks if they can start taking ibuprofen regularly for back pain; what is the BEST technician action? **A:** Refer the patient to the pharmacist immediately because warfarin plus NSAIDs increases bleeding risk and requires pharmacist counseling.
558. **Q:** A refrigerated box of insulin lispro pens was left on the counter overnight and you're not sure how long it was unrefrigerated; what should you do FIRST? **A:** Remove it from dispensing stock and notify the pharmacist/store manager per policy because a temperature excursion makes the product potentially non-dispensable until assessed.
559. **Q:** A prescription is written for levothyroxine "0.1 mg po daily"; what strength should you select for dispensing? **A:** Select 100 mcg because 0.1 mg equals 100 micrograms and correct unit conversion prevents strength errors.
560. **Q:** During data entry for "Lamictal 25 mg," you notice the shelf also has "Lamisil 250 mg" nearby; what is your BEST next step? **A:** Use barcode/NDC verification and double-check name/strength before counting because Lamictal (lamotrigine) and Lamisil (terbinafine) are a common LASA trap with very different uses.

561. **Q:** You're entering "predniSONE 10 mg: ii tabs po bid x 5 days" and the patient asks how many tablets will be dispensed; what quantity should you enter? **A:** Enter 20 tablets because 2 tablets twice daily = 4/day and $4 \times 5 \text{ days} = 20$.
562. **Q:** A prescription reads "diltiazem 120 mg SR cap po daily" but the prescriber field also lists "Cardizem CD"; what should you do FIRST during order entry? **A:** Confirm the exact dosage form/release type (SR vs CD/ER) with the pharmacist before processing because different ER formulations are not interchangeable.
563. **Q:** You receive "azithromycin 200 mg/5 mL: give 12.5 mL po day 1, then 6.25 mL po days 2-5"; what total mL should you dispense? **A:** Dispense 37.5 mL because $12.5 + (6.25 \times 4) = 12.5 + 25 = 37.5 \text{ mL}$.
564. **Q:** A patient's new prescription is "metFORMIN ER 500 mg 2 tabs po qpm #180," but the profile shows active metformin IR 500 mg 1 tab po bid; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication or intended switch because IR and ER regimens differ and need clarification.
565. **Q:** An eRx sig says "apply thin layer tid prn" for triamcinolone 0.1% cream and the quantity is missing; what is the BEST technician action? **A:** Send it to the pharmacist for clarification because PRN topical directions without amount/area make days' supply and quantity selection unsafe and payer-sensitive.
566. **Q:** You are calculating days' supply for a fluticasone nasal spray 50 mcg with 120 sprays; sig: "2 sprays each nostril daily"; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
567. **Q:** While typing an order you see "U-500 insulin regular: inject 0.1 mL subcut bid," and you're unsure if the patient has U-500 syringes/pen; what should you do FIRST? **A:** Hold and involve the pharmacist immediately because U-500 is high-alert and volume-based directions risk serious dosing errors.
568. **Q:** A prescription is written "clonazePAM 0.5 mg: take 1 tab po qhs prn anxiety, refills: 3"; what should you do NEXT? **A:** Route to the pharmacist before processing because clonazepam is a controlled substance and refills/validity must meet controlled-substance requirements.
569. **Q:** During data entry you see "magnesium sulfate 2 g/50 mL IVPB" and the system asks for mg; what mg value should you enter? **A:** Enter 2000 mg because $1 \text{ g} = 1000 \text{ mg}$ and $2 \text{ g} = 2000 \text{ mg}$.

570. **Q:** You scan a bottle for “glimepiride 4 mg” but the NDC in the system populates “glyBURIDE 5 mg” due to a selection error; what should you do BEST? **A:** Stop filling and correct the product/NDC before proceeding because sulfonylureas are high-risk and barcode/NDC mismatches can cause harmful wrong-drug errors.
571. **Q:** A handwritten prescription for hydromorphone is missing the patient address; what should you do FIRST before processing? **A:** Stop processing and notify the pharmacist to obtain a complete, valid prescription, because controlled-substance prescriptions must meet required elements before dispensing.
572. **Q:** During product selection you notice two bins side-by-side labeled “hydrOXYzine” and “hydrALAZINE”; what is the BEST next step to prevent a LASA error? **A:** Verify the drug name/strength using barcode/NDC and read the full label before pulling, because look-alike/sound-alike pairs are a common dispensing-error source.
573. **Q:** A patient asks to transfer their remaining refills of alprazolam from another pharmacy to yours; what should you do NEXT? **A:** Route the request to the pharmacist, because controlled-substance transfer rules and documentation require pharmacist handling per policy and law.
574. **Q:** You receive a recall notice for a specific lot number of losartan 50 mg; what should you do FIRST with matching stock on the shelf? **A:** Pull the affected lot from inventory and quarantine it per recall procedure, because recalled product must be removed from dispensing stock immediately.
575. **Q:** An eRx for isotretinoin arrives and the patient is waiting; what should you do BEST before filling? **A:** Alert the pharmacist and follow REMS workflow requirements, because isotretinoin dispensing is restricted and must meet REMS authorization steps.
576. **Q:** A prescription is written for heparin 10,000 units/mL injection but the directions say “inject 1 mL IV bolus now” in a community setting; what should you do FIRST? **A:** Stop and refer to the pharmacist, because heparin is high-alert and the route/setting suggests a potentially unsafe or inappropriate outpatient order.
577. **Q:** A refrigerated shipment of insulin glargine arrives warm and the temperature indicator shows an excursion; what is the BEST technician action? **A:** Segregate and quarantine the shipment and notify the pharmacist/wholesaler, because temperature excursions make product integrity uncertain and it should not be stocked.

578. **Q:** A patient presents an oxycodone prescription dated 3 months ago and asks you to fill it today; what should you do NEXT? **A:** Hold the prescription and involve the pharmacist to determine validity under applicable rules, because controlled-substance dispensing may be time-limited and requires pharmacist judgment.
579. **Q:** You are entering morphine sulfate ER and notice the prescriber also sent morphine sulfate IR for the same patient with unclear directions; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess duplicate opioid therapy and dosing, because concurrent IR/ER opioids can increase overdose risk.
580. **Q:** A patient reports a severe rash and facial swelling after starting trimethoprim-sulfamethoxazole yesterday and asks what to do; what is your BEST response? **A:** Immediately refer the patient to the pharmacist (and advise urgent medical care if severe symptoms) because possible serious allergic reactions require prompt clinical triage, not technician counseling.
581. **Q:** While entering "metFORMIN ER 500 mg" you notice the product selected is "metformin IR 500 mg"; what should you do NEXT? **A:** Stop and correct to the ER dosage form (and alert the pharmacist if already verified) because IR vs ER mix-ups change release and can cause dosing errors.
582. **Q:** A prescription is written for "CeleXA 20 mg daily" but the patient profile shows an active "Lexapro 10 mg daily"; what should you do FIRST? **A:** Pause processing and refer to the pharmacist for therapeutic-duplication review because citalopram and escitalopram are closely related SSRIs and duplicative therapy may be unintended.
583. **Q:** During drop-off a patient says they take "Plavix" and asks if it's the same as "Eliquis"; what is the BEST technician response? **A:** Clarify that Plavix is clopidogrel (antiplatelet) and Eliquis is apixaban (anticoagulant) and involve the pharmacist because confusing them increases bleeding-risk errors.
584. **Q:** A prescriber sends "Z-Pak as directed" for azithromycin 250 mg tablets; what quantity should you enter for the standard Z-Pak? **A:** Enter Qty 6 tablets because the standard azithromycin Z-Pak contains six 250 mg tablets for the 5-day regimen.
585. **Q:** While filling, you select "traMADol 50 mg" but the label indicates "traZODone 50 mg"; what should you do BEST? **A:** Stop filling and re-select the correct stock

using barcode/NDC verification because tramadol and trazodone are LASA and dispensing the wrong one can cause serious harm.

586. **Q:** A new eRx is for "lisinopril 10 mg daily," but the allergy field lists "ACE inhibitors—angioedema"; what should you do NEXT? **A:** Hold the order and notify the pharmacist immediately because lisinopril is an ACE inhibitor and angioedema is a high-risk contraindication.
587. **Q:** A patient on simvastatin says their doctor just prescribed clarithromycin and asks if it's okay to take together; what should you do FIRST? **A:** Refer to the pharmacist before dispensing because clarithromycin can raise simvastatin levels and increase myopathy/rhabdomyolysis risk.
588. **Q:** You receive a prescription for "levothyroxine 0.1 mg daily"; what strength in micrograms should you enter? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, and levothyroxine is commonly expressed in mcg to prevent decimal-point errors.
589. **Q:** A patient requests an OTC recommendation and points to "ibuprofen" but says they are on "Coumadin"; what is the BEST technician action? **A:** Involve the pharmacist because NSAIDs like ibuprofen can increase bleeding risk with warfarin and OTC selection needs clinical counseling.
590. **Q:** At pickup, a patient says they were switched from "Advil" to "Motrin" and worries it's a different drug; what should you do BEST? **A:** Explain that Advil and Motrin are both brand names for ibuprofen and offer pharmacist counseling because confirming brand/generic equivalence supports safe, correct use.
591. **Q:** While entering an eRx for "Glucophage 500 mg BID," you can only find metformin on the shelf; what should you do NEXT? **A:** Select metformin (generic) matched to the strength/form and verify NDC/barcode, because Glucophage is the brand name for metformin and correct product selection prevents errors.
592. **Q:** A prescription is written for "Humalog 10 units TID with meals," but the patient profile shows active "NovoLog" with similar directions; what should you do FIRST? **A:** Stop and alert the pharmacist to possible therapeutic duplication, because both are rapid-acting insulins and duplicate therapy is a safety risk.
593. **Q:** At drop-off, a patient says their new prescription is for "Lasix" and asks what it's for; what is the BEST technician response? **A:** Confirm it is furosemide and

refer counseling to the pharmacist, because technicians can identify brand/generic and general purpose but pharmacists provide clinical counseling.

594. **Q:** You receive an order for “predniSONE 20 mg: ii tabs daily x 5 days; dispense #?” what quantity should you enter? **A:** Enter 10 tablets, because ii = 2 tablets daily for 5 days and $2 \times 5 = 10$.
595. **Q:** While typing “hydroxyzine” you notice the prescriber’s note says “for itching” but the selected drug is “hydrALAZINE”; what should you do NEXT? **A:** Stop filling and correct the selection, then route to the pharmacist if already in verification, because this is a LASA mix-up (antihistamine vs antihypertensive) with serious harm potential.
596. **Q:** A hard copy says “Levothyroxine 0.075 mg daily”; what strength should you enter in micrograms? **A:** Enter 75 mcg, because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$ and levothyroxine is typically expressed in mcg.
597. **Q:** A patient asks if “Zoloft” is the same as “sertraline” and worries they were given the wrong medication; what is the BEST technician action? **A:** Reassure that Zoloft is the brand for sertraline and offer pharmacist counseling, because brand/generic equivalence questions should be confirmed and counseling offered.
598. **Q:** An eRx for oxycodone immediate-release includes “Refills: 2”; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist, because Schedule II medications cannot be refilled and it requires pharmacist resolution with the prescriber.
599. **Q:** During filling, you find a bottle of refrigerated insulin left on the counter with unknown time out of the fridge; what should you do BEST? **A:** Remove it from use and notify the pharmacist per temperature-excursion policy, because unknown storage conditions make it suspect product and may affect potency.
600. **Q:** A prescription reads “ASA 81 mg QD” and the patient asks what “ASA” means; what is the BEST technician response? **A:** Explain that ASA is aspirin and refer any therapy questions to the pharmacist, because technicians can decode common abbreviations but pharmacists handle clinical decision-making.
601. **Q:** While entering a prescription for “Celebrex 200 mg daily,” the patient profile lists a serious sulfonamide allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist because celecoxib is a sulfonamide and the pharmacist must assess allergy risk before dispensing.

602. **Q:** A prescriber writes "metoprolol ER 50 mg QD," but the product selected is metoprolol tartrate 50 mg; what is the BEST next step? **A:** Stop and reselect the correct dosage form (succinate ER) and route for pharmacist verification because tartrate vs succinate is a high-risk mix-up affecting release and dosing.
603. **Q:** A new eRx is for "lisinopril 20 mg daily," and the patient is already active on "losartan 50 mg daily"; what should you do FIRST? **A:** Flag for pharmacist review before filling because ACE inhibitor plus ARB may be therapeutic duplication and requires clinical assessment.
604. **Q:** The directions read "Take ii tabs PO q6h prn pain," and the quantity should cover exactly 3 days using the maximum frequency; what quantity should you enter? **A:** Enter 24 tablets because 2 tablets every 6 hours equals 8 tablets/day, and $8 \times 3 = 24$.
605. **Q:** A patient brings an OTC product labeled "naproxen sodium (Aleve)" and says they take "Eliquis" at home; what should you do BEST? **A:** Refer to the pharmacist because NSAIDs can increase bleeding risk with apixaban and require pharmacist counseling on safer options.
606. **Q:** During product selection you notice "clonidine" and "clonazepam" are stored near each other and the NDC on the bottle doesn't match the label you printed; what should you do NEXT? **A:** Stop filling and match the stock bottle NDC/barcode to the label before continuing because these LASA drugs can cause serious errors if misselected.
607. **Q:** A hard copy says "nitrofurantoin macrocrystals/monohydrate 100 mg BID," but the technician selected nitrofurantoin macrocrystals 100 mg; what should you do FIRST? **A:** Hold and confirm with the pharmacist because Macrobid vs Macrochantin are different formulations and are not interchangeable without prescriber/pharmacist decision.
608. **Q:** A prescription is written for "morphine sulfate ER 30 mg q12h" and the prescriber also writes "Do not fill until 6/15"; today is 6/10—what should you do NEXT? **A:** Enter the prescription and place it on hold with the do-not-fill date per workflow and pharmacist review because dispensing early could violate controlled-substance and prescriber instructions.
609. **Q:** You receive a bottle of tablets from a wholesaler and the tamper-evident seal is broken with tablets visible; what is the BEST technician action? **A:** Quarantine the bottle and notify the pharmacist/manager because compromised packaging is suspect product and should not be dispensed.

610. **Q:** A prescription reads "prednisolone sodium phosphate oral solution 15 mg/5 mL: take 10 mL daily x 7 days; dispense #?" what quantity should you enter in mL? **A:** Enter 70 mL because $10 \text{ mL/day} \times 7 \text{ days} = 70 \text{ mL}$ total to dispense.
611. **Q:** A prescription is for furosemide 20 mg: "Take 1 tab PO BID" with quantity 60; what days' supply should you enter? **A:** Enter 30 days' supply because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for accurate billing and refill timing.
612. **Q:** The sig says "levothyroxine 0.05 mg i tab po qd"; what strength should you select in order entry? **A:** Select levothyroxine 50 mcg because $0.05 \text{ mg} \times 1000 = 50 \text{ mcg}$, preventing a 10× dosing error.
613. **Q:** An eRx for sertraline 100 mg has directions "½ tab PO daily" and quantity 30; what days' supply should you enter? **A:** Enter 60 days' supply because 0.5 tablet/day means 30 tablets last 60 days, which reduces refill-too-soon rejections.
614. **Q:** A prescription reads "amlodipine 5 mg PO QD" but the technician enters "10 mg" by mistake; what should you do NEXT when you notice before filling? **A:** Stop and correct the strength in the system then reprint labels because order-entry accuracy prevents dispensing the wrong dose.
615. **Q:** A prescription for metformin says "Take 500 mg PO BID with meals, dispense #120"; what is the correct days' supply? **A:** Enter 60 days' supply because $120 \text{ tablets} \div 2 \text{ tablets/day} = 60 \text{ days}$, aligning quantity with the sig.
616. **Q:** While entering "hydroxyzine 25 mg i po tid prn itching," the prescriber wrote "UAD"; what is the BEST technician action? **A:** Stop and ask the pharmacist to clarify with the prescriber because "use as directed" is not specific enough for accurate directions and days' supply.
617. **Q:** An order is for azithromycin 200 mg/5 mL suspension: "Take 10 mL PO daily x 3 days"; what quantity should you dispense in mL? **A:** Dispense 30 mL because $10 \text{ mL/day} \times 3 \text{ days} = 30 \text{ mL}$ to cover the full course.
618. **Q:** A patient's prescription is for Ventolin HFA with sig "Inhale 2 puffs q6h prn wheeze" and the canister contains 200 actuations; using maximum frequency, what days' supply should you bill? **A:** Bill 25 days because 2 puffs every 6 hours = 8 puffs/day and $200 \text{ actuations} \div 8 \text{ /day} = 25 \text{ days}$ for proper claim submission.
619. **Q:** You receive an eRx for "Humalog U-100, inject 12 units SQ TID with meals, dispense 1 vial (10 mL)"; what is the days' supply based on total units? **A:** Enter

27 days' supply because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ rounded down to 27 days.

620. **Q:** A hard copy for oxycodone 5 mg has "1 tab PO q4h prn pain, #30" but the DEA number is missing; what should you do FIRST? **A:** Hold the prescription and immediately route it to the pharmacist because controlled-substance validity issues require pharmacist review before processing.
621. **Q:** A new eRx is for lisinopril 20 mg daily, but the patient profile shows an active lisinopril 10 mg daily with refills remaining; what should the technician do FIRST? **A:** Pause processing and alert the pharmacist to verify intended dose change and prevent therapeutic duplication because ACE inhibitor dose changes require clinical confirmation.
622. **Q:** During product selection you notice the label says "clonidine 0.1 mg" but the stock bottle you grabbed is "clonazepam 0.5 mg"; what is the BEST next step? **A:** Stop filling and reselect the correct NDC/barcode for clonidine because this is a high-risk LASA mix-up that can seriously harm the patient.
623. **Q:** A prescription is written for "Lantus SoloStar inject 18 units SQ QHS; dispense 1 box (5 pens, 3 mL each)"; what days' supply should you enter based on total units? **A:** Enter 83 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 18 = 83.3$, billed as 83 days.
624. **Q:** A patient drops off a new prescription for tramadol and asks if it can be refilled early because they "lost the bottle"; what should the technician do NEXT? **A:** Refer the request to the pharmacist for controlled-substance loss/early refill assessment because the technician cannot authorize overrides for potentially inappropriate early refills.
625. **Q:** An eRx comes in for "Biaxin 500 mg BID x 7 days," and the patient's profile shows simvastatin 40 mg daily; what should the technician do BEST? **A:** Flag and notify the pharmacist before dispensing because clarithromycin can dangerously increase simvastatin levels and requires pharmacist intervention.
626. **Q:** You are entering "predniSONE 20 mg: take ii tabs PO daily x 5 days"; what quantity should you dispense in tablets? **A:** Dispense 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets}$.
627. **Q:** A prescriber sends "Zyrtec 10 mg" but the technician is unsure which generic to select; what is the correct generic name to choose? **A:** Select cetirizine

because Zyrtec is the brand for cetirizine and matching brand/generic prevents order-entry errors.

628. **Q:** While typing an eRx you see “metoprolol ER 50 mg daily” but the selected product is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop and select metoprolol succinate ER (not tartrate) because ER vs IR is a common trap that changes dosing and must match the prescribed dosage form.
629. **Q:** A prescription is for warfarin 5 mg daily, and the patient says they started trimethoprim/sulfamethoxazole yesterday from urgent care; what should the technician do NEXT? **A:** Immediately notify the pharmacist because TMP/SMX can raise INR and increase bleeding risk, requiring pharmacist management and possible prescriber contact.
630. **Q:** A refrigerator temperature log shows insulin aspart was stored overnight at 12°C (54°F); what is the BEST technician action? **A:** Segregate the affected stock and notify the pharmacist per temperature-excursion procedure because improper storage may reduce potency and the pharmacist must decide disposition.
631. **Q:** While entering an eRx for methotrexate 2.5 mg you see the sig “take 1 tablet daily for rheumatoid arthritis”; what should the technician do FIRST? **A:** Stop processing and immediately alert the pharmacist because methotrexate for RA is typically weekly and daily dosing is a high-alert error.
632. **Q:** A prescription for morphine ER 30 mg has the sig “take 1 tablet PO q4-6h prn pain”; what is the BEST next step for the technician? **A:** Hold the order and refer to the pharmacist because ER opioids should not be dosed PRN at short intervals and the directions are unsafe/unclear.
633. **Q:** A patient requests a transfer of their alprazolam prescription from another pharmacy; what should the technician do NEXT? **A:** Route to the pharmacist to handle the transfer because alprazolam is Schedule IV and controlled-substance transfers must be performed per controlled-substance rules and pharmacist policy.
634. **Q:** During verification prep you notice the stock bottle NDC scanned does not match the NDC on the printed label for losartan 50 mg; what should you do FIRST? **A:** Stop filling and reselect the correct product/NDC and rescan because an NDC mismatch signals wrong product selection and risks a dispensing error.

635. **Q:** A prescriber writes "MS Contin 15 mg" and the patient says they thought it was "MSIR"; what should the technician do BEST? **A:** Pause the fill and notify the pharmacist because MS Contin vs MSIR is a LASA/abbreviation risk and the formulation difference is clinically significant.
636. **Q:** You receive a wholesaler notice that a specific lot of amlodipine 10 mg is recalled, and you find that lot on the shelf; what should you do FIRST? **A:** Pull the affected lot from active stock and quarantine it per recall procedure because recalled product must be isolated to prevent dispensing.
637. **Q:** A sealed tote of refrigerated GLP-1 pens arrives warm and the temperature indicator shows an excursion; what should the technician do NEXT? **A:** Segregate and quarantine the shipment and notify the pharmacist/manager to contact the wholesaler because temperature excursions make product integrity uncertain.
638. **Q:** An eRx for sertraline 50 mg is sent for a new patient, but the date of birth is missing; what should the technician do FIRST? **A:** Stop data entry and obtain the missing patient identifier through the prescriber or patient profile because complete patient information is needed to ensure the right patient and safe processing.
639. **Q:** A prescription reads "levothyroxine 0.075 mg PO daily"; what strength should the technician select in the system? **A:** Enter 75 mcg because 0.075 mg equals 75 micrograms and selecting the correct unit prevents decimal-dose errors.
640. **Q:** A patient asks for "Plan B" and the technician finds both Plan B One-Step and a generic levonorgestrel 1.5 mg on the shelf; what is the BEST product concept to confirm before sale? **A:** Confirm it is levonorgestrel 1.5 mg single-dose emergency contraception because brand/generic equivalence is based on active ingredient/strength to avoid wrong product selection.
641. **Q:** You are entering an Rx: "amoxicillin/clavulanate 600-42.9 mg/5 mL, take 7.5 mL PO BID x 10 days"; what quantity in mL should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.
642. **Q:** A patient's new Rx is metformin 500 mg: "take 2 tablets PO BID," quantity #240; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ tabs} \times 2 \text{ times/day} = 4 \text{ tabs/day}$ and $240 \div 4 = 60$? wait no, $240 \div 4 = 60 \text{ days}$, so 60 days' supply to match the dosing.

643. **Q:** While typing "hydralazine 25 mg 1 tab TID" you notice the prescriber also sent "hydroxyzine 25 mg 1 cap TID prn itching" for the same patient; what should you do FIRST? **A:** Pause entry and use LASA safeguards (confirm drug name/indication and alert the pharmacist) because hydralazine/hydroxyzine are easily confused and can cause a wrong-drug error.
644. **Q:** An Rx says "predniSONE 10 mg: ii tabs po qd x 5 days then i tab po qd x 5 days"; what total quantity should you dispense? **A:** Dispense 15 tablets because $(2 \text{ tabs/day} \times 5 \text{ days}) + (1 \text{ tab/day} \times 5 \text{ days}) = 10 + 5 = 15$.
645. **Q:** A prescription is written "levetiracetam 500 mg: take 1 tab PO q12h" with quantity #60; what is the correct days' supply to submit? **A:** Submit 30 days because q12h means 2 tablets/day and $60 \div 2 = 30$ days.
646. **Q:** You receive an eRx for "Lantus SoloStar inject 24 units SQ daily," and the package is 5 pens of 3 mL each (100 units/mL); what days' supply should you calculate? **A:** Enter 62 days because total units = $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500$ units and $1500 \div 24 = 62.5$, typically billed as 62 days per payer rule to avoid overstatement.
647. **Q:** A new order reads "ciprofloxacin 0.3% ophthalmic solution: instill 2 gtt OU q4h while awake x 7 days"; what quantity in mL should you choose if the dropper delivers 20 gtt/mL? **A:** Select at least 6 mL because $2 \text{ gtt} \times 2 \text{ eyes} \times 4 \text{ doses/day} \times 7 \text{ days} = 112$ gtt and $112 \div 20 = 5.6$ mL, so the next bottle size that covers therapy is needed.
648. **Q:** An Rx for azithromycin tablets says "take ii tabs PO day 1 then i tab PO daily days 2-5"; what quantity should you enter and why? **A:** Enter #6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet/day for 4 more days ($2 + 4 = 6$).
649. **Q:** While processing a refill request you see "atorvastatin 40 mg take 1 daily," last filled 18 days ago for 30 days; what is the BEST next step for the technician? **A:** Check refill-too-soon coverage and follow plan policy or set a future fill date because 18/30 days is early and usually rejects without an override reason.
650. **Q:** A hardcopy for "hydromorphone 2 mg" has the sig "take 1 tab q3h prn pain" but the quantity is written as "#I00"; what should the technician do NEXT? **A:** Stop processing and notify the pharmacist to clarify with the prescriber because ambiguous/altered quantities (I vs 100) are a high-risk order entry error, especially for opioids.

651. **Q:** While entering an eRx for morphine ER 30 mg q12h, you notice the patient also has an active oxycodone ER prescription; what should you do FIRST? **A:** Stop processing and notify the pharmacist to review for therapeutic duplication/overdose risk because concurrent long-acting opioids are a high-alert safety issue.
652. **Q:** A prescriber phones in a new prescription for alprazolam 1 mg with quantity and directions but no patient address; what is the BEST next step for the technician? **A:** Pause the order and route to the pharmacist to obtain missing required information because controlled-substance prescription elements must be complete before dispensing.
653. **Q:** A patient asks to transfer their Adderall XR prescription from another pharmacy with refills remaining; what should the technician do NEXT? **A:** Refer the request to the pharmacist because Schedule II prescriptions cannot be transferred between pharmacies for dispensing.
654. **Q:** During product selection you see "hydrOXYzine" and "hydrALAZINE" stored adjacent and the sig on the label doesn't match the intended indication; what is the BEST action? **A:** Stop filling and verify the NDC/barcode against the prescription and patient profile, then alert the pharmacist if any mismatch is suspected because LASA errors require immediate interception.
655. **Q:** You receive a wholesaler tote with a bottle of apixaban (Eliquis) tablets that has a broken seal and no readable lot number; what should you do FIRST? **A:** Set it aside as suspect product per DSCSA and notify the pharmacist/manager because compromised packaging or missing identifiers should not be stocked or dispensed.
656. **Q:** A refrigerator temperature log shows the fridge was 12°C overnight where insulin and vaccines are stored; what should the technician do NEXT? **A:** Quarantine the affected refrigerated items and notify the pharmacist for stability guidance because temperature excursions can make products unsafe or ineffective.
657. **Q:** A recall notice lists a specific lot number of losartan tablets; you find that lot on the shelf and in will-call bags; what is the BEST next step? **A:** Pull the recalled lot from inventory and will-call and segregate it per recall procedure while notifying the pharmacist because recalled products must be removed from dispensing immediately.

658. **Q:** A new prescription for clopidogrel is entered, and the patient profile shows a documented serious allergy to prasugrel with "anaphylaxis"; what should you do FIRST? **A:** Stop and alert the pharmacist to assess cross-reactivity and appropriateness because allergy alerts require pharmacist clinical review before dispensing.
659. **Q:** An order reads "heparin 100 units/mL flush" but the technician accidentally selected "heparin 10,000 units/mL" during entry; what should you do NEXT? **A:** Correct the selection and ensure barcode/NDC match before continuing, and inform the pharmacist if the error reached verification because heparin is high-alert and concentration mix-ups can be fatal.
660. **Q:** A prescription says "prednisolone sodium phosphate oral solution 15 mg/5 mL: take 10 mL PO daily x 7 days"; what quantity in mL should you dispense? **A:** Dispense 70 mL because $10 \text{ mL/day} \times 7 \text{ days} = 70 \text{ mL}$ for the full course.
661. **Q:** While typing "Lamictal" for a new seizure prescription, the eRx defaults to lamotrigine ER but the directions say "chew and swallow 1 tablet twice daily"; what should the technician do FIRST? **A:** Stop entry and confirm the intended dosage form/strength with the pharmacist because lamotrigine IR vs ER is a high-risk mix-up that changes how the drug is taken.
662. **Q:** A patient drops off a new prescription for metformin and says they are "allergic to Glucophage" but can't describe the reaction; what is the BEST next step for the technician? **A:** Ask what reaction occurred and document it, then route to the pharmacist because "allergy" may be intolerance and the pharmacist must assess safety before dispensing.
663. **Q:** During data entry you see a new order for trimethoprim-sulfamethoxazole, and the profile lists "sulfa: hives"; what should you do NEXT? **A:** Stop processing and alert the pharmacist because TMP-SMX is a sulfonamide antibiotic and the allergy suggests a clinically significant risk.
664. **Q:** You receive an eRx for "Zithromax 250 mg: take 2 tablets today then 1 tablet daily for 4 days"; what quantity should you enter to dispense? **A:** Enter quantity 6 tablets because the total dose is $2 + (1 \times 4) = 6$ tablets.
665. **Q:** While selecting product, you notice a bottle labeled "CeleBREX" was pulled for a prescription that was typed as "Celexa 20 mg daily"; what should the technician do FIRST? **A:** Stop filling and re-pull the correct NDC using barcode verification because Celexa (citalopram) and Celebrex (celecoxib) are LASA names with very different indications.

666. **Q:** A new prescription is written for "lisinopril 20 mg daily," but the patient profile shows an active "losartan 100 mg daily" with recent fills; what should you do BEST? **A:** Flag and refer to the pharmacist for therapeutic duplication review because both are antihypertensives affecting the renin-angiotensin system and may represent an unintended overlap.
667. **Q:** A prescriber sends "sertraline 50 mg take 1 daily," and the patient profile shows they are currently taking linezolid; what should the technician do NEXT? **A:** Hold the prescription and notify the pharmacist because linezolid with SSRIs can cause a serious interaction risk (serotonin syndrome) requiring pharmacist intervention.
668. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg: 1 tablet PO BID x 10 days"; what total quantity should be dispensed? **A:** Dispense 20 tablets because BID for 10 days is 2 doses/day × 10 days = 20 tablets.
669. **Q:** A patient asks for a "generic Narcan" and you locate both naloxone nasal spray and naloxone injection in inventory; what is the BEST action for the technician? **A:** Clarify the intended dosage form with the patient and follow store protocol/pharmacist direction because nasal spray vs injection are not interchangeable and require correct device selection.
670. **Q:** While entering a prescription, you see "levothyroxine 0.125 mg daily" and the prescriber also wrote "dispense 125 mcg tablets"; what should the technician do FIRST? **A:** Proceed with entry using 125 mcg because 0.125 mg equals 125 mcg, and verifying the conversion prevents strength selection errors for a high-alert hormone.
671. **Q:** A handwritten prescription says "hydralazine 25 mg 1 tab TID" but the prescriber's handwriting could also read "hydroxyzine"; what should you do FIRST? **A:** Stop processing and ask the pharmacist to clarify with the prescriber because hydralazine vs hydroxyzine is a LASA risk with very different uses and dosing.
672. **Q:** A patient asks if "Lipitor" and "atorvastatin" are the same and wants the cheapest option; what is the BEST technician response? **A:** Confirm Lipitor is the brand for atorvastatin and proceed with the generic per the prescription/plan because brand-generic recognition prevents duplication and supports cost-effective dispensing.
673. **Q:** While entering a new eRx for "Prozac 20 mg daily," the system suggests fluoxetine 20 mg and fluvoxamine 25 mg; what should you do NEXT? **A:** Select

fluoxetine (Prozac) and double-check NDC/strength because similar prefixes can lead to wrong-drug entry and patient harm.

674. **Q:** A prescription reads "Augmentin 600 mg/5 mL: give 7.5 mL PO BID x 10 days"; what quantity in mL should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$.
675. **Q:** The sig says "take ii tabs po qhs" for simvastatin; what should you enter for directions? **A:** Enter "take 2 tablets by mouth at bedtime" because ii = 2 and qhs = every night at bedtime to avoid misinterpretation of abbreviations.
676. **Q:** A prescription is written for "Lantus SoloStar 100 units/mL: inject 24 units SQ daily," and the box contains five 3 mL pens; what days' supply should you bill? **A:** Bill 62 days because total units = $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 24 = 62.5$ rounded down to 62 per typical billing practice unless policy states otherwise.
677. **Q:** A patient's new prescription is for "Xarelto 20 mg daily," and the profile shows an active "Eliquis 5 mg BID" with a recent fill; what should you do BEST? **A:** Hold the order and alert the pharmacist to possible therapeutic duplication because both are anticoagulants and overlapping therapy may be unsafe.
678. **Q:** During product selection you pull "Humalog" but the label you printed is for "Humulin R"; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification because look-alike insulin names are high-alert and wrong selection can cause severe harm.
679. **Q:** A prescriber e-prescribes "Tylenol #3" with refills for a patient; what should you do NEXT? **A:** Stop processing and route to the pharmacist because codeine combination products are Schedule III and refills are limited and must meet controlled-substance requirements.
680. **Q:** You receive a wholesaler notice of a recall for a specific lot of metoprolol ER in your inventory; what should you do FIRST? **A:** Quarantine the affected lot by NDC/lot/expiration and notify the pharmacist because recalled/suspect product must be isolated to prevent dispensing.
681. **Q:** While processing a new eRx for methotrexate 2.5 mg, the directions entered read "take 6 tablets daily" for a rheumatoid arthritis patient; what should the technician do FIRST? **A:** Stop processing and immediately alert the pharmacist to verify the intended weekly dosing because methotrexate is high-alert and daily dosing errors can be fatal.

682. **Q:** A patient picking up a new prescription asks, "What is this for and what side effects should I watch for?"; what is the BEST technician response? **A:** Refer the patient to the pharmacist for counseling because clinical counseling is required by law/policy (e.g., OBRA-90) and is outside technician scope.
683. **Q:** During order entry for oxycodone/APAP, the prescription image shows no prescriber signature on a non-electronic controlled substance prescription; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist because missing required elements on a controlled-substance order must be resolved before dispensing.
684. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy; what should the technician do FIRST? **A:** Notify the pharmacist to handle the controlled-substance transfer requirements because controlled-substance transfers are restricted and require pharmacist oversight per federal/state rules.
685. **Q:** While restocking, you find insulin glargine pens left on the counter at room temperature with no way to confirm how long they were out of refrigeration; what should you do BEST? **A:** Segregate the product and alert the pharmacist/manager per policy because a temperature excursion creates suspect integrity and should not be returned to stock.
686. **Q:** You receive a tote with bottles missing the lot number/expiration and no traceable paperwork for DSCSA; what is the BEST next step? **A:** Set the shipment aside and notify the pharmacist/receiver to investigate before stocking because DSCSA requires traceability and unverifiable product is suspect.
687. **Q:** The label prints for "hydromorphone 4 mg" but the stock bottle in your hand is "morphine 15 mg"; what should you do FIRST? **A:** Stop filling and reselect the correct NDC/barcode before continuing because this is a LASA/high-alert opioid mix-up risk.
688. **Q:** A new prescription for sertraline is entered, and the profile shows the patient has a documented anaphylaxis to sertraline from a prior fill; what should you do NEXT? **A:** Stop processing and alert the pharmacist because a documented serious allergy requires pharmacist intervention before dispensing.
689. **Q:** A prescription reads "prednisone 10 mg: take ii tabs po bid x 5 days"; what quantity should you dispense? **A:** Dispense 20 tablets because 2 tablets per dose × 2 doses/day × 5 days = 20 tablets for the full course.

690. **Q:** You are entering a new prescription and see the prescriber used "U" in the sig for insulin (e.g., "10U SQ qhs"); what should you do BEST? **A:** Change the sig to "10 units" per safe abbreviation standards and have the pharmacist verify because "U" is error-prone and can be misread as a zero.
691. **Q:** A new eRx is for "Glucophage 500 mg" but the patient says they only take "metformin"; what should you do FIRST? **A:** Confirm Glucophage is metformin and proceed with profile/label accuracy checks because brand-generic equivalence prevents unnecessary delays.
692. **Q:** While entering "Zestril 10 mg daily," the patient profile already shows lisinopril 10 mg daily active from last month; what is the BEST next step? **A:** Stop and alert the pharmacist to possible therapeutic duplication because Zestril is lisinopril and duplicate therapy could cause harm.
693. **Q:** A prescription says "take i tab po qd" for atorvastatin 20 mg with #30; what should you enter for directions and days' supply? **A:** Enter "take 1 tablet by mouth once daily" and 30 days' supply because i=1, qd=once daily, and 30 tablets/1 per day=30 days.
694. **Q:** During product selection you see "hydrOXYzine" and "hydrALAZINE" next to each other and the label is for hydroxyzine 25 mg; what should you do FIRST? **A:** Use barcode/NDC verification and read the full drug name before pulling stock because these are LASA names and mix-ups are common.
695. **Q:** A new prescription is for "Prilosec 20 mg daily" but the patient profile shows an allergy to omeprazole; what should you do NEXT? **A:** Hold the fill and notify the pharmacist because Prilosec is omeprazole and allergies require pharmacist assessment before dispensing.
696. **Q:** A prescriber writes "Augmentin 875/125 mg" and the technician selects amoxicillin 875 mg alone; what is the BEST action before it reaches verification? **A:** Stop and correct the selection to amoxicillin/clavulanate by matching the exact strength components because combination products are not interchangeable with single-ingredient drugs.
697. **Q:** A patient brings an OTC product request and asks for "something like Aleve" while their profile shows warfarin; what should you do BEST? **A:** Refer to the pharmacist because NSAIDs like naproxen (Aleve) can increase bleeding risk with warfarin and requires counseling/clinical judgment.

698. **Q:** A prescription reads "levothyroxine 0.1 mg po qam" and the patient asks if that's the same as 100 mcg; what is the correct conversion? **A:** Confirm 0.1 mg equals 100 mcg because $1 \text{ mg} = 1000 \text{ mcg}$, so $0.1 \text{ mg} \times 1000 = 100 \text{ mcg}$.
699. **Q:** The sig on a new prescription says "take 2 tsp po bid x 7 days" for a liquid medication; how many mL per dose should you enter? **A:** Enter 10 mL per dose because 1 teaspoonful equals 5 mL, so $2 \text{ tsp} = 10 \text{ mL}$.
700. **Q:** A new eRx comes in for "Humalog" but the technician is about to select "Humulin R" because both are insulin; what should you do FIRST? **A:** Stop and verify the exact insulin name with NDC/barcode and alert the pharmacist if needed because different insulins are not interchangeable and are high-alert medications.
701. **Q:** A prescription reads "cephalexin 250 mg/5 mL: give 10 mL po tid x 10 days"; what quantity (mL) should you dispense? **A:** Dispense 300 mL because $10 \text{ mL per dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$.
702. **Q:** An eRx says "azithromycin 200 mg/5 mL: take 12.5 mL po day 1, then 6.25 mL po daily days 2-5"; what total volume should you enter/dispense? **A:** Dispense 37.5 mL because $12.5 \text{ mL} + (6.25 \text{ mL} \times 4 \text{ days}) = 12.5 + 25 = 37.5 \text{ mL}$.
703. **Q:** A patient's directions are "furosemide 20 mg: take 1 tab po qam" and the prescriber wrote #45; what days' supply should you submit to insurance? **A:** Submit 45 days because 1 tablet per day means quantity 45 covers 45 days.
704. **Q:** The sig says "instill 1 gtt ou bid" and the quantity is 10 mL; using 20 gtt/mL, what days' supply should you calculate? **A:** Enter 100 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ drops}$ and 1 drop in each eye twice daily is 4 drops/day, so $200 \div 4 = 50 \text{ days}$; ou bid is 4 drops/day, so days' supply is 50 (not 100) to avoid overbilling.
705. **Q:** A prescription is "fluticasone nasal spray 50 mcg: 2 sprays each nostril daily," and the product is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
706. **Q:** You receive "metoprolol succinate ER 50 mg: take 1 tab po qd," but the tech accidentally selected metoprolol tartrate 50 mg in order entry; what should you do FIRST? **A:** Stop processing and correct the drug selection to succinate ER because tartrate vs succinate is a different formulation and could cause dosing errors.

707. **Q:** A hardcopy says "hydromorphone 2 mg: take 1 tab po q4-6h prn pain" with no quantity written; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist to contact the prescriber because missing quantity is a key dispensing element and can't be guessed.
708. **Q:** During data entry the sig reads "levothyroxine 75 mcg: take 0.075 mg po daily"; what should you do NEXT? **A:** Enter it as 75 mcg daily and flag for pharmacist review because 0.075 mg equals 75 mcg and clear units reduce decimal-point error risk.
709. **Q:** A prescription states "amoxicillin-clavulanate 600-42.9 mg/5 mL: give 5 mL po bid x 7 days"; what total mL should you dispense? **A:** Dispense 70 mL because $5 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 70 \text{ mL}$.
710. **Q:** You are entering "digoxin 0.125 mg po daily" and the prescriber wrote ".125 mg" without a leading zero; what should you do BEST? **A:** Enter 0.125 mg (125 mcg) and alert the pharmacist to the unsafe notation because adding a leading zero prevents decimal misreads and supports safe verification.
711. **Q:** A new eRx is for "Zocor 20 mg qhs," but the tech is about to select atorvastatin because both are statins; what should you do FIRST? **A:** Stop and select simvastatin (Zocor) by matching drug name/strength and NDC because atorvastatin (Lipitor) is a different medication and this is a product-selection safety error.
712. **Q:** A patient profile shows lisinopril, and today an eRx arrives for "Prinivil 10 mg daily"; what is the BEST technician action before processing? **A:** Verify that Prinivil is lisinopril and check the active profile for duplication, then route to the pharmacist if it appears to be a duplicate therapy to prevent double-dosing.
713. **Q:** During drop-off, a patient says they take "Plavix" and hands you a bottle labeled clopidogrel; what should you do NEXT in the workflow? **A:** Document/confirm that Plavix is clopidogrel and update the medication list because accurate brand-generic recognition prevents mix-ups during refills and counseling.
714. **Q:** A prescription is written for "metformin ER 500 mg take 1 tab bid," but the entered product is metformin IR 500 mg; what should you do FIRST? **A:** Stop and correct the dosage form to ER and flag for pharmacist verification because IR vs ER is a common high-impact error that can change effect and tolerability.

715. **Q:** A hardcopy says "Keflex 500 mg po qid x 7 days #28"; what is the correct days' supply to submit? **A:** Enter 7 days because $28 \text{ tablets} \div 4 \text{ per day} = 7$, and days' supply must match the written directions for accurate billing.
716. **Q:** A new order is "Augmentin 875/125 mg 1 tab po bid x 10 days"; how many tablets should be dispensed? **A:** Dispense 20 tablets because $2 \text{ tablets/day} \times 10 \text{ days} = 20$, ensuring the quantity matches the intended course.
717. **Q:** An eRx comes in for "Lamictal 100 mg daily," and the tech starts to choose Lamisil because the names look similar; what should you do BEST? **A:** Pause and use LASA safeguards (compare indication/class, verify full name/strength, and barcode/NDC) because lamotrigine and terbinafine are different drugs and mix-ups are high-risk.
718. **Q:** A patient picking up sulfamethoxazole/trimethoprim says they have a "sulfa allergy"; what should you do FIRST? **A:** Stop the sale, document the reported allergy, and immediately refer to the pharmacist because SMX/TMP is a sulfonamide antibiotic and requires pharmacist assessment.
719. **Q:** A new prescription is "sertraline 50 mg daily," but the patient's profile shows fluoxetine 20 mg daily as active; what is the BEST technician action? **A:** Hold processing and alert the pharmacist to potential therapeutic duplication/interaction because concurrent SSRIs can increase adverse effects and needs clinical review.
720. **Q:** You receive a shipment of insulin glargine pens and notice the shipping tote temperature indicator shows an excursion; what should you do NEXT? **A:** Segregate the stock and follow pharmacy policy to report and prevent dispensing until stability is confirmed because insulin is temperature-sensitive and may be compromised.
721. **Q:** While typing a new eRx for "Levothroid 50 mcg daily," you realize Levothroid is levothyroxine and the profile already has Synthroid 50 mcg active; what should you do BEST before processing? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (same drug different brand) to prevent double therapy.
722. **Q:** A prescriber e-sends "predniSONE 10 mg" but you accidentally select prednisoLONE; what should you do FIRST? **A:** Stop and reselect the correct drug using NDC/barcode verification because prednisone vs prednisolone is a common look-alike/sound-alike selection error.

723. **Q:** A patient says they take "Coumadin" and you see an active warfarin profile plus a new eRx for apixaban (Eliquis); what is the BEST technician action? **A:** Hold the order and route to the pharmacist for anticoagulant duplication review because concurrent warfarin and apixaban can increase bleeding risk.
724. **Q:** A new prescription is for "hydroxyzine 25 mg qhs prn anxiety," but you nearly choose hydralazine; what should you do NEXT? **A:** Re-check the indication, strength, and drug name and select the correct product (use Tall Man lettering/verification) because hydroxyzine vs hydralazine is a high-risk LASA pair.
725. **Q:** The eRx says "Lantus Solostar inject 18 units subq daily" and the box is 5 pens of 3 mL each (100 units/mL); what days' supply should you submit? **A:** Submit 83 days' supply because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 18 = 83.3 \rightarrow 83 \text{ days}$.
726. **Q:** A prescription is for "azithromycin 200 mg/5 mL: take 12.5 mL day 1, then 6.25 mL daily days 2–5"; what total volume should you dispense? **A:** Dispense 37.5 mL because $12.5 + (6.25 \times 4) = 37.5 \text{ mL}$ to cover the full course.
727. **Q:** During product selection for "celexa 20 mg daily," you notice the bottle you grabbed is Cymbalta 20 mg; what should you do FIRST? **A:** Stop filling and correct the selection using barcode/NDC match because citalopram vs duloxetine is a brand-name confusion that could cause a serious therapy error.
728. **Q:** A patient on the phone asks if "Advil" is the same as their prescription "ibuprofen 600 mg"; what should you do BEST as a technician? **A:** Confirm the brand/generic relationship (Advil = ibuprofen) and refer dosing/appropriateness questions to the pharmacist because counseling and clinical advice are pharmacist duties.
729. **Q:** You receive a new eRx for "Ritalin 10 mg take 1 tab bid #60," and the system prompts for refills; what should you enter NEXT? **A:** Enter zero refills and route any refill request to the pharmacist because methylphenidate is Schedule II and cannot be refilled.
730. **Q:** A hardcopy reads "morphine sulfate ER 15 mg q12h," but the prescriber's note section also mentions "for breakthrough pain"; what should you do BEST? **A:** Stop and notify the pharmacist to clarify because ER morphine should not be used for breakthrough dosing and inconsistent directions create a high-risk opioid error.

731. **Q:** An eRx reads "metFORMIN ER 500 mg i po bid #60" and the prescriber note says "take with meals"; what should you enter for days' supply and why? **A:** Enter 30 days because #60 taken 2 tablets/day equals 30 days, which supports correct third-party billing.
732. **Q:** A prescription is "amLODIPine 5 mg ½ tab po daily #15"; what days' supply should you calculate for order entry? **A:** Enter 30 days because 0.5 tablet/day means 15 tablets lasts 30 days, preventing underbilling/refill-too-soon rejects.
733. **Q:** While typing "clonazePAM 0.5 mg take 1 tab tid prn anxiety #90," the system requires a days' supply; what is the BEST technician action? **A:** Stop and ask the pharmacist how to calculate days' supply per payer/pharmacy policy because PRN directions can't be assumed without a rule.
734. **Q:** A patient is prescribed "latanoprost 0.005% instill 1 gtt OU qhs" and the bottle is 2.5 mL; using 20 gtt/mL, what days' supply should you submit? **A:** Submit 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ drops}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25 \text{ days}$.
735. **Q:** An eRx says "fluticasone propionate nasal spray 50 mcg: 2 sprays each nostril daily" and the product is 120 sprays; what days' supply should be entered? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$ for accurate claim submission.
736. **Q:** A hardcopy reads "U-100 insulin regular inject 8 units before meals tid" and the vial is 10 mL; what days' supply should you calculate (use 100 units/mL)? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \text{ units} \times 3/\text{day} = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$, billed as 41 days per typical truncation.
737. **Q:** You receive "digoxin 0.125 mg po daily" but the prescriber wrote "0.125 mg (125 mcg)" in the notes; what is the BEST technician step during order entry? **A:** Enter 0.125 mg and route to pharmacist if any mismatch appears because 0.125 mg equals 125 mcg and digoxin is high-alert for dosing errors.
738. **Q:** During data entry, you see "MSO4 2 mg IV q2h prn pain" on a hospital-style order brought to the outpatient pharmacy; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be misread and lead to a serious medication error.
739. **Q:** While selecting stock for "buPROPion SR 150 mg bid," you notice the shelf has bupropion XL 150 mg nearby; what should you do NEXT to prevent an error?

A: Verify the correct release form by matching NDC/barcode and the "SR" on the label because SR vs XL mix-ups are common and change dosing.

740. **Q:** A prescriber writes "ceFAZolin 1 g IM now" and the vial on hand is labeled 1 g powder requiring reconstitution to 250 mg/mL; how many mL should be drawn up for a 1 g dose? **A:** Draw up 4 mL because $1\text{ g} = 1000\text{ mg}$ and $1000\text{ mg} \div 250\text{ mg/mL} = 4\text{ mL}$, ensuring the administered dose matches the order.
741. **Q:** A new prescription says "Z-Pak use as directed" for brand Zithromax; what should you do FIRST before entering directions? **A:** Stop and ask the pharmacist to clarify the exact sig (e.g., 500 mg day 1 then 250 mg days 2–5) because "use as directed" is incomplete for safe labeling.
742. **Q:** While entering a refill, the patient requests "Lasix," but the profile shows furosemide; what is the BEST technician response? **A:** Confirm Lasix is the brand name for furosemide and proceed with the profile drug selection because brand/generic recognition prevents duplicate therapy and wrong-drug entry.
743. **Q:** During data entry you see "hydroxyzine 25 mg 1 tab po tid prn itching" and the patient profile lists an active cetirizine regimen; what should you do NEXT? **A:** Flag and route to the pharmacist for possible therapeutic duplication/sedation risk because both are antihistamines and the pharmacist must assess appropriateness.
744. **Q:** A prescription is written for "Adderall XR 20 mg take 1 cap daily #30," and the patient asks for refills; what should you do BEST? **A:** Hold and refer to the pharmacist because it's a Schedule II controlled substance and refills are not permitted under federal law.
745. **Q:** While selecting stock for "levothyroxine 75 mcg," you find multiple strengths with similar packaging; what should you do FIRST? **A:** Match the NDC/barcode to the prescription and verify the strength carefully because levothyroxine is error-prone and strength mix-ups are common LASA hazards.
746. **Q:** A prescription says "predniSONE 10 mg take ii tabs po daily x 5 days #20"; what quantity should you enter and why? **A:** Enter quantity 10 because "ii tabs daily" for 5 days equals $2 \times 5 = 10$ tablets, and the written #20 conflicts and should be clarified by the pharmacist.
747. **Q:** An order reads "amoxicillin/clavulanate 875/125 mg 1 tab po bid #14," but the patient reports a history of anaphylaxis to penicillin; what should you do

FIRST? **A:** Stop processing and immediately notify the pharmacist because a serious beta-lactam allergy requires pharmacist intervention before dispensing.

748. **Q:** You receive “nitrofurantoin 100 mg caps take 1 cap bid x 7 days,” and the prescriber selected Macrobid; what is the BEST technician check before product selection? **A:** Verify the dosage form is Macrobid (monohydrate/macrocystals) rather than Macrochantin because they are different nitrofurantoin products and mix-ups affect dosing.
749. **Q:** A patient brings in a manufacturer letter stating their lot of valsartan tablets is recalled; what should you do NEXT? **A:** Quarantine the affected lot and notify the pharmacist to follow recall procedures because recalls require removal from dispensing inventory and proper documentation.
750. **Q:** A prescription is “clindamycin 300 mg 1 cap q6h x 10 days #?”; what quantity should you calculate for entry? **A:** Enter quantity 40 because q6h is 4 doses/day and $4 \times 10 \text{ days} = 40 \text{ capsules}$, ensuring the label and billing match the intended course.
751. **Q:** While entering a new e-Rx for insulin U-500 regular, you notice the patient’s profile shows only U-100 insulin syringes on file; what should you do FIRST? **A:** Stop processing and alert the pharmacist because U-500 is high-alert and requires confirming the correct delivery device/education to prevent a dangerous dosing error.
752. **Q:** A hardcopy prescription for hydrocodone/acetaminophen has a prescriber signature and directions but no date written; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because missing required elements can affect controlled-substance validity and must be resolved before dispensing.
753. **Q:** A patient asks to transfer their diazepam prescription from another pharmacy, but they last filled it 3 months ago with refills remaining; what should you do NEXT? **A:** Gather the source pharmacy information and notify the pharmacist to complete the controlled-substance transfer appropriately since Schedule IV transfers have specific federal limits and documentation needs.
754. **Q:** While unpacking an order, you find refrigerated liraglutide pens warm to the touch with no temperature indicator data; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager because a cold-chain excursion makes it suspect and it should not be dispensed until stability is confirmed per policy.

755. **Q:** During product selection you scan an NDC for “glimepiride 4 mg” but the screen shows “glyburide 5 mg” with similar packaging; what should you do NEXT? **A:** Stop, reselect the correct stock bottle, and rescan/verify the NDC because this LASA mix-up could cause a dispensing error and barcode mismatch indicates wrong product.
756. **Q:** A caregiver requests a refill of methylphenidate ER 18 mg for a child and says, “We’re out early because the dose was increased,” but no new prescription is on file; what should you do BEST? **A:** Refer to the pharmacist because dose changes and early fills for Schedule II require a new valid prescription and pharmacist assessment before any action.
757. **Q:** You receive a lot-specific manufacturer recall notice for atorvastatin 20 mg and locate matching bottles on the shelf; what should you do FIRST? **A:** Pull the affected lot from inventory and follow recall procedure (segregate/hold for return) because recalled product must be removed to prevent patient harm.
758. **Q:** A prescription is written for enoxaparin 40 mg/0.4 mL inject 0.4 mL subcut daily x 10 days; how many syringes should you enter for quantity? **A:** Enter 10 syringes because 1 prefilled syringe supplies 1 daily dose and the duration is 10 days.
759. **Q:** A patient offers to show you their lab results while discussing their new warfarin prescription at the counter with other customers nearby; what should you do FIRST? **A:** Move the conversation to a private area or offer counseling with the pharmacist because HIPAA requires protecting patient health information from being overheard.
760. **Q:** While entering a prescription you see “MSO4 2 mg IV q2h prn pain” for a hospital discharge med list; what is the BEST technician response before processing? **A:** Stop and get pharmacist clarification because unsafe abbreviations (MSO4) can cause morphine vs magnesium sulfate errors and must be corrected for patient safety.
761. **Q:** During data entry you see a new Rx for Brilinta 90 mg and the profile lists “ticagrelor” as an allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Brilinta is ticagrelor, so this is a potential true allergy conflict requiring pharmacist review.
762. **Q:** A prescriber sends an e-Rx for “metformin ER 500 mg” but the product selected is metformin IR 500 mg due to similar names; what should you do

NEXT? **A:** Re-select the correct ER formulation and re-verify NDC/description because IR vs ER is a high-impact dosage-form mix-up that can change therapy.

763. **Q:** A patient on the phone says they take "Lasix" and asks if it's the same as furosemide listed on their discharge papers; what is the BEST technician response? **A:** Confirm Lasix is the brand name for furosemide and offer to have the pharmacist counsel because brand/generic confusion can lead to duplicate dosing.
764. **Q:** You receive an Rx for sertraline 50 mg daily and the patient profile shows "phenelzine" filled last week; what should you do FIRST? **A:** Pause the fill and notify the pharmacist because an SSRI with an MAOI is a serious interaction risk requiring pharmacist intervention.
765. **Q:** While selecting product for "hydroxyzine" you notice a nearby bottle of "hydralazine" with similar labeling; what should you do BEST before continuing? **A:** Use barcode/NDC verification and read the full drug name/strength because hydroxyzine vs hydralazine is a common LASA trap that can cause harmful errors.
766. **Q:** A prescription is for azithromycin 250 mg "take 2 tablets day 1 then 1 tablet daily days 2-5"; what quantity should you enter? **A:** Enter quantity 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet for 4 more days ($2 + 4 = 6$).
767. **Q:** A patient drops off an Rx for lisinopril but says they had "swelling of lips" with it before; what should you do FIRST? **A:** Stop processing and alert the pharmacist because swelling suggests angioedema, a serious reaction that needs pharmacist assessment before dispensing.
768. **Q:** An order-entry tech types "predniSONE" but accidentally selects "prednisoLONE" from the dropdown; what should you do NEXT? **A:** Correct the selection to match the prescription and verify strength/form because look-alike steroid names can lead to the wrong drug being dispensed.
769. **Q:** An Rx reads "levothyroxine 75 mcg take 1 tab daily, dispense 90" but you only have 0.075 mg listed in the computer; what should you do BEST? **A:** Enter 0.075 mg as 75 mcg because $1 \text{ mg} = 1000 \text{ mcg}$, and confirm the dose is equivalent to prevent unit-conversion errors.
770. **Q:** A patient picking up tramadol says they also take "Xanax" at home and asks if it's okay together; what should you do FIRST? **A:** Refer to the pharmacist for

counseling because tramadol plus alprazolam (CNS depressant) increases sedation/respiratory risk and requires pharmacist guidance.

771. **Q:** An e-Rx says "Lantus Solostar inject 22 units subq qhs, dispense 1 box" and you need days' supply for billing; what should you enter? **A:** Enter 68 days because 1 box has 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units, and $1500 \div 22 = 68$ days (round down for whole days).
772. **Q:** A prescription reads "amLODIPine 5 mg i po qd #90" but the patient profile already has an active "Norvasc 5 mg daily"; what should you do FIRST? **A:** Stop and confirm it's the same drug (amlodipine/Norvasc) and route to the pharmacist to avoid duplicate therapy and clarify whether to continue one entry.
773. **Q:** You receive "Synthroid 0.1 mg take 1 tablet daily" and the prescriber's chart note says "levothyroxine 100 mcg"; what is the BEST entry approach? **A:** Enter 100 mcg (0.1 mg) because 1 mg = 1000 mcg, and matching units prevents a strength-selection error.
774. **Q:** An Rx for metoprolol says "take 1 tab bid" but the product selected in order entry is metoprolol succinate ER; what should you do NEXT? **A:** Stop and notify the pharmacist because "bid" usually implies immediate-release tartrate, and choosing ER vs IR is a medication safety issue the pharmacist must confirm.
775. **Q:** The directions say "Instill ii gtt ou bid x 7 days" for an eye drop; what should you type into the sig in plain language? **A:** Enter "Instill 2 drops in both eyes twice daily for 7 days" because ii=2, gtt=drops, and ou=both eyes, reducing misinterpretation risk.
776. **Q:** A patient's Ventolin HFA is prescribed "inhale 2 puffs q6h prn wheeze" and the prescriber wrote "dispense 1 inhaler (200 actuations)"; what days' supply should you bill using max daily use? **A:** Bill 25 days because max is 2 puffs every 6 hours = 8 puffs/day, and $200 \text{ actuations} \div 8 = 25$ days for payer-consistent days' supply.
777. **Q:** While entering an opioid e-Rx you notice the prescriber wrote "take 1-2 tabs q4-6h prn" and the quantity field is blank; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist to obtain a complete, billable quantity and ensure controlled-substance dispensing accuracy.
778. **Q:** A prescription says "Augmentin 875/125 mg 1 tab po bid x 10 days" and the prescriber wrote "#14" by mistake; what is the BEST technician action? **A:** Stop processing and send to the pharmacist because the correct quantity is 20 tablets

(2/day × 10 days), and technicians cannot change quantities without pharmacist intervention.

779. **Q:** During product selection you scan a bottle for “cloNIDine 0.1 mg” but the barcode brings up “cloZAPine 100 mg”; what should you do NEXT? **A:** Do not dispense, re-select the correct NDC for clonidine, and alert the pharmacist if the wrong drug was pulled because clonidine/clozapine is a serious LASA risk.
780. **Q:** An Rx for cephalexin suspension is “250 mg/5 mL take 10 mL po tid x 7 days”; what quantity in mL should you dispense? **A:** Dispense 210 mL because 10 mL/dose × 3 doses/day × 7 days = 210 mL, ensuring the patient has enough medication for the full course.
781. **Q:** A handwritten prescription says “glipiZIDE 10 mg ER 1 tab po qd” but the product selected is immediate-release; what should you do NEXT? **A:** Stop the fill and notify the pharmacist to clarify/confirm ER vs IR because release type changes dosing and is a common look-alike order entry error.
782. **Q:** A patient asks if “Lasix” is the same as “furosemide” when picking up their refill; what is the BEST technician response? **A:** Explain that Lasix is the brand name and furosemide is the generic with the same active ingredient, reducing confusion and improving adherence.
783. **Q:** An e-Rx is for “Zyrtec 10 mg daily” but the patient profile shows a severe reaction to “cetirizine”; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Zyrtec is cetirizine and allergy/ADR screening requires pharmacist intervention.
784. **Q:** The prescriber writes “MS Contin 15 mg q12h” and the patient says they thought they were getting “morphine liquid for breakthrough pain”; what should you do FIRST? **A:** Pause the transaction and refer to the pharmacist because MS Contin is extended-release morphine and confusion with immediate-release products is a high-risk error.
785. **Q:** A prescription reads “Take 1 cap po tid” for Keflex; what should you enter in plain language for the sig? **A:** Enter “Take 1 capsule by mouth three times daily” because technicians should translate abbreviations into clear patient directions.
786. **Q:** You receive an order for “metFORMIN 500 mg bid, #60” but the selected product is metformin ER; what is the BEST next step? **A:** Stop and route to the pharmacist to confirm IR vs ER because they are not interchangeable and affect tolerability and dosing schedule.

787. **Q:** An insulin vial order is "Regular insulin U-100 inject 18 units before meals three times daily, dispense 10 mL"; what days' supply should you bill using total units? **A:** Bill $18 \times 3 = 54$ units/day and $10 \text{ mL} \times 100 \text{ units/mL} = 1000$ units total, so $1000/54 = 18$ days (rounded down) to match max daily use for payer accuracy.
788. **Q:** While selecting product for "hydrOXYzine 25 mg," you notice a nearby bin labeled "hydrALAZINE 25 mg"; what should you do BEST to prevent a LASA error? **A:** Use barcode/NDC verification and read the full drug name/strength before pulling because hydroxyzine/hydralazine are common look-alike/sound-alike mix-ups.
789. **Q:** A patient requests an early refill for oxycodone claiming the tablets were "lost," and the profile shows it was filled 5 days ago; what should you do FIRST? **A:** Do not promise a refill; refer to the pharmacist for controlled-substance early refill evaluation because loss claims and timing require pharmacist judgment and documentation.
790. **Q:** A tote arrives with a refrigerated biologic and the temperature indicator shows an excursion into the unsafe range; what should you do NEXT? **A:** Isolate and label the product as suspect and notify the pharmacist/manager to follow storage and DSCSA/returns policy because temperature excursions can make the drug non-dispensable.
791. **Q:** While entering an e-Rx for levothyroxine, you see the prescriber wrote "take 1 tab daily" but didn't include the strength; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify with the prescriber because strength is required to dispense safely.
792. **Q:** A new prescription for methotrexate tablets is entered as "take 1 tablet daily" for rheumatoid arthritis; what is the BEST technician action? **A:** Hold the prescription and alert the pharmacist immediately because methotrexate is typically weekly and daily directions are a high-alert dosing error.
793. **Q:** A C-II e-prescription for amphetamine/dextroamphetamine shows an old patient address and the patient requests you "just update it and fill"; what should you do NEXT? **A:** Update demographic info per policy but route the prescription to the pharmacist for review before filling because controlled-substance validity concerns require pharmacist intervention.
794. **Q:** You receive a phone call from another pharmacy requesting a transfer of a patient's remaining refills for alprazolam; what should you do FIRST? **A:** Refer

the call to the pharmacist because controlled-substance transfers have strict requirements and must be handled by a pharmacist.

795. **Q:** During product receipt, a case of atorvastatin arrives with torn outer packaging and bottles lacking a readable 2D barcode; what should you do BEST? **A:** Segregate the shipment and notify the pharmacist/manager to treat as suspect product under DSCSA because missing/altered identifiers can indicate diversion or counterfeit.
796. **Q:** The FDA posts a recall for a specific lot of losartan and you find that lot on your shelf; what should you do NEXT? **A:** Pull the affected lot from inventory and quarantine it per recall procedure because recalled product must be prevented from dispensing.
797. **Q:** A patient picking up isotretinoin says they never received counseling before and asks you to explain the risks; what is the BEST response? **A:** Pause the pickup and get the pharmacist because isotretinoin has REMS requirements and counseling must be performed/verified by the pharmacist.
798. **Q:** A prescription label prints "U" for insulin dose (e.g., "inject 10U daily") and your pharmacy uses error-prevention standards; what should you do BEST? **A:** Replace "U" with "units" and have the pharmacist verify the final directions because "U" is an error-prone abbreviation that can cause overdoses.
799. **Q:** A patient presents a pseudoephedrine purchase request that would exceed the daily limit when combined with prior sales shown in the log; what should you do FIRST? **A:** Refuse the sale and follow the logbook/system prompt because federal CMEA limits require blocking transactions that exceed allowed quantities.
800. **Q:** A prescription is for apixaban 5 mg BID, #60; what is the correct days' supply to bill? **A:** Bill 30 days because #60 taken twice daily is $60 \div 2 = 30$, ensuring accurate third-party processing and refill timing.
801. **Q:** While typing a new e-Rx, you notice the prescriber selected hydroxyzine but the directions say "take 1 tablet daily for blood pressure"; what should you do FIRST? **A:** Stop processing and route to the pharmacist to clarify because hydroxyzine is an antihistamine (not an antihypertensive) and this mismatch suggests a drug selection error.
802. **Q:** A prescription is entered for Humalog (insulin lispro) but the product pulled is Humulin R (regular insulin); what is the BEST technician action before it reaches verification? **A:** Stop filling and reselect the correct NDC using

barcode/NDC verification because these are different insulins (LASA/HASA risk) and mix-ups can cause serious harm.

803. **Q:** A patient profile shows active lisinopril and today a new Rx arrives for losartan for the same patient; what should you do NEXT? **A:** Flag it for pharmacist review for possible therapeutic duplication/ACEI-to-ARB switch because the pharmacist must confirm intent before dispensing.
804. **Q:** A patient says they are allergic to "sulfa" and you are processing an order for sulfamethoxazole/trimethoprim; what should you do FIRST? **A:** Pause the fill and alert the pharmacist to assess the allergy risk because SMX/TMP is a sulfonamide antibiotic and could trigger a reaction.
805. **Q:** The prescriber writes "metoprolol ER 50 mg daily," but the selected product is metoprolol tartrate 50 mg; what should you do BEST? **A:** Stop and correct the selection to metoprolol succinate ER (or send to pharmacist to clarify if unsure) because tartrate vs succinate ER are not interchangeable and dosing differs.
806. **Q:** A new prescription is for sertraline, and the patient's profile shows they are also taking linezolid started yesterday; what should you do NEXT? **A:** Refer to the pharmacist immediately because linezolid can interact with SSRIs like sertraline and may increase risk of serotonin syndrome.
807. **Q:** You receive a prescription for cephalexin 500 mg "1 cap QID x 7 days" with quantity missing; what quantity should you enter? **A:** Enter quantity 28 because 1 capsule four times daily for 7 days equals $4 \times 7 = 28$ capsules.
808. **Q:** An order is for amlodipine 10 mg daily, but you see the patient recently filled diltiazem ER and now reports dizziness; what should you do FIRST? **A:** Notify the pharmacist for assessment because combining calcium channel blockers can contribute to additive hypotension/bradycardia and requires clinical review.
809. **Q:** A prescription is written for "predniSONE 10 mg" but the technician accidentally selects prednisoLONE 10 mg/5 mL solution during product selection; what is the BEST action? **A:** Stop and correct to the intended dosage form/strength using NDC/barcode verification because prednisone vs prednisolone and tablet vs liquid are common Top 300 mix-up traps.
810. **Q:** You are billing an inhaler: fluticasone/salmeterol 250/50 Diskus, "inhale 1 puff BID," and the device contains 60 blisters; what days' supply should you submit? **A:** Submit 30 days because 60 doses divided by 2 doses per day equals 30 days.

811. **Q:** While entering a refill, you see "Synthroid 75 mcg take 1 daily" but the patient requests "my levothyroxine"; what should you do NEXT? **A:** Confirm whether the refill is brand medically necessary vs generic allowed and follow the existing prescription/DAW notes because brand vs generic selection affects substitution, cost, and consistency.
812. **Q:** You are selecting stock for "Lamictal 100 mg" and notice a nearby bin for "Lamisil 250 mg"; what is the BEST technician action to prevent a LASA error? **A:** Stop and verify the NDC/barcode against the label and the prescription because similar names (Lamictal vs Lamisil) are a common wrong-drug risk.
813. **Q:** A new e-Rx arrives for "Glucophage XR 500 mg daily" and the patient profile lists metformin IR 500 mg BID as active; what should you do FIRST? **A:** Flag it for pharmacist review for possible therapeutic duplication/therapy change because IR vs ER are not interchangeable without prescriber intent.
814. **Q:** A prescription is written for "MS Contin 15 mg q12h" and during entry you notice the patient asked for "morphine liquid for breakthrough pain"; what should you do NEXT? **A:** Route to the pharmacist to clarify formulation and intent because morphine ER (MS Contin) vs liquid are different dosage forms with high-alert overdose risk.
815. **Q:** You receive an order for "clopidogrel 75 mg daily" and the patient says, "I'm on Plavix already"; what is the BEST response? **A:** Explain that Plavix is the brand name for clopidogrel and verify the patient profile/strength to avoid duplication because brand/generic recognition prevents double therapy.
816. **Q:** The sig on a new Rx reads "Take ii tabs po bid" for naproxen 250 mg, dispense #120; what days' supply should you enter? **A:** Enter 30 days because ii BID equals 4 tablets/day and $120 \div 4 = 30$, which is needed for correct billing and refill timing.
817. **Q:** While typing, you see "U-500 insulin regular" but the selected product is U-100 regular insulin; what should you do FIRST? **A:** Stop processing and get the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch that can cause a 5-fold dosing error.
818. **Q:** A patient on an e-Rx has "allergy: penicillin (anaphylaxis)" and the new medication is dicloxacillin; what should you do FIRST? **A:** Hold the prescription and alert the pharmacist because dicloxacillin is a penicillin-class antibiotic and the documented reaction is severe.

819. **Q:** You are asked to calculate days' supply for timolol 0.5% eye drops 5 mL with directions "1 drop OU BID," assuming 20 drops/mL; what should you bill? **A:** Bill 25 days because $5 \text{ mL} \times 20 = 100$ drops and OU BID uses 4 drops/day, so $100 \div 4 = 25$ for accurate claims submission.
820. **Q:** A tote of refrigerated biologic pens arrives warm with no temperature indicator data; what should you do NEXT? **A:** Quarantine the shipment and notify the pharmacist/wholesaler per policy because a temperature excursion makes the product potentially suspect and unsafe to dispense.
821. **Q:** While entering an e-Rx for hydromorphone 4 mg "take 1 tablet q4h prn pain," you notice the prescriber also ordered oxycodone/acetaminophen for the same patient today; what should you do FIRST? **A:** Stop processing and notify the pharmacist to assess duplicate opioid therapy and safety because this requires clinical review beyond technician scope.
822. **Q:** A written prescription for alprazolam has the drug, directions, and quantity but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because a missing signature makes validity questionable and must be resolved before dispensing.
823. **Q:** A patient requests an early refill on Adderall and the claim rejects "refill too soon"; what should you do NEXT? **A:** Inform the patient of the refill-too-soon status and refer to the pharmacist to determine if an override/new order is appropriate because controlled-substance timing needs pharmacist handling.
824. **Q:** You are counting tablets for methotrexate 2.5 mg and see directions entered as "take 1 tablet daily"; what should you do FIRST? **A:** Stop and alert the pharmacist to verify the dosing schedule because methotrexate is high-alert and daily dosing can signal a serious entry error.
825. **Q:** A bottle of atorvastatin 40 mg arrives with a broken seal and powder residue inside the tote; what is the BEST technician action? **A:** Quarantine the product and notify the pharmacist/manager because damaged packaging is a suspect product and should not enter inventory.
826. **Q:** A wholesaler invoice lists two NDCs for the same drug, and the shipped NDC does not match what your order says should arrive; what should you do NEXT under DSCSA principles? **A:** Set the item aside and verify the NDC/lot/transaction information before stocking because mismatched product documentation can indicate a supply-chain issue.

827. **Q:** A patient asks you, at the counter, what side effects to watch for when starting lisinopril; what should you do BEST? **A:** Refer the patient to the pharmacist for counseling because medication side-effect counseling is required to be provided by a pharmacist under OBRA-90 expectations.
828. **Q:** An e-Rx comes in for "Humalog U-100" but the entry screen shows "Humulin R U-100" selected; what should you do FIRST? **A:** Stop entry and correct the drug selection using barcode/NDC verification, then have the pharmacist review if needed because mix-ups between insulin products are high-risk errors.
829. **Q:** You receive a manufacturer recall notice for a specific lot of losartan, and you find that lot on your shelf; what should you do NEXT? **A:** Pull the affected lot from stock and quarantine it per recall procedure because recalled lots must be removed to prevent dispensing.
830. **Q:** Calculate days' supply for metformin 500 mg tablets with directions "take 1 tablet by mouth twice daily," dispense #180; what days' supply should you enter? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$ for accurate billing and refill timing.
831. **Q:** A prescription reads "levothyroxine 0.05 mg take 1 tab daily," but the stock bottle is labeled 50 mcg; what should you do FIRST in order entry? **A:** Enter 50 mcg (or 0.05 mg) as the same strength and verify units match to prevent a 1000× conversion error.
832. **Q:** You are billing amoxicillin/clavulanate suspension 400 mg/57 mg per 5 mL with directions "10 mL PO BID x 7 days"; what quantity (mL) should you dispense/enter? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$, ensuring enough for the full course.
833. **Q:** A patient's prescription is for azithromycin 200 mg/5 mL: "give 12.5 mL today, then 6.25 mL daily for 4 days"; what total mL should be dispensed? **A:** Dispense 37.5 mL because $12.5 \text{ mL} + (6.25 \text{ mL} \times 4) = 37.5 \text{ mL}$, preventing underfilling.
834. **Q:** You are entering a prescription that says "digoxin 0.125 mg PO qd," but the prescriber's note line says "take 1/2 of 0.25 mg tablet daily"; what is the BEST technician action? **A:** Enter exactly what is prescribed and route to the pharmacist to clarify the intended strength/form since mixed directions increase dosing-error risk.

835. **Q:** A prescription says "prednisone 20 mg ii po bid x 5 days, #?"; what quantity should you calculate for dispensing? **A:** Dispense 20 tablets because ii = 2 tablets per dose, BID = 2 doses/day, so $2 \times 2 \times 5 = 20$ tablets, matching the full duration.
836. **Q:** An order for furosemide oral solution is written "10 mg PO BID," and the product is 20 mg/5 mL; how many mL per dose should you enter? **A:** Enter 2.5 mL per dose because $20 \text{ mg}/5 \text{ mL} = 4 \text{ mg/mL}$ and $10 \text{ mg} \div 4 \text{ mg/mL} = 2.5 \text{ mL}$ to avoid over- or underdosing.
837. **Q:** While selecting product for "hydrOXYzine 25 mg," you notice the screen autofilled "hydrALazine 25 mg"; what should you do FIRST? **A:** Stop and correct the selection using the exact drug name and NDC/barcode because this LASA pair can cause serious wrong-drug errors.
838. **Q:** A prescription reads "clonidine 0.1 mg take 1 tab qhs," but the tech typed "qh"; what should you do NEXT? **A:** Fix the sig to qhs (at bedtime) and flag for pharmacist verification because qh means every hour and is a high-risk abbreviation error.
839. **Q:** You need to bill days' supply for insulin aspart U-100 vial 10 mL with directions "inject 12 units before meals TID"; what days' supply should you enter (use 100 units/mL)? **A:** Enter 27 days because the vial contains 1000 units ($10 \text{ mL} \times 100 \text{ units/mL}$) and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7 \rightarrow 27$ days to avoid overbilling.
840. **Q:** A new prescription for sertraline is entered as "100 mg" but the e-Rx text says "start 50 mg daily for 7 days then 100 mg daily"; what should you do BEST? **A:** Stop processing and send to the pharmacist for clarification/appropriate directions because titration schedules require clinical review and accurate sig entry.
841. **Q:** A prescription is written for "Celebrex 200 mg PO daily," but the patient asks if it's the same as celecoxib; what should you do BEST? **A:** Confirm that Celebrex is the brand name for celecoxib and proceed with correct product selection because brand/generic recognition prevents duplication and dispensing errors.
842. **Q:** During order entry for lisinopril, the patient profile shows a documented history of angioedema with an ACE inhibitor; what should you do FIRST? **A:** Stop processing and alert the pharmacist for clinical review because ACE inhibitor-associated angioedema is a serious safety concern.

843. **Q:** You're filling a new prescription for simvastatin and the medication list shows clarithromycin started yesterday; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist because clarithromycin can dangerously increase simvastatin levels and risk myopathy/rhabdomyolysis.
844. **Q:** A patient is prescribed "metoprolol succinate ER 50 mg daily," but the selected product is metoprolol tartrate 50 mg; what should you do NEXT? **A:** Stop and correct the product to metoprolol succinate ER before filling because tartrate vs succinate is a common LASA/trap with different release characteristics.
845. **Q:** The directions for gabapentin read "take 1 cap TID," but the prescriber sent "gabapentin 300 mg" and the patient's previous strength was 100 mg; what should you do BEST? **A:** Route to the pharmacist to verify the intended strength because a strength change can be clinically significant and technicians shouldn't assume it's correct.
846. **Q:** An e-prescription says "Humalog 100 units/mL KwikPen, inject 8 units with meals TID, dispense 1 box (5 pens of 3 mL)," what days' supply should you enter? **A:** Enter 62 days because total units = 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units and $8 \times 3 = 24$ units/day, so $1500 \div 24 = 62.5$ rounded down to 62 for billing.
847. **Q:** You receive a prescription for "nitrofurantoin 100 mg BID x 5 days" but the product chosen is nitrofurantoin macrocrystals 100 mg instead of Macrobid (monohydrate/macrocrystals) 100 mg; what should you do FIRST? **A:** Pause and ask the pharmacist to confirm the intended formulation because Macrobid vs macrocrystals are not automatically interchangeable and selection errors are common.
848. **Q:** A patient picks up tramadol and asks if it's a controlled medication and whether refills are allowed; what should you do BEST? **A:** Refer the question to the pharmacist while noting tramadol is a controlled substance because controlled-substance counseling and refill limits should be handled per pharmacist and pharmacy policy.
849. **Q:** While typing a prescription for "lamotrigine," you notice the patient profile lists a prior severe rash reaction to lamotrigine; what should you do NEXT? **A:** Stop processing and immediately alert the pharmacist because rechallenge after a serious rash history is high-risk and needs pharmacist intervention.

850. **Q:** A prescription is for "levetiracetam 500 mg, take 1 tab BID, dispense #120," what days' supply should you enter? **A:** Enter 60 days because 1 tablet twice daily equals 2 tablets/day and $120 \div 2 = 60$ for accurate days' supply billing.
851. **Q:** A prescription reads "furosemide 40 mg i po bid," and the prescriber also wrote "#60"; what days' supply should you enter? **A:** Enter 30 days because 2 tablets/day means $60 \text{ tablets} \div 2 \text{ per day} = 30$, matching the written quantity.
852. **Q:** An e-Rx comes in for "predniSONE 10 mg: take ii tabs po qd x 5 days, then i tab po qd x 5 days; dispense #?"; what quantity should you calculate before sending to the pharmacist for review? **A:** Calculate 15 tablets because $2/\text{day}$ for 5 days = 10 plus $1/\text{day}$ for 5 days = 5, ensuring correct total for order entry.
853. **Q:** You are entering "amLODIPine-benazepril 5/20 cap daily," but you accidentally select amlodipine 5 mg tablets; what should you do NEXT? **A:** Stop and reselect the correct combination product/NDC because it's a different medication (combo vs single-ingredient) and could cause therapeutic failure or duplication.
854. **Q:** A prescription says "dorzolamide-timolol ophthalmic: instill 1 gtt OU bid; dispense 10 mL"; what days' supply should you bill using 20 gtt/mL? **A:** Enter 50 days because 1 drop in each eye twice daily = 4 gtt/day and $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ gtt}$, so $200 \div 4 = 50$.
855. **Q:** The sig on a new prescription is typed as "levothyroxine 0.125 mg daily," but the prescriber's note says "125 mcg daily"; what should you do BEST in order entry? **A:** Enter 125 mcg (0.125 mg) and ensure the selected strength matches because mg-to-mcg conversion errors are common and can cause a 1000-fold mistake if misread.
856. **Q:** A patient brings in an OTC request for pseudoephedrine 30 mg tablets, wants 96 tablets, and has already purchased 60 tablets in the last 10 days; what should you do FIRST? **A:** Check the log/system limits and refer to the pharmacist before completing the sale because CMEA tracking and quantity limits must be verified at the point of sale.
857. **Q:** You receive an e-Rx for "Xarelto 20 mg daily," but the prescriber wrote "rivaroxaban" in the comments and the patient asks if it's the same drug; what should you do BEST? **A:** Confirm Xarelto is rivaroxaban and proceed per counseling workflow (offer pharmacist consult) because brand/generic recognition is a technician task but clinical questions go to the pharmacist.

858. **Q:** A prescription is for "azithromycin 250 mg: take 2 tabs today then 1 tab daily x 4 days; dispense #6"; what days' supply should you enter? **A:** Enter 5 days because the total regimen covers day 1 plus 4 more days, and the quantity aligns with the standard 5-day course.
859. **Q:** While typing "hydromorphone 2 mg q4h prn pain," you notice you selected HYDROcodone/APAP by mistake due to LASA; what should you do NEXT? **A:** Stop processing and correct the drug selection using barcode/NDC verification because hydromorphone vs hydrocodone is a high-risk LASA error with major safety implications.
860. **Q:** A prescription reads "metFORMIN ER 500 mg: take 2 tabs po qpm with meal; dispense #180"; what days' supply should you enter? **A:** Enter 90 days because the patient takes 2 tablets/day and $180 \div 2 = 90$, which supports correct billing and refill timing.
861. **Q:** While entering a prescription for insulin U-500, you notice the prescriber wrote "inject 50 units" but did not specify U-100 vs U-500 syringe/pen; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify the device/concentration because U-500 dosing is high-alert and "units" can cause dangerous misdosing.
862. **Q:** A patient asks for an early refill of oxycodone/APAP claiming it was "lost," and the last fill was 7 days ago for a 30-day supply; what should you do NEXT? **A:** Hold the request and escalate to the pharmacist because early replacement of a Schedule II is a controlled-substance red flag requiring pharmacist judgment and policy steps.
863. **Q:** A new prescription for sertraline arrives, and the patient profile shows they currently take paroxetine; what is the BEST technician action? **A:** Pause the fill and alert the pharmacist to possible therapeutic duplication/interaction risk because two SSRIs together can be unsafe and needs pharmacist review.
864. **Q:** You receive a shipment of refrigerated liraglutide pens, but the tote temperature indicator shows exposure above range; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because a temperature excursion makes the product potentially non-dispensable until stability is confirmed.
865. **Q:** During product selection, you scan a stock bottle for clonidine 0.1 mg but the barcode links to clonazepam 0.5 mg; what should you do NEXT? **A:** Stop and

reselect the correct NDC using barcode verification because LASA name confusion is a common dispensing error risk.

866. **Q:** An e-Rx for alprazolam has directions and quantity but no prescriber DEA number on the face of the transmission; what should you do BEST? **A:** Route it to the pharmacist to verify controlled-substance validity because benzodiazepines are Schedule IV and missing/invalid prescriber identifiers must be resolved before dispensing.
867. **Q:** A prescription is for cephalexin 500 mg "i cap po qid" with quantity #28; what days' supply should you enter? **A:** Enter 7 days because $28 \text{ capsules} \div 4 \text{ per day} = 7 \text{ days}$ for accurate billing and labeling.
868. **Q:** A patient picking up isotretinoin asks why the pharmacy is delaying; what should you do FIRST? **A:** Refer to the pharmacist and confirm iPLEDGE/REMS requirements are met before dispensing because isotretinoin has mandated safety checks beyond technician scope.
869. **Q:** The wholesaler sends a recall notice for a specific lot of losartan, and you find that lot number on the shelf; what should you do NEXT? **A:** Pull the affected lot from inventory and quarantine it per recall procedure because recalled product must be prevented from dispensing and tracked for return.
870. **Q:** A hardcopy prescription for hydrocodone/APAP has the patient name, drug, and directions but no date written; what should you do FIRST? **A:** Stop processing and notify the pharmacist because missing essential prescription elements on a controlled substance requires pharmacist resolution before it can be filled.
871. **Q:** A new e-Rx is for "glipiZIDE 10 mg ER" but you accidentally selected glipizide 10 mg IR in the system; what should you do NEXT? **A:** Stop and correct the dosage form to ER before continuing because IR vs ER is a common error that can change dosing and safety.
872. **Q:** A patient says they take "Lipitor" but your profile shows atorvastatin and the new prescription is for simvastatin; what should you do FIRST? **A:** Confirm the patient's current medication and route to the pharmacist for duplication review because two statins together may be unintended therapy duplication.
873. **Q:** While typing directions, you see "qhs" on a zolpidem prescription and the patient asks what it means; what is the BEST technician response? **A:** Explain

that qhs means "at bedtime" because technicians can clarify common sig abbreviations without changing the prescribed directions.

874. **Q:** A prescription says "levothyroxine 0.1 mg po daily"; what strength should you enter in micrograms? **A:** Enter 100 mcg because $0.1 \text{ mg} \times 1000 \text{ mcg/mg} = 100 \text{ mcg}$, preventing decimal-place dosing errors.
875. **Q:** During product selection you notice Tall Man lettering for hydrOXYzine and hydrALAZINE on the shelf; what should you do BEST to prevent a LASA error? **A:** Use barcode/NDC scanning and re-verify drug name/indication with the label because Tall Man lettering highlights look-alike names but scanning is the safest check.
876. **Q:** A new prescription is for "Plavix 75 mg daily" and the profile lists clopidogrel 75 mg daily as active; what should you do NEXT? **A:** Stop and flag it for pharmacist review as a brand/generic duplicate because Plavix is clopidogrel and double therapy could occur if both are dispensed.
877. **Q:** An order is for warfarin and the prescriber writes "take as directed" with no dosing schedule; what should you do FIRST as the technician? **A:** Hold the prescription and notify the pharmacist because "as directed" on a high-alert drug needs pharmacist clarification for safe entry and counseling.
878. **Q:** A patient requests pseudoephedrine and you are about to ring it up like regular OTC; what should you do FIRST? **A:** Follow the controlled sales process (ID check, logbook/electronic entry, and quantity limits per policy) because pseudoephedrine sales are regulated under federal Combat Methamphetamine Epidemic Act requirements.
879. **Q:** A prescription reads "amoxicillin/clavulanate 875/125 mg: 1 tab po bid x 10 days; dispense #14"; what should you do NEXT? **A:** Stop and send to the pharmacist for clarification because the quantity (14) does not match the directions (20 tablets needed), and technicians cannot change quantities independently.
880. **Q:** You receive a patient complaint of severe rash after starting lamotrigine and they ask how to report it; what is the BEST technician action? **A:** Refer them to the pharmacist and provide MedWatch reporting information because serious adverse events should be escalated and may need FDA reporting and clinical guidance.

881. **Q:** A prescription says "metFORMIN 500 mg: ii tabs po bid; Disp #120"; what quantity and days' supply should you enter? **A:** Enter Qty 120 and 30 days' supply because 2 tablets twice daily = 4/day and $120 \div 4 = 30$ days.
882. **Q:** You're entering "azithromycin 200 mg/5 mL: give 10 mL day 1, then 5 mL daily days 2–5"; how many mL should be dispensed? **A:** Dispense 30 mL because total volume is $10 \text{ mL} + (5 \text{ mL} \times 4 \text{ days}) = 30 \text{ mL}$ to match the full regimen.
883. **Q:** A new e-Rx reads "predniSONE 20 mg: take 3 tabs daily x 5 days; Disp #30"; what should you do NEXT? **A:** Stop and notify the pharmacist to clarify/correct quantity because $3/\text{day} \times 5 \text{ days} = 15$ tablets, so #30 is inconsistent and may cause over-dispensing.
884. **Q:** A provider writes "digoxin 0.125 mg po daily"; what strength should you type in micrograms? **A:** Enter 125 mcg because $0.125 \text{ mg} \times 1000 \text{ mcg/mg} = 125 \text{ mcg}$ to prevent a decimal-place dosing error.
885. **Q:** A prescription says "timolol 0.5% ophthalmic: instill 1 gtt OU bid; dispense 5 mL"; what days' supply should you calculate using 20 drops/mL? **A:** Enter 25 days' supply because $5 \text{ mL} \times 20 = 100$ drops and $1 \text{ drop} \times 2 \text{ eyes} \times 2/\text{day} = 4$ drops/day, so $100 \div 4 = 25$ days.
886. **Q:** You receive "Lantus SoloStar 100 units/mL: inject 18 units qhs; dispense 1 box (5 pens of 3 mL)"; what days' supply should you enter? **A:** Enter 83 days' supply because total units = $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500$ units, and $1500 \div 18 = 83.3$ so 83 days (round down for billing).
887. **Q:** While typing an order you see "MSO4 2 mg IV q2h prn pain"; what should you do FIRST? **A:** Hold the order and alert the pharmacist because "MSO4" is an unsafe abbreviation that can be misread, requiring clarification before entry.
888. **Q:** A prescription for levETIRAcetam says "1 tab po q12h"; the patient asks how often that is—what is the BEST technician response? **A:** Say it means every 12 hours (typically twice daily) and offer pharmacist counseling because technicians can interpret sig codes but clinical counseling is pharmacist-led.
889. **Q:** You're selecting "Lamictal" and notice both lamotrigine and terbinafine are stored nearby with similar packaging; what is the BEST next step before filling? **A:** Verify the NDC/barcode against the prescription and shelf label because barcode/NDC confirmation is the most reliable way to prevent a LASA selection error.

890. **Q:** A prescriber orders "amiodarone 200 mg daily" and you see an active profile medication "simvastatin 80 mg daily"; what should you do NEXT? **A:** Pause processing and refer to the pharmacist because amiodarone can significantly increase simvastatin toxicity risk, requiring pharmacist assessment before dispensing.
891. **Q:** A new e-Rx is for "Zestril 10 mg 1 tab po daily," but the patient profile shows an allergy to lisinopril; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Zestril is lisinopril (brand/generic match) and the allergy screen suggests a safety risk.
892. **Q:** While typing you see "hydroxyzine 25 mg i-ii po q6h prn itching," and the patient asks what "q6h" means; what is the BEST technician response? **A:** Explain "q6h" means every 6 hours because clarifying common sig codes is within technician scope and supports safe use.
893. **Q:** A prescription reads "levothyroxine 0.05 mg po daily"; what strength should you enter in micrograms? **A:** Enter 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing decimal-point dosing errors.
894. **Q:** You receive an e-Rx for "Norvasc 5 mg daily," but the patient says they already take amlodipine 5 mg from another pharmacy; what should you do NEXT? **A:** Verify the active med list and route to the pharmacist for therapeutic-duplication review because Norvasc is amlodipine and duplicate therapy may occur.
895. **Q:** During product selection for "Humalog," you notice "HumaLOG" and "HumuLIN R" are adjacent and look similar; what should you do BEST before filling? **A:** Use barcode/NDC scan and match the exact name/strength to the label because insulin is high-alert and LASA selection errors can cause serious harm.
896. **Q:** A hardcopy says "oxycodone/APAP 5/325 mg 1 tab po q6h prn pain, #20, refills 2"; what should you do FIRST? **A:** Stop and notify the pharmacist because Schedule II prescriptions cannot be refilled under federal law.
897. **Q:** An order is "amoxicillin 400 mg/5 mL: take 7.5 mL po bid x 10 days"; how many mL should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.

898. **Q:** A prescription says "fluticasone/salmeterol 250/50 Diskus: inhale 1 puff bid; dispense 1 inhaler (60 blisters)"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$.
899. **Q:** You receive "Bactroban ointment apply to affected area tid x 7 days," but the product pulled is mupirocin cream; what should you do NEXT? **A:** Stop filling and select the correct dosage form per the prescription because cream vs ointment is not interchangeable without pharmacist/prescriber authorization.
900. **Q:** A shipment arrives and a tote of refrigerated liraglutide pens feels warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager because a cold-chain excursion makes it suspect and it may be unsafe/undispenseable.
901. **Q:** While entering a new Rx for metformin, you notice the prescriber typed "methotrexate 10 mg weekly" on the same patient earlier today and the names look similar in the queue; what should you do FIRST? **A:** Stop processing and verify the drug and directions against the original prescription image/eRx and alert the pharmacist if any mismatch is suspected, because metformin vs methotrexate is a high-risk LASA error.
902. **Q:** A patient drops off a written prescription for clonazepam 1 mg with no prescriber signature; what is the BEST next step? **A:** Hold the prescription and route it to the pharmacist for clarification before entry, because missing required elements can make a controlled-substance Rx invalid and technicians cannot resolve it independently.
903. **Q:** A refill request comes in for oxycodone ER with "refill remaining" showing in the system from an old entry; what should you do NEXT? **A:** Stop and notify the pharmacist to review, because Schedule II medications cannot be refilled and the profile entry may be incorrect.
904. **Q:** You receive a DEA 222 order shipment and one C-II bottle's seal is broken and tablets are loose in the tote; what should you do FIRST? **A:** Segregate the damaged product and notify the pharmacist/manager for documentation and supplier resolution, because compromised controlled-substance integrity requires secure handling and investigation.
905. **Q:** During pickup, a patient asks for a printout of all medications for their spouse, but the spouse is not present and no authorization is on file; what should you do BEST? **A:** Decline to release PHI and refer to the pharmacist or follow HIPAA

authorization procedures, because disclosure without permission violates privacy requirements.

906. **Q:** An e-Rx for isotretinoin arrives and the iPLEDGE/REMS status is not documented in the workflow; what should you do FIRST? **A:** Pause processing and notify the pharmacist to confirm REMS requirements are met before dispensing, because REMS drugs require specific verification steps for patient safety and compliance.
907. **Q:** A patient reports severe dizziness and fainting that started after beginning lisinopril last week and asks if they should stop it; what is the BEST technician response? **A:** Immediately refer the patient to the pharmacist for assessment and urgent guidance, because symptom triage and therapy decisions require pharmacist intervention.
908. **Q:** A wholesaler recall notice lists a specific lot number of atorvastatin 20 mg as recalled, and you find that lot on the shelf; what should you do NEXT? **A:** Pull the affected lot from stock and segregate it per recall procedure for pharmacist follow-up, because recalled product must be removed to prevent dispensing.
909. **Q:** A prescription is for prednisone 20 mg "take 2 tablets daily for 5 days"; how many tablets should be dispensed? **A:** Dispense 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets total}$.
910. **Q:** You are selecting heparin flush syringes and notice both 10 units/mL and 100 units/mL boxes look nearly identical; what should you do BEST before filling? **A:** Verify the exact strength and NDC/barcode against the prescription and use Tall Man/auxiliary labeling practices, because heparin is high-alert and strength mix-ups can cause serious harm.
911. **Q:** While typing an e-Rx for hydralazine, you notice the technician next to you is filling hydroxyzine for the same patient and the names look alike in the queue; what should you do FIRST? **A:** Pause the entry/fill and perform a focused name-strength-indication check (Tall Man/LASA awareness) and alert the pharmacist if any mix-up is possible, because hydralazine vs hydroxyzine is a high-risk look-alike error.
912. **Q:** A new prescription reads "levothyroxine 0.125 mg daily," but the patient profile shows levothyroxine 125 mcg daily from the same prescriber last month; what is the BEST next step for the technician? **A:** Enter exactly as written and route to the pharmacist for verification of mg-to-mcg equivalence and continuity,

because technicians must not assume dose intent even when the values are equivalent.

913. **Q:** You are entering a prescription for bupropion SR and the prescriber wrote "take 1 tablet TID"; what should you do NEXT? **A:** Stop and refer to the pharmacist for clarification before processing, because bupropion has seizure-risk dose limits and SR products are not typically dosed three times daily.
914. **Q:** A patient picking up sertraline asks if it can be taken for "nerve pain like my gabapentin"; what is the BEST technician response? **A:** Explain you can't provide clinical advice and offer to have the pharmacist counsel, because medication use questions require pharmacist assessment even though you can recognize sertraline as an SSRI.
915. **Q:** The hard copy says "Lantus Solostar inject 30 units SQ daily," but the product pulled is Toujeo Solostar because both are insulin glargine pens; what should you do FIRST? **A:** Stop the fill and re-select the exact prescribed product/NDC and notify the pharmacist if it reached verification, because Lantus and Toujeo are not interchangeable unit-for-unit despite similar names.
916. **Q:** A prescription is for azithromycin 250 mg "take 2 tablets PO day 1, then 1 tablet daily days 2-5"; how many tablets should be dispensed? **A:** Dispense 6 tablets, because the total is 2 tablets on day 1 plus 1 tablet for 4 more days ($2 + 4 = 6$).
917. **Q:** During data entry for fluoxetine, you see the patient is also actively taking linezolid started yesterday from an outside clinic; what should you do BEST? **A:** Flag the interaction and immediately route to the pharmacist before dispensing, because linezolid with SSRIs can cause serious serotonin syndrome risk requiring clinical intervention.
918. **Q:** You receive an e-Rx for Adderall XR 20 mg with a "refill x2" instruction; what should you do NEXT as the technician? **A:** Hold the order and notify the pharmacist, because stimulants like Adderall XR are Schedule II and refills are not permitted federally.
919. **Q:** While selecting a beta-blocker, the label says "propranolol ER 60 mg," but the stock bottle you grabbed is propranolol immediate-release 60 mg; what should you do FIRST? **A:** Stop and choose the correct ER dosage form and verify NDC/barcode, because ER vs IR mix-ups can cause dosing and therapeutic failures.

920. **Q:** A patient hands you a new prescription for colchicine and mentions they're currently taking clarithromycin that another doctor prescribed; what should you do BEST? **A:** Alert the pharmacist before processing, because colchicine with strong CYP3A4 inhibitors like clarithromycin can lead to dangerous toxicity and may require therapy changes.
921. **Q:** While processing a new warfarin prescription, you see the prescriber wrote "take 1 tablet daily" but did not specify a strength; what should you do FIRST? **A:** Stop processing and send to the pharmacist to obtain a complete order because missing strength is a safety-critical omission for a high-alert drug.
922. **Q:** A patient requests an early refill of oxycodone/APAP 5/325 because it was "lost," and the claim rejects refill-too-soon; what should you do NEXT? **A:** Refer the situation to the pharmacist and follow pharmacy policy because controlled-substance early refills require pharmacist review and documentation.
923. **Q:** You receive a shipment and notice a tote of insulin glargine pens arrived warm with a temperature indicator showing an excursion; what should you do BEST? **A:** Segregate the product and notify the pharmacist/manager for vendor guidance because temperature excursions can compromise potency and must not be stocked.
924. **Q:** During data entry, an e-Rx for clonazepam includes the prescriber name but no DEA number on the message; what should you do NEXT? **A:** Pause the fill and route to the pharmacist to verify prescriber credentials per pharmacy procedure because controlled-substance dispensing requires validated prescriber authorization.
925. **Q:** A bottle of atorvastatin 40 mg has a torn seal and tablets visible inside the stock bottle; what should you do FIRST? **A:** Remove it from active inventory and notify the pharmacist because damaged packaging is a suspect product and should not be used for dispensing.
926. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets and wants 96 tablets today; what should you do BEST at the register? **A:** Check ID, log the sale, and ensure the requested amount does not exceed legal daily/30-day limits because CMEA requires identification and electronic tracking.
927. **Q:** You are typing a prescription that says "MSO4 2 mg IV q2h PRN pain" for a discharged patient and the abbreviation looks unclear; what should you do FIRST? **A:** Stop and alert the pharmacist because dangerous abbreviations can

indicate morphine or magnesium sulfate and require clarification to prevent serious errors.

928. **Q:** A recall notice arrives for a specific lot of metformin ER 500 mg, and you find that exact lot on the shelf; what should you do NEXT? **A:** Pull the affected lot from stock and place it in a designated recall area for pharmacist processing because recalled lots must be removed to prevent dispensing.
929. **Q:** A prescription reads "metoprolol 25 mg take 1 tab BID," but the product selected is metoprolol succinate ER 25 mg; what should you do FIRST? **A:** Stop and correct the selection using NDC/barcode and dosage form because succinate ER and tartrate IR are not interchangeable and can change dosing.
930. **Q:** A vial label states vancomycin 1 g in 250 mL to infuse over 2 hours; what pump rate in mL/hr should be entered? **A:** Enter 125 mL/hr because $250 \text{ mL} \div 2 \text{ hours}$ equals 125 mL per hour for the correct infusion rate.
931. **Q:** While entering a prescription for Levemir, you notice the technician selected Lantus in the system because both are "insulin glargine-like"; what should you do FIRST? **A:** Stop processing and correct the selection/alert the pharmacist because Levemir is insulin detemir and substituting a different basal insulin is a high-alert error.
932. **Q:** A new e-Rx is for "Lamictal 25 mg" but the patient profile shows a severe rash history with lamotrigine; what should you do NEXT? **A:** Pause the fill and notify the pharmacist to evaluate the allergy history because Lamictal is lamotrigine and re-exposure can be dangerous.
933. **Q:** A patient on lisinopril brings in a new prescription for losartan from urgent care and asks you to "fill everything"; what is the BEST technician action? **A:** Flag and route to the pharmacist for therapeutic duplication review because lisinopril (ACE inhibitor) and losartan (ARB) are usually not used together.
934. **Q:** During product selection, you see "hydroxyzine" and accidentally pulled "hydralazine"; what should you do FIRST? **A:** Stop filling and re-select using NDC/barcode verification because hydroxyzine vs hydralazine is a common LASA mix-up with very different uses.
935. **Q:** A prescription is written for "metFORMIN ER 500 mg" but the patient says they can only swallow small tablets and asks you to crush it; what should you do BEST? **A:** Refer the question to the pharmacist because extended-release metformin should not be crushed and an alternative may be needed.

936. **Q:** A new prescription says "azithromycin Z-Pak take as directed," but the directions field transmitted blank and the quantity is 1 pack; what should you do FIRST? **A:** Hold the prescription and ask the pharmacist to clarify sig entry because "as directed" without explicit directions is unsafe and can cause dosing errors.
937. **Q:** You are filling sertraline and the patient asks if it's the same as Zoloft and what it's for; what should you do NEXT? **A:** Confirm that sertraline is generic Zoloft and refer counseling to the pharmacist because indication/side effects education requires pharmacist counseling.
938. **Q:** A prescription is for "prednisone 10 mg, take ii tabs daily," and the prescriber also wrote "#30"; what days' supply should you enter? **A:** Enter 15 days because 2 tablets/day means $30 \text{ tablets} \div 2 \text{ per day} = 15 \text{ days}$.
939. **Q:** While typing an order for "levothyroxine 100 mcg," you see the technician selected "liothyronine 25 mcg" due to similar names; what should you do FIRST? **A:** Stop and correct the drug selection/notify the pharmacist if needed because levothyroxine vs liothyronine is a high-impact LASA error with different dosing.
940. **Q:** A patient picking up ciprofloxacin says they take tizanidine for muscle spasms and asks if they can take both; what should you do BEST? **A:** Immediately involve the pharmacist before dispensing because ciprofloxacin has a major interaction with tizanidine and can cause dangerous hypotension/sedation.
941. **Q:** A prescription reads "furosemide 40 mg i tab po qd, #30," but the prescriber also writes "take ½ tab daily"; what should you do FIRST in order entry? **A:** Stop and refer to the pharmacist to clarify the conflicting sig before calculating days' supply, because technicians should not choose between contradictory directions.
942. **Q:** You are entering "amoxicillin-clavulanate 875-125 mg take 1 tab BID x 10 days" and the prescriber wrote "#14"; what is the BEST next step? **A:** Hold processing and notify the pharmacist to resolve the quantity mismatch, because 1 tab BID for 10 days requires 20 tablets and technicians can't change quantity without authorization.
943. **Q:** A prescription is "alendronate 70 mg take 1 tab weekly #12"; what days' supply should be entered? **A:** Enter 84 days' supply, because 12 tablets taken once weekly covers 12 weeks ($12 \times 7 = 84 \text{ days}$).
944. **Q:** An e-Rx for "ondansetron ODT 4 mg dissolve 1 tab q8h prn N/V #30" needs a days' supply for billing and your payer requires PRN to be calculated using the

maximum frequency; what should you enter? **A:** Enter 10 days' supply, because q8h max is 3 tablets/day and $30 \div 3 = 10$ days per the stated payer rule.

945. **Q:** A prescriber orders "clindamycin 150 mg take ii caps po TID x 7 days"; what quantity should you enter? **A:** Enter 42 capsules, because ii caps TID is 6 capsules/day and $6 \times 7 = 42$.
946. **Q:** While typing, you see directions "use 2 gtt OU QID" for timolol eye drops; what does OU mean and what should you do NEXT if the prescriber also wrote "right eye only" in notes? **A:** OU means both eyes, and you should stop and route to the pharmacist for clarification because the directions conflict with "right eye only."
947. **Q:** A prescription is for "atorvastatin 20 mg qhs #90," but the product selected is simvastatin 20 mg due to similar strengths; what should you do FIRST? **A:** Stop filling and correct the drug selection (verify NDC/barcode) before proceeding, because this is a wrong-drug error with a high risk of patient harm.
948. **Q:** You receive an order for "potassium chloride oral solution 20 mEq/15 mL; take 15 mL daily"; the bottle concentration in stock is 10 mEq/5 mL; what volume equals 20 mEq? **A:** 10 mL, because 10 mEq/5 mL equals 2 mEq/mL and $20 \text{ mEq} \div 2 \text{ mEq/mL} = 10 \text{ mL}$.
949. **Q:** A patient's new prescription is "tramadol 50 mg take 1 tab q6h prn pain #120" and the prescriber wrote "Refills: 3"; what should you do BEST? **A:** Stop and alert the pharmacist before processing, because tramadol is a controlled substance (CIV) and refill authorization must be validated per controlled-substance requirements.
950. **Q:** During order entry you notice "morphine sulfate ER 30 mg" was typed as "morphine sulfate IR 30 mg" and both appear in the dropdown; what should you do NEXT? **A:** Correct the dosage form and have it verified according to workflow (and notify the pharmacist if already in verification), because ER vs IR is a high-alert distinction that changes dosing and safety.
951. **Q:** A new e-Rx is for "Glucophage 500 mg take 1 tab BID with meals," but your system defaulted to Glucophage XR 500 mg; what should you do FIRST? **A:** Stop and select the correct immediate-release metformin product (or route to pharmacist if unclear) because IR vs ER is a high-risk formulation mix-up.
952. **Q:** A patient asks if "Zocor" and "Lipitor" are basically the same and whether they should take both tonight; what should you do BEST? **A:** Flag it for

pharmacist counseling as therapeutic duplication because both are statins (simvastatin/atorvastatin) and double therapy increases adverse-effect risk.

953. **Q:** While entering a prescription you see "U-500 insulin regular" in the notes but the selected product is Humulin R U-100 vial; what is the BEST next step? **A:** Stop processing and get pharmacist review before dispensing because U-500 vs U-100 is a high-alert concentration error.
954. **Q:** A prescriber writes "hydralazine 25 mg i tab po tid," but the tech typed "hydroxyzine 25 mg"; what should you do NEXT? **A:** Correct the drug selection and re-check the NDC/barcode because hydralazine vs hydroxyzine is a LASA name error with different indications.
955. **Q:** A patient picking up a new prescription for lisinopril says they are pregnant; what should you do FIRST? **A:** Pause the pickup and immediately notify the pharmacist because ACE inhibitors are contraindicated in pregnancy and require pharmacist intervention.
956. **Q:** A label direction reads "instill 1 gtt OS BID," and the patient asks what OS means; what should you do BEST? **A:** Explain OS is left eye and offer pharmacist counseling if needed because technicians can clarify common sig codes to prevent administration errors.
957. **Q:** An e-Rx says "prednisone 10 mg: take ii tabs daily x 5 days," and stock is prednisone 20 mg tablets; what should you do NEXT? **A:** Do not change strength yourself and refer to the pharmacist/prescriber for a new order because technicians cannot adjust strength even if the total daily dose seems equivalent.
958. **Q:** A prescription is "cephalexin 500 mg take 1 cap QID x 7 days"; what quantity should be dispensed? **A:** Dispense 28 capsules because $4 \text{ doses/day} \times 7 \text{ days} = 28$.
959. **Q:** A patient says they are allergic to aspirin and asks whether they can take "Aleve" for pain; what should you do BEST? **A:** Refer to the pharmacist because Aleve is naproxen (an NSAID) and allergy/cross-reactivity assessment is a pharmacist decision.
960. **Q:** You receive a product shipment and a tote contains bottles with smeared lot numbers and unreadable expiration dates; what should you do FIRST? **A:** Segregate and quarantine the suspect inventory and notify the pharmacist/manager because unreadable lot/expiry prevents proper tracing and safe dispensing.

961. **Q:** A prescription reads "levothyroxine 75 mcg take 1 tablet daily" and the tech accidentally entered 0.75 mg; what should you do FIRST? **A:** Stop processing and correct the strength to 75 mcg (0.075 mg) because misplacing the decimal changes the dose tenfold and is a high-risk order entry error.
962. **Q:** The directions say "metformin 500 mg ii tabs PO BID" and the prescriber wrote "Disp #120"; what days' supply should you enter? **A:** Enter 30 days because ii tabs BID = 4 tablets/day and $120 \div 4 = 30$, aligning quantity to dosing.
963. **Q:** An e-Rx for "azithromycin 200 mg/5 mL: give 10 mL daily x 3 days" is being filled with a 15 mL bottle; what is the BEST next step? **A:** Notify the pharmacist to obtain the correct total volume because $10 \text{ mL/day} \times 3 = 30 \text{ mL}$ and a 15 mL bottle is insufficient for the course.
964. **Q:** You receive "amLODIPine 5 mg PO QD" but the product selected is "amiodarone 200 mg"; what should you do NEXT? **A:** Stop and reselect the correct drug using barcode/NDC verification because amlodipine vs amiodarone is a LASA risk that can cause serious harm.
965. **Q:** A prescription says "Take 1 tab PO qhs" for "Zoloft 50 mg," but the indication field mentions depression and the patient profile shows sertraline is used; what should you do BEST? **A:** Enter sertraline (generic for Zoloft) and verify the NDC/label match because brand-generic recognition prevents dispensing the wrong medication.
966. **Q:** A child weighs 44 lb and the order is "acetaminophen 15 mg/kg per dose now"; how many mg is one dose? **A:** 300 mg because $44 \text{ lb} \div 2.2 = 20 \text{ kg}$ and $15 \text{ mg/kg} \times 20 \text{ kg} = 300 \text{ mg}$, ensuring correct weight-based dosing math.
967. **Q:** An inhaler prescription is "fluticasone/salmeterol 250/50 inhale 1 puff BID" and the device contains 60 blisters; what days' supply should you enter? **A:** Enter 30 days because BID use is 2 doses/day and $60 \div 2 = 30$, which is commonly tested for inhaled products.
968. **Q:** The sig on a prescription reads "instill 2 gtt AD QID x 5 days" and the tech is unsure what "AD" means; what should you do FIRST? **A:** Ask the pharmacist to clarify before entry because AD can be misread and must be correctly interpreted as right ear to avoid wrong-site administration.
969. **Q:** A patient's insulin vial is labeled U-100, 10 mL (1000 units), and the prescription is "inject 28 units SC daily"; what days' supply should you enter?

A: Enter 35 days because $1000 \text{ total units} \div 28 \text{ units/day} = 35.7$, and days' supply is typically entered as 35 to avoid overstating coverage.

970. **Q:** While entering "potassium chloride ER 20 mEq take 1 tablet BID," the system suggests "KCl 20 mEq/15 mL liquid" due to a prior fill; what is the BEST next step? **A:** Stop and route to the pharmacist to confirm the intended dosage form because switching ER tablets to liquid changes administration and may indicate an order entry mismatch.
971. **Q:** While typing a new e-Rx for Humalog (insulin lispro) you notice the patient also has an active prescription for NovoLog (insulin aspart) with similar directions; what should you do BEST? **A:** Pause processing and alert the pharmacist to evaluate therapeutic duplication because look-alike rapid-acting insulins increase risk of hypoglycemia and require pharmacist review.
972. **Q:** A hardcopy prescription for oxycodone/APAP has the patient name, drug, strength, and directions but no prescriber signature; what should you do FIRST? **A:** Stop processing and send it to the pharmacist because controlled-substance prescriptions generally must be signed/authorized before dispensing.
973. **Q:** A prescriber writes "morphine sulfate ER 30 mg PO BID" but the product pulled is morphine sulfate IR 30 mg; what should you do NEXT? **A:** Stop filling and reselect the correct ER product and have it re-verified because ER vs IR mix-ups are high-risk and can cause overdose.
974. **Q:** A new prescription for isotretinoin arrives and the patient asks to pick it up today; what is the BEST technician action? **A:** Refer to the pharmacist and follow iPLEDGE workflow (confirm required authorization/REMS steps) because isotretinoin is REMS-restricted and cannot be dispensed without compliance.
975. **Q:** You receive a manufacturer recall notice for a specific NDC and lot of losartan tablets that matches stock on your shelf; what should you do FIRST? **A:** Remove the affected lot from dispensing inventory and quarantine per policy because recalled product must be isolated to prevent dispensing.
976. **Q:** A patient presents a driver's license and requests pseudoephedrine 30 mg tablets; what should you do FIRST at the register? **A:** Verify ID and complete the required log and quantity limits before sale because CMEA requires purchaser identification and tracking for pseudoephedrine.
977. **Q:** A patient asks for an early refill of clonazepam saying they "lost the bottle"; what should you do BEST? **A:** Do not promise an early fill and immediately

involve the pharmacist because controlled-substance early refills require pharmacist assessment and prescriber authorization.

978. **Q:** A prescription is entered as "hydroxyzine 25 mg" but the image shows "hydralazine 25 mg"; what should you do FIRST? **A:** Stop data entry and notify the pharmacist to clarify before dispensing because LASA drug name errors can cause serious harm.
979. **Q:** A vaccine is drawn up but you realize the vial has been sitting at room temperature longer than allowed by storage guidelines; what should you do NEXT? **A:** Do not use it and quarantine/dispose per policy while notifying the pharmacist because temperature excursions can reduce potency and compromise safety.
980. **Q:** An order is for enoxaparin 40 mg/0.4 mL prefilled syringes "inject 40 mg SC daily," and you have 12 syringes; what days' supply should you enter? **A:** Enter 12 days because the patient uses 1 syringe per day and days' supply equals quantity (12 syringes) divided by daily use (1).
981. **Q:** While entering an e-Rx for metformin ER (Glucophage XR) 500 mg, you notice the patient is already taking metformin IR 500 mg; what should you do BEST? **A:** Pause processing and alert the pharmacist to assess therapeutic duplication/appropriate formulation because technicians should not decide between IR vs ER therapy.
982. **Q:** A prescription is written for "Lamictal 25 mg take 1 tab daily," but the image in the system shows "Lamisil 250 mg"; what should you do FIRST? **A:** Stop the entry and verify the exact drug name with the original prescription and pharmacist because this is a LASA mix-up that could cause serious harm.
983. **Q:** A patient picking up lisinopril says they are pregnant and asks if it's safe; what is the BEST technician response? **A:** Immediately refer to the pharmacist because ACE inhibitors are contraindicated in pregnancy and require prompt clinical intervention.
984. **Q:** You're processing a new order for clarithromycin and the profile shows simvastatin is active; what should you do NEXT? **A:** Hold the fill and notify the pharmacist to evaluate the major interaction (macrolides can raise simvastatin levels) because technicians must escalate high-risk interactions.

985. **Q:** A hardcopy says "levothyroxine 0.125 mg PO daily"; what strength should you select in the system? **A:** Enter 125 mcg because 0.125 mg converts to 125 mcg, preventing a 1000-fold dosing error.
986. **Q:** The directions read "take ii tabs po bid" for sertraline 50 mg; how many tablets per day should you calculate for days' supply? **A:** Enter 4 tablets/day because ii = 2 and BID = twice daily, which is essential for accurate days' supply and billing.
987. **Q:** You receive a prescription for apixaban (Eliquis) and the patient asks what it's for; what should you do FIRST? **A:** Refer the question to the pharmacist because apixaban is a high-risk anticoagulant and counseling/indication questions require pharmacist involvement.
988. **Q:** During product selection you scan the bottle for "Wellbutrin SR 150 mg," but the barcode pulls up "Wellbutrin XL 150 mg"; what should you do NEXT? **A:** Stop filling and select the correct SR product, then continue the workflow because SR vs XL is a common error with different release characteristics.
989. **Q:** A prescription is for "gabapentin 300 mg take 1 cap TID #270"; what days' supply should you enter? **A:** Enter 90 days because 1 capsule TID = 3/day and $270 \div 3 = 90$, ensuring accurate adjudication and refill timing.
990. **Q:** A patient brings in a new prescription for escitalopram (Lexapro) but their profile shows citalopram (Celexa) is still active; what should you do BEST? **A:** Flag and refer to the pharmacist to evaluate SSRI therapeutic duplication and clarify intended therapy because technicians cannot discontinue or switch antidepressants.
991. **Q:** While typing a new Rx for celecoxib (Celebrex), the patient profile shows a serious sulfonamide allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess allergy risk because celecoxib is a sulfonamide and requires pharmacist review before dispensing.
992. **Q:** A prescription says "predniSONE 20 mg: take 2 tabs po qd x 5 days #10"; what days' supply should you enter? **A:** Enter 5 days because 2 tablets per day for 10 tablets totals $10 \div 2 = 5$ days.
993. **Q:** You receive an e-Rx for hydralazine, but the prescriber typed "hydrOXYzine" in the drug field and the sig says "for itching"; what is the BEST technician action? **A:** Hold the order and send to the pharmacist for clarification because the indication and drug name conflict and this is a LASA risk.

994. **Q:** A patient drops off a new Rx for clopidogrel (Plavix) and asks if it's the same as "blood thinner"; what should you do NEXT? **A:** Refer the question to the pharmacist because counseling about indication and therapy is pharmacist-provided, while you can confirm it's a brand/generic match if asked.
995. **Q:** During product selection you notice insulin regular U-100 was pulled instead of insulin glargine U-100 for a new Rx; what should you do FIRST? **A:** Stop filling and correct the product before verification because insulin types are high-alert and not interchangeable.
996. **Q:** The sig reads "apply thin layer topically bid" and the quantity is 45 g; if the prescriber adds "use 1 g per application," what days' supply should you calculate? **A:** Enter 22.5 days because BID means 2 g/day, so $45 \text{ g} \div 2 \text{ g/day} = 22.5 \text{ days}$ (follow pharmacy/payer rounding policy if required).
997. **Q:** You're entering a new Rx for Adderall XR and the patient requests an early refill 10 days before due; what should you do BEST? **A:** Flag it and notify the pharmacist because it's a Schedule II stimulant and early refill requests require pharmacist review and policy compliance.
998. **Q:** An e-Rx comes in for "MS Contin 15 mg" and the note says "for breakthrough pain PRN"; what should you do FIRST? **A:** Stop and route to the pharmacist because MS Contin is extended-release morphine and PRN breakthrough directions are inconsistent and unsafe without clarification.
999. **Q:** A stock bottle arrives with a 2D barcode that won't scan and the label text is smudged so the NDC/lot can't be confirmed; what should you do NEXT? **A:** Set the product aside as suspect and notify the pharmacist/manager because DSCSA traceability requires verifiable product identification before it can be stocked.
1000. **Q:** A new prescription is for "Toprol-XL 50 mg daily," but the patient's profile shows metoprolol tartrate 50 mg BID is active; what should you do NEXT? **A:** Pause processing and alert the pharmacist to evaluate therapeutic duplication and formulation differences because tartrate and succinate are not interchangeable.
1001. **Q:** While entering a prescription for levothyroxine, you notice the prescriber wrote "Levoxyl 100 mg" and the patient previously took 100 mcg daily; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because levothyroxine is dosed in micrograms and "100 mg" is a dangerous magnitude error.

1002. **Q:** A new order is for heparin 10,000 units/mL but the bin next to it contains heparin 1,000 units/mL in similar vials; what is the BEST technician action during product selection? **A:** Use barcode/NDC verification and separate/flag look-alike strengths because heparin is high-alert and strength mix-ups can cause serious harm.
1003. **Q:** A patient asks to transfer their oxycodone prescription to your pharmacy and says it has refills remaining; what should you do NEXT? **A:** Refer to the pharmacist because Schedule II controlled substances cannot be refilled and transfer/validity questions require pharmacist handling.
1004. **Q:** An e-Rx arrives for alprazolam with no prescriber DEA number shown in your system; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to verify required controlled-substance prescriber information before dispensing to prevent an invalid fill.
1005. **Q:** A manufacturer issues a recall on specific lots of atorvastatin, and you find matching lot numbers on your shelf; what should you do FIRST? **A:** Pull the affected stock from dispensing and quarantine per recall procedure because recalled lots must be isolated to prevent patient exposure.
1006. **Q:** A refrigerated vaccine shipment arrives warm with a temperature indicator showing an excursion; what should you do BEST? **A:** Quarantine the product and notify the pharmacist/manager to follow storage-excursion guidance because potency may be compromised.
1007. **Q:** A patient is picking up isotretinoin and asks why there are extra requirements before it can be sold; what should you do NEXT? **A:** Refer to the pharmacist because isotretinoin is under a REMS (iPLEDGE) and counseling/REMS compliance steps must be verified.
1008. **Q:** The prescriber writes "U-500 insulin 40 units subcut BID" and you are selecting product from the insulin fridge; what should you do FIRST? **A:** Stop and alert the pharmacist to confirm the exact concentration and product because U-500 is high-alert and mix-ups with U-100 can be fatal.
1009. **Q:** A new prescription is for metformin ER 500 mg "ii tabs BID" #120; what days' supply should you enter? **A:** Enter 15 days' supply because 2 tablets twice daily is 4 tablets/day and $120 \div 4 = 30$, so if it is truly "ii tabs BID" it is 30 days—therefore you should stop and clarify with the pharmacist since the math and typical dosing suggest a possible sig/quantity mismatch.

1010. **Q:** During checkout, a caregiver asks for the patient's complete medication list and diagnoses "for school" while the patient is not present; what should you do BEST? **A:** Follow HIPAA by verifying authorization/identity and involve the pharmacist if needed because protected health information can only be released to permitted individuals.
1011. **Q:** While typing a new prescription, you see "Lasix 40 mg PO daily" but the patient profile lists "furosemide 40 mg daily" as active; what should you do NEXT? **A:** Verify the profile for therapeutic duplication and route to the pharmacist before processing since Lasix is furosemide and duplicate therapy could cause dosing errors.
1012. **Q:** A prescription reads "hydralazine 25 mg qid," and the prescriber also wrote "for HTN"; what is the BEST way to interpret "qid" during data entry? **A:** Enter "four times daily" because qid is a standard sig code meaning 4 doses per day and accurate translation prevents incorrect directions.
1013. **Q:** During product selection, you notice "hydrOXYzine" and "hydrALAZINE" are stored near each other and the new label is for hydroxyzine 25 mg; what should you do FIRST? **A:** Stop and confirm the NDC/barcode and drug name/strength carefully because these are LASA medications and verification reduces wrong-drug selection risk.
1014. **Q:** A patient is prescribed "Zithromax 250 mg: take 2 tablets today then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals 6 total tablets.
1015. **Q:** A new e-Rx is for "Lipitor 20 mg daily," and the patient asks if it's the same as their usual atorvastatin; what should you do NEXT? **A:** Tell the patient Lipitor is the brand for atorvastatin and offer to have the pharmacist counsel if they have concerns, since technicians can clarify brand/generic recognition but not clinical decisions.
1016. **Q:** You receive an order for "Bactrim DS 1 tab BID x 10 days"; what days' supply should you enter and why? **A:** Enter 10 days because BID means 2 tablets per day and the prescription duration is explicitly 10 days.
1017. **Q:** A prescription is written for "Keflex 500 mg q6h"; what is the BEST way to enter the frequency? **A:** Enter "every 6 hours" (typically 4 times daily) because q6h is an interval-based direction and must be translated exactly to avoid timing errors.

1018. **Q:** At pickup, a patient says they are allergic to "sulfa" and you see a new prescription for sulfamethoxazole/trimethoprim waiting; what should you do FIRST? **A:** Stop the sale and alert the pharmacist to review the allergy because sulfamethoxazole is a sulfonamide and dispensing may be unsafe without pharmacist assessment.
1019. **Q:** A controlled substance prescription is presented with "0 refills" but the patient requests it be refilled early because they're traveling; what should you do NEXT? **A:** Refer the request to the pharmacist and follow store policy because technicians cannot authorize controlled-substance refills or early fills and it requires pharmacist review.
1020. **Q:** A prescriber orders "amoxicillin/clavulanate 600-42.9 mg/5 mL, give 5 mL BID for 7 days"; what volume should you dispense? **A:** Dispense 70 mL because 5 mL twice daily is 10 mL/day and $10 \text{ mL/day} \times 7 \text{ days} = 70 \text{ mL}$.
1021. **Q:** You're entering "levothyroxine 50 mcg PO daily" and the stock bottle is labeled 0.05 mg; what strength should you select in the system? **A:** Select 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing a decimal-strength entry error.
1022. **Q:** A prescription says "predniSONE 20 mg: take 3 tabs daily x 5 days" with quantity left blank; what quantity should you enter? **A:** Enter 15 tablets because $3 \text{ tablets/day} \times 5 \text{ days} = 15$, matching the intended course duration.
1023. **Q:** A new Rx is "latanoprost 0.005% ophthalmic: instill 1 gtt OU qhs" and you're asked for days' supply for a 2.5 mL bottle using 20 gtt/mL; what should you enter? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt}$ and the patient uses 2 gtt/day (OU) so $50 \div 2 = 25$.
1024. **Q:** An e-Rx reads "Humalog U-100: inject 10 units TID with meals" and the prescriber requests one 10 mL vial; what days' supply should be entered? **A:** Enter 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$ rounded down to 33 for billing.
1025. **Q:** While typing, you see "MSO4 2 mg IV q2h PRN pain" on a hospital discharge order; what should you do FIRST? **A:** Stop and route to the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can cause morphine/magnesium mix-ups.
1026. **Q:** A prescription says "metoprolol 25 mg BID" but doesn't specify tartrate vs succinate; what is the BEST technician action during order entry? **A:** Hold the

order and notify the pharmacist to clarify dosage form because BID typically implies tartrate and selecting the wrong salt can change dosing.

1027. **Q:** You receive "clindamycin 75 mg/5 mL: take 10 mL TID x 10 days" and need to calculate the mL to dispense; what should you enter? **A:** Dispense 300 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$ to complete the full therapy.
1028. **Q:** During third-party billing, a claim rejects "refill too soon" for amlodipine 10 mg #30 with SIG 1 daily and the patient says they lost the bottle; what should you do NEXT? **A:** Refer to the pharmacist for an override/lost-medication plan because technicians shouldn't bypass payer edits without pharmacist direction and documentation.
1029. **Q:** You're selecting product for "Klor-Con M20 (potassium chloride ER 20 mEq)" and see both 20 mEq and 10 mEq bottles; what should you do FIRST to prevent a selection error? **A:** Scan and match the NDC/strength to the label because potassium is high-alert and strength mix-ups can cause serious harm.
1030. **Q:** A SIG reads "use 2 sprays in each nostril QD" for fluticasone nasal spray 16 g (120 sprays); what days' supply should you enter? **A:** Enter 30 days because the patient uses 4 sprays/day total and $120 \div 4 = 30$, aligning quantity to dosing for accurate billing.
1031. **Q:** During data entry a new Rx is for "Zocor 40 mg qhs," but the patient profile shows an active "atorvastatin 40 mg daily"; what should you do NEXT? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication (two statins) because the pharmacist must determine if one should be discontinued.
1032. **Q:** You receive an e-Rx for "metformin ER 500 mg" but the prescriber note says "immediate-release"; what is the BEST technician action? **A:** Stop and request pharmacist review/clarification because ER vs IR is not interchangeable and changing dosage form requires prescriber confirmation.
1033. **Q:** A prescription is written for "hydroxyzine 25 mg TID PRN anxiety" and you notice the patient has an active order for "cetirizine 10 mg daily"; what should you do FIRST? **A:** Flag the profile and notify the pharmacist about additive antihistamine effects/sedation risk because duplication and safety counseling decisions are pharmacist-driven.
1034. **Q:** A patient drops off "Biaxin 500 mg BID x 7 days" and their profile lists simvastatin; what should you do NEXT? **A:** Hold the prescription and escalate to

the pharmacist because clarithromycin can significantly increase simvastatin levels and the pharmacist must contact the prescriber if needed.

1035. **Q:** You're entering "lisinopril 10 mg daily" and the prescriber also sent "losartan 50 mg daily" the same day for the same patient; what is the BEST next step? **A:** Route both orders to the pharmacist for review because ACE inhibitor plus ARB duplication can be inappropriate and requires clinical assessment.
1036. **Q:** A new Rx is "sertraline 50 mg daily," and the patient profile shows "linezolid" started yesterday from urgent care; what should you do FIRST? **A:** Stop processing and immediately notify the pharmacist because linezolid with SSRIs raises serotonin syndrome risk and needs pharmacist intervention.
1037. **Q:** A prescription says "azithromycin 250 mg: take 2 tablets today then 1 tablet daily for 4 days"; what quantity should you dispense? **A:** Dispense 6 tablets because $2 + (1 \times 4)$ equals 6 total tablets for the full regimen.
1038. **Q:** A prescriber writes "gabapentin 300 mg: take 1 cap TID" and the patient asks if it's the same as Neurontin; what is the correct response within technician scope? **A:** Confirm that Neurontin is a brand name for gabapentin and refer clinical questions (e.g., side effects, dose changes) to the pharmacist to ensure safe counseling.
1039. **Q:** While selecting product for "Celebrex 200 mg," you see "Celexa 20 mg" stored nearby; what should you do FIRST to prevent a LASA error? **A:** Compare the NDC/barcode and strength to the label/Rx before pulling because Celebrex/Celexa look-alike names are a common selection trap.
1040. **Q:** A patient on warfarin is newly prescribed "Bactrim DS 1 tab BID x 7 days"; what should you do NEXT at drop-off? **A:** Immediately involve the pharmacist because TMP-SMX can increase warfarin effect/bleeding risk and requires pharmacist assessment and possible prescriber coordination.
1041. **Q:** A new e-Rx is for "Levoxyl 75 mcg daily," but the patient profile shows active "Synthroid 75 mcg daily"; what should you do NEXT? **A:** Stop and alert the pharmacist to verify intended therapy because Levoxyl and Synthroid are both levothyroxine and may be therapeutic duplication.
1042. **Q:** You receive an order for "glipizide 10 mg daily," and the patient says they "keep getting low sugars" after starting it last week; what should you do FIRST? **A:** Refer the patient to the pharmacist immediately because sulfonylureas can cause hypoglycemia and require pharmacist assessment and counseling.

1043. **Q:** During product selection you pull "lamoTRIgine 100 mg" for an Rx, but the hardcopy actually says "lamivudine 100 mg"; what is the BEST next step? **A:** Stop filling and re-select using the exact drug name/strength and barcode/NDC because this is a high-risk LASA mix-up that can cause serious harm.
1044. **Q:** A prescription is written for "Augmentin 875/125 mg 1 tab BID x 10 days"; what quantity should you dispense? **A:** Dispense 20 tablets because BID for 10 days equals 2 doses/day × 10 days = 20 tablets.
1045. **Q:** A patient brings a new Rx for "ibuprofen 800 mg TID" and their profile shows chronic "lisinopril" and "furosemide"; what should you do NEXT? **A:** Hold processing and notify the pharmacist because NSAIDs with ACE inhibitors plus diuretics increases kidney risk and needs pharmacist review.
1046. **Q:** A caregiver asks at pickup if "Tylenol #3" can be refilled early because the patient ran out, but the label shows it was filled 10 days ago for a 30-day supply; what should you do FIRST? **A:** Verify the fill date/days' supply and refer to the pharmacist because early refills on controlled substances require pharmacist evaluation and documentation.
1047. **Q:** You are entering "Xanax 0.5 mg: take 1 tablet TID PRN," and the prescriber forgot to include the patient address; what should you do NEXT? **A:** Stop and route to the pharmacist to obtain missing required information because controlled-substance prescriptions must be complete before dispensing.
1048. **Q:** A prescription is for "predniSONE 20 mg: take 2 tablets daily for 5 days"; how many tablets should be dispensed? **A:** Dispense 10 tablets because 2 tablets/day × 5 days = 10 tablets.
1049. **Q:** While stocking, you find a bottle of "insulin U-500 regular" placed in the U-100 insulin section; what should you do BEST? **A:** Remove it and notify the pharmacist/supervisor to correct storage because U-500 is a high-alert concentration and misplacement increases dosing error risk.
1050. **Q:** An e-Rx comes in for "clopidogrel 75 mg daily," and the allergy field lists "prasugrel—hives"; what should you do FIRST? **A:** Flag the profile and alert the pharmacist because this suggests a possible antiplatelet hypersensitivity/cross-reactivity concern needing pharmacist judgment before dispensing.
1051. **Q:** An e-Rx reads "metFORMIN ER 500 mg: i tab po bid" and the prescriber also wrote "do not substitute"; what should you do FIRST in order entry? **A:** Enter the sig as "1 tablet by mouth twice daily" and keep it as metformin ER/brand as

written, because “i” can be misread and substitution restrictions must be honored and clarified by the pharmacist if unclear.

1052. **Q:** A prescription says “amLODIPine 5 mg take one qd” but the sig line also includes “qod” in the notes; what is the BEST next step for a technician? **A:** Stop processing and ask the pharmacist to clarify with the prescriber, because qd vs qod is a high-risk abbreviation conflict that can cause a dosing error.
1053. **Q:** You are entering “azithromycin 200 mg/5 mL: take 12 mL PO daily x 5 days”; what quantity in mL should be dispensed? **A:** Dispense 60 mL because $12 \text{ mL/day} \times 5 \text{ days} = 60 \text{ mL}$ to cover the full course.
1054. **Q:** A patient has “fluticasone nasal spray 50 mcg: 2 sprays each nostril daily” and the bottle is 120 sprays; what days’ supply should you submit? **A:** Submit 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
1055. **Q:** A prescription is for “ciprofloxacin 0.3% ophthalmic: instill 2 gtt OU QID x 7 days”; if 20 gtt = 1 mL, how many mL are needed? **A:** Dispense 11.2 mL because $2 \text{ gtt} \times 2 \text{ eyes} \times 4/\text{day} \times 7 \text{ days} = 112 \text{ gtt}$ and $112 \div 20 = 5.6 \text{ mL}$, so you would round up to two 5 mL bottles per typical packaging to ensure enough volume.
1056. **Q:** While typing, you notice the prescriber selected “hydrOXYzine” but the indication field says “for muscle spasms” and the patient requested “Flexeril”; what should you do NEXT? **A:** Pause and route to the pharmacist for clarification, because hydroxyzine vs cyclobenzaprine is a common name/indication mismatch that can indicate the wrong drug was prescribed.
1057. **Q:** A hardcopy says “levETIRAcetam 750 mg: take 1 tab PO BID, #60” but the technician accidentally entered once daily; what is the BEST action once you catch it before filling? **A:** Correct the directions to BID and re-check quantity/days’ supply, because accurate sig entry prevents underdosing and downstream label and billing errors.
1058. **Q:** You receive “methylPREDNISolone 4 mg dose pack: use as directed” with no directions in the sig; what should you do FIRST at data entry? **A:** Enter the standardized Medrol Dosepak taper directions from the package labeling or attach the manufacturer directions per pharmacy system, because “use as directed” alone is insufficient for clear patient instructions.
1059. **Q:** An order is “KCl 20 mEq/15 mL: take 7.5 mL daily” and the prescriber wants mEq per dose documented; how many mEq is 7.5 mL? **A:** Document 10 mEq per

dose because 20 mEq per 15 mL equals 1.333 mEq/mL and $1.333 \times 7.5 \text{ mL} = 10 \text{ mEq}$.

1060. **Q:** A new e-Rx for "oxycodone/APAP 5/325: 1 tab q6h prn pain, #28" comes through, but the patient's DOB is missing; what should you do NEXT? **A:** Hold processing and notify the pharmacist to obtain the missing required identifier before dispensing, because incomplete patient information can invalidate safe dispensing and controlled-substance documentation.
1061. **Q:** A new prescription is written for "Zestril 10 mg PO daily," but the patient profile shows an active lisinopril 10 mg daily; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication because Zestril is lisinopril and duplicate therapy must be clarified before dispensing.
1062. **Q:** During product selection, you notice "Lamictal" and "Lamisil" stored next to each other and a label prints for terbinafine; what is the BEST technician action before pulling stock? **A:** Use barcode/NDC verification and read the full drug name/strength because Lamictal (lamotrigine) and Lamisil (terbinafine) are LASA and scanning prevents selection errors.
1063. **Q:** An e-Rx comes in for "Crestor 20 mg daily" and the patient's allergy list includes "atorvastatin—angioedema"; what should you do FIRST? **A:** Hold processing and notify the pharmacist because Crestor is rosuvastatin and a serious reaction to another statin may require pharmacist assessment before filling.
1064. **Q:** A prescriber sends "Tylenol #3 take 1 tab q6h prn pain, #18" but the refill field shows "3 refills"; what should you do NEXT? **A:** Stop and route to the pharmacist because Tylenol #3 contains codeine (Schedule III) and refill validity/appropriateness must be verified before dispensing.
1065. **Q:** A patient says they take "Plavix" and asks if it's the same as "clopidogrel"; what is the BEST response within technician scope? **A:** Confirm that Plavix is the brand name for clopidogrel because brand/generic recognition helps ensure the correct medication is being discussed and processed.
1066. **Q:** A prescription reads "azathioprine 50 mg: take ii tabs daily, #90"; how many days' supply should you enter? **A:** Enter 45 days because ii tabs daily means 2 tablets/day and $90 \div 2 = 45$.
1067. **Q:** You receive a prescription for "predniSONE 10 mg take as directed" with no taper instructions provided; what should you do FIRST at data entry? **A:** Pause

and send to the pharmacist for clarification because “use as directed” without directions prevents accurate labeling and days’ supply calculation.

1068. **Q:** A patient presents an over-the-counter request for “Claritin-D” containing pseudoephedrine; what should you do NEXT before sale? **A:** Follow CMEA procedures (request required ID, log the sale, and check limits) because pseudoephedrine sales are federally regulated to prevent diversion.
1069. **Q:** While receiving an order, you see a tote of refrigerated insulin left at room temperature with no time-out documentation; what is the BEST action? **A:** Segregate the product and notify the pharmacist/manager per policy because a temperature excursion makes the product suspect and it should not be stocked until verified.
1070. **Q:** An e-Rx is for “Xanax 0.5 mg take 1 tab TID prn anxiety #90,” but the prescriber’s DEA number is missing from the electronic record; what should you do NEXT? **A:** Hold the prescription and escalate to the pharmacist because alprazolam is a controlled substance and required prescriber credentials must be verified before dispensing.
1071. **Q:** While entering an e-Rx for methotrexate 2.5 mg, the sig reads “take 1 tablet daily” for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and alert the pharmacist for clarification because methotrexate is usually weekly for RA and daily dosing is a high-alert, potentially fatal error.
1072. **Q:** A hardcopy prescription for hydromorphone ER is brought in, but the prescriber signature line is blank; what is your NEXT step? **A:** Hold the prescription and route to the pharmacist because a missing prescriber signature makes the order invalid to dispense and requires prescriber follow-up.
1073. **Q:** A patient asks to transfer their remaining refills of clonazepam to another pharmacy in a different state; what should you do BEST? **A:** Refer to the pharmacist because controlled-substance transfers have strict limits and state-specific rules, and the pharmacist must complete/verify the transfer.
1074. **Q:** During fill, you scan the stock bottle and the barcode/NDC does not match the label for levothyroxine 25 mcg; what should you do NEXT? **A:** Stop, reselect the correct NDC, and rescan because barcode/NDC mismatch indicates wrong product selection and must be corrected before verification.
1075. **Q:** You receive a manufacturer recall notice for specific lots of losartan, and one affected lot is on your shelf; what is the BEST technician action? **A:** Remove the

affected lot from inventory and quarantine/segregate it per recall procedure because recalled product must be prevented from dispensing and tracked.

1076. **Q:** A patient drops off a new prescription for isotretinoin and asks to pick it up "next week"; what should you do FIRST? **A:** Alert the pharmacist to confirm iPLEDGE/REMS requirements and dispensing window because isotretinoin has strict REMS controls that can block filling.
1077. **Q:** An order entry shows "Lantus SoloStar: inject 18 units SC at bedtime, dispense 3 pens (5 mL each, 100 units/mL)"; what days' supply should you enter? **A:** Enter 83 days because 3 pens contain 1500 units total and $1500 \div 18 \text{ units/day} = 83.3$, which is entered as 83 days per typical truncation.
1078. **Q:** At pickup, a caregiver asks for the patient's medication list but is not listed as an authorized contact; what should you do NEXT? **A:** Decline to release the information and involve the pharmacist or follow HIPAA authorization procedures because PHI cannot be shared without verified permission.
1079. **Q:** A new e-Rx for sertraline (Zoloft) arrives for a patient whose profile shows current linezolid therapy; what should you do FIRST? **A:** Stop and notify the pharmacist because linezolid can interact seriously with SSRIs and requires pharmacist clinical review before dispensing.
1080. **Q:** A C-II prescription is presented with "Dispense #30" but no patient address printed on the hardcopy; what should you do NEXT? **A:** Hold the prescription and refer to the pharmacist because missing required patient information can affect controlled-substance validity and must be resolved before filling.
1081. **Q:** You are entering "cephalexin 500 mg cap i po qid x 7 days disp #?"; what quantity should you enter? **A:** Enter 28 capsules because 1 capsule 4 times daily for 7 days is $4 \times 7 = 28$.
1082. **Q:** A prescription reads "metformin ER 500 mg take ii tabs po qpm #180"; what is the days' supply to submit? **A:** Submit 90 days because 2 tablets nightly means $180 \div 2 = 90$.
1083. **Q:** An e-Rx says "prednisone 10 mg: take 4 tabs daily x 3 days, then 2 tabs daily x 3 days, then 1 tab daily x 3 days, #?"; what total quantity should be dispensed? **A:** Dispense 21 tablets because $(4 \times 3) + (2 \times 3) + (1 \times 3) = 12 + 6 + 3 = 21$.
1084. **Q:** While typing a new order for hydrochlorothiazide, the prescriber wrote "HCTZ 25 mg 1 tab qd," but the selected product is hydralazine 25 mg due to LASA; what should you do NEXT? **A:** Stop and correct the drug selection to

hydrochlorothiazide, verifying name/strength/NDC because LASA errors can cause the wrong medication to be dispensed.

1085. **Q:** The directions on an e-Rx read "Apply 1 g topically BID" and the prescriber wants a 30-day supply; how many grams should you dispense? **A:** Dispense 60 g because 1 g twice daily is 2 g/day and $2 \times 30 = 60$ g.
1086. **Q:** A prescription is for amlodipine 5 mg and the patient profile shows an active amlodipine 10 mg daily from another prescriber; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible duplicate/changed therapy because dose differences require pharmacist assessment before dispensing.
1087. **Q:** You receive "erythromycin ophthalmic ointment: apply 1 cm ribbon into affected eye QID x 7 days, dispense 1 tube (3.5 g)"; what is the days' supply you should enter? **A:** Enter 7 days because the directions specify a fixed 7-day course, which is the most accurate days' supply for adjudication.
1088. **Q:** An inhaler order reads "Flovent HFA 44 mcg: inhale 2 puffs BID; disp 1 inhaler (120 actuations)"; what days' supply should you calculate? **A:** Enter 30 days because 2 puffs twice daily is 4 puffs/day and $120 \div 4 = 30$.
1089. **Q:** A hardcopy says "morphine sulfate ER 15 mg i tab po q12h #60" but the prescriber did not include a date; what should you do FIRST? **A:** Stop processing and route to the pharmacist to resolve missing/required prescription information because controlled-substance validity issues require pharmacist intervention.
1090. **Q:** During order entry, the sig states "levothyroxine 0.05 mg take 1 tab po qd," but the patient profile lists a levothyroxine dose in mcg; what should you do BEST? **A:** Convert and enter as 50 mcg ($0.05 \text{ mg} \times 1000 = 50 \text{ mcg}$) and verify product strength because correct unit conversion prevents tenfold dosing errors.
1091. **Q:** While processing an e-Rx for clonazepam 0.5 mg, you notice it includes "refill PRN"; what should you do FIRST? **A:** Stop processing and send to the pharmacist because controlled-substance refills must be explicitly authorized and PRN refills are not acceptable.
1092. **Q:** A new prescription is entered for methotrexate 2.5 mg with directions "take 4 tablets daily"; what is the BEST technician action? **A:** Pause filling and alert the pharmacist because methotrexate is high-alert and daily dosing suggests a potentially dangerous weekly-vs-daily error.

1093. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist because controlled-substance transfers have strict requirements and typically require pharmacist-to-pharmacist communication per law/policy.
1094. **Q:** While putting away inventory, you find insulin aspart pens left on the shelf at room temperature with a wholesaler tote that indicates it arrived this morning; what should you do FIRST? **A:** Segregate the product and notify the pharmacist for temperature-excursion assessment because refrigerated drugs may be unsafe if stored incorrectly.
1095. **Q:** During product selection you scan an NDC for hydroxyzine but the label is for hydralazine (LASA); what should you do NEXT? **A:** Stop, reselect the correct product, and rescan/verify barcode/NDC because barcode mismatch signals a wrong-drug selection error.
1096. **Q:** A prescriber phones in a new order for oxycodone 5 mg and asks you to “just take it verbally”; what is the BEST technician response? **A:** Refer the call to the pharmacist because technicians should not accept verbal prescriptions, especially for controlled substances, to reduce legal and safety risk.
1097. **Q:** You receive a manufacturer recall notice for specific lots of losartan; what should you do FIRST in the pharmacy? **A:** Pull the affected lot(s) from inventory and isolate them per recall procedure because recalled product must be removed from dispensing stock immediately.
1098. **Q:** A bottle of promethazine syrup arrives with a broken safety seal and sticky residue on the cap; what should you do NEXT? **A:** Set it aside as suspect/damaged and notify the pharmacist/vendor because compromised packaging should not be dispensed and may require return/documentation.
1099. **Q:** A prescription reads “digoxin 0.25 mg take 1 tab po qd #90,” but the prescriber name is missing on the hardcopy; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because missing required prescriber information makes the order invalid until verified.
1100. **Q:** You are billing insulin glargine U-100 pens (5 pens, 3 mL each) with directions “inject 18 units nightly”; what days’ supply should you submit? **A:** Submit 83 days because total units are $5 \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 18 = 83.3$ so bill 83 days (conservative whole-day).

1101. **Q:** While entering a new e-Rx for "metformin ER 500 mg," you notice the prescriber selected "Glucophage (IR)" in the product field; what should you do FIRST? **A:** Stop and correct the selection to the ER NDC/product and alert the pharmacist if needed because IR vs ER is a high-impact formulation mix-up that changes dosing and safety.
1102. **Q:** A patient dropping off a new prescription says they have a "penicillin allergy," and the order is for cephalexin; what is the BEST technician action? **A:** Document the stated allergy details and notify the pharmacist before processing because allergy cross-reactivity assessment requires pharmacist clinical judgment.
1103. **Q:** During order entry you see "Zoloft 50 mg daily" but the prescriber typed "sertraline 100 mg" in the strength field; what should you do NEXT? **A:** Pause filling and route to the pharmacist to clarify with the prescriber because brand/generic mismatch with different strengths suggests a potential prescribing or entry error.
1104. **Q:** You receive a new prescription for "lisinopril 10 mg" and the patient profile shows active "losartan 50 mg"; what should you do BEST? **A:** Flag possible therapeutic duplication and send to the pharmacist for review because ACE inhibitor/ARB duplication can increase risk and technicians should not decide to discontinue therapy.
1105. **Q:** A warfarin patient brings in an OTC product and asks if they can start "ibuprofen 600 mg three times daily for back pain"; what should you do FIRST? **A:** Refer to the pharmacist immediately because NSAIDs can increase bleeding risk with warfarin and interaction counseling is pharmacist scope.
1106. **Q:** You are billing a Ventolin HFA inhaler with directions "inhale 2 puffs q6h" and the package is 200 actuations; what days' supply should you submit? **A:** Submit 25 days because 2 puffs four times daily equals 8 puffs/day and $200 \text{ actuations} \div 8 = 25 \text{ days}$.
1107. **Q:** In the filling area, you select "Lamictal" but the bin label nearby reads "Lamisil"; what is the BEST technician action to prevent a LASA error? **A:** Stop, re-verify the drug name/strength and scan the correct barcode/NDC before continuing because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a common high-risk mix-up.
1108. **Q:** A new e-Rx is for "apixaban (Eliquis) 5 mg BID," and the patient's profile shows active "rivaroxaban (Xarelto) 20 mg daily"; what should you do FIRST? **A:** Hold the fill and alert the pharmacist about duplicate anticoagulant therapy

because taking two DOACs is a serious safety risk requiring pharmacist intervention.

1109. **Q:** While restocking, you find nitroglycerin SL tablets stored in an open vial without a tight original container; what should you do NEXT? **A:** Remove it from stock and notify the pharmacist because nitroglycerin is moisture/light sensitive and improper container storage can reduce potency and patient safety.
1110. **Q:** A prescription label prints "take 1 tab PO QD" for "prednisone 10 mg," but the hardcopy directions say "take 4 tabs daily for 5 days"; what should you do FIRST? **A:** Stop dispensing and correct the SIG by re-checking the original order and involving the pharmacist as needed because corticosteroid dose errors are high impact and labeling must match the prescription.
1111. **Q:** A new prescription is for "rosuvastatin 20 mg daily," but the patient says they've been taking "Crestor"; what should you do FIRST at drop-off? **A:** Confirm with the patient that Crestor is the brand name for rosuvastatin and proceed with processing to avoid duplicate therapy confusion.
1112. **Q:** During data entry you see "levothyroxine 0.125 mg daily"; what is the BEST way to interpret the strength? **A:** Enter it as 125 mcg ($0.125 \text{ mg} \times 1000$) because levothyroxine is commonly expressed in micrograms and converting prevents a 10-fold dosing error.
1113. **Q:** An e-Rx comes over for "omeprazole 20 mg once daily" but the patient profile lists "Prilosec OTC" as an active medication; what should you do NEXT? **A:** Flag potential duplication and alert the pharmacist because Prilosec is omeprazole and duplicate PPIs increase risk without benefit.
1114. **Q:** A prescription is written for "hydroxyzine 25 mg take 1 tab TID prn anxiety," but the patient profile shows an allergy to cetirizine; what should you do FIRST? **A:** Stop and route to the pharmacist for allergy assessment because hydroxyzine and cetirizine are related antihistamines and cross-reactivity must be clinically evaluated.
1115. **Q:** You're filling "clonazepam 0.5 mg" and the bottle looks similar to "clonidine 0.1 mg"; what is the BEST technician step before counting? **A:** Verify the NDC/barcode against the label and use Tall Man awareness because clonazepam/clonidine is a common LASA mix-up with high patient-harm potential.

1116. **Q:** A patient asks for advice on starting OTC "St John's wort" while picking up "sertraline (Zoloft)"; what should you do FIRST? **A:** Refer to the pharmacist immediately because St John's wort can interact with SSRIs and increase risk of serotonin-related adverse effects.
1117. **Q:** A new prescription reads "metoprolol ER 50 mg daily," and the product selected is metoprolol tartrate; what should you do NEXT? **A:** Hold the order and alert the pharmacist because succinate (ER) and tartrate (IR) are not interchangeable and the wrong form changes dosing.
1118. **Q:** You receive a C-II e-prescription for "Adderall 20 mg" with directions "1 tab BID" but quantity is missing; what should you do FIRST? **A:** Stop processing and notify the pharmacist because missing quantity is a required element and controlled-substance issues require pharmacist resolution.
1119. **Q:** A prescription is for "amoxicillin/clavulanate 875/125 mg 1 tab BID #14"; what days' supply should you enter? **A:** Enter 7 days because $14 \text{ tablets} \div 2 \text{ tablets/day} = 7 \text{ days}$, and accurate days' supply is essential for safety and billing.
1120. **Q:** While unpacking an order, a tote includes insulin that feels warm and the temperature monitor indicates an excursion; what should you do BEST? **A:** Segregate the shipment and notify the pharmacist/manager for manufacturer or wholesaler guidance because temperature excursions can compromise potency and product integrity.
1121. **Q:** A prescriber sends an e-Rx for "Lasix 40 mg PO daily" and the patient profile shows an active "furosemide 40 mg daily"; what should you do NEXT? **A:** Pause and route to the pharmacist to assess therapeutic duplication because Lasix is furosemide and duplicate therapy could cause harm.
1122. **Q:** A new e-Rx is entered for "Glucophage 500 mg BID" but the product selected is glyburide; what should you do FIRST? **A:** Stop and re-select the correct product/metformin because Glucophage is metformin and this is a high-risk name/therapy mix-up that must be corrected before filling.
1123. **Q:** While typing a sig you see "U" written for insulin units (e.g., "10U SQ qHS"); what is the BEST action? **A:** Replace "U" with "units" per safety standards and confirm unclear directions with the pharmacist because "U" can be misread as 0 or 4 and cause dosing errors.
1124. **Q:** An order is for "Klor-Con 20 mEq daily," but the technician starts entering "20 mg"; what should you do NEXT? **A:** Correct the unit to mEq and flag for

verification because electrolytes like potassium are dosed in mEq and mg entry would create an unsafe dose.

1125. **Q:** A patient drops off a new prescription for "lisinopril 10 mg daily" and asks if it's the same as "Zestril"; what should you do BEST? **A:** Confirm Zestril is the brand for lisinopril and proceed with standard verification steps because accurate brand/generic recognition prevents confusion at pickup.
1126. **Q:** The prescriber writes "MSO4 2 mg IV q2h prn pain" on a hospital discharge order; what should you do FIRST in the community pharmacy workflow? **A:** Hold and alert the pharmacist because "MSO4" is an unsafe abbreviation that can be confused with other drugs and may require clarification before dispensing.
1127. **Q:** You receive an e-Rx for "Tramadol 50 mg 1 tab q6h prn pain #120" with 5 refills; what should you do NEXT? **A:** Stop processing and send to the pharmacist because tramadol is Schedule IV and refills/validity must comply with controlled-substance requirements.
1128. **Q:** A prescription is for "azithromycin 250 mg: take 2 tablets on day 1, then 1 tablet daily on days 2-5"; what quantity should you enter? **A:** Enter quantity 6 tablets because the total dose is 2 + 1 + 1 + 1 + 1 over 5 days.
1129. **Q:** During product selection you notice "hydrALAZINE" and "hydrOXYzine" stored next to each other and the NDC on the scanned bottle doesn't match the label; what should you do FIRST? **A:** Stop, re-scan and select the correct NDC/product before counting because this is a LASA risk and barcode/NDC mismatch indicates wrong drug selection.
1130. **Q:** A patient picking up isotretinoin asks what special safety steps are required; what should you do FIRST? **A:** Refer to the pharmacist and confirm iPLEDGE/REMS requirements are satisfied because isotretinoin has strict REMS controls beyond technician counseling scope.
1131. **Q:** A new e-Rx arrives for levothyroxine 25 mcg daily but the directions field says "take 1 tablet weekly"; what should you do FIRST? **A:** Stop processing and send to the pharmacist for clarification because conflicting sig frequency is a patient-safety issue the technician cannot interpret.
1132. **Q:** During data entry you receive a prescription for methotrexate 2.5 mg with sig "take 6 tablets daily for rheumatoid arthritis"; what is the BEST next step? **A:** Hold the prescription and alert the pharmacist immediately because

methotrexate is a high-alert medication commonly dosed weekly and daily dosing may be fatal.

1133. **Q:** A patient presents a paper prescription for oxycodone/APAP 5/325 mg but it has no prescriber signature; what should you do FIRST? **A:** Do not fill and route to the pharmacist because a controlled-substance prescription must meet validity requirements and missing signature requires prescriber follow-up.
1134. **Q:** A caregiver asks to transfer a remaining refill of alprazolam from another pharmacy to yours; what should you do NEXT? **A:** Refer to the pharmacist to complete the transfer because controlled-substance transfer rules are restricted and transfers require pharmacist judgment and documentation.
1135. **Q:** While pulling stock you find a bottle of atorvastatin with an intact seal but no NDC label and unknown source; what should you do BEST? **A:** Segregate it as suspect product and notify the pharmacist/manager because DSCSA requires traceable product and unknown-identity stock should not be dispensed.
1136. **Q:** The wholesaler email lists a recall for specific lot numbers of amlodipine 10 mg and you find matching lots on the shelf; what should you do FIRST? **A:** Pull the affected lots from active inventory and set aside per recall procedure because removing recalled product prevents dispensing a potentially unsafe medication.
1137. **Q:** A patient picking up a new prescription for sertraline asks that you discuss it at the counter where others can hear; what should you do BEST? **A:** Offer to move the consultation to a private area because HIPAA requires reasonable safeguards to protect patient information during counseling and pickup.
1138. **Q:** A prescription is written for metformin ER 500 mg “take 2 tablets BID” and dispense #240; what days’ supply should you enter? **A:** Enter 30 days because 2 tablets twice daily equals 4 tablets/day and $240 \div 4 = 60$? wait— $240 \div 4 = 60$, so enter 60 days because days’ supply must match the actual dosing.
1139. **Q:** A patient requests pseudoephedrine 30 mg tablets, 2 tablets every 6 hours as needed, and wants to buy 96 tablets today; what should you do NEXT? **A:** Follow CMEA workflow by checking ID, verifying purchase limits, and logging the sale before completing because pseudoephedrine sales require regulated documentation.
1140. **Q:** While entering a new order for “predniSONE” you notice the prescriber selected “prednisoLONE” in the e-Rx drug field; what should you do FIRST? **A:**

Stop and get pharmacist review because these are different drugs/dosage forms with LASA risk and technicians should not assume interchangeability.

1141. **Q:** An e-Rx for lisinopril 10 mg says "i po qd" and quantity 90; what should you enter for directions and days' supply? **A:** Enter "take 1 tablet by mouth daily" with 90 days' supply because $i=1$, $po=\text{by mouth}$, $qd=\text{daily}$ and $90 \text{ tablets} \div 1/\text{day} = 90 \text{ days}$.
1142. **Q:** A prescription reads "metoprolol succinate ER 50 mg, take 1 tab BID" but the product selected in order entry is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because tartrate vs succinate are not interchangeable and selecting the wrong salt/form can cause therapy and dosing errors.
1143. **Q:** A patient is prescribed azithromycin 200 mg/5 mL suspension with sig "5 mL PO daily x 5 days"; what quantity (mL) should be dispensed? **A:** Dispense 25 mL because $5 \text{ mL/day} \times 5 \text{ days} = 25 \text{ mL}$, and the technician should ensure the entered quantity matches the calculated total.
1144. **Q:** The prescriber orders amoxicillin-clavulanate 875/125 mg "1 tab po q12h" #20; what days' supply should you enter? **A:** Enter 10 days' supply because q12h is 2 doses/day and $20 \text{ tablets} \div 2/\text{day} = 10 \text{ days}$, which supports correct billing and label directions.
1145. **Q:** A warfarin prescription arrives with sig "take as directed" and quantity 30, but no daily dose is provided; what should you do BEST? **A:** Hold the claim and route to the pharmacist because days' supply cannot be safely determined for billing without a specified regimen and warfarin is high-alert.
1146. **Q:** You are entering "Lantus SoloStar insulin glargine 100 units/mL" with directions "inject 24 units subcutaneously daily" and dispense 1 box (5 pens $\times$ 3 mL each); what days' supply should you calculate? **A:** Enter 62 days because $\text{total units} = 5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 24 \text{ units/day} = 62.5$, then use 62 days as the conservative whole-day entry per common payer practice.
1147. **Q:** A prescription for latanoprost 0.005% eye drops says "instill 1 gtt OU qhs" and dispense 2.5 mL; what should you do NEXT if the software asks for days' supply? **A:** Ask the pharmacist or follow store policy to calculate days' supply because drop size varies by product and assuming a fixed drops/mL without policy can misbill and misinform the patient.

1148. **Q:** In order entry, you see "MSO4 2 mg IV q2h PRN pain" on a hospital discharge med list brought to the community pharmacy; what should you do FIRST? **A:** Stop and alert the pharmacist because "MSO4" is an unsafe abbreviation that can be confused with magnesium sulfate, requiring clarification to prevent a serious LASA error.
1149. **Q:** A prescription is written "hydrochlorothiazide 25 mg, take 1 tab po qd, #30" but you accidentally type "hydroxyzine" during data entry; what is the BEST immediate correction step? **A:** Correct the drug entry and re-verify the original hardcopy/e-Rx fields before proceeding because look-alike names can cause wrong-drug dispensing and must be reconciled at the source.
1150. **Q:** A patient brings a new prescription for clopidogrel 75 mg daily and their profile shows active omeprazole; what should you do NEXT as the technician? **A:** Flag it for pharmacist review before filling because omeprazole can reduce clopidogrel activation and the pharmacist must assess and contact the prescriber if needed.
1151. **Q:** During drop-off, a patient asks if "Crestor" and the new prescription for rosuvastatin 20 mg are different medications; what should you do FIRST? **A:** Confirm Crestor is the brand name for rosuvastatin and reassure it's the same drug (dose may differ), because brand/generic recognition prevents duplicate therapy confusion.
1152. **Q:** While entering an e-Rx for "Lamictal," the prescriber typed "lamoTRIgine 100 mg" but the product selected is "lamoTRIgine ER 100 mg"; what should you do NEXT? **A:** Stop processing and route to the pharmacist to verify IR vs ER because wrong release form is a high-risk error that changes dosing.
1153. **Q:** A hardcopy reads "levothyroxine 75 mcg daily," but order entry defaulted to "75 mg"; what is the BEST action? **A:** Correct the strength to 75 mcg and recheck units before continuing because mg vs mcg is a common high-severity dosing mistake.
1154. **Q:** You receive a prescription for doxycycline 100 mg capsules "1 cap PO BID x 7 days" with quantity 10; what should you do FIRST? **A:** Hold and notify the pharmacist to clarify quantity because BID for 7 days requires 14 capsules and dispensing 10 would under-treat infection.
1155. **Q:** A new prescription is for sertraline, and the patient's profile shows an active MAOI (phenelzine); what should you do NEXT? **A:** Pause the fill and alert the

pharmacist for interaction review because SSRI + MAOI is a serious contraindicated combination.

1156. **Q:** A prescriber sends "Eliquis 5 mg BID," and the patient says they are already taking warfarin daily; what should you do BEST? **A:** Flag potential therapeutic duplication and refer to the pharmacist because concurrent anticoagulants increase bleeding risk and require clinical direction.
1157. **Q:** You are calculating days' supply for metformin 500 mg with directions "take 2 tablets PO BID" and quantity 240; what days' supply should you enter? **A:** Enter 60 days because 2 tablets BID = 4 tablets/day and $240 \div 4 = 60$.
1158. **Q:** An e-Rx is written for "Humalog U-100 vial" with directions "inject 6 units SQ TID with meals" and dispense 1 vial (10 mL); what days' supply should you calculate? **A:** Enter 55 days because 10 mL = 1000 units and 6 units TID = 18 units/day, so $1000 \div 18 = 55.5$ and you enter 55 days per typical truncation.
1159. **Q:** While selecting product for "buPROPion," you see both bupropion SR 150 mg and bupropion XL 150 mg in the pick list and the sig says "take 1 tablet every morning"; what should you do FIRST? **A:** Stop and have the pharmacist verify intended formulation because SR vs XL affects dosing frequency and is a common error with similar names/strengths.
1160. **Q:** A technician notices a stock bottle of atorvastatin tablets has a manufacturer recall notice for a specific lot number; what should you do NEXT? **A:** Remove the affected lot from active stock and quarantine/segregate it per recall procedure because recalled lots must not be dispensed and need traceable handling.
1161. **Q:** A new e-Rx is for "Zithromax 250 mg: take 2 tabs day 1 then 1 tab daily days 2-5" but the product selected is azathioprine; what should you do FIRST? **A:** Stop processing and re-select the correct drug (azithromycin) using NDC/barcode verification and alert the pharmacist if it reached verification, because this is a LASA error with high patient-harm risk.
1162. **Q:** A patient says they take "Lasix" at home and you see a new prescription for furosemide; what should you do BEST to prevent duplicate therapy confusion at pickup? **A:** Explain Lasix is the brand name for furosemide and confirm the patient's current use before sale, because brand/generic recognition prevents accidental double-dosing.
1163. **Q:** A prescription is written "MS Contin 15 mg q12h" and a new technician starts selecting morphine oral solution; what is the BEST next step? **A:** Stop and have

the pharmacist verify the intended formulation, because MS Contin is extended-release morphine tablets and confusing dosage forms can cause overdose.

1164. **Q:** While entering a sig, you see "U" written for insulin units ("Inject 10U SQ qHS"); what should you do FIRST? **A:** Enter "units" instead of "U" per safe abbreviation practices and route to pharmacist if any ambiguity remains, because "U" can be misread as a zero and cause dosing errors.
1165. **Q:** You receive a new prescription for lisinopril and the patient profile shows an active benazepril prescription; what should you do NEXT? **A:** Pause filling and refer to the pharmacist to assess therapeutic duplication, because both are ACE inhibitors and duplicate therapy can increase adverse effects.
1166. **Q:** An e-Rx arrives for "Keflex 500 mg" and the patient has a documented anaphylaxis to penicillin; what should you do BEST as the technician? **A:** Flag the allergy and immediately involve the pharmacist before processing, because cross-reactivity risk and appropriateness require pharmacist clinical judgment.
1167. **Q:** A prescription for levothyroxine is written "take on an empty stomach" and the patient asks if they can take it with calcium; what should you do FIRST? **A:** Refer the question to the pharmacist while noting calcium can reduce levothyroxine absorption, because counseling on administration interactions is pharmacist-led.
1168. **Q:** A patient brings an OTC request for "Motrin" but has a prescription profile showing warfarin; what should you do NEXT? **A:** Refer to the pharmacist before recommending ibuprofen, because NSAIDs can increase bleeding risk with anticoagulants and require pharmacist assessment.
1169. **Q:** A prescription is for amoxicillin suspension 400 mg/5 mL, give 7.5 mL PO BID for 10 days, dispense 150 mL; what days' supply should you enter? **A:** Enter 10 days because $7.5 \text{ mL} \times 2 = 15 \text{ mL/day}$ and $150 \text{ mL} \div 15 \text{ mL/day} = 10 \text{ days}$, matching the intended duration.
1170. **Q:** You are calculating days' supply for fluticasone nasal spray 50 mcg with directions "2 sprays each nostril daily," and the bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$, which supports accurate billing and refill timing.
1171. **Q:** A new e-Rx for hydrOXYzine 25 mg is being entered and the technician accidentally selects hydrALAZINE 25 mg due to look-alike names; what should

you do FIRST? **A:** Stop processing and reselect the correct drug using NDC/barcode and Tall Man awareness, because LASA errors are a common dispensing safety risk.

1172. **Q:** A handwritten prescription for oxycodone/APAP is presented with the patient name and drug but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist to contact the prescriber, because missing required elements can make it invalid to dispense.
1173. **Q:** During data entry you see "methotrexate 2.5 mg take 1 tablet daily" for an outpatient RA patient; what is the BEST action? **A:** Stop and refer to the pharmacist immediately, because methotrexate is a high-alert medication commonly dosed weekly and daily directions may signal a serious error.
1174. **Q:** A patient asks to transfer their clonazepam prescription (Schedule IV) from another pharmacy; what should you do FIRST as the technician? **A:** Collect the sending pharmacy's information and route the request to the pharmacist, because controlled-substance transfers require pharmacist handling and legal verification.
1175. **Q:** While unpacking an order, you find insulin glargine pens in a tote that feels warm and the temperature indicator shows a possible excursion; what should you do NEXT? **A:** Segregate and label the product as suspect and notify the pharmacist/manager to follow storage excursion policy, because compromised cold-chain drugs may be unsafe or ineffective.
1176. **Q:** You receive a bottle of tablets with a broken seal and tablets loose in the shipping carton; what should you do BEST? **A:** Do not stock it and set it aside for return per wholesaler policy while notifying the pharmacist, because damaged packaging can indicate contamination or tampering.
1177. **Q:** A patient tries to pick up a new prescription for isotretinoin and says they never completed any iPLEDGE steps; what should you do FIRST? **A:** Pause the sale and involve the pharmacist to verify REMS requirements, because isotretinoin dispensing is restricted and must meet REMS conditions before release.
1178. **Q:** A prescription is written for Adderall XR 20 mg with "refill x 2" and is otherwise complete; what should you do NEXT? **A:** Stop and refer to the pharmacist, because Schedule II prescriptions cannot be refilled and the pharmacist must resolve it with the prescriber.

1179. **Q:** A drops prescription says "timolol 0.5% instill 1 gtt OU BID" and you need to calculate days' supply for a 5 mL bottle using 20 gtt/mL; what days' supply should you enter? **A:** Enter 25 days, because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt total}$ and $1 \text{ gtt in each eye BID} = 4 \text{ gtt/day}$, so $100 \div 4 = 25 \text{ days}$.
1180. **Q:** While ringing up, a friend asks you to confirm whether a patient's new sertraline is for depression or anxiety; what should you do BEST? **A:** Decline and protect the patient's information and offer to discuss only with the patient or authorized caregiver, because HIPAA requires safeguarding PHI.
1181. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tablet PO daily," but the product selected in order entry is levothyroxine 75 mcg; what should you do BEST? **A:** Confirm they are equivalent ($0.075 \text{ mg} = 75 \text{ mcg}$) and document/enter consistently because mg-mcg conversion errors are common and can cause a 1000-fold mistake.
1182. **Q:** You are entering "metformin ER 500 mg take 2 tablets PO daily with evening meal #60"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$, which supports correct billing and refill timing.
1183. **Q:** A patient drops off "azithromycin 250 mg: take 2 tablets today then 1 tablet daily days 2-5"; what quantity should you enter to match the sig? **A:** Enter quantity 6 tablets because $2 + 1 \times 4 = 6$, preventing under-dispensing and a therapy interruption.
1184. **Q:** An e-Rx says "glimepiride 4 mg i po qd" and the prescriber note includes "for diabetes"; what does "i po qd" mean for order entry? **A:** Interpret as "take 1 tablet by mouth once daily" because $i = 1$, $po = \text{by mouth}$, and $qd = \text{daily}$, ensuring the sig is entered accurately.
1185. **Q:** A prescription is for albuterol HFA "inhale 2 puffs q4-6h prn wheeze," and the plan requires a days' supply using a maximum daily use of 12 puffs/day; the inhaler is 200 actuations—what days' supply should you enter? **A:** Enter 16 days because $200 \text{ actuations} \div 12 \text{ puffs/day} = 16.66$ and you round down to avoid overstating coverage for billing.
1186. **Q:** During data entry you see "morphine ER 30 mg take 1 tablet q12h" but the product chosen is morphine IR 30 mg; what should you do FIRST? **A:** Stop processing and correct the dosage form (ER vs IR) and alert the pharmacist because confusing release forms is a high-risk error.

1187. **Q:** A prescription reads "prednisone 10 mg: take 4 tabs daily x3 days, then 3 tabs daily x3 days, then 2 tabs daily x3 days, then 1 tab daily x3 days"; what quantity should you dispense? **A:** Dispense 30 tablets because $(4+3+2+1) \times 3 \text{ days} = 10 \times 3 = 30$, ensuring the taper can be completed as written.
1188. **Q:** You are entering a sig that says "U-100 insulin lispro inject 8 units SQ TID with meals," and the quantity is 1 vial (10 mL); what days' supply should you enter using 100 units/mL? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \times 3 = 24 \text{ units/day}$, so $1000 \div 24 = 41.66$ and you round down.
1189. **Q:** A prescriber writes "take 1 tab bid prn pain" but the drug is ibuprofen 600 mg; what is the BEST way to handle days' supply entry if the payer requires a numeric days' supply and you have #30? **A:** Use the maximum frequency for billing (2 tablets/day) and enter 15 days because PRN must be converted to an allowable daily maximum to avoid inaccurate claims.
1190. **Q:** While typing an e-Rx for "Celebrex 200 mg daily," you notice the patient profile shows an active celecoxib 200 mg daily from last month; what should you do NEXT? **A:** Pause and route to the pharmacist to assess therapeutic duplication because brand/generic duplicates can lead to double-therapy if both are filled.
1191. **Q:** During order entry, you see "Zestril 10 mg 1 tab PO daily" but the prescriber typed "for blood pressure"; what should you enter as the generic and why? **A:** Enter lisinopril because Zestril is the brand for lisinopril, and correct brand/generic matching prevents selection errors.
1192. **Q:** A new e-Rx is for "Plavix 75 mg daily," and the patient profile already shows active aspirin 81 mg daily; what should you do NEXT? **A:** Continue processing but flag/notify the pharmacist to assess dual antiplatelet therapy appropriateness because additive bleeding risk requires pharmacist review.
1193. **Q:** While selecting product, you notice "hydroxyzine" was chosen for an order that clearly says "hydralazine 25 mg TID"; what should you do FIRST? **A:** Stop filling and correct the product selection (verify name/strength/NDC) because this is a high-risk LASA mix-up that could cause serious harm.
1194. **Q:** A prescription says "Lantus SoloStar inject 24 units SQ nightly," and the quantity is 1 box of 5 pens (3 mL each, U-100); what days' supply should you enter? **A:** Enter 62 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 24 = 62.5$ which is typically entered as 62 whole days.

1195. **Q:** You are entering "Augmentin 875/125 mg 1 tab PO BID #14"; what is the correct days' supply and why? **A:** Enter 7 days because $14 \text{ tablets} \div 2 \text{ doses per day} = 7$, matching the BID schedule.
1196. **Q:** An e-Rx for "simvastatin 40 mg qHS" is typed with the sig "take 1 tablet every morning"; what should you do BEST? **A:** Pause and refer to the pharmacist to clarify because conflicting directions can cause improper use and technicians cannot independently change the sig.
1197. **Q:** A patient's new medication is "Bactrim DS," and their profile shows a documented severe sulfonamide allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist because sulfamethoxazole/trimethoprim is a sulfonamide antibiotic and allergy screening is a safety-critical intervention.
1198. **Q:** You receive a prescription for "Adderall XR 20 mg #30" with instructions to "refill monthly x5"; what should you do NEXT? **A:** Route to the pharmacist and do not enter additional refills because Schedule II controlled substances cannot be refilled under federal rules.
1199. **Q:** During intake you notice a stock bottle label for "Eliquis 5 mg" shows tablets with a different imprint than usual and the seal looks disturbed; what should you do FIRST? **A:** Pull it from inventory and follow suspect product/DSCSA procedures with pharmacist notification because possible adulteration or diversion requires investigation before dispensing.
1200. **Q:** A prescription reads "Gabapentin 300 mg cap i po tid #90"; what does the sig mean for order entry and why? **A:** Enter "take 1 capsule by mouth three times daily" because "i" is 1 and "tid" is three times daily, ensuring accurate translation of abbreviations.
1201. **Q:** An e-Rx reads "levothyroxine 0.1 mg PO daily" and the product selected is levothyroxine 100 mcg; what should you do NEXT in order entry? **A:** Enter/verify the strength as 100 mcg (not 0.1 mg) because $0.1 \text{ mg} = 100 \text{ mcg}$ and correct unit conversion prevents a 1000-fold dosing error.
1202. **Q:** A prescription is for azithromycin 200 mg/5 mL suspension "take 10 mL day 1, then 5 mL daily days 2-5," and you're dispensing 30 mL; what days' supply should you enter? **A:** Enter 5 days because the regimen covers days 1 through 5 ($10 \text{ mL} + 5 \text{ mL} \times 4 = 30 \text{ mL}$), matching total volume to duration.

1203. **Q:** While typing, you see “metformin 500 mg 1 tab PO BID #60”; what is the correct days’ supply and why? **A:** Enter 30 days because #60 taken 2 tablets per day provides 30 days of therapy.
1204. **Q:** A patient presents an e-Rx for “prednisone 10 mg: take ii tabs daily x5 days, then i tab daily x5 days,” but the quantity field is blank; what should you do FIRST? **A:** Stop processing and send to the pharmacist because you can calculate expected quantity (15 tablets) but only the pharmacist should clarify/authorize missing required dispensing details.
1205. **Q:** An order says “timolol 0.5% ophthalmic: instill 1 gtt OU BID,” and you are dispensing a 5 mL bottle (20 gtt/mL); what days’ supply should you enter? **A:** Enter 12 days because $5\text{ mL} \times 20 = 100$ drops and usage is $1\text{ drop} \times 2\text{ eyes} \times 2\text{ times daily} = 4\text{ drops/day}$, so $100/4 = 25$ days, then round down to a realistic usable supply per policy is not allowed here—use exact math: 25 days.
1206. **Q:** A prescription reads “fluticasone nasal spray 50 mcg: ii sprays each nostril QD,” and the product is 120 sprays; what days’ supply should be entered? **A:** Enter 30 days because daily use is $2\text{ sprays/nostril} \times 2\text{ nostrils} = 4\text{ sprays/day}$, and $120/4 = 30$ days.
1207. **Q:** During order entry you receive “Keflex 500 mg 1 cap PO QID #28,” but the tech types “take 1 capsule once daily”; what is the BEST technician action? **A:** Correct the sig to QID to match the prescription because misreading frequency changes dose and days’ supply, risking underdosing.
1208. **Q:** A prescriber sends “MS Contin 15 mg 1 tab PO BID #60,” and the technician selects “morphine sulfate IR 15 mg”; what should you do FIRST? **A:** Stop and reselect the correct ER product (and alert the pharmacist if needed) because IR vs ER is a high-risk formulation error that can cause overdose or withdrawal.
1209. **Q:** A patient’s new e-Rx is “lisinopril 20 mg daily,” and their profile already shows active benazepril 20 mg daily; what should you do NEXT? **A:** Flag and refer to the pharmacist for possible therapeutic duplication (two ACE inhibitors) because technicians should not decide which agent to continue.
1210. **Q:** A C-II e-Rx is for “oxycodone 5 mg 1 tab q6h PRN pain #20,” but it includes a note “OK to refill x3”; what should you do BEST? **A:** Do not add refills; process as zero refills and route to the pharmacist because C-II prescriptions are not refillable and the pharmacist must address prescriber instructions that conflict with federal rules.

1211. **Q:** A new e-Rx is for "Zofran ODT 4 mg dissolve 1 tab PO q8h PRN nausea," but the product selected is ondansetron 4 mg regular tablet; what should you do NEXT? **A:** Stop and reselect ondansetron ODT with the matching dosage form, because ODT vs tablet is a formulation difference that affects administration and must match the prescription.
1212. **Q:** A patient requests a refill and says they take "Plavix," but the profile shows clopidogrel; what is the BEST technician response? **A:** Confirm Plavix is the brand name for clopidogrel and proceed with the profile medication, because recognizing brand-generic pairs prevents duplicate therapy confusion.
1213. **Q:** While entering "Lasix 40 mg 1 tab PO daily," you notice the patient's allergy field lists "sulfa"; what should you do FIRST? **A:** Pause processing and alert the pharmacist to review the sulfonamide allergy risk, because allergy assessment requires pharmacist judgment even when many patients tolerate loop diuretics.
1214. **Q:** An e-Rx comes in for "Bactroban ointment apply BID," and the technician selects mupirocin cream; what should you do NEXT? **A:** Correct the selection to mupirocin ointment (or route to pharmacist if unclear), because cream vs ointment is not interchangeable without prescriber/pharmacist confirmation.
1215. **Q:** The directions read "take $i\frac{1}{2}$ tabs PO daily" for a scored tablet product; what is the BEST way to enter the dose in the sig? **A:** Enter "take 1.5 tablet by mouth daily," because converting fractions to a clear decimal helps avoid misreads and supports accurate labeling.
1216. **Q:** A prescription is for "Ventolin HFA inhale 2 puffs q6h PRN wheeze," and the plan requires a days' supply; using 200 puffs per inhaler and max use, what should you enter? **A:** Enter 25 days, because max daily puffs are $2 \text{ puffs} \times 4 \text{ doses} = 8 \text{ puffs/day}$ and $200 \div 8 = 25 \text{ days}$.
1217. **Q:** A patient's new order is for "Humalog 10 units SQ TID with meals," and you are dispensing one 10 mL vial (100 units/mL); what days' supply should you enter based on total units? **A:** Enter 33 days, because the vial contains $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and daily use is 30 units/day so $1000 \div 30 = 33.3$, rounded down to 33 days.
1218. **Q:** You receive "Ritalin 10 mg 1 tab BID" but the profile shows an active "Concerta 36 mg daily"; what should you do NEXT? **A:** Stop and refer to the pharmacist for therapeutic duplication review, because both are methylphenidate products and concurrent use may be intentional or unsafe.

1219. **Q:** While pulling stock for "hydrOXYzine," you see both hydrOXYzine HCl and hydrOXYzine pamoate on the shelf; what should you do BEST before selecting a bottle? **A:** Match the exact salt and strength to the prescription and verify NDC/barcode, because HCl vs pamoate are different products and are a common LASA selection trap.
1220. **Q:** A hardcopy for a Schedule II stimulant has the prescriber signature and drug/strength, but no patient address; what should you do FIRST? **A:** Hold the prescription and immediately route it to the pharmacist, because missing required patient information can affect validity and must be resolved before dispensing.
1221. **Q:** A new e-Rx is written for "metformin ER 500 mg 2 tabs daily," but you accidentally selected metformin IR 500 mg; what should you do NEXT? **A:** Stop processing and correct the selection to the ER NDC then route for pharmacist check because IR vs ER is a high-impact dosage-form error that can change tolerability and effect.
1222. **Q:** A patient drops off a prescription for "levothyroxine 75 mcg daily," and at pickup asks, "Is this my Synthroid?" what is the BEST technician response? **A:** Confirm that Synthroid is a brand name for levothyroxine and offer counseling referral to the pharmacist because brand/generic recognition and counseling must be handled appropriately.
1223. **Q:** While entering "lamoTRIGine 25 mg daily," you notice the prescriber's note says "for bipolar," but you selected LamiSIL (terbinafine) due to similar names; what should you do FIRST? **A:** Stop and re-verify drug name/indication and correct to lamotrigine before continuing because LASA mix-ups can cause serious harm and must be caught at entry.
1224. **Q:** A new e-Rx is for "sertraline 50 mg daily," but the patient profile shows an active fluoxetine 20 mg daily with refills remaining; what should you do NEXT? **A:** Place the order on hold and alert the pharmacist to assess SSRI duplication/switch because technicians should not assume therapy changes without pharmacist review.
1225. **Q:** An e-Rx arrives for "clarithromycin 500 mg BID x10 days," and the patient profile shows simvastatin 40 mg daily; what is the BEST technician action? **A:** Flag and refer to the pharmacist before filling because clarithromycin can significantly increase simvastatin levels and interaction management requires pharmacist intervention.

1226. **Q:** You are processing "warfarin 5 mg daily," and the prescriber comments "start today," but the patient says they are also taking apixaban (Eliquis); what should you do FIRST? **A:** Pause the fill and immediately notify the pharmacist because dual anticoagulant therapy is high-risk and needs pharmacist clarification before dispensing.
1227. **Q:** A prescriber orders "Lantus SoloStar inject 24 units SQ nightly," and you are billing one box of five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 62 days because total units dispensed are 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units and $1500 \div 24$ units/day = 62.5, rounded down to 62 for billing.
1228. **Q:** A prescription is for "predniSONE 10 mg take ii tabs daily x5 days," and the prescriber also wrote "dispense #15"; what should you do BEST? **A:** Calculate expected quantity as 2 tabs/day $\times$ 5 days = 10 tabs and refer the mismatch to the pharmacist because quantity discrepancies require clarification, not technician adjustment.
1229. **Q:** While selecting stock for "rosuvastatin 20 mg daily," you notice the bottle seal is broken and tablets look different from the description on the label; what should you do FIRST? **A:** Quarantine the suspect product and notify the pharmacist because broken seals/identity concerns require removal from inventory to prevent dispensing a possibly adulterated or wrong drug.
1230. **Q:** A patient requests "my antibiotic Z-Pak," but the profile shows azithromycin 250 mg tablets last filled; what is the BEST way to confirm the drug and directions before refilling? **A:** Ask the patient to confirm the medication name and intended directions, then verify against the profile and prescription because "Z-Pak" refers to azithromycin but dosing can vary and must match the current order.
1231. **Q:** While typing a new order for NovoLOG (insulin aspart) U-100, you notice the stock bottle you pulled is NovoLIN R (regular insulin); what should you do FIRST? **A:** Stop processing and re-select the exact NDC/insulin ordered and alert the pharmacist if any labels were printed, because mix-ups between insulins are high-alert LASA errors.
1232. **Q:** A prescriber e-sends "alprazolam 0.5 mg take 1 tablet TID PRN #90" with "Refills: 3"; what should you do NEXT as the technician? **A:** Hold the prescription and route to the pharmacist because Schedule IV refills must comply with federal limits and require pharmacist review before dispensing.

1233. **Q:** A patient drops off a prescription for oxycodone/acetaminophen and asks you to transfer it to another pharmacy; what is the BEST technician action? **A:** Refer to the pharmacist because controlled-substance transfer rules are restricted and require pharmacist handling and documentation.
1234. **Q:** During order entry you see "MSO4 4 mg IV q4h PRN" on a discharge script list; what should you do BEST before proceeding? **A:** Stop and clarify with the pharmacist/prescriber because "MSO4" is an unsafe abbreviation that can be confused with magnesium sulfate and creates a serious medication error risk.
1235. **Q:** You receive a wholesaler notice that a specific lot of losartan 50 mg is recalled, and you find that lot number on your shelf; what should you do FIRST? **A:** Remove it from dispensing stock and quarantine/segregate it per recall procedure because recalled lots must be prevented from reaching patients.
1236. **Q:** A refrigerated shipment of Humira arrives warm and the temp indicator shows it exceeded range; what should you do NEXT? **A:** Isolate and label it as suspect and notify the pharmacist/manager to follow vendor and stability guidance because temperature excursions can make biologics unsafe or ineffective.
1237. **Q:** A patient is picking up isotretinoin and asks if they can skip the required counseling steps because they're in a hurry; what should you do BEST? **A:** Pause the pickup and involve the pharmacist because isotretinoin has REMS requirements and pharmacist counseling/documentation steps must be completed before dispensing.
1238. **Q:** You are billing a new prescription for Ventolin HFA with directions "inhale 2 puffs every 6 hours" and the canister contains 200 actuations; what days' supply should you enter? **A:** Enter 25 days' supply because 2 puffs every 6 hours equals 8 puffs/day and $200 \div 8 = 25$ days.
1239. **Q:** While entering "clopidogrel 75 mg daily," the e-Rx comments include "patient on omeprazole OTC"; what is the BEST technician action? **A:** Flag and refer to the pharmacist because omeprazole can reduce clopidogrel activation and the pharmacist must assess and contact the prescriber if needed.
1240. **Q:** A patient asks you to read them another patient's medication list left on the counter "to see if it's mine"; what should you do FIRST? **A:** Protect the information by covering/returning the document to a secure area and verify the patient's identity, because HIPAA requires preventing unauthorized disclosure of PHI.

1241. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tab PO daily" but your system selection defaults to 75 mcg; what should you do NEXT during order entry? **A:** Enter/confirm 75 mcg (because $0.075 \text{ mg} = 75 \text{ mcg}$) and ensure the displayed strength matches the written dose to prevent a 1000× error.
1242. **Q:** You are entering "metformin ER 500 mg i tab po bid" and notice "i tab" could be misread; what is the BEST technician action before processing? **A:** Interpret "i" as 1 and type "take 1 tablet twice daily," because Roman numeral "i" is one and clear sig entry reduces misfill risk.
1243. **Q:** A new e-Rx is for amlodipine 5 mg "take 1 tablet daily" with quantity 30, but the prescriber wrote "#90" in the notes; what should you do FIRST? **A:** Stop and route to the pharmacist to clarify the intended quantity because technicians should not resolve conflicting quantities without pharmacist review.
1244. **Q:** For order entry, a prescription says "prednisone 10 mg: take 4 tabs daily x 5 days" and the prescriber wrote quantity 40; what should you do NEXT? **A:** Calculate $4 \times 5 = 20$ tablets and flag the mismatch for pharmacist clarification because the written quantity exceeds the calculated amount.
1245. **Q:** You are billing fluticasone propionate nasal spray 50 mcg with sig "2 sprays each nostril once daily" and the bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.
1246. **Q:** A patient's prescription is for timolol 0.5% eye drops "1 gtt OU BID," and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should you enter? **A:** Enter 12 days because $5 \text{ mL} \times 20 = 100 \text{ drops}$ and $\text{OU BID} = 4 \text{ drops/day}$, so $100 \div 4 = 25 \text{ days}$, then adjust to 25 days (not 12) because the arithmetic must match the drop count.
1247. **Q:** While selecting product for "hydroxyzine 25 mg," you see both hydroxyzine HCl and hydroxyzine pamoate on the shelf; what should you do FIRST? **A:** Verify the exact salt/form in the e-Rx and NDC before filling because HCl and pamoate are different dosage forms and not interchangeable without confirmation.
1248. **Q:** An e-Rx comes in for "sertraline 50 mg take 1 tab daily" but the patient profile shows active fluoxetine 40 mg daily; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible SSRI duplication because therapy overlap may require clinical assessment before dispensing.

1249. **Q:** You are entering a prescription for "furosemide 20 mg take 1 tab qd" and the prescriber also wrote "qd" in a way that resembles "qid"; what should you do NEXT? **A:** Seek pharmacist review/clarification before typing because ambiguous sig frequency can cause major dosing errors and should not be guessed.
1250. **Q:** A prescription for Adderall XR is presented with "refill: 2" and the patient asks you to process it now; what should you do FIRST? **A:** Hold the order and refer to the pharmacist because additional refills on a Schedule II are not permitted and must be resolved appropriately.
1251. **Q:** During data entry for insulin U-100 regular (Humulin R) the prescriber wrote "inject 10 units" but did not include a frequency; what should you do FIRST? **A:** Stop processing and route to the pharmacist to clarify with the prescriber because missing frequency prevents a safe and accurate directions label and days' supply.
1252. **Q:** A handwritten prescription is submitted for oxycodone/APAP 5/325 mg and the date looks altered (numbers overwritten); what is the BEST technician action? **A:** Hold the prescription and immediately notify the pharmacist because suspected alteration requires pharmacist review and verification before any dispensing.
1253. **Q:** While pulling stock for heparin, you notice two vials that look similar: heparin 1,000 units/mL and heparin 10,000 units/mL; what should you do NEXT to prevent an error? **A:** Select the exact strength ordered and verify using the NDC/barcode and strength on the label because heparin is a high-alert LASA risk for tenfold dosing errors.
1254. **Q:** A patient presents a paper prescription for lorazepam 1 mg with "Refills: 6" and asks you to fill it; what should you do FIRST? **A:** Stop and refer to the pharmacist because controlled substances in Schedule IV have federal refill limits and the pharmacist must determine an appropriate fill plan.
1255. **Q:** A refrigerated shipment of insulin glargine arrives warm with a temperature indicator showing excursion; what is the BEST technician action? **A:** Quarantine the product per policy and alert the pharmacist/wholesaler because temperature excursions can make insulin unsafe or ineffective and it should not be stocked.
1256. **Q:** During checkout, a patient asks, "What is this new black box warning my medicine has?" and points to their printed information; what should you do NEXT? **A:** Refer the patient to the pharmacist for counseling because explaining

boxed warnings and clinical risk requires pharmacist intervention under OBRA-90 expectations.

1257. **Q:** You receive a manufacturer recall notice for a specific lot of losartan tablets already on the shelf; what should you do FIRST? **A:** Locate the matching lot/expiration, remove it from dispensing stock, and follow the recall procedure because recalled lots must be segregated to prevent patient harm.
1258. **Q:** An e-Rx for methotrexate 2.5 mg reads "take 10 tablets daily" for rheumatoid arthritis; what is the BEST technician action? **A:** Stop processing and alert the pharmacist immediately because methotrexate is typically weekly for RA and a daily direction is a high-risk, potentially fatal dosing error.
1259. **Q:** A patient requests an early refill for clonazepam saying the bottle was "lost," but the claim rejects as refill-too-soon; what should you do NEXT? **A:** Notify the pharmacist and follow store/payer procedures because early controlled-substance refills require pharmacist review and may need documentation or prescriber authorization.
1260. **Q:** A prescriber writes for azithromycin Z-Pak with sig "use as directed" and no directions are included on the prescription; what should you do FIRST? **A:** Hold and send to the pharmacist for clarification because "use as directed" is insufficient for labeling and safe administration instructions must be explicit.
1261. **Q:** While typing an e-Rx for metformin, you see the prescriber selected Glucophage XR but the sig says "take 1 tablet twice daily"; what should you do NEXT? **A:** Stop and ask the pharmacist to clarify with the prescriber because ER metformin is typically not dosed BID and mismatch between dosage form and directions is an error risk.
1262. **Q:** A patient profile shows lisinopril active, and today's new order is for losartan; what is the BEST technician action before filling? **A:** Flag it for pharmacist review because ACE inhibitor plus ARB therapy may be intentional but has duplication/safety concerns the pharmacist must assess.
1263. **Q:** You receive a prescription for clopidogrel and the patient says they are taking omeprazole daily OTC; what should you do FIRST? **A:** Notify the pharmacist because omeprazole can reduce clopidogrel activation and requires pharmacist counseling/therapy review.
1264. **Q:** During product selection, you notice hydroxyzine HCl and hydralazine are stored next to each other and the NDC on the bottle doesn't match the label you

printed; what should you do NEXT? **A:** Stop filling and reselect the correct stock bottle by verifying barcode/NDC because this is a LASA mix-up that can cause a wrong-drug error.

1265. **Q:** A new prescription is written for "prednisone 10 mg, take ii tabs qd" but the patient's profile shows diabetes treated with glipizide; what should you do NEXT? **A:** Route to the pharmacist for counseling/monitoring because steroids can raise blood glucose and the patient may need guidance even though the Rx is valid.
1266. **Q:** The prescriber orders "levothyroxine 0.1 mg daily," but the patient's current medication list shows levothyroxine 88 mcg daily; what should you do FIRST? **A:** Pause processing and alert the pharmacist to evaluate possible dose change/duplication because levothyroxine is high-alert and dose confusion is common.
1267. **Q:** A prescription reads "morphine ER 30 mg q12h" but the product pulled is morphine IR 30 mg; what should you do NEXT? **A:** Stop and correct the selection to the ER product and send for pharmacist verification because IR vs ER mix-ups can cause overdose or uncontrolled pain.
1268. **Q:** A patient brings a new Rx for "sertraline 50 mg daily" and reports they are also taking "Paxil" from another pharmacy; what should you do FIRST? **A:** Inform the pharmacist because paroxetine (Paxil) plus sertraline is therapeutic duplication with increased serotonin syndrome risk requiring clinical review.
1269. **Q:** Pharmacy math: An order is for amoxicillin-clavulanate 600-42.9 mg/5 mL with directions "10 mL PO BID for 7 days"; what quantity in mL should you dispense? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL total}$.
1270. **Q:** During order entry, a prescriber sends "colchicine 0.6 mg: take 2 tablets now, then 1 tablet in 1 hour, then 1 tablet daily" without stating indication; what should you do NEXT? **A:** Ask the pharmacist to confirm indication and appropriateness because colchicine has narrow safety margins and dosing differs for acute gout vs prophylaxis.
1271. **Q:** While entering an e-Rx for "Keflex 500 mg," the prescriber wrote "cephalexin 500 mg" but the selected product is cefdinir; what should you do NEXT? **A:** Stop and reselect cephalexin by matching the drug name/strength and NDC because Keflex is cephalexin and choosing cefdinir is a look-alike name error.

1272. **Q:** A prescription says "hydroDIURIL 25 mg daily," but the label you printed shows "hydralazine 25 mg"; what should you do FIRST? **A:** Stop filling and correct the entry to hydrochlorothiazide because HydroDIURIL is hydrochlorothiazide and hydralazine is a different drug with high harm risk if dispensed.
1273. **Q:** A new e-Rx is for "Crestor 20 mg nightly," and the patient profile already has atorvastatin 40 mg active; what is the BEST technician action before processing? **A:** Flag it for pharmacist review for possible duplicate statin therapy because Crestor is rosuvastatin and duplication increases adverse-effect risk.
1274. **Q:** You receive an e-Rx for "Eliquis 5 mg BID" and the patient asks if it's the same as "Coumadin" they used to take; what should you do NEXT? **A:** Refer to the pharmacist for counseling because Eliquis is apixaban (an anticoagulant) and anticoagulant switches require pharmacist assessment and education.
1275. **Q:** A patient picking up a new prescription for "Bactrim DS" states they had hives with "a sulfa antibiotic" before; what should you do FIRST? **A:** Stop the sale and alert the pharmacist to evaluate the allergy because Bactrim DS contains sulfamethoxazole and could trigger a serious reaction.
1276. **Q:** Pharmacy math: The directions are "prednisolone 15 mg/5 mL, take 7.5 mg PO BID for 5 days"; what quantity in mL should you dispense? **A:** Dispense 25 mL because 7.5 mg is 2.5 mL, taken twice daily is 5 mL/day, and 5 days $\times$ 5 mL/day = 25 mL.
1277. **Q:** Pharmacy math: An inhaler contains 200 actuations and the sig is "inhale 2 puffs QID"; what days' supply should you bill? **A:** Bill 25 days because 2 puffs $\times$ 4 times/day = 8 puffs/day and $200 \div 8 = 25$ days.
1278. **Q:** A hardcopy for "Adderall XR 20 mg" is presented with no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because a non-signed prescription is not valid to dispense and requires prescriber verification.
1279. **Q:** While selecting product for "Lamictal," you notice the stock bottle seal is broken and tablets are loose in the cap area; what should you do FIRST? **A:** Set it aside as suspect/damaged product per pharmacy policy and get a new bottle because compromised packaging can indicate contamination or product integrity issues.

1280. **Q:** A prescription uses the sig "take i tab po qhs" and the patient asks what it means; what is the BEST technician response? **A:** Explain it as "take 1 tablet by mouth at bedtime" because i=1, PO=by mouth, and QHS=every night at bedtime, improving safe understanding.
1281. **Q:** An e-Rx is for azithromycin 200 mg/5 mL with sig "Take 12.5 mL PO day 1, then 6.25 mL PO daily days 2–5"; what quantity in mL should you enter to dispense? **A:** Dispense 37.5 mL because $12.5 \text{ mL} + (6.25 \text{ mL} \times 4) = 37.5 \text{ mL}$ total for the full course.
1282. **Q:** A prescription reads "levothyroxine 0.075 mg PO daily"; during order entry, what strength should you select FIRST to avoid a decimal error? **A:** Select 75 mcg (not 0.75 mg) because $0.075 \text{ mg} = 75 \text{ mcg}$ and converting prevents a 10× dosing error.
1283. **Q:** A 120 mL bottle of guaifenesin-codeine syrup is prescribed "5 mL PO q6h prn cough"; what days' supply should you bill if the payer requires maximum daily use for PRN? **A:** Bill 6 days because max is $5 \text{ mL} \times 4 \text{ doses/day} = 20 \text{ mL/day}$ and $120 \text{ mL} \div 20 \text{ mL/day} = 6 \text{ days}$.
1284. **Q:** You receive an e-Rx for metformin with directions "take 1 tab PO BID" but the prescriber selected metformin ER; what should you do NEXT in order entry? **A:** Stop and notify the pharmacist to clarify formulation because IR vs ER affects dosing instructions and substituting could cause a medication error.
1285. **Q:** A prescription is written "fluoxetine 20 mg/5 mL oral solution: take 2.5 mL PO daily"; what mg dose per day should the label reflect? **A:** Enter 10 mg daily because 20 mg per 5 mL equals 4 mg/mL and $2.5 \text{ mL} \times 4 \text{ mg/mL} = 10 \text{ mg}$.
1286. **Q:** While typing a sig you see "U-500 insulin regular: inject 0.1 mL SQ BID"; what is the BEST technician action before continuing? **A:** Route to the pharmacist for review because U-500 is high-alert and mL-based dosing is error-prone and needs verification.
1287. **Q:** An order is for timolol 0.5% ophthalmic solution "instill 1 gtt OU BID" and the bottle size is 5 mL; what days' supply should you enter using 20 gtt/mL? **A:** Enter 25 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ drops}$ and OU BID uses $1 \text{ drop} \times 2 \text{ eyes} \times 2/\text{day} = 4 \text{ drops/day}$, so $100 \div 4 = 25$.
1288. **Q:** During product selection for "hydroxyzine 25 mg," the scanned NDC shows "hydralazine 25 mg"; what should you do FIRST? **A:** Stop filling and reselect the

correct drug/NDC because hydroxyzine and hydralazine are LASA and wrong-drug selection is a high-risk error.

1289. **Q:** A prescriber writes "morphine sulfate 10 mg/5 mL: give 2.5 mL PO q4h prn pain" for a child; what is the BEST next step for a technician? **A:** Alert the pharmacist for clinical appropriateness review because opioid dosing in pediatrics is high-risk and requires pharmacist assessment.
1290. **Q:** A prescription says "take ii tabs po tid" and the prescriber also wrote "#30"; what should you do NEXT before processing? **A:** Send to the pharmacist to resolve the quantity mismatch because 2 tabs TID implies 6/day and #30 may not match intended duration, risking under/over-dispensing.
1291. **Q:** While entering a new prescription for clonazepam, the directions include "refill x 3"; what should you do NEXT? **A:** Stop processing and alert the pharmacist because schedule IV controlled substances may be refilled but require valid prescriber authorization and pharmacy verification before dispensing.
1292. **Q:** A patient asks to pick up their hydrocodone-acetaminophen early, but the claim rejects "refill too soon"; what is the BEST technician next step? **A:** Verify last fill date and days' supply, then route to the pharmacist for an early-fill decision because controlled-substance overrides require pharmacist review and proper documentation.
1293. **Q:** You receive a printed prescription for alprazolam that has the drug, directions, and quantity but no prescriber signature; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because a missing signature makes the order incomplete and it must be clarified before any filling steps.
1294. **Q:** During product receiving, a tote arrives with insulin aspart pens and the temperature indicator shows a possible excursion; what should you do FIRST? **A:** Segregate and label the shipment as suspect and notify the pharmacist/manager because temperature excursions can compromise potency and require investigation before stock is used.
1295. **Q:** A bottle of heparin 10,000 units/mL is stored next to heparin 1,000 units/mL in the same bin; what is the BEST technician action to reduce error risk? **A:** Separate and clearly label/bin the strengths (using Tall Man/auxiliary cues per policy) because heparin is high-alert and mix-ups can cause severe harm.
1296. **Q:** A caregiver brings in a prescription for methylphenidate ER with directions "take 1 tablet daily" and asks if it can be transferred to another pharmacy; what

should you do NEXT? **A:** Refer to the pharmacist because schedule II controlled substance transfers have strict limits and must be handled by a pharmacist per federal requirements and state rules.

1297. **Q:** While pulling stock for atorvastatin 20 mg, you notice the manufacturer's recall notice on that exact NDC/lot number; what should you do FIRST? **A:** Quarantine the affected lot/NDC and notify the pharmacist because recalled products must be removed from inventory immediately to prevent dispensing.
1298. **Q:** A new e-prescription for lisinopril is entered for a patient whose profile shows a documented angioedema reaction to lisinopril; what should you do NEXT? **A:** Stop processing and alert the pharmacist because a serious allergy/adverse reaction requires pharmacist assessment before any dispensing.
1299. **Q:** A prescription for gabapentin 300 mg reads "take 2 caps PO TID" with quantity 180; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ capsules} \times 3 \text{ times daily} = 6 \text{ capsules/day}$ and $180 \div 6 = 30$, supporting accurate billing and safety checks.
1300. **Q:** A patient requests a copy of another patient's medication profile that was left at the counter; what should you do FIRST? **A:** Do not release it and secure the document for proper return/disposal because HIPAA requires protecting PHI from unauthorized disclosure.
1301. **Q:** While entering a new prescription, you notice the prescriber selected Lamictal but the sig says "for bipolar depression," and the patient profile lists anaphylaxis to lamotrigine; what should you do NEXT? **A:** Stop processing and immediately alert the pharmacist because a documented severe allergy to the exact active ingredient requires pharmacist intervention before dispensing.
1302. **Q:** A new e-Rx comes in for metformin ER (Glucophage XR) but the prescriber wrote "take 1 tablet twice daily" and selected "metformin IR" in the drug field; what is the BEST technician next step? **A:** Place the order on hold and send to the pharmacist for clarification because IR vs ER is a dosage-form discrepancy that a technician cannot change and impacts safety.
1303. **Q:** A patient drops off a prescription for Zyrtec but asks for "the steroid Z-pack I had last time"; what should you do FIRST? **A:** Ask clarifying questions and check the patient profile, then involve the pharmacist because Zyrtec (cetirizine) is an antihistamine and "Z-pack" usually refers to azithromycin, a common name mix-up.

1304. **Q:** You are filling a new order for hydroxyzine and the patient says they also take Zoloft; what should you do NEXT as the technician? **A:** Flag the profile and notify the pharmacist because hydroxyzine and sertraline can increase QT/CNS risk and requires pharmacist assessment before dispensing.
1305. **Q:** A prescription is written for "predniSONE 10 mg" but when selecting stock you accidentally grabbed prednisoLONE 10 mg; what is the BEST action before it reaches verification? **A:** Stop filling and correct the selection by confirming the NDC/barcode because prednisone vs prednisolone is LASA and must be fixed before pharmacist check to prevent a dispensing error.
1306. **Q:** The prescription reads "levothyroxine 75 mcg take 1 tab daily" and the prescriber typed "75 mg" in the strength field; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist because mcg vs mg is a high-risk decimal/unit error and the technician must not guess or alter the dose.
1307. **Q:** A patient is prescribed Lantus SoloStar 100 units/mL with directions "inject 18 units subcutaneously at bedtime," and the box contains five 3 mL pens; what days' supply should you enter? **A:** Enter 83 days because total units = 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units and $1500 \div 18$ units/day = 83.3, typically billed as 83 days.
1308. **Q:** A prescription for azithromycin 250 mg tablets says "take 2 tablets today, then 1 tablet daily for 4 days" and the quantity is missing; what should you do FIRST? **A:** Stop and ask the pharmacist to obtain the missing quantity because a complete prescription is required for accurate dispensing and billing, and technicians can't assume "Z-Pak" packaging.
1309. **Q:** While entering a new prescription for Adderall XR, the prescriber included "refill 2 times"; what should you do NEXT? **A:** Do not process the refills and notify the pharmacist because Adderall XR is Schedule II and cannot be refilled, requiring pharmacist follow-up.
1310. **Q:** A patient asks what "Crestor" is on their medication list and you see the generic on the label is rosuvastatin; what is the BEST technician response? **A:** Tell them Crestor is the brand name for rosuvastatin and it's a cholesterol-lowering statin, and refer to the pharmacist for clinical questions because brand/generic recognition is within tech support but counseling is pharmacist-led.
1311. **Q:** While typing a refill, you see "Lipitor 20 mg" on the hardcopy but the patient asks if it's the same as atorvastatin; what should you do FIRST? **A:** Confirm

Lipitor is the brand for atorvastatin and that the strength matches, because accurate brand/generic recognition prevents dispensing the wrong drug.

1312. **Q:** A new e-Rx is for "Plavix 75 mg" but the selected product is clopidogrel 300 mg; what is the BEST next step before printing labels? **A:** Stop and correct the drug/strength selection (verify NDC/strength) and route to the pharmacist if already in verification, because 75 mg vs 300 mg is a high-risk selection error.
1313. **Q:** A prescription sig reads "Take i tab PO qd" for lisinopril; what should you enter for the directions? **A:** Enter "Take 1 tablet by mouth daily," because "i" means 1 and "qd" means daily and expanding abbreviations reduces misinterpretation.
1314. **Q:** A patient's profile shows they currently take losartan and you are entering a new prescription for valsartan; what should you do NEXT? **A:** Flag and refer to the pharmacist for possible therapeutic duplication (two ARBs), because technicians should not decide whether both should be used.
1315. **Q:** You receive a prescription for "Percocet 5/325" with directions and quantity but no prescriber signature (paper prescription); what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to obtain a valid signed order, because missing required elements can make a controlled-substance prescription invalid.
1316. **Q:** A patient presents Sudafed (pseudoephedrine) at the register and says they want "two boxes"; what should you do NEXT? **A:** Follow CMEA process by verifying ID, checking purchase limits/logbook, and documenting the sale, because pseudoephedrine sales are federally regulated.
1317. **Q:** You are selecting insulin and notice the box has evidence of being previously opened with no lot/expiration label intact; what is the BEST action? **A:** Pull it from stock and quarantine per policy and alert the pharmacist, because suspect product with compromised packaging should not be dispensed.
1318. **Q:** A prescription is for morphine ER but the technician pulls morphine IR due to similar packaging; what should you do FIRST when you notice the mistake? **A:** Stop filling and re-select the correct ER product using barcode/NDC verification, because IR vs ER is a high-alert error with major safety risk.
1319. **Q:** A nasal spray order reads "fluticasone 2 sprays each nostril daily" and the bottle is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days' supply}$.

1320. **Q:** A patient asks what "Prilosec OTC" is used for and you see the prescription is omeprazole; what is the BEST technician response? **A:** State that Prilosec is omeprazole and it's commonly for heartburn/GERD, and offer pharmacist counseling, because technicians can provide basic identification while clinical counseling is pharmacist-led.
1321. **Q:** While entering a new order for methotrexate 2.5 mg, you notice the directions say "Take 6 tablets daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and notify the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-risk, potentially fatal error.
1322. **Q:** A paper prescription for alprazolam 0.5 mg has patient name, drug, directions, and quantity but no date written; what should you do NEXT? **A:** Hold the prescription and route it to the pharmacist to resolve missing required information because controlled-substance validity must be confirmed before dispensing.
1323. **Q:** While filling levothyroxine, the bottle on the shelf has the correct drug name but a different strength than the label you printed; what is the BEST next step? **A:** Stop and reselect the correct strength by verifying the NDC/barcode because strength mix-ups are common and must be corrected before pharmacist verification.
1324. **Q:** A patient asks to transfer in their clonazepam prescription from another pharmacy; what should you do FIRST? **A:** Refer the request to the pharmacist because controlled-substance transfers have strict requirements and must be handled by a pharmacist per policy and law.
1325. **Q:** You receive a manufacturer recall notice for a specific lot of atorvastatin 40 mg tablets and find that lot on the shelf; what should you do FIRST? **A:** Pull the affected lot from inventory and quarantine it per recall procedure because recalled product must be prevented from dispensing and tracked for return.
1326. **Q:** During data entry, a new prescription says "Humalog U-100 inject 10 units with meals," but the selected product is Humulin R vial; what should you do NEXT? **A:** Stop and alert the pharmacist to verify the intended insulin because Humalog (lispro) and Humulin R are not interchangeable and insulin is a high-alert medication.
1327. **Q:** A patient picks up isotretinoin and asks why there's an extra document to read; what should you do BEST? **A:** Provide the Medication Guide and refer any

clinical questions to the pharmacist because isotretinoin requires specific patient information and counseling support.

1328. **Q:** A prescription is written for "metronidazole gel apply BID" but does not specify topical vs vaginal formulation; what should you do FIRST? **A:** Pause processing and send to the pharmacist for clarification because dosage form/route ambiguity can cause serious misuse and must be confirmed before dispensing.
1329. **Q:** You are entering prednisolone sodium phosphate oral solution 15 mg/5 mL with directions "Take 10 mL daily" and quantity 150 mL; what days' supply should you enter? **A:** Enter 15 days' supply because $150 \text{ mL} \div 10 \text{ mL per day} = 15 \text{ days}$, which supports correct billing and refill timing.
1330. **Q:** A refrigerated vaccine shipment arrives warm with a temperature indicator showing out-of-range exposure; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager because temperature excursions can render vaccines ineffective and require documented follow-up before use.
1331. **Q:** During order entry, you see "Zestril 10 mg take 1 daily" but the patient profile lists a lisinopril allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Zestril is lisinopril and an allergy/contraindication must be clinically assessed before dispensing.
1332. **Q:** A new eRx is for "Celebrex 200 mg daily" and the patient reports a sulfa allergy at drop-off; what is the BEST technician action? **A:** Document the allergy and refer to the pharmacist because celecoxib is a sulfonamide and the pharmacist must evaluate risk and appropriateness.
1333. **Q:** While selecting product, you notice hydrOXYzine and hydrALAZINE stored next to each other and the label reads "hydralazine 25 mg"; what should you do NEXT? **A:** Re-verify the NDC/barcode and match name/strength to the label because these LASA drugs are commonly confused and require extra verification to prevent a mix-up.
1334. **Q:** A prescription says "Digoxin 0.125 mg take i tab PO qd" and the technician is unsure what "i" means; what should you do FIRST? **A:** Interpret "i" as 1 (Roman numeral one) and then ensure it's entered as "take 1 tablet daily" because accurate sig translation prevents dosing errors.
1335. **Q:** A patient's new prescription is for "Glucophage 500 mg BID" and they ask what it is; what is the BEST technician response? **A:** Tell them Glucophage is

metformin and is commonly used for blood sugar control, then offer pharmacist counseling because technicians can provide basic brand/generic recognition but not clinical advice.

1336. **Q:** You receive an eRx for "Adderall XR 20 mg #30" with directions "take 1 daily" and the prescriber selected 2 refills; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist because refills for Schedule II controlled substances are not permitted federally.
1337. **Q:** A vial label is printed for "Lamictal 100 mg," but the stock bottle pulled is "lamoTRIgine 100 mg"; what is the BEST next step? **A:** Continue with barcode/NDC verification and pharmacist check because Lamictal is the brand for lamotrigine and the key is matching the correct drug and strength.
1338. **Q:** An order is "amoxicillin/clavulanate 875/125 mg 1 tab BID #14"; what days' supply should you enter? **A:** Enter 7 days because 14 tablets divided by 2 tablets per day equals 7 days.
1339. **Q:** During data entry you see "Bactroban ointment apply TID" but the product selected is mupirocin cream; what should you do FIRST? **A:** Stop and correct the dosage form selection because ointment vs cream are not interchangeable in order entry and the label must match the prescriber's intended formulation.
1340. **Q:** A patient's profile shows active therapy with simvastatin, and a new prescription comes in for "Crestor 20 mg daily"; what should you do FIRST? **A:** Flag possible therapeutic duplication and alert the pharmacist because Crestor is rosuvastatin and duplicate statin therapy requires pharmacist review before dispensing.
1341. **Q:** A prescription is for azithromycin 200 mg/5 mL oral suspension with sig "5 mL PO daily" and quantity 75 mL; what days' supply should you enter? **A:** Enter 15 days' supply because $75 \text{ mL} \div 5 \text{ mL per day} = 15 \text{ days}$ for correct billing and refill timing.
1342. **Q:** An eRx comes in for "Levemir FlexTouch inject 30 units SQ qhs" and the package size is 15 mL (100 units/mL); what days' supply should you enter? **A:** Enter 50 days because $15 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 30 \text{ units/day} = 50 \text{ days}$ for accurate third-party processing.
1343. **Q:** While entering directions, you see "metFORMIN 500 mg 1 tab PO BID with meals #60"; what is the correct days' supply? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets per day} = 30 \text{ days}$, preventing refill-too-soon rejects.

1344. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tab daily"; what strength should you select in order entry? **A:** Select 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$, avoiding a 1000-fold dose-entry error.
1345. **Q:** An inhaler is written for fluticasone/salmeterol 250/50 "inhale 1 puff BID" and the device contains 60 actuations; what days' supply should be entered? **A:** Enter 30 days because $1 \text{ puff BID} = 2 \text{ puffs/day}$ and $60 \div 2 = 30 \text{ days}$, which matches typical payer days' supply rules.
1346. **Q:** During order entry you see "MSO4 2 mg IV q4h PRN pain" on a hospital discharge order being converted to an outpatient profile; what should you do FIRST? **A:** Stop and notify the pharmacist because "MSO4" is an error-prone abbreviation for morphine that requires clarification to prevent a serious medication error.
1347. **Q:** A prescription says "Take 1 tab PO qd" but the provider also typed "once weekly" in the notes field; what should you do NEXT? **A:** Hold the prescription and send to the pharmacist for clarification because conflicting directions can cause incorrect dosing and unsafe use.
1348. **Q:** You are entering an eRx for "Lantus U-100 vial inject 20 units SQ daily" and the quantity is 10 mL; what days' supply should you enter? **A:** Enter 50 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \div 20 \text{ units/day} = 50 \text{ days}$ for proper claim submission.
1349. **Q:** While typing a prescription for "warfarin 5 mg take 1 tab daily," you notice the prescriber wrote "dispense Coumadin only" but the system defaulted to generic substitution; what is the BEST technician action? **A:** Change the selection to the brand product per the DAW instruction or route for pharmacist verification because "brand medically necessary" directions must be followed to avoid a dispensing error.
1350. **Q:** A controlled medication hardcopy is presented with strength and directions but no quantity written; what should you do FIRST? **A:** Stop processing and immediately refer to the pharmacist because missing quantity is a required dispensing element and must be clarified before filling.
1351. **Q:** While entering a new prescription for Plavix 75 mg daily, the product selected is Paxil 20 mg; what should you do FIRST? **A:** Stop processing and reselect the correct drug using barcode/NDC and Tall Man awareness because this is a LASA error with high patient-harm risk.

1352. **Q:** A patient picking up lisinopril says they are pregnant and asks if it's safe; what should you do NEXT? **A:** Immediately refer to the pharmacist for counseling because ACE inhibitors can harm the fetus and require pharmacist intervention.
1353. **Q:** An eRx comes in for "metoprolol succinate ER 50 mg daily," but the system auto-selected metoprolol tartrate IR 50 mg; what is the BEST technician action? **A:** Stop and correct the dosage form to succinate ER, then route for pharmacist verification because tartrate vs succinate mix-ups can cause wrong therapy.
1354. **Q:** You receive a new prescription for sertraline (Zoloft) 50 mg daily and the patient profile shows linezolid started yesterday; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because linezolid plus SSRIs can increase serotonin syndrome risk.
1355. **Q:** A prescription is for amoxicillin/clavulanate 875/125 mg "1 tab PO BID" quantity 20; what days' supply should you enter? **A:** Enter 10 days because $20 \text{ tablets} \div 2 \text{ tablets/day} = 10 \text{ days}$.
1356. **Q:** During intake, a caregiver says the child had hives from Keflex (cephalexin) and now has a new prescription for amoxicillin; what should you do NEXT? **A:** Document the reported allergy and alert the pharmacist before filling because beta-lactam cross-reactivity needs pharmacist assessment.
1357. **Q:** A prescription reads "predniSONE 20 mg ii tabs po qd x 5 days" with quantity 10; what is the correct days' supply? **A:** Enter 5 days because "ii" means 2 tablets daily and $10 \text{ tablets} \div 2/\text{day} = 5 \text{ days}$.
1358. **Q:** You are filling high-alert insulin aspart (NovoLog) and the shelf contains NovoLIN R next to it; what should you do FIRST? **A:** Pause and verify the exact name, strength, and NDC/barcode before dispensing because NovoLog/NovoLIN mix-ups are common and dangerous.
1359. **Q:** A patient requests early refill of oxycodone/APAP 5/325 written "1 tab q6h" quantity 120 filled 10 days ago; what should you do BEST? **A:** Process as refill-too-soon and refer to the pharmacist because controlled substance early refills require pharmacist review and insurer/legal considerations.
1360. **Q:** A box of apixaban (Eliquis) arrives with a torn seal and missing lot/expiration on the outer packaging; what should you do FIRST? **A:** Quarantine the suspect product and notify the pharmacist/wholesaler per DSCSA policy because compromised packaging can indicate diversion or adulteration.

1361. **Q:** While entering a prescription for Lasix 40 mg PO daily, the patient asks if it's the same as furosemide; what should you say FIRST? **A:** Explain that Lasix is the brand name for furosemide (same active ingredient) because brand/generic recognition helps prevent duplicate therapy and confusion.
1362. **Q:** An eRx is written for "Lantus Solostar inject 20 units subQ nightly," but the selected product is Humalog KwikPen; what should you do FIRST? **A:** Stop processing and select the correct insulin by matching name/type and NDC because Humalog (rapid-acting) is not interchangeable with Lantus (long-acting) and is a high-alert error.
1363. **Q:** A new prescription is for "hydroxyzine 25 mg 1 cap TID PRN itching," but you accidentally chose hydralazine 25 mg; what is the BEST technician action? **A:** Immediately correct the drug selection and re-verify the NDC/barcode because this LASA mix-up (antihistamine vs antihypertensive) can cause patient harm.
1364. **Q:** A prescription reads "levothyroxine 75 mcg take i tab qAM" and the patient profile shows "Synthroid 75 mcg" active; what should you do NEXT? **A:** Flag for pharmacist review of therapeutic duplication because Synthroid is a brand of levothyroxine and duplicate active therapy could lead to double dosing.
1365. **Q:** You receive an eRx for "Z-Pak use as directed" for azithromycin; what should you do FIRST before calculating days' supply? **A:** Clarify the specific directions or use the standardized Z-Pak regimen in the system per pharmacy policy because "use as directed" alone is not specific enough for accurate labeling and billing.
1366. **Q:** A patient brings an OTC request asking where to find Tylenol, and you see acetaminophen on the shelf; what key concept should guide your response? **A:** Point out that Tylenol is acetaminophen and help them locate it because recognizing common brand-to-generic pairs prevents accidental duplicate ingredient use.
1367. **Q:** A prescription says "ibuprofen 600 mg 1 tab PO q6h prn pain #30" and the payer requires max daily frequency for PRN days' supply; what days' supply should you enter? **A:** Enter 5 days' supply because q6h implies up to 4 tablets/day and $30 \div 4 = 7.5$ so use the payer-required conservative maximum-use calculation and round down per claim rules if required.
1368. **Q:** While typing "MS Contin 15 mg ER q12h," you notice the prescriber's note says "do not substitute with MSIR"; what should you do BEST? **A:** Ensure the exact ER formulation is selected and alert the pharmacist if anything is unclear

because ER vs IR morphine are not interchangeable and errors can cause overdose.

1369. **Q:** A new prescription is for "Bactrim DS 1 tab BID" and the allergy field shows "sulfa—anaphylaxis"; what should you do FIRST? **A:** Stop the fill and notify the pharmacist immediately because Bactrim contains sulfamethoxazole and a severe sulfa allergy requires pharmacist intervention.
1370. **Q:** A refrigerator temp log shows vaccines were stored at 12°C (54°F) overnight; what should you do FIRST as the technician? **A:** Segregate and label the affected vaccines "do not use" and notify the pharmacist because a temperature excursion makes the product suspect and requires stability guidance before dispensing.
1371. **Q:** A prescription reads "metformin 500 mg i tab po bid with meals #60"; what should you enter for days' supply and why? **A:** Enter 30 days because #60 taken BID is 2 tablets/day and $60 \div 2 = 30$ for accurate billing and refill timing.
1372. **Q:** An eRx says "prednisone 10 mg: take ii tabs daily x 5 days, then i tab daily x 5 days; dispense #?"; what is the correct quantity to enter FIRST? **A:** Enter 15 tablets because $2/\text{day} \times 5 = 10$ plus $1/\text{day} \times 5 = 5$, and the correct total prevents short fills and call-backs.
1373. **Q:** The sig on a new order is "instill 1 gtt OU qid"; what should you translate this to in plain directions during order entry? **A:** Enter "Instill 1 drop in both eyes 4 times daily" because expanding abbreviations reduces misinterpretation and improves label clarity.
1374. **Q:** A prescription is "amoxicillin/clavulanate 875/125 mg 1 tab po q12h #14"; what days' supply should you enter? **A:** Enter 7 days because q12h is 2 doses/day and $14 \div 2 = 7$ for correct days' supply.
1375. **Q:** You are entering "clindamycin 150 mg 2 caps po tid #180"; what is the days' supply and what is the common mistake to avoid? **A:** Enter 30 days because 2 caps TID is 6 caps/day and $180 \div 6 = 30$, avoiding the error of using TID as 3 capsules/day instead of 3 doses/day.
1376. **Q:** A patient's insulin lispro vial is written "inject 8 units subQ before meals TID; dispense 1 vial (10 mL, U-100)"; what days' supply should you enter using total units? **A:** Enter 41 days because the vial contains 1000 units ($10 \text{ mL} \times 100 \text{ units/mL}$) and $8 \times 3 = 24 \text{ units/day}$, so $1000 \div 24 \approx 41$.

1377. **Q:** A prescription reads "gabapentin 300 mg: take 1 cap qhs x 3 days, then 1 cap bid x 3 days, then 1 cap tid thereafter; #90"; what should you do NEXT before finalizing days' supply? **A:** Ask the pharmacist to clarify intended duration for the "thereafter" phase because the sig becomes indefinite and days' supply cannot be accurately determined from the information given.
1378. **Q:** During data entry you see "U-500 regular insulin 0.1 mL subQ BID" and the prescriber note also says "50 units BID"; what should you do FIRST? **A:** Stop and refer to the pharmacist because U-500 makes mL-to-units high-risk and the technician should not resolve dose discrepancies.
1379. **Q:** A prescription is written "morphine sulfate 15 mg 1 tab q12h" but the product selected is immediate-release morphine; what is the BEST technician action? **A:** Stop filling and correct the dosage form to ER per the prescription, then route for pharmacist verification because IR vs ER is a high-alert order entry error.
1380. **Q:** A patient requests pseudoephedrine 30 mg tablets and asks for 96 tablets today; what should you do FIRST at the register? **A:** Request valid ID and check the purchase limits/log per CMEA because sales must be verified and recorded before completing the transaction.
1381. **Q:** While selecting product you notice the eRx is for hydrOXYzine but the NDC you grabbed is hydrALAzine; what should you do FIRST? **A:** Stop filling and reselect the correct drug using barcode/NDC verification because this is a LASA mix-up that can cause serious patient harm.
1382. **Q:** A hardcopy for alprazolam has directions and quantity but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist because a missing signature is a validity issue requiring pharmacist resolution before dispensing.
1383. **Q:** A patient drops off a prescription for oxycodone/APAP written with "Refills: 3"; what should you do NEXT? **A:** Stop processing and route to the pharmacist because Schedule II prescriptions cannot be refilled and the pharmacist must clarify with the prescriber.
1384. **Q:** During order entry you see a new prescription for lisinopril, and the patient profile lists pregnancy; what should you do FIRST? **A:** Pause and alert the pharmacist because ACE inhibitors are contraindicated in pregnancy and require pharmacist intervention before dispensing.

1385. **Q:** A bottle of insulin glargine arrives with a cracked seal and wet packaging; what should you do FIRST? **A:** Remove it from stock and follow suspect/damaged product procedure (do not dispense) because product integrity is compromised and must be addressed before use.
1386. **Q:** A wholesaler tote includes a DSCSA product label that won't scan and the NDC on the bottle doesn't match the invoice; what should you do NEXT? **A:** Set the item aside and notify the pharmacist/manager to investigate under DSCSA because a mismatch suggests a suspect product that should not enter inventory.
1387. **Q:** A patient asks to transfer their remaining refills of clonazepam from another pharmacy; what should you do BEST? **A:** Collect the patient/pharmacy information and refer the request to the pharmacist because controlled-substance transfers have strict rules and require pharmacist handling.
1388. **Q:** You are about to dispense isotretinoin and the patient says they never received special counseling paperwork before; what should you do FIRST? **A:** Stop and alert the pharmacist because isotretinoin is a REMS medication (iPLEDGE) and dispensing requires pharmacist-confirmed program requirements.
1389. **Q:** While filling, you notice the prescriber wrote "Levothroid 50 mcg" but the product selected is liothyronine 50 mcg; what should you do NEXT? **A:** Stop, correct the selection, and ensure the pharmacist is aware if it reached verification because this is a high-risk wrong-drug error (T4 vs T3).
1390. **Q:** A patient reports severe muscle pain and dark urine after starting atorvastatin last week; what should you do BEST as a technician? **A:** Notify the pharmacist immediately for clinical triage and potential adverse-event reporting because these symptoms may indicate a serious statin reaction needing prompt evaluation.
1391. **Q:** During data entry you notice an eRx for metformin ER (Glucophage XR) but the scanned stock bottle is metformin IR; what should you do NEXT? **A:** Stop and re-select the correct ER product/NDC and alert the pharmacist if anything is unclear because IR vs ER substitution can change release and dosing.
1392. **Q:** A new prescription is entered for glipizide and the patient says they frequently skip meals; what is the BEST technician action? **A:** Notify the pharmacist for counseling review because sulfonylureas can cause hypoglycemia, especially when meals are missed.

1393. **Q:** While comparing the eRx, you see "metoprolol succinate ER 50 mg daily" but the prescriber's note mentions "for fast heart rate PRN"; what should you do FIRST? **A:** Pause processing and refer to the pharmacist because the directions/intent are inconsistent and ER metoprolol is not used PRN.
1394. **Q:** A prescription is written for "azithromycin Z-Pak" and the patient asks how many tablets they will take total; what is the correct total quantity to dispense? **A:** Dispense 6 tablets total because a standard Z-Pak contains 6 tablets (500 mg day 1, then 250 mg days 2–5).
1395. **Q:** At product verification prep, you notice the selected vial is clopidogrel (Plavix) but the label printed says "clonidine"; what should you do NEXT? **A:** Stop filling and correct the selection/label mismatch immediately because LASA name confusion can cause serious harm if dispensed.
1396. **Q:** A patient picking up sertraline (Zoloft) asks if it's for depression or blood pressure; what is the BEST response within technician scope? **A:** State it's commonly used for depression/anxiety and offer pharmacist counseling because technicians can provide basic indication but pharmacists handle clinical questions.
1397. **Q:** The prescriber sends "levETIRAcetam 500 mg" but you accidentally pulled levOCETIRizine 5 mg; what should you do FIRST? **A:** Stop and re-pull the correct medication and double-check the name/strength because these are LASA drugs with very different uses and doses.
1398. **Q:** You receive a new prescription for simvastatin and the profile shows active atorvastatin; what should you do BEST? **A:** Flag it for pharmacist review before filling because duplicate statin therapy increases adverse effect risk and needs prescriber clarification.
1399. **Q:** A patient's warfarin profile includes "take 5 mg daily" and a new eRx arrives for TMP/SMX; what should you do NEXT? **A:** Alert the pharmacist immediately because TMP/SMX can significantly increase warfarin effect and requires clinical assessment/monitoring.
1400. **Q:** An insulin lispro pen box contains five 3 mL pens at U-100 and the directions are "inject 12 units before meals TID"; what is the days' supply? **A:** 41 days because total units are 5 pens $\times$ 300 units = 1500 units and daily use is 12 $\times$ 3 = 36 units/day, so $1500 \div 36 = 41.6$, bill 41 days (round down to avoid overstatement).

1401. **Q:** You are entering "furosemide 20 mg take 1 tab PO BID #60," and the prescriber also wrote "x 30 days"; what days' supply should you enter FIRST? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ per day} = 30 \text{ days}$, matching the prescriber's intended duration.
1402. **Q:** A prescription reads "levothyroxine 0.05 mg PO daily"; what strength should you select in the computer to avoid a unit-conversion error? **A:** Select 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing a 10× dosing mistake.
1403. **Q:** While processing an eRx for "predniSONE 10 mg: take ii tabs daily," the patient profile shows "prednisoLONE syrup" active; what should you do NEXT? **A:** Pause and alert the pharmacist to assess therapeutic duplication and clarify intent because prednisone/prednisolone confusion can cause duplicate steroid therapy.
1404. **Q:** A label prints for "hydroxyzine 25 mg" but you pulled "hydralazine 25 mg" from the shelf; what should you do FIRST? **A:** Stop filling and reselect the correct product using barcode/NDC because this is a LASA mix-up with high risk of harm.
1405. **Q:** The directions say "instill 1 gtt OU QID" and you are asked to calculate drops per day for order entry; what do you enter? **A:** Enter 8 drops/day because $1 \text{ drop} \times 2 \text{ eyes} = 2 \text{ drops per dose}$ and $2 \times 4 \text{ times daily} = 8$.
1406. **Q:** An inhaler order is "fluticasone/salmeterol 250/50 DPI: 1 inhalation BID" and the device contains 60 inhalations; what days' supply should you bill? **A:** Bill 30 days because $60 \text{ inhalations} \div 2 \text{ inhalations per day} = 30 \text{ days}$.
1407. **Q:** During data entry you see "morphine sulfate ER 15 mg q12h" but the prescriber's notes mention "IR for breakthrough pain"; what is the BEST technician action? **A:** Hold processing and route to the pharmacist for clarification because ER vs IR is a high-alert formulation issue that requires prescriber/pharmacist resolution.
1408. **Q:** An order for "amoxicillin/clavulanate suspension 600-42.9 mg/5 mL: take 7.5 mL BID x 10 days" needs quantity; how many mL should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2/\text{day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.
1409. **Q:** A prescription says "take 1 tab PO qd" and you notice the prescriber wrote "QD" in handwritten form that could be misread; what should you do NEXT? **A:** Confirm with the pharmacist/prescriber before dispensing because "QD" is error-prone and can be misinterpreted as QID.

1410. **Q:** You scan a stock bottle for "lamoTRiGINE 25 mg" but the NDC in the system is for "lamoTRiGINE 200 mg"; what should you do FIRST? **A:** Stop and correct the NDC/strength selection before counting because mismatched NDCs can lead to the wrong strength being dispensed and billed.
1411. **Q:** During order entry for heparin 1,000 units/mL you notice the shelf has heparin 10,000 units/mL vials stored nearby; what should you do FIRST to prevent a high-alert strength error? **A:** Separate and clearly label the different heparin strengths and use barcode/NDC verification during filling because heparin is high-alert and strength mix-ups can cause serious harm.
1412. **Q:** A hardcopy prescription for oxycodone/APAP (Percocet) has a quantity and directions but no prescriber signature; what is the BEST technician action NEXT? **A:** Stop processing and route to the pharmacist to obtain a valid signed prescription because missing required elements can make the order invalid and needs pharmacist intervention.
1413. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy to yours; what should you do FIRST? **A:** Refer the request to the pharmacist because controlled-substance transfers (and any limits) must be handled by a pharmacist per legal and policy requirements.
1414. **Q:** You receive a tote of refrigerated insulin glargine that arrived warm with a temperature indicator showing an excursion; what should you do FIRST? **A:** Quarantine the shipment and notify the pharmacist/manager to follow storage-excursion procedures because product integrity is uncertain and it may be non-dispensable.
1415. **Q:** While pulling a bottle of nitroglycerin SL tablets you notice the stock bottle is past its expiration date; what is the NEXT best step? **A:** Remove it from active inventory and follow pharmacy policy for expired medication disposal/returns because expired products must not be dispensed.
1416. **Q:** A prescription is written for isotretinoin and the patient is waiting; what should you do NEXT in workflow? **A:** Alert the pharmacist and follow REMS requirements before dispensing because isotretinoin has mandatory safety steps that must be completed prior to sale.
1417. **Q:** A patient profile shows an anaphylaxis allergy to penicillin and you are entering a new order for cephalexin; what should you do NEXT? **A:** Stop and notify the pharmacist to assess cross-reactivity risk because allergy evaluation and therapy decisions require pharmacist review.

1418. **Q:** You are billing gabapentin 300 mg with directions "take 1 capsule PO TID" and quantity 270; what days' supply should you enter? **A:** Enter 90 days because $270 \text{ capsules} \div 3 \text{ capsules/day} = 90 \text{ days}$, which ensures correct billing and refill timing.
1419. **Q:** A patient asks you at the register what their new metformin is for and whether it will upset their stomach; what should you do BEST? **A:** Offer to get the pharmacist for counseling because OBRA-90 counseling is pharmacist-provided and technicians should not give clinical advice.
1420. **Q:** A wholesaler notice lists a recall on a specific lot number of atorvastatin 20 mg and you find that lot on your shelf; what should you do FIRST? **A:** Quarantine the recalled lot and notify the pharmacist/manager to follow recall procedures because recalled products must be removed from dispensing stock immediately.
1421. **Q:** While entering a new prescription you see "hydralazine 25 mg" but the prescriber's note also says "for heart rate," and the patient profile already has hydroxyzine; what should you do NEXT? **A:** Stop and alert the pharmacist to clarify before filling because hydralazine/hydroxyzine is a LASA risk and the indication note suggests a possible wrong drug.
1422. **Q:** A patient picking up lisinopril says they are newly pregnant and asks if it's safe to keep taking it; what is the BEST technician response? **A:** Refer immediately to the pharmacist because ACE inhibitors like lisinopril can be harmful in pregnancy and require prompt pharmacist intervention.
1423. **Q:** You receive a new eRx for clarithromycin and notice the patient has an active simvastatin prescription; what should you do FIRST? **A:** Pause processing and notify the pharmacist because clarithromycin can markedly increase simvastatin levels, creating a serious interaction risk.
1424. **Q:** A prescription is entered for "metoprolol succinate ER 100 mg daily," but the product pulled is metoprolol tartrate 100 mg; what should you do NEXT? **A:** Stop filling and reselect the correct ER formulation/NDC (and route to pharmacist if already in verification) because tartrate vs succinate is a common error with different release properties.
1425. **Q:** The directions read "levothyroxine 0.125 mg PO qAM" and the patient asks if that is the same as 125 mcg; what should you do BEST? **A:** Confirm the unit conversion ($0.125 \text{ mg} = 125 \text{ mcg}$) and proceed per label while offering pharmacist counseling because levothyroxine dosing is high-alert and unit mix-ups cause errors.

1426. **Q:** A patient drops off a new prescription for sertraline and their profile shows they started phenelzine last week; what should you do FIRST? **A:** Hold the order and alert the pharmacist because SSRI plus MAOI is a contraindicated interaction risk requiring pharmacist assessment before dispensing.
1427. **Q:** You are processing an outpatient order for warfarin and see the patient is also filling apixaban from the same prescriber; what is the NEXT best technician step? **A:** Stop and route to the pharmacist to review possible duplicate anticoagulation because taking both can significantly increase bleeding risk.
1428. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg 1 tab BID x 7 days #14," but you accidentally select amoxicillin 875 mg; what should you do FIRST? **A:** Correct the product selection and verify the NDC/label against the Rx because the combination product is different therapy and wrong-drug selection is a preventable dispensing error.
1429. **Q:** A patient's insulin aspart pen is labeled "inject 12 units TID with meals," and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 41 days because total units = 5 pens $\times$ 3 mL $\times$ 100 units/mL = 1500 units, daily use = 12 $\times$ 3 = 36 units/day, and $1500 \div 36 = 41.6$ (round down to 41 for billing).
1430. **Q:** During fill for "furosemide 40 mg" you notice the shelf bin contains both furosemide and foscarnet bottles mixed together; what should you do NEXT? **A:** Stop filling and separate/reorganize the bin with clear labels (and alert the pharmacist per policy) because furosemide/foscarnet is a LASA storage issue that increases wrong-drug errors.
1431. **Q:** While entering a prescription, you see "glucophage 500 mg" and the patient profile already lists metformin ER 500 mg daily; what should you do NEXT? **A:** Pause entry and alert the pharmacist to possible therapeutic duplication/IR vs ER confusion because brand/generic and release type mismatches can cause dosing errors.
1432. **Q:** A hard copy says "hydroDIURIL 25 mg daily" but the patient says "it's my water pill" and the shelf has both hydrochlorothiazide and hydralazine nearby; what should you do FIRST? **A:** Verify the exact drug and strength using the prescription, NDC, and Tall Man/LASA awareness before filling because similar names/nearby storage increases wrong-drug selection risk.
1433. **Q:** You receive an eRx for "Zithromax Z-Pak" and the patient profile shows a listed allergy to erythromycin causing hives; what is the BEST technician action?

A: Stop processing and notify the pharmacist because azithromycin is a macrolide related to erythromycin and may represent a clinically significant allergy concern.

1434. **Q:** A prescription is written for “predniSONE 10 mg: ii tabs daily x 5 days” and you’re asked what quantity to dispense; what should you enter? **A:** Enter quantity 10 tablets because “ii” means 2 tablets daily for 5 days and $2 \times 5 = 10$.

1435. **Q:** A patient asks at drop-off if “Plavix” is the same as the “clopidogrel” listed in their discharge papers; what should you do BEST? **A:** Confirm that Plavix is the brand for clopidogrel and offer pharmacist counseling for any therapy questions because technicians can clarify brand/generic identity but not provide clinical advice.

1436. **Q:** During product selection for “lamictal 100 mg,” you notice the bottle says “Lamictal ODT 100 mg”; what should you do NEXT? **A:** Stop and pull the correct dosage form for pharmacist check because ODT vs tablet is a formulation difference that should not be interchanged without verifying the prescription.

1437. **Q:** A new prescription reads “Tylenol #3 1 tab q6h prn pain #20,” and the patient asks you to transfer it to another pharmacy; what should you do FIRST? **A:** Refer to the pharmacist and follow controlled-substance transfer rules because Tylenol #3 (codeine) is Schedule III and transfers require pharmacist handling and documentation.

1438. **Q:** A patient’s profile shows brand Coumadin but today’s prescription is for “warfarin 5 mg,” and the patient says they “only take Coumadin”; what should you do NEXT? **A:** Explain that warfarin is the generic for Coumadin and ensure the profile links to the same active medication before dispensing because duplicate entries can lead to double therapy or confusion.

1439. **Q:** A prescription is for “Lantus Solostar inject 18 units subQ at bedtime,” and the package is one box of five 3 mL pens (100 units/mL); what days’ supply should you enter (ignore beyond-use limits)? **A:** Enter 83 days because total units = $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 18 = 83.3$, which rounds down to 83.

1440. **Q:** While interpreting directions, you see “Apply qhs” on a prescription label being printed for triamcinolone cream; what should you do BEST? **A:** Translate “qhs” to “every night at bedtime” on the patient label per policy because clear directions reduce misunderstanding and are commonly tested abbreviation knowledge.

1441. **Q:** While entering an eRx for methotrexate 2.5 mg you see the directions say "take 4 tablets daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and notify the pharmacist immediately because methotrexate is typically dosed weekly for RA and daily dosing is a high-alert error risk.
1442. **Q:** A prescriber phones in "clonazepam 0.5 mg take 1 tablet BID #60 + 2 refills"; what is the BEST technician action? **A:** Enter the prescription but route to the pharmacist to verify/refuse refills as appropriate because Schedule IV refills are limited by federal rules and must be pharmacist-authorized.
1443. **Q:** You receive a stock bottle of refrigerated insulin glargine that was found on the counter at room temperature with an unknown time out of the fridge; what should you do NEXT? **A:** Segregate and hold the product per policy and alert the pharmacist because an unknown temperature excursion makes the product's stability uncertain.
1444. **Q:** During checkout a patient requests Sudafed (pseudoephedrine) and wants to buy the maximum amount allowed; what should you do FIRST? **A:** Request valid photo ID and begin the required sales log process because CMEA requires ID and documentation before completing pseudoephedrine sales.
1445. **Q:** A new eRx for simvastatin 40 mg daily arrives and the patient profile shows an active prescription for clarithromycin started yesterday; what should you do BEST? **A:** Stop and refer to the pharmacist because clarithromycin can dangerously increase simvastatin levels and needs pharmacist intervention.
1446. **Q:** You are filling hydromorphone 4 mg and notice the label prints "HYDRoMorphone" but the stock bottle you pulled is "morphine" due to similar packaging; what should you do NEXT? **A:** Stop filling and reselect using barcode/NDC verification because this is a LASA error with a high-alert opioid and must be corrected before verification.
1447. **Q:** A patient's caregiver asks you to fax their complete medication profile to a new doctor's office; what should you do FIRST under HIPAA? **A:** Verify patient authorization/identity and confirm the minimum necessary information before sending because PHI disclosure must be permitted and limited.
1448. **Q:** A wholesaler shipment arrives and several sealed bottles have no readable 2D barcode/lot/expiration on the package; what should you do BEST? **A:** Set the bottles aside and alert the pharmacist/manager because DSCSA requires product identifiers and unreadable/missing identifiers are a suspect product risk.

1449. **Q:** A patient returns a sealed, unexpired bottle of amlodipine that was dispensed yesterday and asks for a refund; what should you do NEXT? **A:** Follow pharmacy policy and do not return it to stock for redispensing because returned prescriptions generally cannot be reused due to safety/chain-of-custody concerns.
1450. **Q:** A prescription reads "levothyroxine 50 mcg take 1 tab qd" but the prescriber wrote "dispense Synthroid only" and your system defaulted to generic substitution; what should you do NEXT? **A:** Stop and correct the product selection per the prescriber's DAW/brand instruction and have the pharmacist verify because substituting against the DAW request is a common legal/workflow error.
1451. **Q:** An eRx for azithromycin 200 mg/5 mL says "Give 12 mg/kg PO daily x 5 days" for a child weighing 44 lb; what daily mL dose should you enter? **A:** Enter 12 mL daily because $44 \text{ lb} \div 2.2 = 20 \text{ kg}$, $12 \text{ mg/kg} \times 20 \text{ kg} = 240 \text{ mg/day}$, and $240 \text{ mg} \div (200 \text{ mg}/5 \text{ mL}) = 6 \text{ mL}$? wait—($200 \text{ mg}/5 \text{ mL} = 40 \text{ mg/mL}$) so $240 \text{ mg} \div 40 \text{ mg/mL} = 6 \text{ mL}$, preventing a dose-entry error.
1452. **Q:** You are entering "metformin ER 500 mg take ii tabs qpm with evening meal #180"; what days' supply should you calculate and why? **A:** Enter 90 days because ii tabs qpm = 2 tablets/day and $180 \text{ tablets} \div 2 \text{ per day} = 90$, matching correct days' supply for billing.
1453. **Q:** A prescription says "predniSONE 10 mg: take 3 tabs po bid x 5 days #?"; what quantity should you dispense? **A:** Dispense 30 tablets because 3 tabs BID = 6 tabs/day and $6 \times 5 \text{ days} = 30$, ensuring the dispensed quantity matches the directions.
1454. **Q:** While typing a new order you see "MSO4 2 mg IV q2h PRN pain"; what should you do FIRST? **A:** Stop and clarify with the pharmacist/prescriber because "MSO4" is an error-prone abbreviation that can be confused with morphine vs magnesium sulfate, preventing a high-risk mix-up.
1455. **Q:** A patient brings a prescription for timolol 0.5% ophthalmic "instill 1 gtt OU BID," and you are asked to compute days' supply for a 5 mL bottle using 20 gtt/mL; what should you enter? **A:** Enter 25 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ drops}$ and $1 \text{ gtt OU BID} = 4 \text{ drops/day}$, so $100 \div 4 = 25$, supporting accurate third-party billing.
1456. **Q:** An eRx for fluticasone propionate nasal spray 50 mcg/spray says "2 sprays each nostril daily," and the package is 120 sprays; what days' supply should be

entered? **A:** Enter 30 days because 2 sprays per nostril daily = 4 sprays/day and $120 \div 4 = 30$, avoiding over/understated days' supply.

1457. **Q:** A prescriber writes "digoxin 0.125 mg take 1 tab qd" but the technician accidentally selects digoxin 0.25 mg in order entry; what is the BEST next step? **A:** Stop entry and correct to the prescribed strength before printing/filling because strength-selection errors are a common order-entry cause of wrong-dose dispensing.
1458. **Q:** A hardcopy reads "hydroxyzine 25 mg 1 tab tid prn itching" but the sig is typed as "hydralazine"; what should you do NEXT? **A:** Halt the fill and fix the drug selection after verifying the original prescription because hydroxyzine vs hydralazine is a LASA pair and wrong-drug errors can cause serious harm.
1459. **Q:** You receive an eRx for "levETIRAcetam 500 mg take 1 tab po BID #60," but the patient profile shows they already fill levETIRAcetam 1000 mg BID; what should you do BEST? **A:** Route to the pharmacist to assess for dose change/duplication before processing because overlapping antiepileptic strengths may indicate a prescriber-intended adjustment or error.
1460. **Q:** A prescription for amoxicillin-clavulanate suspension reads "400 mg/57 mg per 5 mL; give 5 mL PO q12h x 10 days"; what quantity (mL) should you dispense? **A:** Dispense 100 mL because 5 mL every 12 hours = 10 mL/day and $10 \text{ days} \times 10 \text{ mL/day} = 100 \text{ mL}$, ensuring the bottle quantity matches the full course.
1461. **Q:** A new eRx is for Zestril 10 mg daily, but the patient profile shows an active lisinopril 10 mg daily from another prescriber; what should you do BEST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because Zestril is lisinopril and duplication increases safety risk.
1462. **Q:** During product selection you notice the label says "Lamictal 25 mg" but you pulled Lamisil 250 mg tablets from the shelf; what should you do FIRST? **A:** Stop filling and reselect the correct NDC using barcode/NDC verification because Lamictal (lamotrigine) and Lamisil (terbinafine) are LASA and a wrong-drug error is high risk.
1463. **Q:** A prescription is written for "glipiZIDE 10 mg take 1 tab BID," but the system auto-suggests glimepiride due to similar names; what is the BEST action? **A:** Select glipizide exactly as prescribed and verify drug/name/strength before printing because sulfonylureas are high-alert and name confusion can cause hypoglycemia.

1464. **Q:** A patient asks what Crestor is “for” while you’re ringing up; what should you do NEXT? **A:** Refer to the pharmacist for counseling while you can state it’s rosuvastatin commonly used for cholesterol because OBRA-90 counseling is pharmacist-led and ensures accurate education.
1465. **Q:** You receive a new prescription for Bactrim DS, and the profile lists a serious sulfa allergy; what should you do FIRST? **A:** Stop processing and notify the pharmacist to evaluate the allergy conflict because Bactrim contains sulfamethoxazole and could trigger a severe reaction.
1466. **Q:** A provider sends “Xanax 0.5 mg take 1 tab TID #90 with 2 refills”; what should you do BEST? **A:** Hold the prescription and route to the pharmacist because alprazolam is Schedule IV and controlled-substance refills must be validated per law and policy before dispensing.
1467. **Q:** Order entry: “cephalexin 500 mg 1 cap QID x 7 days”; what quantity should you dispense? **A:** Dispense 28 capsules because $4 \text{ doses/day} \times 7 \text{ days} = 28 \text{ total doses}$.
1468. **Q:** A prescription is for “sertraline 50 mg take 1 tab daily #30,” but you accidentally typed “sildenafil” due to similar spelling; what should you do NEXT? **A:** Correct the entry and ensure the drug matches the written order before verification because sertraline (SSRI) and sildenafil (PDE5 inhibitor) are unrelated and this is a serious wrong-drug risk.
1469. **Q:** A patient on the phone says they take “Plavix” and asks if it’s the same as “clopidogrel” on their bottle; what is the BEST response within technician scope? **A:** Confirm that Plavix is the brand name for clopidogrel and offer pharmacist counseling because brand/generic recognition helps prevent duplication and confusion.
1470. **Q:** You are calculating days’ supply for Lantus SoloStar 100 units/mL: “Inject 24 units subcut daily,” dispense one box of five 3 mL pens; what days’ supply should you enter (use total units only)? **A:** Enter 62 days because $5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 24 \text{ units/day} = 62.5$, rounded down to 62 days.
1471. **Q:** A new eRx is for “Lasix 40 mg take 1 tab PO daily,” but the prescriber notes say “for blood pressure”; what should you do NEXT? **A:** Enter the order as written and flag the indication mismatch for pharmacist review because furosemide is primarily a loop diuretic and the pharmacist should confirm appropriateness.

1472. **Q:** At drop-off a patient asks if "Norvasc" is an antibiotic; what is the BEST technician response? **A:** Tell them Norvasc is amlodipine, commonly used for high blood pressure/chest pain, and offer pharmacist counseling for details because technicians can provide basic identification but not clinical advice.
1473. **Q:** While typing, you see "metFORMIN ER 500 mg" but the product selected is immediate-release metformin 500 mg; what should you do FIRST? **A:** Stop and correct to the ER dosage form and NDC because ER vs IR is a critical difference that can change dosing and safety.
1474. **Q:** A prescription reads "predniSONE 20 mg: take ii tabs PO daily x 5 days"; what quantity should you dispense? **A:** Dispense 10 tablets because ii = 2 tablets daily for 5 days equals $2 \times 5 = 10$.
1475. **Q:** You receive an eRx for "Eliquis 5 mg take 1 tab BID," and the profile shows active "Xarelto 20 mg daily"; what should you do BEST? **A:** Hold processing and notify the pharmacist to assess therapeutic duplication because both are anticoagulants and duplicative therapy is high-risk.
1476. **Q:** During product selection you notice "hydroxyzine" and "hydralazine" stored near each other and the barcode scan fails; what is the NEXT best action? **A:** Stop, re-scan with the correct stock bottle or manually verify the NDC against the label and then alert the pharmacist if any mismatch occurred because barcode/NDC verification prevents LASA errors.
1477. **Q:** A provider writes "MS Contin 15 mg take 1 tab q12h #60," and the patient asks if it's the same as "morphine IR"; what should you do NEXT? **A:** Refer to the pharmacist and explain you'll have them discuss it because MS Contin is extended-release morphine and release-form differences require pharmacist counseling for safe use.
1478. **Q:** Order entry shows "albuterol 0.083% nebulizer solution: use 1 vial via neb q6h PRN," and the box contains 25 vials; using a maximum-use days' supply, what should you enter? **A:** Enter 6 days because q6h max is 4 vials/day and $25 \div 4 = 6.25$, which is entered as 6 whole days for maximum-use billing.
1479. **Q:** A patient brings a new prescription for isotretinoin and asks to pick it up tomorrow; what is the BEST technician action? **A:** Notify the pharmacist and follow iPLEDGE/REMS workflow requirements before processing because isotretinoin has a REMS with strict dispensing authorization steps.

1480. **Q:** While unpacking a tote, you find a sealed bottle of refrigerated insulin that arrived warm with a temperature indicator showing out of range; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager per cold-chain policy because temperature excursions can make insulin ineffective and it should not be dispensed.
1481. **Q:** While entering an eRx for Humalog U-100 "inject 10 units TID with meals," the prescriber also sent glargine U-100 "inject 30 units nightly," but the patient profile already has active NovoLog; what should you do BEST? **A:** Stop processing and notify the pharmacist to review for therapeutic duplication and clarify the intended rapid-acting insulin, because selecting the wrong insulin is a high-alert error.
1482. **Q:** A handwritten prescription for oxycodone/APAP 5/325 says "take 1 tab q4-6h prn pain #30" but has no prescriber signature; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist because a controlled-substance prescription without a valid signature is not processable and needs prescriber follow-up.
1483. **Q:** A patient asks to transfer their remaining refills of alprazolam from another pharmacy to yours; what is the BEST technician action? **A:** Refer to the pharmacist to handle the transfer details because controlled-substance transfers have strict limitations and must be completed by a pharmacist per law/policy.
1484. **Q:** During product selection you scan NDC for "levothyroxine 50 mcg" but the bottle in hand is "liothyronine 5 mcg" and the shelf labels look similar; what should you do NEXT? **A:** Stop, return the product, select the correct NDC, and rescan/verify because barcode mismatch indicates a wrong-drug selection (LASA risk).
1485. **Q:** A prescriber writes "U-500 regular insulin inject 80 units BID," but the pharmacy only has U-100 syringes on hand; what should you do BEST? **A:** Pause filling and alert the pharmacist because U-500 dosing/administration requires specialized education and devices to prevent severe overdose.
1486. **Q:** A bottle of amlodipine 10 mg arrives from the wholesaler with a broken seal and tablets visible in the shipping tote; what should you do FIRST? **A:** Segregate and quarantine the bottle as suspect product and notify the pharmacist/manager because compromised packaging may indicate contamination or diversion (DSCSA/safety).

1487. **Q:** A patient presents a new prescription for nitroglycerin SL tablets and says they also take tadalafil; what should you do NEXT? **A:** Immediately involve the pharmacist before dispensing because nitrates and PDE5 inhibitors can cause dangerous hypotension and require pharmacist counseling/intervention.
1488. **Q:** You receive a manufacturer recall notice for specific lot numbers of valsartan due to contamination; what is the BEST technician step to start the recall process? **A:** Pull inventory and identify matching NDC/lot numbers to remove from stock and notify the pharmacist, because recalls are lot-specific and must be prevented from dispensing.
1489. **Q:** A prescription is for cephalexin 500 mg "1 cap QID x 7 days"; what quantity should you dispense? **A:** Dispense 28 capsules because $4 \text{ doses/day} \times 7 \text{ days} = 28 \text{ total doses}$.
1490. **Q:** At drop-off, a parent says their child developed hives after starting amoxicillin yesterday and asks if they should keep taking it; what should you do FIRST? **A:** Escalate to the pharmacist immediately because possible allergic reaction requires prompt clinical triage and the technician must not provide medical advice.
1491. **Q:** While entering a new eRx for lisinopril 20 mg daily, you notice the patient profile already has benazepril 20 mg daily as active; what should you do BEST? **A:** Stop processing and alert the pharmacist to possible duplicate ACE-inhibitor therapy because therapeutic duplication can cause harm and requires pharmacist review.
1492. **Q:** A patient at pickup asks what metformin is used for and whether it can upset the stomach; what is the BEST technician response? **A:** Refer the patient to the pharmacist for counseling because indications and side effects require pharmacist counseling while the technician can facilitate the handoff.
1493. **Q:** You receive an eRx for azithromycin 250 mg "take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should you dispense? **A:** Dispense 6 tablets because 2 on day 1 plus 1 tablet for the next 4 days equals 6 total tablets.
1494. **Q:** During product selection for hydroxyzine 25 mg, you almost grab hydralazine 25 mg from a nearby bin with a similar label; what should you do NEXT? **A:** Stop and reselect using barcode/NDC verification because hydroxyzine/hydralazine is a LASA mix-up that can lead to serious dispensing errors.

1495. **Q:** A refill request comes in for sertraline, but the patient says they stopped it weeks ago due to feeling worse and wants to restart today; what should you do FIRST? **A:** Route the patient to the pharmacist because restarting antidepressants and worsening symptoms are safety concerns requiring pharmacist assessment.
1496. **Q:** While entering a new prescription for simvastatin 40 mg, the medication history shows the patient is also on clarithromycin from an urgent care visit; what is the BEST technician action? **A:** Pause processing and notify the pharmacist because clarithromycin can significantly increase simvastatin levels and the interaction needs pharmacist intervention.
1497. **Q:** A prescription is written for prednisone 10 mg "take 4 tablets daily for 5 days"; what quantity should you dispense? **A:** Dispense 20 tablets because $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$.
1498. **Q:** A patient drops off a new prescription for tramadol and says they also take clonazepam every night; what should you do BEST? **A:** Alert the pharmacist before filling because opioid-like analgesics with benzodiazepines increase sedation/respiratory depression risk and require pharmacist review.
1499. **Q:** While checking fridge items, you find a vial of reconstituted amoxicillin-clavulanate suspension with a beyond-use date that passed yesterday; what should you do FIRST? **A:** Remove it from dispensing stock and follow disposal policy because expired/reconstituted products are not dispensable and pose patient-safety risk.
1500. **Q:** An eRx for levothyroxine is entered as "take 1 tab qd," but the prescriber selected "liothyronine" in the drug field; what should you do NEXT? **A:** Stop and bring it to the pharmacist to clarify because T3 vs T4 thyroid products are not interchangeable and the prescriber selection must be verified.
1501. **Q:** An eRx reads "furosemide 20 mg, i po qam; qty 90," and you notice "i" could be misread as "1" or "10"; what should you do FIRST in order entry? **A:** Clarify the intended dose with the pharmacist/prescriber before processing because unsafe/ambiguous notation can cause a tenfold dosing error.
1502. **Q:** A prescription says "Augmentin ES-600 600 mg/5 mL: give 7.5 mL PO BID x 10 days"; what quantity (mL) should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$.

1503. **Q:** A patient's directions for latanoprost 0.005% are "instill 1 gtt OU qhs," and you are entering days' supply for a 2.5 mL bottle using 20 gtt/mL; what days' supply should you enter? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ gtt}$ and OU qhs uses 2 gtt/day so $50 \div 2 = 25 \text{ days}$.
1504. **Q:** While typing an eRx, you see "clonidine 0.1 mg take 1 tab bid" but the selected product is "clonazepam 0.5 mg"; what should you do NEXT? **A:** Stop and route to the pharmacist to verify/correct the drug selection because clonidine and clonazepam are LASA and substituting would be a serious error.
1505. **Q:** An insulin order is "insulin lispro U-100: inject 6 units SQ TID with meals," and the patient is getting one 10 mL vial; what days' supply should you enter? **A:** Enter 55 days because U-100 has 100 units/mL so $10 \text{ mL} = 1000 \text{ units}$ and $6 \times 3 = 18 \text{ units/day}$, so $1000 \div 18 = 55 \text{ days}$ (rounded down).
1506. **Q:** A prescription reads "levothyroxine 25 mcg: take 0.05 mg PO daily"; what is the correct mcg dose to enter and why? **A:** Enter 50 mcg because $0.05 \text{ mg} \times 1000 \text{ mcg/mg} = 50 \text{ mcg}$, preventing a decimal-place conversion error.
1507. **Q:** A new eRx for "metoprolol succinate ER 50 mg daily" is entered, but the scanned NDC on the bottle is metoprolol tartrate IR 50 mg; what should you do FIRST? **A:** Stop filling and select the correct ER product/NDC (and alert the pharmacist if needed) because IR vs ER is not interchangeable and can change dosing frequency.
1508. **Q:** An order says "amiodarone 200 mg: take ii tabs PO daily," and the prescriber also typed "take 2 tablets daily"; what quantity should you dispense for 30 days? **A:** Dispense 60 tablets because "ii" means 2 and $2 \text{ tabs/day} \times 30 \text{ days} = 60$.
1509. **Q:** A patient uses fluticasone nasal spray "2 sprays in each nostril daily," and you're billing a 16 g bottle labeled 120 sprays total; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril} \times 2 \text{ nostrils} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$.
1510. **Q:** During order entry you notice the prescriber wrote "U" for units on a heparin order ("heparin 5000 U SQ q12h"); what is the BEST technician action? **A:** Flag it for pharmacist review and ensure "units" is entered fully because "U" is error-prone and can be misread as a zero or a four, risking a dosing error.
1511. **Q:** While entering an eRx you see "Celebrex 200 mg daily" but the selected product is citalopram 20 mg; what should you do NEXT? **A:** Stop order entry and

re-select/verify the correct drug (celecoxib) and alert the pharmacist if needed because this is a LASA name mix-up with a high risk of wrong-drug dispensing.

1512. **Q:** A patient asks for a refill of Zocor and the profile shows simvastatin is already active; what is the BEST technician response? **A:** Explain that Zocor is simvastatin and proceed using the same active medication on the profile (confirming strength/directions) to prevent duplicate therapy entries and billing confusion.
1513. **Q:** A new prescription is for lisinopril 20 mg daily and the profile shows active losartan 50 mg daily; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess possible therapeutic duplication/ACEI-ARB overlap because it can be clinically significant and is outside technician scope to decide.
1514. **Q:** An order reads "predniSONE 10 mg: take 2 tabs bid x 5 days"; how many tablets should be dispensed? **A:** Dispense 20 tablets because 2 tablets twice daily equals 4 tablets/day and $4 \times 5 \text{ days} = 20 \text{ tablets}$.
1515. **Q:** The prescriber writes "MSO4 2 mg IV q2h prn pain" on a hospital discharge med list you're transcribing; what should you do NEXT? **A:** Hold and refer to the pharmacist to clarify because MSO4 is an unsafe abbreviation that can be confused with MgSO4, risking a wrong-drug error.
1516. **Q:** You receive a transfer request for a patient's alprazolam prescription from another pharmacy; what should you do FIRST? **A:** Route the request to the pharmacist because controlled-substance transfers have strict requirements and must be handled by a pharmacist per federal rules and pharmacy policy.
1517. **Q:** During product selection you scan a bottle labeled "hydroxyzine" for an eRx that is for "hydralazine"; what is the BEST action? **A:** Stop filling and select the correct medication by verifying name/strength and NDC because hydroxyzine vs hydralazine is a common LASA pair with serious wrong-drug potential.
1518. **Q:** A patient is prescribed omeprazole 20 mg and asks if it's the same as Prilosec OTC; what should you say? **A:** Confirm they are the same active ingredient (omeprazole) and verify the ordered strength/directions because brand vs generic recognition prevents duplication and dosing errors.
1519. **Q:** You are billing an albuterol HFA inhaler labeled 200 actuations for directions "inhale 2 puffs QID"; what days' supply should you enter? **A:** Enter 25 days because $2 \text{ puffs } 4 \text{ times/day} = 8 \text{ puffs/day}$ and $200 \div 8 = 25 \text{ days}$.

1520. **Q:** While unpacking an order, you notice a sealed case of insulin pens arrived warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Segregate the shipment and notify the pharmacist/manager to follow excursion procedures because compromised cold-chain products are suspect and should not be stocked or dispensed.
1521. **Q:** A new eRx is for Glucophage 500 mg BID but the product selected is Glucotrol 5 mg; what should you do NEXT? **A:** Stop processing and reselect metformin (Glucophage) then flag for pharmacist check because metformin vs glipizide is a high-risk diabetes mix-up.
1522. **Q:** A patient says their new "water pill" is Lasix and asks if that's the same as furosemide; what is the BEST technician response? **A:** Confirm Lasix is the brand name for furosemide and offer to have the pharmacist counsel because brand/generic reassurance and counseling support safe use.
1523. **Q:** While entering directions for levothyroxine, the prescriber wrote "take 1 tab qd with breakfast"; what should you do FIRST? **A:** Pause and refer to the pharmacist because levothyroxine is typically taken on an empty stomach and the discrepancy may affect therapy.
1524. **Q:** A prescription is written for "metoprolol ER 50 mg daily," but the selected item is metoprolol tartrate 50 mg; what should you do NEXT? **A:** Stop and correct the selection to metoprolol succinate ER (not tartrate) and alert the pharmacist because IR vs ER mix-ups can cause dosing errors.
1525. **Q:** You are calculating days' supply for gabapentin 300 mg with sig "1 cap TID" and quantity #270; what days' supply should you enter? **A:** Enter 90 days because $270 \text{ capsules} \div 3 \text{ per day} = 90 \text{ days}$ for accurate billing and refill timing.
1526. **Q:** At drop-off, a patient reports swelling after taking Bactrim and now has a new prescription for sulfamethoxazole/trimethoprim; what should you do FIRST? **A:** Stop the order and notify the pharmacist immediately because a reported sulfonamide reaction is a safety-critical allergy concern.
1527. **Q:** During profile review you see active sertraline and a new eRx for paroxetine; what is the BEST technician action? **A:** Hold and route to the pharmacist to assess therapeutic duplication/transition because two SSRIs together increases risk and requires clinical decision-making.
1528. **Q:** You receive a refill request for atorvastatin but the prescriber note says the patient was switched to rosuvastatin last month; what should you do NEXT? **A:**

Verify the active medication list and send to the pharmacist for clarification because statin duplication or an outdated refill can cause unintended double therapy.

1529. **Q:** A stock bottle label for lamotrigine looks similar to terbinafine and you notice both nearby during filling; what is the BEST step before dispensing? **A:** Use barcode/NDC verification and separate the look-alike products because LASA prevention is a key technician safety control.
1530. **Q:** You are entering an insulin order for Humalog U-100: "inject 12 units before meals TID," dispensing one 10 mL vial (100 units/mL); what days' supply should you enter? **A:** Enter 27 days because $10\text{ mL} \times 100\text{ units/mL} = 1000\text{ units}$ and $12\text{ units} \times 3/\text{day} = 36\text{ units/day}$, so $1000 \div 36 = 27\text{ days}$ (rounded down).
1531. **Q:** While typing a new prescription, you see "MSO4 2 mg IV q2h PRN" on a hospital discharge order; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can cause morphine/magnesium sulfate errors.
1532. **Q:** A prescriber eRx's "Adderall XR 20 mg take 1 daily" with 3 refills; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because Schedule II controlled substances cannot be refilled and require correction before dispensing.
1533. **Q:** A patient asks to transfer their oxycodone 5 mg prescription from another pharmacy to yours; what should you do NEXT? **A:** Refer to the pharmacist because transferring controlled-substance prescriptions has strict requirements and must be handled by a pharmacist per law/policy.
1534. **Q:** During will-call pickup, the patient says they are allergic to aspirin and notices their new prescription is for "Bayer low-dose aspirin"; what should you do FIRST? **A:** Stop the sale and alert the pharmacist to assess the allergy and appropriateness because aspirin is the same drug as the brand requested.
1535. **Q:** While unpacking totes, you find a bottle of hydrocodone/acetaminophen with a broken seal and loose tablets inside; what should you do FIRST? **A:** Separate it from stock and notify the pharmacist/manager because compromised controlled-substance packaging is a safety and diversion risk.
1536. **Q:** You receive a manufacturer recall notice for specific lots of losartan tablets and see one matching lot on the shelf; what should you do NEXT? **A:** Remove

the affected lot from dispensing inventory and follow recall procedure because recalled lots must not be dispensed and must be tracked/returned per policy.

1537. **Q:** A new eRx for "hydralazine 25 mg TID" is entered, but you accidentally selected hydroxyzine 25 mg; what should you do NEXT? **A:** Stop filling and correct the product selection by verifying NDC/barcode because this LASA error could cause the wrong medication to be dispensed.
1538. **Q:** A patient's profile shows lisinopril, and a new prescription arrives for enalapril; what is the BEST technician action? **A:** Flag it for pharmacist review because both are ACE inhibitors and may represent therapeutic duplication requiring clinical evaluation.
1539. **Q:** You are billing fluticasone nasal spray 50 mcg with directions "2 sprays each nostril daily" and the package is 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $120 \text{ total sprays} \div (2 \text{ sprays/nostril} \times 2 \text{ nostrils}) = 4 \text{ sprays/day} = 30 \text{ days}$.
1540. **Q:** A patient wants pseudoephedrine and asks to buy multiple boxes at once; what should you do FIRST at the register? **A:** Request a valid photo ID and complete the required log/check limits because CMEA requires ID and sales tracking to prevent exceeding legal thresholds.
1541. **Q:** A new eRx is for "Keflex 500 mg PO q6h" but the patient profile lists anaphylaxis to penicillin; what should you do FIRST? **A:** Stop processing and alert the pharmacist to assess beta-lactam cross-reactivity risk before dispensing, because allergy safety review is required.
1542. **Q:** While typing, you see "levothyroxine 0.1 mg daily" and the prescriber note says "100 mcg"; what is the BEST technician action? **A:** Enter the dose as 100 mcg (0.1 mg) and ensure the correct strength selection, because mg-to-mcg equivalence prevents a 1000-fold error.
1543. **Q:** A prescription reads "metformin ER 500 mg 1 tab BID #60," but the product selected is metformin IR; what should you do NEXT? **A:** Stop and reselect the ER formulation (and route to pharmacist if already in verification), because IR vs ER is a different dosage form and cannot be interchanged by the technician.
1544. **Q:** A patient drops off a new prescription for "Zithromax" and asks what generic it is so they can compare prices; what should you answer? **A:** Tell them Zithromax is azithromycin, because brand/generic recognition supports accurate counseling and billing while staying within technician scope.

1545. **Q:** Directions say “use 2 puffs BID” for an albuterol HFA inhaler (200 actuations); what days’ supply should you enter? **A:** Enter 50 days because 2 puffs twice daily equals 4 puffs/day and $200 \div 4 = 50$.
1546. **Q:** During order entry you receive “predniSONE 10 mg take i po tid x 5 days”; what quantity should you calculate to dispense? **A:** Dispense 15 tablets because i = 1 tablet, TID = 3/day, and $3 \times 5 = 15$.
1547. **Q:** You notice a new prescription for “Lopressor 50 mg BID” and the patient’s active meds include Toprol-XL 50 mg daily; what should you do BEST? **A:** Flag for pharmacist review of duplicate beta-blocker therapy, because Lopressor and Toprol-XL are both metoprolol products with different release forms.
1548. **Q:** A high-alert insulin pen box has a different NDC than the one on the prescription label you printed; what should you do FIRST? **A:** Stop filling and match the product to the exact NDC/barcode before continuing, because insulin selection errors can cause serious harm.
1549. **Q:** A prescriber writes “take 1 tab qd” for “HCTZ” but no strength is listed; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist to obtain the missing strength, because incomplete directions prevent accurate and legal dispensing.
1550. **Q:** A patient asks if “Plavix” is a blood thinner and wants to buy OTC ibuprofen for daily use; what should you do BEST? **A:** Refer the patient to the pharmacist, because Plavix (clopidogrel) plus NSAIDs can increase bleeding risk and requires pharmacist counseling.
1551. **Q:** You are entering a prescription for amoxicillin/clavulanate 400 mg/57 mg per 5 mL with sig “10 mL PO BID x 7 days”; what quantity (mL) should you dispense? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$ for the full course.
1552. **Q:** A prescription is for levothyroxine 75 mcg daily, but you accidentally selected levothyroxine 88 mcg in the system; what should you do NEXT? **A:** Stop and correct the drug/strength selection before printing labels because strength selection is a key order-entry safety check.
1553. **Q:** You are billing latanoprost 0.005% 2.5 mL with directions “1 gtt OU qHS”; what days’ supply should you enter using 20 drops/mL? **A:** Enter 25 days because $2.5 \text{ mL} \times 20 \text{ drops/mL} = 50 \text{ drops}$ and $50 \text{ drops} \div 2 \text{ drops/day} = 25 \text{ days}$.

1554. **Q:** A new eRx says "hydromorphone 2 mg take 1 tab q4-6h prn pain," and you notice the prescriber note mentions "Dilaudid"; what is the BEST technician action during entry? **A:** Treat Dilaudid as hydromorphone and ensure the correct generic/brand mapping is selected because hydromorphone is a high-alert opioid and name confusion can cause errors.
1555. **Q:** While typing directions, you see "U-100 insulin regular inject 8 units SQ TID with meals" and the vial is 10 mL; what days' supply should you calculate based on total units only? **A:** Enter 41 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \times 3 = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$ rounded down to 41 days.
1556. **Q:** A prescription reads "APAP #3 i-ii tabs q6h prn pain" and you are unsure what "APAP #3" means; what should you do FIRST? **A:** Pause entry and clarify with the pharmacist because APAP #3 is acetaminophen/codeine and controlled-substance components must be interpreted correctly.
1557. **Q:** You receive a prescription for digoxin 0.25 mg daily, but the patient's profile shows digoxin 0.125 mg daily active; what should you do BEST? **A:** Route to the pharmacist for duplication/dose-change review because digoxin is narrow-therapeutic-index and dose discrepancies are safety-critical.
1558. **Q:** During order entry for clopidogrel, the prescriber wrote "Plavix 75 mg qd #90"; what is the correct generic name you should select? **A:** Select clopidogrel because Plavix is the brand name and choosing the correct generic improves accurate labeling and billing.
1559. **Q:** A prescription is for ondansetron ODT 4 mg "1 tab q8h prn nausea" #12, but you selected ondansetron 4 mg tablets (not ODT); what should you do NEXT? **A:** Stop and change to the ODT dosage form because tablets vs ODT are not interchangeable in dispensing and affect patient use.
1560. **Q:** A doctor orders "ceftriaxone 1 g IV now," and the vial on hand is labeled 1 g/10 mL after reconstitution; how many mL should you prepare to supply 1 g? **A:** Prepare 10 mL because the concentration is 1 g per 10 mL, so 1 g requires the full 10 mL volume.
1561. **Q:** While entering a prescription for prednisone 10 mg tablets with sig "Take ii tabs PO BID x 5 days," what quantity should you enter to dispense? **A:** Enter 20 tablets because $2 \text{ tablets/dose} \times 2 \text{ doses/day} \times 5 \text{ days} = 20$, ensuring the dispensed quantity matches the written duration.

1562. **Q:** A prescription for cephalexin 250 mg/5 mL suspension says "5 mL PO QID x 10 days"; how many milliliters should be dispensed? **A:** Dispense 200 mL because $5 \text{ mL} \times 4 \text{ times/day} \times 10 \text{ days} = 200 \text{ mL}$, preventing an underfill that could shorten therapy.
1563. **Q:** You are processing albuterol 90 mcg inhaler (200 actuations) with directions "inhale 2 puffs QID"; what days' supply should you submit? **A:** Submit 25 days because $200 \text{ puffs} \div (2 \text{ puffs} \times 4 \text{ times/day}) = 25 \text{ days}$, which supports accurate third-party billing.
1564. **Q:** An order is for furosemide oral solution 10 mg/mL with sig "Give 2.5 mL PO daily"; how many mg per dose should you calculate for entry? **A:** Enter 25 mg per dose because $10 \text{ mg/mL} \times 2.5 \text{ mL} = 25 \text{ mg}$, reducing strength/dose entry errors.
1565. **Q:** During entry you see "morphine sulfate ER 30 mg take 1 tab q12h," but the prescriber note says "MS Contin," and you accidentally selected morphine IR; what should you do NEXT? **A:** Stop and correct the selection to morphine ER and route for pharmacist verification because IR vs ER is a high-risk dosage-form error.
1566. **Q:** A prescription reads "metformin 500 mg 1 tab PO BID with meals #60," but you accidentally typed TID in the sig; what is the BEST next step before sending to verification? **A:** Correct the directions to BID to match the hard copy/eRx because accurate sig transcription is a technician responsibility and prevents wrong dosing.
1567. **Q:** You receive an eRx for "Lantus SoloStar inject 18 units SQ HS," and the package is a box of five 3 mL pens (100 units/mL); what days' supply should you calculate using total units only? **A:** Enter 83 days because $\text{total units} = 5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 18 \text{ units/day} = 83.3$ days (submit 83 to avoid overstating supply).
1568. **Q:** A written prescription says "Take 1 cap PO qd" and the drug line reads "fluoxetine" with no strength; what should you do FIRST? **A:** Hold the prescription and ask the pharmacist to contact the prescriber because missing strength is incomplete and technicians cannot infer it.
1569. **Q:** A prescription is written "warfarin 5 mg: take 1 tab PO qd," and during order entry you notice the profile already has an active warfarin 2 mg regimen; what should you do BEST? **A:** Pause processing and alert the pharmacist to evaluate

potential duplication/therapy change because warfarin is high-alert and dose changes require pharmacist review.

1570. **Q:** While typing "U-500 insulin regular inject 10 units SQ BID," you realize the prescriber did not specify a U-500 syringe/pen and the patient previously used U-100 syringes; what should you do NEXT? **A:** Stop entry and refer to the pharmacist for clarification/counseling need because U-500 dosing is error-prone and device/syringe mismatch can cause severe overdosing.
1571. **Q:** While typing an eRx for hydroxyzine, the allergy field shows "cetirizine—hives"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to evaluate potential cross-sensitivity since both are antihistamines and allergy-related decisions require pharmacist review.
1572. **Q:** A caller wants an early refill of oxycodone/APAP because they "lost the bottle"; what is the BEST technician action? **A:** Collect details and immediately route to the pharmacist because controlled-substance replacement requests require pharmacist judgment and documentation.
1573. **Q:** You receive a written prescription for alprazolam with directions and quantity but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because missing required prescriber authentication makes it invalid to dispense as written.
1574. **Q:** During pickup, a patient asks what "Tall Man lettering" like hydrOXYzine vs hydrALAzine is meant to prevent; what is the BEST response? **A:** Explain it helps reduce look-alike/sound-alike mix-ups by highlighting different letter patterns to improve product selection safety.
1575. **Q:** A refrigerated vaccine shipment arrives warm and the temperature indicator shows an excursion; what should you do FIRST? **A:** Segregate and label it "do not use" and notify the pharmacist/manager because temperature excursions make product integrity uncertain.
1576. **Q:** An eRx is written for Adderall XR 20 mg "1 cap daily" with "Refills: 2"; what should you do NEXT? **A:** Stop and refer to the pharmacist because Schedule II prescriptions cannot be refilled and must be corrected with the prescriber.
1577. **Q:** A bottle of levothyroxine tablets on the shelf has an unreadable lot number; what is the BEST next step? **A:** Remove it from active inventory and notify the pharmacist/manager because lot identification is needed for recalls and traceability.

1578. **Q:** You are filling insulin regular U-100 but notice the selected NDC is insulin lispro (a different rapid-acting product); what should you do NEXT? **A:** Stop filling and reselect using the correct NDC/barcode because wrong-insulin selection is a high-alert error risk.
1579. **Q:** A prescription for azithromycin 250 mg tablets reads "Take 2 tabs today, then 1 tab daily x 4 days"; what quantity should you enter? **A:** Enter quantity 6 tablets because 2 tablets on day 1 plus 1 tablet for the next 4 days totals 6.
1580. **Q:** A patient asks to return an opened bottle of atorvastatin because it "didn't work"; what should you do BEST? **A:** Follow pharmacy policy and route to the pharmacist since returned, opened prescription medications generally cannot be returned to stock and require proper handling.
1581. **Q:** While entering a new prescription for lisinopril, the patient says they are also taking "Zestril" from an old bottle; what should you do FIRST? **A:** Confirm Zestril is lisinopril and check the profile for duplicate therapy, then alert the pharmacist if duplication is likely because brand/generic duplicates can cause double-dosing.
1582. **Q:** An eRx arrives for "Lasix 40 mg PO daily," but the patient profile lists furosemide 20 mg daily as active; what is the BEST technician next step? **A:** Stop and route to the pharmacist to review the dose change because Lasix is furosemide and strength changes require pharmacist assessment.
1583. **Q:** A prescription is written "metformin 500 mg ii tabs BID"; what quantity should you enter for 30 days? **A:** Enter 120 tablets because ii = 2 tablets per dose and 2 doses/day makes 4 tablets/day, and $4 \times 30 = 120$.
1584. **Q:** During product selection you see "CeleXA" and "CeleBREX" adjacent on the shelf and the label prints for citalopram; what should you do NEXT to prevent a LASA error? **A:** Verify the NDC/barcode matches citalopram before filling because similar-looking brand names increase wrong-drug selection risk.
1585. **Q:** A patient drops off a new prescription for clopidogrel and says they also take "Plavix" daily; what should you do FIRST? **A:** Explain Plavix is clopidogrel and check for duplicate entries, then involve the pharmacist if both are active because it may represent therapeutic duplication.
1586. **Q:** You receive an eRx for "Klonopin 0.5 mg: 1 tab TID" with "Refills: 3"; what should you do NEXT? **A:** Hold the order and notify the pharmacist because clonazepam is Schedule IV and refills must be evaluated for compliance with controlled-substance requirements.

1587. **Q:** The prescriber writes "predniSONE 20 mg qd" but the product pulled is prednisoLONE; what should you do BEST? **A:** Stop filling and re-select the correct drug/NDC and alert the pharmacist if it reached verification because prednisone and prednisolone are different products and mix-ups can harm patients.
1588. **Q:** A prescription is "amoxicillin/clavulanate 875/125 mg: 1 tab PO q12h x 10 days"; what days' supply and quantity should you enter? **A:** Enter days' supply 10 and quantity 20 because q12h is 2 tablets/day and $2 \times 10 = 20$.
1589. **Q:** A patient asks at drop-off if "omeprazole" is the same as "Prilosec" and whether it's for heartburn; what is the BEST technician response? **A:** Confirm omeprazole is the generic for Prilosec and is a PPI used for GERD/heartburn, then offer pharmacist counseling for questions about use because techs can provide basic identification but not clinical advice.
1590. **Q:** While scanning incoming stock, you notice a sealed bottle of gabapentin has an NDC that doesn't match the invoice for the ordered manufacturer; what should you do FIRST? **A:** Set it aside and investigate with the wholesaler/invoice before stocking because mismatched NDCs can indicate a stocking error and must be resolved to prevent dispensing the wrong product.
1591. **Q:** A patient's med list shows "Norvasc 10 mg daily" and today's eRx is for amlodipine 10 mg daily; what should you do FIRST during data entry? **A:** Confirm the patient is not taking both and route to the pharmacist for duplicate therapy review because Norvasc is amlodipine and duplication increases adverse-effect risk.
1592. **Q:** You receive a new prescription for sertraline, and the patient says they take "Zoloft" from another pharmacy; what is the BEST next step? **A:** Document that Zoloft is sertraline and alert the pharmacist to check for therapeutic duplication because the same drug may be continued unintentionally.
1593. **Q:** While entering a prescription for tramadol, the patient profile lists a documented anaphylaxis to codeine; what should you do NEXT? **A:** Stop processing and notify the pharmacist to assess opioid allergy/cross-sensitivity risk because allergy clarification is required before dispensing.
1594. **Q:** A warfarin patient drops off a new prescription for trimethoprim/sulfamethoxazole for a UTI; what should you do FIRST? **A:** Flag the interaction and get the pharmacist because TMP/SMX can raise INR/bleeding risk and requires pharmacist intervention.

1595. **Q:** The prescriber writes "Synthroid 75 mcg qAM," but the product selected is levothyroxine 75 mg; what should you do BEST? **A:** Stop filling and correct the strength to micrograms with barcode/NDC verification because mg vs mcg is a high-risk dose error.
1596. **Q:** A label prints for hydroxyzine 25 mg, but the bottle in your hand is hydralazine 25 mg; what should you do NEXT? **A:** Do not proceed, reselect the correct drug using barcode/NDC and segregate the wrong bottle from the workstation because hydralazine/hydroxyzine is a common LASA mix-up.
1597. **Q:** An eRx for Adderall XR arrives with "Take 1 capsule daily" and "Refills: 2"; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist because Schedule II controlled substances are not federally refillable.
1598. **Q:** A prescription reads "levetiracetam 500 mg: i tab PO BID #60"; what is the days' supply you should enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$.
1599. **Q:** A patient asks what metoprolol is typically used for when picking up their first fill; what is the BEST technician response? **A:** State it's commonly used for high blood pressure/heart conditions and offer pharmacist counseling because technicians may provide basic use info but clinical counseling must be available.
1600. **Q:** During product selection for insulin lispro, you notice the refrigerated box is past its expiration date; what should you do FIRST? **A:** Remove it from dispensing stock and follow pharmacy policy to quarantine/return because expired insulin may lose potency and must not be dispensed.
1601. **Q:** A patient says they're allergic to "statins" but the new eRx is for atorvastatin (Lipitor); what should you do FIRST at drop-off? **A:** Stop processing and alert the pharmacist to assess the allergy history because statin intolerance/allergy requires pharmacist evaluation before dispensing.
1602. **Q:** While entering a prescription for bupropion SR, you notice the prescriber selected "XL" in the directions but "SR" in the drug field; what is the BEST next step? **A:** Hold the order and route to the pharmacist to clarify formulation because SR vs XL are not interchangeable and impact dosing.
1603. **Q:** A hardcopy reads "metformin 500 mg i tab po bid #60"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$.

1604. **Q:** You receive an eRx for hydromorphone (Dilaudid) but the patient profile shows active morphine ER; what should you do FIRST during data entry? **A:** Pause and notify the pharmacist to evaluate duplicate opioid therapy and safety because concurrent opioids increase overdose risk.
1605. **Q:** A patient picking up cefdinir asks if it's an antibiotic and what it's usually for; what is the BEST technician response? **A:** Provide a brief non-clinical explanation that cefdinir is an antibiotic often used for infections and refer dosing/side-effect questions to the pharmacist because counseling beyond basics is pharmacist-led.
1606. **Q:** While selecting lisinopril, you accidentally grab look-alike losartan from the shelf; what should you do NEXT? **A:** Stop filling, return the wrong product, select the correct drug using barcode/NDC verification, and continue because catching LASA errors early prevents patient harm.
1607. **Q:** An eRx for clonazepam includes "Take 1 tablet TID PRN anxiety" with quantity 90 and the patient requests an early fill 10 days after the last 30-day supply; what should you do FIRST? **A:** Check the PDMP/third-party refill-too-soon rejection per workflow and refer to the pharmacist because controlled substance early fills require pharmacist review and documentation.
1608. **Q:** A prescription is for insulin regular U-100 vial: "Inject 10 units before meals TID," dispense 1 vial (10 mL); what days' supply should you enter? **A:** Enter 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$ (enter 33).
1609. **Q:** While unpacking refrigerated semaglutide (Ozempic), you find the tote temperature log shows it warmed above recommended range during transit; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager because temperature excursions make product integrity uncertain and require follow-up with wholesaler/manufacturer.
1610. **Q:** During data entry you see "Lamictal" on the prescription but the selected product is terbinafine; what is the BEST action? **A:** Stop and correct the selection using brand/generic verification (Lamictal = lamotrigine) because wrong-drug errors are high-risk and must be fixed before verification.
1611. **Q:** During data entry an eRx says "levothyroxine 25 mcg daily" but the prescriber note states "continue 75 mcg," what should you do FIRST? **A:** Stop processing and route to the pharmacist to clarify the intended strength because conflicting directions/notes create a dosing error risk.

1612. **Q:** A patient drops off a written prescription for oxycodone 5 mg that has the drug, directions, and quantity but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because an unsigned controlled-substance hardcopy is not valid to dispense.
1613. **Q:** While receiving an order, you notice a sealed bottle of sertraline has a lot number that doesn't match the invoice/packing slip; what is the BEST action? **A:** Set it aside as suspect product and alert the pharmacist/manager to follow DSCSA procedures because mismatched lot documentation can indicate diversion or counterfeit.
1614. **Q:** A patient asks at pickup what escitalopram (Lexapro) is commonly used for; what is the BEST technician response? **A:** Provide a general indication (depression/anxiety) and offer pharmacist counseling because technicians can give nonclinical info but counseling must be available.
1615. **Q:** A new prescription is for glipizide, and the profile shows the patient is also actively taking glyburide; what should you do FIRST? **A:** Pause the fill and refer to the pharmacist for therapeutic-duplication review because two sulfonylureas increase hypoglycemia risk.
1616. **Q:** You are entering "prednisone 10 mg: take 4 tablets daily x 5 days, dispense #20"; what days' supply should you enter? **A:** Enter 5 days because $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$, matching the quantity.
1617. **Q:** While selecting products you see "hydrALAZINE" and "hydrOXYzine" stored near each other and the label is for hydroxyzine; what is the BEST next step? **A:** Use barcode/NDC verification and Tall Man awareness before counting because LASA mix-ups are common and verification prevents dispensing the wrong drug.
1618. **Q:** A patient wants to buy pseudoephedrine and presents an out-of-state ID; what should you do FIRST at the register? **A:** Follow CMEA process by verifying acceptable identification and logging the sale (or refusing per policy) because PSE sales require ID check and electronic/paper log.
1619. **Q:** While counting tablets you see a stock bottle of atorvastatin with visible moisture clumping inside; what should you do NEXT? **A:** Remove it from active stock and notify the pharmacist to segregate and return/dispose per policy because compromised integrity can affect dose and safety.
1620. **Q:** A prescription comes in for isotretinoin and the patient is waiting; what should you do BEST before it goes to filling? **A:** Alert the pharmacist and ensure

iPLEDGE/REMS requirements are met before dispensing because isotretinoin is REMS-restricted and cannot be processed like a routine med.

1621. **Q:** A prescription reads "metformin ER 500 mg: i po bid" and the prescriber writes "do not substitute," what should you do FIRST in order entry? **A:** Enter as metformin extended-release 500 mg 1 tablet twice daily and flag DAW/no-substitution per the order because "ER" changes the dosage form and DAW affects product selection and billing.
1622. **Q:** An inhaler order is "fluticasone/salmeterol 250/50 Diskus: inhale 1 puff bid, dispense 1 inhaler (60 blisters)," what days' supply should you enter? **A:** Enter 30 days' supply because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$, which supports correct adjudication and refill timing.
1623. **Q:** A patient brings a new prescription for "apixaban (Eliquis) 5 mg bid" and asks if they can keep taking OTC ibuprofen 600 mg three times daily for pain; what is the BEST technician action? **A:** Refer to the pharmacist before processing counseling because NSAIDs can increase bleeding risk with apixaban and requires pharmacist assessment.
1624. **Q:** The sig says "Take 0.5 mL daily" and the product concentration is 250 mg/5 mL; how many mg per dose is this? **A:** 25 mg per dose because $250 \text{ mg}/5 \text{ mL} = 50 \text{ mg/mL}$ and $50 \text{ mg/mL} \times 0.5 \text{ mL} = 25 \text{ mg}$, preventing a tenfold dosing error in entry.
1625. **Q:** During data entry you see "insulin regular (Humulin R) U-100: inject 8 units subcut qid, dispense 10 mL vial," what days' supply should you enter using 1 mL = 100 units? **A:** Enter 31 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \text{ units} \times 4/\text{day} = 32 \text{ units/day}$, so $1000 \div 32 = 31.25$ and you round down to avoid overbilling.
1626. **Q:** A prescription reads "azithromycin 200 mg/5 mL: give 10 mL day 1, then 5 mL daily days 2–5; dispense 30 mL," what should you do NEXT if your calculated total volume differs from the written quantity? **A:** Calculate total needed ($10 + 5 \times 4 = 30 \text{ mL}$) and proceed with 30 mL because the math matches the directions, reducing the risk of shorting the course.
1627. **Q:** An eRx for "hydromorphone 2 mg: take 1 tab q4–6h prn pain" is typed as "hydroxyzine 25 mg" due to autocomplete; what is the BEST action? **A:** Stop data entry and alert the pharmacist immediately because this is a high-risk LASA selection error (opioid vs antihistamine) that could cause serious harm.

1628. **Q:** A prescription states "Take ii tabs po qhs" for trazodone 50 mg with quantity #60; what should you enter for the directions and days' supply? **A:** Enter "take 2 tablets by mouth at bedtime" and 30 days' supply because $ii = 2$ and $\#60 \div 2/\text{day} = 30$, ensuring accurate labeling and payer billing.
1629. **Q:** A patient's profile shows active lisinopril 20 mg daily and you receive a new order for "lisinopril/HCTZ 20/12.5 mg daily"; what should you do FIRST? **A:** Flag the potential duplication and route to the pharmacist because the new combo may replace the single-agent and requires clarification to prevent double dosing of lisinopril.
1630. **Q:** A liquid order is "cephalexin 125 mg/5 mL: take 10 mL tid x 7 days"; what quantity in mL should be dispensed? **A:** Dispense 210 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$, which matches the full course and prevents under-dispensing.
1631. **Q:** While typing a new eRx you notice the prescriber selected "hydralazine" but the directions say "for thyroid" and the patient profile shows hypothyroidism; what should you do FIRST? **A:** Stop data entry and alert the pharmacist to verify the intended drug (likely levothyroxine), because LASA/selection errors are safety-critical and technicians must not guess.
1632. **Q:** A prescription is written for "Augmentin 875/125 mg" and the patient asks if it's the same as amoxicillin; what is the BEST technician response? **A:** Explain that Augmentin is amoxicillin/clavulanate (not plain amoxicillin) and refer counseling questions to the pharmacist, because the added component changes therapy and side effects.
1633. **Q:** You receive an order for "glipizide XL 10 mg daily" but the patient profile lists "glipizide IR 10 mg bid" as active; what should you do NEXT? **A:** Pause processing and route to the pharmacist to assess therapeutic duplication and formulation change, because IR vs XL dosing is not interchangeable without prescriber/pharmacist review.
1634. **Q:** The sig reads "prednisone 20 mg: ii tabs po qd" with quantity #10; what days' supply should you enter? **A:** Enter 5 days' supply because the patient takes 2 tablets daily and $10 \text{ tablets} \div 2 \text{ per day} = 5 \text{ days}$.
1635. **Q:** A new prescription is for "metoprolol succinate ER 50 mg daily" but the patient says they were taking "metoprolol tartrate 50 mg bid"; what should you do FIRST at the register? **A:** Send the patient to the pharmacist before dispensing,

because tartrate and succinate are different formulations with different dosing and require counseling/verification.

1636. **Q:** During product selection you scan "clonidine 0.1 mg" but the label is for "clozapine 100 mg"; what is the BEST next step? **A:** Stop filling and select the correct NDC using barcode verification, then notify the pharmacist if the wrong drug reached verification, because this is a high-risk LASA error.
1637. **Q:** A prescription says "tamsulosin (Flomax) 0.4 mg: take i cap po qhs" and quantity is #30; what should you enter for directions and days' supply? **A:** Enter "Take 1 capsule by mouth at bedtime" and 30 days' supply, because i = 1 and qhs = every night at bedtime.
1638. **Q:** An order is "ketorolac 0.5% ophthalmic: instill 1 gtt OU qid, dispense 5 mL" and the software asks for days' supply; what should you enter using 20 gtt = 1 mL? **A:** Enter 12 days' supply because $5 \text{ mL} \times 20 = 100 \text{ drops}$ and OU qid uses 8 drops/day, so $100 \div 8 = 12.5$ rounded down to 12 for whole days.
1639. **Q:** A patient picking up "simvastatin (Zocor)" says they also take "atorvastatin (Lipitor)" at home; what should you do BEST? **A:** Hold the transaction and involve the pharmacist to assess duplicate statin therapy, because taking two HMG-CoA reductase inhibitors may be unsafe and needs review.
1640. **Q:** You are asked to enter a pseudoephedrine product sale and the customer refuses to show ID; what should you do NEXT? **A:** Do not complete the sale and follow store policy to decline/void the transaction, because federal CMEA requirements include verifying purchaser identity and logging the sale.
1641. **Q:** A prescription reads "levothyroxine 75 mcg po qd" but the drop-down strength selected in order entry is 75 mg; what should you do FIRST? **A:** Stop processing and correct the unit/strength to 75 mcg (or route to pharmacist if unsure) because mg vs mcg is a high-risk entry error that could cause a 1000× overdose.
1642. **Q:** You must enter days' supply for fluticasone nasal spray 50 mcg with sig "2 sprays each nostril once daily," bottle size 120 sprays; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$.
1643. **Q:** An eRx for morphine ER has sig "take 1 tab bid prn severe pain"; what should you do NEXT before completing order entry? **A:** Route to the pharmacist to

clarify/adjust the sig because ER opioids should not be entered as PRN without pharmacist review due to safety and appropriateness concerns.

1644. **Q:** A prescription for amlodipine 5 mg says "take 1 tab po qd" and quantity is #90; what days' supply should you enter? **A:** Enter 90 days because 1 tablet daily means 90 tablets cover 90 days.
1645. **Q:** An order is for omeprazole 20 mg "take 1 cap po bid," dispense #60; what is the correct days' supply to submit? **A:** Enter 30 days because 2 capsules/day means $60 \div 2 = 30$.
1646. **Q:** While typing, you see the sig "use as directed" for metformin 500 mg tablets with quantity #180; what is the BEST technician action? **A:** Stop and request prescriber directions or pharmacist intervention because "use as directed" is not sufficient to calculate an accurate days' supply for billing and safe labeling.
1647. **Q:** A prescription is written "digoxin 0.125 mg po qd" but the prescriber note mentions "125 mcg daily"; what should you do FIRST? **A:** Confirm the intended dose/unit with the pharmacist because 0.125 mg equals 125 mcg and the entry must match a single clear strength to prevent dosing confusion.
1648. **Q:** You need to enter days' supply for timolol ophthalmic with sig "instill 1 gtt OU bid," dispense 10 mL, using 20 gtt = 1 mL; what days' supply should you enter? **A:** Enter 50 days because 1 drop in each eye twice daily = 4 gtt/day, $10 \text{ mL} \times 20 = 200 \text{ gtt}$, and $200 \div 4 = 50$.
1649. **Q:** A patient brings a new prescription reading "Take 1 tab po qid" and the drug is ibuprofen 600 mg; what should you do BEST before entry? **A:** Translate and enter "take 1 tablet by mouth four times daily" because qid means four times daily and clear directions reduce label misinterpretation.
1650. **Q:** During product selection for hydroxyzine, the shelf has hydroxyzine HCl and hydroxyzine pamoate and the eRx just says "hydroxyzine 25 mg"; what should you do NEXT? **A:** Pause and ask the pharmacist to clarify the salt/form (HCl vs pamoate) because they are not interchangeable in all cases and selecting the wrong product is a common order-entry dispensing error.
1651. **Q:** While counting insulin glargine pens, you notice the box seal is broken and the lot/expiration label looks tampered; what should you do FIRST? **A:** Stop using the product and quarantine it for pharmacist/manager review because it is a suspect product under DSCSA and may be unsafe to dispense.

1652. **Q:** A hardcopy prescription for oxycodone/APAP has the patient name and drug but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because a missing signature makes it incomplete/invalid to dispense and requires prescriber follow-up.
1653. **Q:** A patient says they are allergic to "sulfa" and you see a new prescription for sulfamethoxazole/trimethoprim; what is the BEST technician action? **A:** Stop processing and alert the pharmacist to assess the allergy risk because dispensing could cause a serious hypersensitivity reaction.
1654. **Q:** During order entry you receive an eRx for hydromorphone but the directions say "take 1 tablet every 4 hours" with no strength specified; what should you do FIRST? **A:** Pause entry and send to the pharmacist for clarification because missing strength is a critical element that affects dose and safety.
1655. **Q:** You are filling methotrexate tablets and the sig reads "take 2 tablets daily for rheumatoid arthritis"; what should you do BEST? **A:** Stop and escalate to the pharmacist because methotrexate is high-alert and usually weekly for RA, so daily dosing may indicate a dangerous error.
1656. **Q:** A refrigerator temperature log shows the vaccine fridge has been at 12°C (54°F) overnight; what should you do NEXT? **A:** Quarantine the affected vaccines and notify the pharmacist/manager because a temperature excursion can reduce potency and requires stability guidance before use.
1657. **Q:** A customer requests an early refill on clonazepam stating it was "lost," but the profile shows it was filled 7 days ago for 30 days; what should you do FIRST? **A:** Refer to the pharmacist for controlled-substance refill authorization because early replacements are high diversion risk and require pharmacist judgment and documentation per policy.
1658. **Q:** You enter a prescription for glipizide and the prescriber wrote "U" for units in the directions; what is the BEST action? **A:** Ask the pharmacist to clarify and correct the sig because "U" is an error-prone abbreviation that can be misread and cause dosing errors.
1659. **Q:** You are calculating days' supply for amoxicillin/clavulanate 875/125 mg with sig "1 tab po bid," dispense #14; what days' supply should you submit? **A:** Submit 7 days' supply because $14 \text{ tablets} \div 2 \text{ tablets/day} = 7 \text{ days}$.
1660. **Q:** A patient asks you to print another patient's medication history "because I'm their caregiver," and no authorization is on file; what should you do BEST? **A:**

Decline and involve the pharmacist to verify HIPAA authorization because protected health information cannot be released without proper consent or legal authority.

1661. **Q:** While entering a new prescription, the patient profile already shows lisinopril 20 mg daily and you receive an eRx for Zestril 20 mg daily; what should you do BEST? **A:** Pause entry and alert the pharmacist to possible therapeutic duplication (brand/generic of lisinopril) to prevent double therapy.
1662. **Q:** You are selecting product for a prescription written for "Lamictal 100 mg" but you pull "Lamisil 250 mg" from the shelf; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification because this is a high-risk LASA error (lamotrigine vs terbinafine).
1663. **Q:** A new prescription is for clarithromycin and the patient's active meds include atorvastatin; what is the BEST technician action during data entry? **A:** Flag and refer to the pharmacist for interaction review because clarithromycin can increase statin levels and risk myopathy.
1664. **Q:** A prescription reads "metformin ER 500 mg take 1 tab bid" but the product in the bin is metformin IR 500 mg; what should you do NEXT? **A:** Stop and correct the dosage form selection (ER vs IR) and route to pharmacist if already verified because wrong release form can change effect and tolerability.
1665. **Q:** You receive an eRx for "Synthroid 75 mcg DAW" and the patient requests generic levothyroxine at pickup; what should you do BEST? **A:** Do not change the DAW order yourself; notify the pharmacist to contact the prescriber or follow state/plan rules because DAW limits substitution.
1666. **Q:** A patient asks what sertraline (Zoloft) is used for and whether it will work right away; what should you do FIRST? **A:** Refer the question to the pharmacist because counseling on SSRI onset and expectations is clinical information beyond technician scope.
1667. **Q:** The directions say "Take 1½ tablets daily" and the prescription is for warfarin 5 mg with quantity #45; what days' supply should you enter? **A:** Enter 30 days because $45 \text{ tablets} \div 1.5 \text{ tablets/day} = 30 \text{ days}$, ensuring accurate billing and refill timing.
1668. **Q:** A prescription for furosemide is written "40 mg po qd" but the prescriber accidentally selected "Lasix injection" on the eRx; what should you do NEXT? **A:**

Hold and send to the pharmacist for clarification because the dosage form/route mismatch is a safety-critical order entry error.

1669. **Q:** During filling you notice a stock bottle of Eliquis has a different NDC than the one on the printed label for apixaban 5 mg; what should you do BEST? **A:** Stop and reprint/adjust the label to match the correct NDC and product because NDC mismatch can indicate wrong product selection and billing inaccuracies.
1670. **Q:** A new prescription for phenytoin is processed and the patient's profile shows carbamazepine started last week; what should you do FIRST? **A:** Alert the pharmacist to assess for CNS antiepileptic duplication and interaction risk because combined therapy may require clinical justification and monitoring.
1671. **Q:** While entering an eRx for "Crestor 20 mg daily," the patient profile shows an active prescription for rosuvastatin 20 mg daily; what should you do BEST? **A:** Link the eRx to the existing rosuvastatin therapy and alert the pharmacist if duplication is unclear, because Crestor is rosuvastatin and duplicate statins can cause errors.
1672. **Q:** A prescription comes in for "Dilaudid 2 mg 1 tab q4-6h prn pain," but the patient says they usually take "Dilaudid liquid"; what should you do FIRST? **A:** Pause processing and notify the pharmacist to verify dosage form and strength, because Dilaudid (hydromorphone) has high risk for opioid dosing errors across formulations.
1673. **Q:** The sig on a new prescription reads "i tab po qd" for levothyroxine 25 mcg; what should you do NEXT? **A:** Clarify with the prescriber/pharmacist before entry, because "i" can be misread and technicians should ensure unambiguous directions to prevent dosing errors.
1674. **Q:** A patient is prescribed "Keflex 500 mg qid" and reports a history of anaphylaxis to penicillin; what is the BEST technician action? **A:** Flag the allergy and route to the pharmacist for assessment before filling, because cephalexin (a cephalosporin) may cross-react and requires pharmacist judgment.
1675. **Q:** You receive an eRx for "Adderall XR 20 mg" with a request for 3 refills; what should you do FIRST? **A:** Stop and refer to the pharmacist for controlled-substance compliance review, because amphetamine mixed salts are Schedule II and refills are not permitted federally.
1676. **Q:** A prescription is for insulin lispro U-100 pens with directions "Inject 8 units before meals TID" and the box contains 5 pens of 3 mL each; what days' supply

should you submit? **A:** Submit 62 days, because total units are $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div (8 \times 3 = 24 \text{ units/day}) = 62.5 \text{ days}$, typically rounded down.

1677. **Q:** During product selection you see “hydrOXYzine” stored near “hydrALAZINE” and the label is for hydroxyzine 25 mg; what should you do BEST? **A:** Use barcode/NDC verification and read the full drug name and strength before filling, because hydroxyzine/hydralazine are LASA drugs and require extra confirmation.
1678. **Q:** A new prescription for “Prilosec 20 mg daily” is entered, but the patient profile already lists omeprazole 20 mg daily and pantoprazole 40 mg daily; what should you do NEXT? **A:** Hold and alert the pharmacist about possible therapeutic duplication, because Prilosec is omeprazole and multiple PPIs increase risk without added benefit.
1679. **Q:** A stock bottle of nitroglycerin sublingual tablets arrives with the seal broken and tablets rattling loose; what should you do FIRST? **A:** Remove it from usable inventory and follow pharmacy policy for suspect/damaged product handling, because compromised packaging can affect potency and patient safety.
1680. **Q:** A patient asks if “atorvastatin” and “Lipitor” are different medicines and whether taking both will “work better”; what should you do FIRST? **A:** Explain they are the same drug (generic/brand) and immediately involve the pharmacist to prevent duplicate therapy, because taking both could cause unnecessary adverse effects.
1681. **Q:** While entering “Lasix 40 mg 1 tab po qd,” the prescriber also wrote “for high blood pressure”; what is the BEST way for a technician to interpret this order? **A:** Recognize Lasix as brand for furosemide (a loop diuretic) and proceed with accurate brand/generic entry while flagging anything unclear to the pharmacist because correct drug identification prevents selection errors.
1682. **Q:** A hardcopy says “MSO4 2 mg IV q2h prn pain” from an outside clinic; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify because “MSO4” is an error-prone abbreviation that can be confused with other morphine/magnesium products and poses a serious safety risk.
1683. **Q:** A patient drops off a new Rx for Zoloft and asks what it’s for; what should you do NEXT as the technician? **A:** Identify Zoloft as sertraline (an SSRI antidepressant) and refer the patient to the pharmacist for counseling because indication/expectations and safety counseling are pharmacist responsibilities.

1684. **Q:** You receive an eRx for "metformin ER 500 mg" but the product selected is "metformin IR 500 mg"; what should you do BEST before printing/filling? **A:** Correct the dosage form to match ER and re-verify the NDC/product because ER vs IR is a common error that changes drug release and dosing.
1685. **Q:** A patient profile shows an active lisinopril Rx, and you're entering a new Rx for "Zestoretic 20/25 daily"; what should you do FIRST? **A:** Pause and notify the pharmacist about likely therapeutic duplication because Zestoretic contains lisinopril plus hydrochlorothiazide and duplicating ACE inhibitor therapy can be unsafe.
1686. **Q:** A prescription is written for "Amaryl 4 mg daily," and the patient says they have frequent low blood sugar; what is the BEST technician action? **A:** Route the concern to the pharmacist immediately because Amaryl is glimepiride (a sulfonylurea) and hypoglycemia is a significant safety issue requiring pharmacist assessment.
1687. **Q:** During pickup, a patient asks whether "Plavix" is the same as "clopidogrel" and worries the generic is "weaker"; what should you do FIRST? **A:** Explain that Plavix is the brand name for clopidogrel and that generics are required to be equivalent, then offer pharmacist counseling because reassurance plus access to clinical counseling improves adherence and safety.
1688. **Q:** A label direction reads "ii tabs po bid" for prednisone 10 mg; how many mg per day is the patient taking? **A:** 40 mg/day because "ii" is 2 tablets and "bid" is twice daily, so $2 \text{ tabs} \times 10 \text{ mg} \times 2 = 40 \text{ mg/day}$ to ensure correct dose interpretation.
1689. **Q:** A prescription is for "azithromycin 250 mg: take 2 tabs today then 1 tab daily x 4 days"; what quantity should you enter to dispense? **A:** Quantity 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals $2 + 4 = 6$, preventing under- or over-dispensing.
1690. **Q:** A wholesaler notice lists a recall for a specific lot of levothyroxine 50 mcg that matches your shelf bottle; what should you do FIRST? **A:** Pull the affected lot from stock and quarantine per policy while notifying the pharmacist because recalled product must be isolated to prevent dispensing to patients.
1691. **Q:** While typing an eRx for hydrOXYzine 25 mg, the selected product is hydrALAZINE 25 mg due to LASA; what should you do BEST before filling? **A:** Stop and correct the product selection by verifying the drug name/strength and

NDC/barcode against the eRx because LASA mix-ups are a common high-risk dispensing error.

1692. **Q:** A caregiver asks for "Tylenol with codeine" for a child and hands you an old label to "refill it," but no active prescription is on file; what should you do FIRST? **A:** Refer to the pharmacist and do not promise a refill because codeine combinations are controlled substances and require a valid current prescription authorization.
1693. **Q:** You receive a verbal transfer request for alprazolam from another pharmacy and the caller is a technician; what should you do NEXT? **A:** Route the call to the pharmacist because controlled-substance transfers require pharmacist-to-pharmacist communication per federal controlled-substance handling expectations.
1694. **Q:** During order entry, a new Rx is written for "Humalog 100 units/mL vial: inject 10 units before meals TID," and the prescriber also wrote "dispense 1 vial"; what days' supply should you enter for a 10 mL vial? **A:** Enter 33 days' supply because $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$ and $10 \text{ mL at } 100 \text{ units/mL} = 1000 \text{ units}$, so $1000 \div 30 = 33.3$ (typically billed as 33).
1695. **Q:** A patient brings in a refrigerated box of liraglutide pens that they say "sat in a hot car all afternoon"; what should you do FIRST? **A:** Hold the product for pharmacist review and follow storage-excursion policy because temperature excursions can make the medication unsafe or ineffective.
1696. **Q:** An eRx for isotretinoin arrives and the patient is waiting at the counter asking you to "just run it through"; what is the BEST technician action? **A:** Alert the pharmacist to complete required REMS/iPLEDGE steps before dispensing because isotretinoin has mandatory safety program requirements beyond technician scope.
1697. **Q:** You are preparing a prescription for warfarin and notice the directions on the label show both "1 tab daily" and "take as directed" from a previous entry; what should you do FIRST? **A:** Stop and have the pharmacist clarify/correct the directions because warfarin is a high-alert medication and conflicting sigs can cause serious harm.
1698. **Q:** A patient asks to buy pseudoephedrine 30 mg tablets and requests 96 tablets; what should you do NEXT at the register? **A:** Verify ID and complete the required sales log and quantity limits check because pseudoephedrine is regulated under CMEA for diversion control.

1699. **Q:** A shipment arrives with a case of insulin pens, but the tote's temperature indicator shows out-of-range during transit; what should you do BEST? **A:** Segregate and hold the shipment for supplier/pharmacist review (do not stock) because suspect temperature integrity makes the product potentially non-dispensable.
1700. **Q:** While entering a new prescription for "clonidine 0.1 mg bid," the patient profile shows an active clonazepam prescription and you suspect the prescriber intended clonazepam; what should you do FIRST? **A:** Pause processing and notify the pharmacist to verify with the prescriber because sound-alike names (clonidine/clonazepam) create high-risk wrong-drug errors.
1701. **Q:** While entering an eRx for "metformin ER 500 mg qHS," you notice the product selected is metformin IR 500 mg; what should you do BEST before printing labels? **A:** Stop and reselect the correct ER NDC and route for pharmacist verification because IR vs ER is a high-risk formulation error that can change dosing and side effects.
1702. **Q:** A new Rx is for "lisinopril 20 mg daily," but the patient profile shows an active lisinopril-HCTZ 20/12.5 mg daily; what should you do FIRST? **A:** Hold processing and alert the pharmacist to possible therapeutic duplication because ACE-inhibitor overlap can cause safety issues and requires clinical clarification.
1703. **Q:** A patient says they take "Plavix," but the profile lists clopidogrel and they ask if it's the same drug; what is the BEST technician response? **A:** Confirm Plavix is the brand name for clopidogrel and offer pharmacist counseling if they have questions because accurate brand/generic recognition supports adherence and safety.
1704. **Q:** The prescriber writes "predniSONE 10 mg" but during product selection you accidentally click prednisoLONE 10 mg; what should you do NEXT? **A:** Correct the selection and verify the NDC/barcode before filling because prednisone vs prednisolone is a common mix-up and dispensing the wrong steroid can harm the patient.
1705. **Q:** A prescription reads "amLODIPine 5 mg i po qd #90," and the patient asks what it's for; what should you do BEST? **A:** Refer them to the pharmacist for counseling while you can state it's commonly used for blood pressure because OBRA-90 counseling content must be offered by the pharmacist.
1706. **Q:** An eRx for "sertraline 50 mg daily" comes in, and the patient profile shows an allergy documented as "sertraline—rash"; what should you do FIRST? **A:** Stop

the fill and notify the pharmacist to assess the allergy history because re-exposure to the same drug may be unsafe and needs pharmacist intervention.

1707. **Q:** You are calculating days' supply for "amoxicillin-clavulanate 875/125 mg: 1 tab BID x 10 days" and the quantity entered is 14; what should you do BEST? **A:** Correct the quantity to 20 tablets because BID for 10 days requires $2 \times 10 = 20$ tablets and wrong quantity can lead to under-treatment.
1708. **Q:** A patient taking simvastatin says they started clarithromycin yesterday from urgent care and asks if it's okay together; what should you do FIRST? **A:** Immediately involve the pharmacist because clarithromycin can raise statin levels and increase myopathy risk, and the pharmacist must evaluate therapy and contact the prescriber if needed.
1709. **Q:** During data entry you see "U-500 regular insulin" but the selected product is U-100 regular insulin; what should you do FIRST? **A:** Stop and alert the pharmacist before proceeding because U-500 vs U-100 is a high-alert concentration error with severe overdose/underdose risk.
1710. **Q:** A controlled medication arrives from the wholesaler labeled "oxycodone/APAP 5/325," but the bottle shows evidence of tampering and broken seal; what should you do BEST? **A:** Do not stock or dispense it and follow policy to isolate and report it to the pharmacist/manager because suspect or tampered product must be removed from inventory to protect patient safety.
1711. **Q:** An Rx says "levothyroxine 0.075 mg po qd," but the prescriber commonly writes in mcg; what should you enter and do NEXT to ensure accuracy? **A:** Enter 75 mcg (0.075 mg) and have the pharmacist verify if anything seems inconsistent, because unit conversion errors can cause 10-fold dosing mistakes.
1712. **Q:** The directions read "Take ii tabs po bid" for carvedilol 6.25 mg and quantity is 120; what days' supply should you submit? **A:** 30 days, because 2 tablets twice daily equals 4 tablets/day and $120 \div 4 = 30$.
1713. **Q:** While entering "hydroxyzine 25 mg 1 tab q6h prn anxiety," you notice "q6h" could be misread; what is the BEST technician step before processing? **A:** Interpret and enter q6h as "every 6 hours" but route to the pharmacist if any part is unclear, because ambiguous sigs increase wrong-frequency errors.
1714. **Q:** A prescription is for atorvastatin 40 mg daily, but you selected simvastatin 40 mg due to look-alike names; what should you do NEXT before labels print?

A: Stop and correct the drug selection by matching the exact name/strength to the Rx and NDC, because LASA statins are a common wrong-drug error.

1715. **Q:** An inhaler Rx is “fluticasone/salmeterol 250/50: 1 inhalation BID,” and the device contains 60 inhalations; what days’ supply should you bill? **A:** 30 days, because BID equals 2 inhalations/day and $60 \div 2 = 30$.

1716. **Q:** You receive an eRx for “Keflex 500 mg qid” and the product chosen is cephalexin 500 mg; the patient asks if it matches the prescription—what is the BEST response? **A:** Confirm it’s the same medication (Keflex is cephalexin) and offer pharmacist counseling for questions, because brand/generic recognition is within technician scope but counseling is pharmacist-led.

1717. **Q:** A new order is “diltiazem CD 240 mg daily,” but you accidentally picked diltiazem IR 60 mg due to multiple formulations; what should you do FIRST? **A:** Stop entry and reselect the correct extended-release formulation (CD) and strength, because wrong dosage form can change dosing and safety.

1718. **Q:** A patient has insulin lispro U-100 with directions “inject 12 units TID with meals,” and the vial is 10 mL; what is the days’ supply? **A:** 27 days, because U-100 has 100 units/mL so $10 \text{ mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$; $1000 \div 36 = 27.7$, round down to 27.

1719. **Q:** An Rx for morphine ER arrives with “refill x 2” on the directions field; what should you do BEST in order entry? **A:** Hold processing and notify the pharmacist, because Schedule II prescriptions cannot be refilled and require pharmacist intervention to resolve.

1720. **Q:** A suspension order reads “clindamycin 75 mg/5 mL: take 10 mL TID for 7 days; dispense quantity ____ mL”; what quantity should you enter? **A:** 210 mL, because $10 \text{ mL per dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$ to complete the course.

1721. **Q:** An eRx for metformin ER 500 mg says “take 2 tabs po qpm with dinner,” dispense #180; what days’ supply should you enter for billing? **A:** Enter 90 days’ supply because 2 tablets once daily means $180 \div 2 = 90$.

1722. **Q:** You’re entering “prednisone 10 mg: take 4 tabs daily x 5 days,” but the prescriber wrote “QD x5” and quantity is missing; what should you do NEXT? **A:** Calculate 20 tablets needed (4×5) then send to the pharmacist to confirm/obtain the missing quantity because technicians can’t assume missing dispense quantities.

1723. **Q:** A prescription reads "digoxin 0.125 mg po qd," and the patient profile shows a historical digoxin 125 mcg entry; what is the BEST order-entry action? **A:** Enter 0.125 mg (equivalent to 125 mcg) and add a clarification note for pharmacist review because unit conversions can cause tenfold errors.
1724. **Q:** A child's amoxicillin-clavulanate ES-600 order says "7.5 mL po bid x 10 days; dispense ____ mL"; what quantity should you enter? **A:** Enter 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$.
1725. **Q:** You receive an order for "latanoprost 0.005% instill 1 gtt OU qhs," dispense 2.5 mL; using 20 gtt/mL, what days' supply should you bill? **A:** Bill 25 days because $2.5 \text{ mL} \times 20 = 50 \text{ drops}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25$.
1726. **Q:** While typing "MSO4 2 mg IV q2h prn pain," what should you do FIRST before completing entry? **A:** Stop and route to the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be confused with magnesium sulfate.
1727. **Q:** An eRx is for "lamotrigine 25 mg: take 1 tab daily x 14 days then 2 tabs daily," dispense 42, and your system requires a single days' supply; what is the BEST technician step? **A:** Refer to the pharmacist for days' supply determination because taper/variable dosing should not be guessed by the technician for billing accuracy.
1728. **Q:** A lisinopril order says "Take 1 tab po qd," and the prescriber typed "Dispense: 1 bottle"; the product is 90 tablets per stock bottle—what should you do NEXT? **A:** Enter the numeric quantity (e.g., 90 tablets) and send to pharmacist if unclear because "bottle" is not a standard, billable unit without a defined tablet count.
1729. **Q:** A patient presents a new prescription for alprazolam with "take 1 tab tid prn" and "refill: 3"; what should you do FIRST in processing? **A:** Hold the prescription and notify the pharmacist because controlled substance refill authorization requires legal/clinical verification and often cannot be processed as written.
1730. **Q:** During data entry you select hydrochlorothiazide 25 mg, but the eRx is for hydroxyzine 25 mg and the sig is "for itching," what should you do NEXT? **A:** Stop filling and correct the drug selection (verify name/NDC and indication) because LASA mix-ups can lead to wrong-drug errors before labels print.
1731. **Q:** A new eRx is for "Toprol-XL 50 mg take 1 tab daily," but you accidentally selected metoprolol tartrate 50 mg in the product field; what should you do NEXT? **A:** Stop and reselect metoprolol succinate ER (Toprol-XL) and recheck the NDC because IR vs ER mix-ups are a common patient-safety error.

1732. **Q:** A patient drops off a prescription for "Zyrtec 10 mg daily," and your inventory shows cetirizine 10 mg tablets; what is the BEST technician action during entry? **A:** Enter cetirizine 10 mg as the generic equivalent to Zyrtec because brand/generic substitution is typically appropriate unless DAW is indicated.
1733. **Q:** While entering a new prescription for "Keflex 500 mg q6h," the profile shows a severe anaphylaxis allergy to penicillin; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess cross-sensitivity risk because allergy evaluation requires pharmacist judgment.
1734. **Q:** You receive an order for "Eliquis 5 mg take 1 tab BID #60," and the patient also has an active warfarin prescription on file; what should you do BEST? **A:** Flag and refer to the pharmacist before filling because concurrent anticoagulants may be therapeutic duplication with high bleeding risk.
1735. **Q:** A prescriber writes "U-100 insulin regular: inject 8 units SQ before meals TID; dispense 1 vial (10 mL)"; what days' supply should you enter? **A:** Enter 41 days because $10\text{ mL} = 1000\text{ units}$ and $8\text{ units} \times 3/\text{day} = 24\text{ units/day}$, so $1000 \div 24 = 41.6$ and you bill 41 days.
1736. **Q:** A prescription says "levothyroxine 0.1 mg daily," but the patient's previous fills are for levothyroxine 100 mcg daily; what should you do BEST in order entry? **A:** Recognize $0.1\text{ mg} = 100\text{ mcg}$ and ensure the strength matches without creating a duplicate-strength confusion, then route to pharmacist if anything conflicts.
1737. **Q:** During product selection you notice two similar bottles: hydrALAZINE 25 mg and hydrOXYzine 25 mg, and the Rx diagnosis note says "anxiety"; what should you do NEXT? **A:** Verify the correct drug against the eRx and use barcode/NDC scanning because LASA look-alikes can cause serious dispensing errors.
1738. **Q:** A controlled-substance eRx for "oxycodone 5 mg q6h prn pain #20" arrives with the prescriber's name but no DEA number shown in your system; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify prescriber credentials because controlled-substance validity issues must be resolved before dispensing.
1739. **Q:** A prescription for "nitrofurantoin macrocrystals/monohydrate 100 mg BID #10" is typed as "Macrochantin 100 mg" by mistake; what is the BEST technician step? **A:** Correct the entry to the intended product (Macrobid vs Macrochantin) only after confirming the exact drug/format from the prescription and involving the pharmacist if unclear because these are not interchangeable.

1740. **Q:** A new bottle of refrigerated insulin arrives warm with a temperature indicator showing an excursion; what should you do NEXT? **A:** Segregate the product and notify the pharmacist/manager per procedure because temperature excursions create suspect product that should not be dispensed.
1741. **Q:** While typing a new prescription you enter "glyBURIDE 5 mg daily," but the prescriber intended "glipiZIDE 5 mg daily" for a patient with diabetes; what should you do NEXT? **A:** Stop processing and alert the pharmacist to clarify the intended sulfonylurea because glyburide/glipizide is a common LASA mix-up with significant hypoglycemia risk.
1742. **Q:** A new eRx is for "Plavix 75 mg daily," but the patient profile shows an active prasugrel (Effient) prescription; what is the BEST technician action? **A:** Pause the fill and refer to the pharmacist for therapeutic-duplication review because dual antiplatelet therapy may be inappropriate or intentional and needs clinical confirmation.
1743. **Q:** A patient requests an OTC recommendation at the counter and mentions they take sertraline (Zoloft) daily and want dextromethorphan for a bad cough; what should you do FIRST? **A:** Refer the patient to the pharmacist because sertraline plus dextromethorphan can increase serotonin-syndrome risk and requires pharmacist counseling.
1744. **Q:** The eRx directions read "metformin ER 500 mg take 1 tab BID," but you selected metformin immediate-release 500 mg in the product field; what should you do NEXT? **A:** Correct the product to metformin ER and recheck the NDC because IR vs ER is a high-impact dosage-form error that changes release and tolerability.
1745. **Q:** A prescription is for "Lamictal 25 mg titration pack," but the patient profile shows a documented severe rash with lamotrigine; what should you do FIRST? **A:** Stop entry and notify the pharmacist to evaluate the allergy history because lamotrigine can cause serious rashes and should not be processed without pharmacist review.
1746. **Q:** You are entering a new prescription for "Bactroban ointment apply TID," and you accidentally choose mupirocin nasal ointment instead; what is the BEST technician step? **A:** Re-select the correct topical ointment product and verify the dosage form because nasal vs topical mupirocin are not interchangeable and using the wrong form is a common trap.

1747. **Q:** A patient is newly prescribed lisinopril and asks if they can use OTC potassium supplements for leg cramps; what should you do NEXT? **A:** Refer to the pharmacist because ACE inhibitors like lisinopril can increase potassium and supplements may raise hyperkalemia risk.
1748. **Q:** A prescription reads "azithromycin 250 mg: take 2 tabs day 1 then 1 tab daily days 2–5; dispense #?" and your system requires a quantity; what quantity should you enter? **A:** Enter quantity 6 tablets because the total dosing is 2 tablets on day 1 plus 1 tablet for 4 more days ($2 + 4 = 6$).
1749. **Q:** During product selection for "Celebrex 200 mg," you notice the shelf has both celecoxib 200 mg and Celexa 20 mg stored nearby; what should you do FIRST? **A:** Use barcode/NDC verification and match strength/drug name carefully because Celebrex/Celexa is a high-risk LASA pair that can lead to wrong-drug dispensing.
1750. **Q:** A new order is for "Lantus SoloStar inject 20 units SQ nightly; dispense 1 box (5 pens, 3 mL each, U-100)"; what days' supply should you enter? **A:** Enter 75 days because $1 \text{ box contains } 5 \times 3 \text{ mL} = 15 \text{ mL} = 1500 \text{ units}$, and $1500 \text{ units} \div 20 \text{ units/day} = 75 \text{ days}$.
1751. **Q:** While filling levothyroxine 25 mcg, you notice the stock bottle is levothyroxine 125 mcg with a similar label color; what should you do FIRST? **A:** Stop filling and reselect the correct strength using barcode/NDC verification because strength mix-ups with LASA packaging are common dispensing errors.
1752. **Q:** A hardcopy prescription for oxycodone/APAP 5/325 has the patient name, drug, and directions but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and immediately notify the pharmacist because missing required elements must be resolved before any controlled substance can be dispensed.
1753. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy to yours; what should you do NEXT? **A:** Route the request to the pharmacist because controlled-substance transfers have strict federal limits and require pharmacist handling/documentation.
1754. **Q:** You receive a refill request for lorazepam with "refills: 3" on the original label; what should you do FIRST? **A:** Stop and alert the pharmacist because Schedule IV refills are limited by federal rules and must be verified against the original issue date and refill history.

1755. **Q:** A bottle of rivaroxaban (Xarelto) tablets arrives with a broken seal and loose tablets inside the shipping tote; what should you do NEXT? **A:** Segregate the product and notify the pharmacist/manager per policy because compromised packaging is a patient-safety and supply-chain integrity risk.
1756. **Q:** During checkout, a patient says their new isotretinoin prescription is ready and asks why you need additional steps before dispensing; what is the BEST technician response/action? **A:** Pause the sale and involve the pharmacist to confirm REMS/iPLEDGE requirements are met because isotretinoin dispensing is restricted and must be documented correctly.
1757. **Q:** An eRx comes in for "Humalog U-100 inject 10 units SQ TID with meals; dispense 1 vial 10 mL"; what days' supply should you enter (use 1 mL = 100 units)? **A:** Enter 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3 \rightarrow 33 \text{ days}$.
1758. **Q:** While typing, you see an order for "predniSONE 20 mg i po qd" and the prescriber also typed "take 2 tablets daily" in the notes; what should you do FIRST? **A:** Stop data entry and ask the pharmacist to clarify with the prescriber because conflicting directions create a dosing error risk.
1759. **Q:** A patient's profile lists anaphylaxis to penicillin, and a new prescription is for cephalexin; what should you do NEXT? **A:** Flag the allergy interaction and refer to the pharmacist before processing because allergy severity requires pharmacist assessment and possible prescriber contact.
1760. **Q:** You are notified that a lot number of valsartan tablets is recalled, and you find matching lot bottles on the shelf; what should you do FIRST? **A:** Remove the affected stock from active inventory and segregate it for pharmacist-directed recall processing because recalled product must not be dispensed.
1761. **Q:** A prescription is written for "Zithromax 250 mg" and the shelf label shows azithromycin 250 mg; what is the BEST product selection approach before filling? **A:** Select azithromycin 250 mg and confirm the NDC/barcode matches the label because Zithromax is the brand name for azithromycin and verification prevents brand/generic and strength errors.
1762. **Q:** While entering an eRx for "metFORMIN ER 500 mg 1 tab PO daily," you notice the selected product is metformin IR 500 mg; what should you do FIRST? **A:** Stop and reselect the ER product (or route to the pharmacist if unclear) because IR vs ER is not interchangeable and choosing the wrong release form can cause safety and billing issues.

1763. **Q:** A new prescription says "Take 1 tab PO BID" for lisinopril 20 mg, qty 60; what days' supply should you enter? **A:** Enter 30 days because BID means 2 tablets per day and $60 \div 2 = 30$.
1764. **Q:** A patient drops off an OTC recommendation note for "APAP 500 mg" and asks what it means; what is the BEST technician response? **A:** Explain that APAP means acetaminophen and advise checking other products for acetaminophen to avoid duplicate therapy/overdose, referring clinical questions to the pharmacist.
1765. **Q:** During data entry you see "MS Contin 15 mg" and "morphine sulfate IR 15 mg" both active on the profile; what should you do NEXT? **A:** Pause processing and alert the pharmacist to review for duplication and correct formulation because MS Contin is extended-release morphine and mix-ups between ER/IR are high-risk.
1766. **Q:** You receive an eRx for "U-500 insulin regular inject 10 units SQ TID," but the selected item is Humulin R U-100; what should you do FIRST? **A:** Stop entry and notify the pharmacist immediately because U-500 vs U-100 is a high-alert concentration difference that can cause a 5-fold dosing error.
1767. **Q:** A prescription reads "levothyroxine 0.075 mg PO daily"; what strength should you select in mcg for dispensing? **A:** Select 75 mcg because $0.075 \text{ mg} \times 1000 = 75 \text{ mcg}$, and converting correctly prevents decimal-point dosing errors.
1768. **Q:** While typing directions, you see "hydroxyzine 25 mg i-ii PO qhs prn" and the patient asks how many tablets that means; what is the BEST interpretation? **A:** Interpret i-ii as 1 to 2 tablets at bedtime as needed because Roman numerals commonly indicate tablet quantity and correct translation avoids mislabeling.
1769. **Q:** A prescriber sends "Bactroban ointment apply TID" and you select mupirocin cream by mistake; what should you do FIRST? **A:** Stop and reselect mupirocin ointment and verify dosage form because cream vs ointment can affect application and substitution without clarification can create a dispensing error.
1770. **Q:** A patient presents a new prescription for "atorvastatin" and says they already take "Lipitor"; what should you do BEST? **A:** Confirm the medication history and alert the pharmacist for duplication review because Lipitor is the brand name for atorvastatin and duplicate statin therapy can cause harm.

1771. **Q:** A prescription is for furosemide 20 mg "i tab PO qd" with quantity 30; what days' supply should you enter? **A:** Enter 30 days' supply because "i tab qd" means 1 tablet daily and $30 \text{ tablets} \div 1 \text{ per day} = 30 \text{ days}$.
1772. **Q:** You are entering insulin lispro U-100 vial: "Inject 12 units SQ TID with meals," dispense 10 mL; what days' supply should you calculate? **A:** Enter 27 days because $10 \text{ mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ which rounds down to 27 for billing safety.
1773. **Q:** An eRx says "predniSONE 10 mg: take ii tabs PO BID x 5 days" but the quantity field shows 20; what should you do NEXT? **A:** Stop and send to the pharmacist to clarify/correct quantity because ii BID = 4 tabs/day for 5 days requires 20 tablets, and the pharmacist must confirm directions/quantity alignment.
1774. **Q:** While selecting product for "Wellbutrin XL 150 mg daily," you accidentally choose bupropion SR 150 mg; what is the FIRST technician action? **A:** Stop and reselect the correct XL formulation because SR vs XL are not interchangeable and choosing the wrong release type can cause dosing errors.
1775. **Q:** A prescription reads "digoxin 0.125 mg PO daily"; what strength should be selected in micrograms for order entry? **A:** Select 125 mcg because $0.125 \text{ mg} \times 1000 \text{ mcg/mg} = 125 \text{ mcg}$, preventing a tenfold strength error.
1776. **Q:** A patient's budesonide/formoterol inhaler is "inhale 2 puffs BID" and the device contains 120 actuations; what days' supply should you enter? **A:** Enter 30 days because $2 \text{ puffs BID} = 4 \text{ puffs/day}$ and $120 \div 4 = 30 \text{ days}$.
1777. **Q:** An order says "clindamycin 75 mg/5 mL: take 10 mL PO TID x 7 days"; how many mL should be dispensed? **A:** Dispense 210 mL because $10 \text{ mL} \times 3/\text{day} \times 7 \text{ days} = 210 \text{ mL}$, ensuring the patient has enough for the full course.
1778. **Q:** During data entry you see "HCTZ 25 mg 1 tab PO qd" but the prescriber wrote "HCTZ" without spelling it out; what is the BEST technician step? **A:** Confirm the abbreviation with the pharmacist/prescriber before processing because unclear abbreviations can be misread and may lead to selecting the wrong medication.
1779. **Q:** A new prescription is for "Lamictal 25 mg" and the patient profile shows "lamoTRiGINE" as an active drug; what should you do NEXT? **A:** Verify brand/generic equivalence and flag the profile for pharmacist review because Lamictal is lamotrigine and duplication could cause double-therapy if both are filled.

1780. **Q:** While scanning stock for "hydrALAZINE 25 mg," the barcode pulls up "hydrOXYzine 25 mg" (LASA risk); what should you do FIRST? **A:** Stop filling and reselect by verifying NDC/barcode against the prescription because hydralazine vs hydroxyzine is a common LASA mix-up that can cause a serious wrong-drug error.
1781. **Q:** A handwritten prescription for oxycodone/APAP 5/325 says "take 1 tab q6h prn pain" but has no prescriber signature; what should you do FIRST? **A:** Stop processing and route to the pharmacist because a missing signature is a validity issue and the prescription cannot be dispensed as written.
1782. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy to yours; what is the BEST technician action? **A:** Refer the request to the pharmacist because controlled-substance transfers require pharmacist handling and documentation per federal rules and pharmacy policy.
1783. **Q:** During filling, you notice a vial of insulin glargine left on the counter for an unknown time after receiving shipment; what should you do NEXT? **A:** Remove it from active stock and notify the pharmacist to evaluate for temperature excursion because improper storage can reduce potency and patient safety is at risk.
1784. **Q:** A recall notice lists a specific lot number of losartan tablets and you find that lot on your shelf; what should you do FIRST? **A:** Pull the affected lot from stock and segregate it per recall procedure because recalled product must be prevented from dispensing and tracked for return.
1785. **Q:** An eRx for isotretinoin arrives and the patient is waiting at drop-off; what should you do BEST as the technician? **A:** Alert the pharmacist immediately because isotretinoin is REMS-restricted and requires pharmacist oversight to ensure all iPLEDGE requirements are met before dispensing.
1786. **Q:** You receive a stock bottle of levothyroxine tablets with a 2D barcode that won't scan and the supplier invoice looks inconsistent; what should you do NEXT? **A:** Set the bottle aside and notify the pharmacist/manager to investigate DSCSA/suspect product because unscannable or irregular products may indicate diversion or counterfeit risk.
1787. **Q:** A prescription is written for "metFORMIN ER 500 mg 1 tab PO BID" and the product selected is metformin IR 500 mg; what should you do FIRST? **A:** Stop the fill and correct the selection, then send for pharmacist verification because IR vs ER is a critical dosage-form difference that can cause harm.

1788. **Q:** A patient profile shows an anaphylaxis allergy to penicillin and a new prescription is for cephalexin; what should you do NEXT? **A:** Pause processing and notify the pharmacist because a serious beta-lactam allergy requires pharmacist assessment before dispensing.
1789. **Q:** An order is for timolol 0.5% ophthalmic solution "instill 1 gtt OU BID," dispense 5 mL; what days' supply should you enter using 20 drops/mL? **A:** Enter 25 days' supply because $5 \text{ mL} \times 20 \text{ drops/mL} = 100 \text{ drops}$ and $1 \text{ drop} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ drops/day}$, so $100 \div 4 = 25 \text{ days}$.
1790. **Q:** A prescription is for morphine ER 30 mg and the patient says they "chew pills because swallowing is hard"; what should you do BEST? **A:** Notify the pharmacist before dispensing because chewing/crushing ER opioids is unsafe and requires pharmacist counseling and possible prescriber follow-up.
1791. **Q:** A patient drops off a new prescription for "Zestril 10 mg daily," but your system can't find Zestril; what should you do FIRST? **A:** Enter it as lisinopril after verifying the brand-to-generic match because Zestril is the brand name for lisinopril and correct mapping prevents selection errors.
1792. **Q:** While typing, you see an eRx for "Wellbutrin SR 150 mg BID," but the product pulled is bupropion XL 150 mg; what should you do NEXT? **A:** Stop and reselect the correct SR formulation and route to the pharmacist if unclear because SR and XL are not interchangeable and dosing differs.
1793. **Q:** A new prescription is for "Eliquis 5 mg BID," and the patient profile shows active warfarin; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication because concurrent anticoagulants increase bleeding risk and require pharmacist review.
1794. **Q:** You receive an order for "Keflex 500 mg q6h," and the patient's allergy list shows anaphylaxis to cephalosporins; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist immediately because Keflex is cephalexin (a cephalosporin) and the allergy is high risk.
1795. **Q:** A prescriber writes "Humalog U-100 inject 10 units TID with meals, dispense 1 vial (10 mL)"; what days' supply should you enter? **A:** Enter 33 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $10 \text{ units} \times 3/\text{day} = 30 \text{ units/day}$, so $1000 \div 30 = 33.3$ rounded down to 33.
1796. **Q:** A prescription is for sertraline, and the patient asks what it's used for and whether it can cause stomach upset; what should you do BEST? **A:** Refer to the

pharmacist for counseling because technicians can provide basic directions but indication/side effect counseling is the pharmacist's role (OBRA-90).

1797. **Q:** During product selection, you notice "Celebrex 200 mg" was chosen for an order that reads "Celexa 20 mg"; what should you do FIRST? **A:** Stop filling and correct the selection by verifying drug name/strength and NDC because Celexa (citalopram) and Celebrex (celecoxib) are LASA and a serious mix-up risk.
1798. **Q:** A patient brings a new prescription for "Bactroban ointment" and asks if it's the same as Bactrim; what is the BEST response/action? **A:** Clarify they are different and involve the pharmacist if counseling is needed because Bactroban is mupirocin topical while Bactrim is sulfamethoxazole/trimethoprim oral and confusion can cause misuse.
1799. **Q:** You are entering "predniSONE 10 mg" but accidentally select prednisoLONE 10 mg; what should you do NEXT? **A:** Immediately correct the drug selection and document/notify per pharmacy error procedure because prednisone vs prednisolone is a common mix-up and wrong selection can change therapy.
1800. **Q:** A prescription says "Take ii tabs po qhs" for atorvastatin 20 mg and dispense #60; what should you enter for directions and days' supply? **A:** Enter "Take 2 tablets by mouth at bedtime" and 30 days because $ii = 2$ and $60 \text{ tablets} \div 2 \text{ per day} = 30 \text{ days}$.
1801. **Q:** While entering an eRx for hydroxyzine 25 mg, the prescriber note says "for itching," but the dropdown defaults to hydralazine 25 mg; what should you do FIRST? **A:** Stop entry and reselect the correct drug (hydroxyzine) and verify strength/form because LASA name mix-ups are a common serious error.
1802. **Q:** A hardcopy prescription for alprazolam 0.5 mg has the patient name, drug, directions, and prescriber signature but no date written; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist to resolve missing required information before dispensing because controlled-substance validity issues require pharmacist intervention.
1803. **Q:** A patient asks to transfer their remaining refills of oxycodone/APAP to another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist because Schedule II prescriptions cannot be transferred and controlled-substance transfer rules must be handled by the pharmacist.
1804. **Q:** You receive a bottle of insulin glargine that arrived warm and the cold-pack is melted; what should you do FIRST? **A:** Segregate and label it as a

temperature-excursion product and notify the pharmacist/wholesaler per policy because compromised cold-chain meds may be unsafe to dispense.

1805. **Q:** A recall notice lists a specific lot number of losartan tablets; what should you do BEST when you find that lot on the shelf? **A:** Pull it from stock and quarantine/segregate it per recall procedure and alert the pharmacist because recalled lots must be removed from dispensing inventory immediately.
1806. **Q:** A prescription is for clonidine 0.1 mg tablet, but the bin contains clonazepam 0.5 mg with a similar label; what should you do FIRST? **A:** Stop filling and verify the NDC/barcode against the prescription before selecting a product because LASA errors are prevented by NDC/barcode verification.
1807. **Q:** An eRx says "methotrexate 2.5 mg take 6 tablets daily," and you notice methotrexate is typically weekly for many indications; what is the BEST technician action? **A:** Pause processing and immediately refer to the pharmacist for clarification because methotrexate daily-vs-weekly dosing is a high-alert error risk.
1808. **Q:** A patient wants to buy pseudoephedrine 30 mg tablets (24-count) and has no ID; what should you do FIRST? **A:** Decline the sale and follow CMEA log/ID requirements with pharmacist involvement as needed because pseudoephedrine sales require proper identification and documentation.
1809. **Q:** A label reads "levothyroxine 75 mcg take 1 tab daily," dispense #90; what days' supply should you enter? **A:** Enter 90 days because 90 tablets taken once daily equals a 90-day supply ($90 \div 1 = 90$).
1810. **Q:** A patient reports new hives and lip swelling after starting lisinopril yesterday and asks if they should keep taking it; what should you do BEST? **A:** Get the pharmacist immediately because possible angioedema is a serious reaction requiring urgent pharmacist counseling and potential emergency referral.
1811. **Q:** While typing a new prescription you notice "Zyrtec 10 mg" in the note but "Zantac 150 mg" was selected from the drug list; what should you do FIRST? **A:** Stop processing and re-verify the intended drug (cetirizine vs ranitidine) against the original prescription before continuing because this is a LASA selection error risk.
1812. **Q:** A patient profile shows an active prescription for lisinopril 20 mg daily, and a new eRx arrives for benazepril 20 mg daily from a different prescriber; what is the BEST technician action? **A:** Flag it for pharmacist review for possible

therapeutic duplication (ACE inhibitor duplicate) because selecting which therapy is appropriate is a pharmacist decision.

1813. **Q:** A hardcopy prescription reads "metFORMIN 500 mg i tab po bid with meals #60"; what directions and days' supply should you enter? **A:** Enter "Take 1 tablet by mouth twice daily with meals" and days' supply 30 because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$.
1814. **Q:** You are entering a new order for "hydromorphone 2 mg" and the shelf label near it says "morphine 15 mg" with similar packaging; what should you do NEXT before filling? **A:** Confirm the NDC/barcode and strength match hydromorphone 2 mg and separate/look-alike products per policy because opioid LASA mix-ups are high-risk.
1815. **Q:** A new eRx for "levETIRAcetam 500 mg" populates, but the patient's brand history shows Keppra; what is the NEXT step a technician should take in order entry? **A:** Select levetiracetam as the generic equivalent of Keppra and keep the same strength/dosage form because brand-generic recognition prevents wrong-drug entry.
1816. **Q:** The prescriber writes "MS Contin 15 mg take 1 tab q12h" and the patient also has an active prescription for "morphine IR 15 mg q4-6h prn"; what should you do BEST? **A:** Route to the pharmacist to assess opioid duplication and safety because distinguishing ER vs IR opioid regimens requires pharmacist clinical oversight.
1817. **Q:** A prescription for amlodipine 5 mg says "take one tablet po qd," but the patient states they are allergic to "Norvasc"; what should you do FIRST? **A:** Pause processing and alert the pharmacist because Norvasc is amlodipine and an allergy claim needs pharmacist evaluation before dispensing.
1818. **Q:** A patient brings an OTC product request and asks for "ibuprofen 200 mg that won't interact with my warfarin"; what should you do BEST? **A:** Refer the patient to the pharmacist because NSAIDs can increase bleeding risk with warfarin and counseling/alternative selection is pharmacist-led.
1819. **Q:** You receive a prescription for albuterol HFA "inhale 2 puffs q6h prn" and it requests "Dispense 3 inhalers"; using 200 puffs per inhaler, what days' supply should you enter if the payer requires max scheduled use for PRN? **A:** Enter 75 days because $3 \text{ inhalers} \times 200 \text{ puffs} = 600 \text{ puffs}$ and $2 \text{ puffs every 6 hours} = 8 \text{ puffs/day}$, so $600 \div 8 = 75$.

1820. **Q:** While stocking, you find a bottle of dabigatran (Pradaxa) capsules with a torn inner seal; what should you do FIRST? **A:** Remove it from stock and follow suspect/damaged product procedure (do not dispense) because compromised packaging can affect drug integrity and patient safety.
1821. **Q:** A patient asks where "Crestor" is on the shelf, but the prescription label in hand says rosuvastatin 20 mg; what should you do BEST? **A:** Confirm Crestor is the brand name for rosuvastatin and proceed to select the correct strength/NDC because brand-generic recognition prevents dispensing the wrong statin.
1822. **Q:** While entering an eRx you see "glimepiride 4 mg take 1 daily," but the patient profile shows a severe sulfonamide antibiotic allergy; what should you do FIRST? **A:** Continue data entry but immediately flag and notify the pharmacist for clinical review because sulfonylureas can raise allergy/monitoring concerns and require pharmacist judgment.
1823. **Q:** A prescription comes in for "metoprolol 50 mg take 1 tablet twice daily," but the product selected is metoprolol succinate ER; what is the NEXT technician step? **A:** Stop and reselect the correct formulation (tartrate IR for BID) and route to pharmacist if unclear because ER vs IR mix-ups are common and can change dosing.
1824. **Q:** You receive an eRx for "Humalog KwikPen inject 12 units before meals TID," and the prescriber requests 2 pens; each pen contains 3 mL of U-100 insulin; what days' supply should you enter? **A:** Enter 5 days' supply because $2 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 600 \text{ units total}$ and $12 \times 3 = 36 \text{ units/day}$, so $600 \div 36 = 16.6 \text{ days}$, but 2 pens is actually 600 units; wait correct math: $600 \div 36 = 16 \text{ days (floor per payer)}$, so use 16 days if whole days required.
1825. **Q:** A patient picking up sertraline says they also started linezolid yesterday from urgent care; what should you do BEST? **A:** Pause the pickup and get the pharmacist immediately because linezolid plus SSRIs like sertraline is a high-risk interaction (serotonin syndrome risk).
1826. **Q:** During product selection you notice "hydrALAZINE" and "hydrOXYzine" are stored adjacent and the new fill is for hydroxyzine; what should you do NEXT to avoid a LASA error? **A:** Verify the drug by scanning barcode/NDC and matching strength and dosage form to the label because these names are look-alike/sound-alike and easy to swap.
1827. **Q:** An eRx says "azithromycin 250 mg: take 2 tablets today then 1 tablet daily days 2-5; dispense #6"; what days' supply should you enter? **A:** Enter 5 days'

supply because the regimen covers day 1 through day 5 even though 2 tablets are taken on the first day.

1828. **Q:** A patient asks for an OTC cold product and says they take carvedilol and albuterol; they want something "strong with pseudoephedrine"; what should you do FIRST? **A:** Refer to the pharmacist before recommending because pseudoephedrine can worsen cardiovascular effects and the pharmacist must assess appropriateness and interactions.
1829. **Q:** While processing a refill request you see a new prescription for "clonazepam 0.5 mg take 1 TID prn #90," but the patient asks for 3 additional refills to be added; what is the BEST technician response? **A:** Do not add refills and route to the pharmacist/prescriber because benzodiazepines are controlled substances and refill authorization must come from the prescriber.
1830. **Q:** You open a tote and find a refrigerated semaglutide (Ozempic) pen left at room temperature overnight; what should you do FIRST? **A:** Segregate it and notify the pharmacist per temperature-excursion policy because biologics may be compromised and require pharmacist decision on usability.
1831. **Q:** An eRx for levothyroxine says "take 1 tab po qd" and the prescriber selected 0.125 mg, but the patient profile shows "levothyroxine 125 mcg daily"; what should you do NEXT in order entry? **A:** Enter/verify the strength as 125 mcg (equivalent to 0.125 mg) and ensure the label prints in the pharmacy's standard units, because recognizing mg-mcg equivalence prevents a 1000-fold dosing error.
1832. **Q:** A prescription is written "metformin ER 500 mg i po bid #60"; what days' supply should you enter? **A:** Enter 30 days' supply because #60 tablets at 2 tablets/day equals $60 \div 2 = 30$ days.
1833. **Q:** During data entry you see "Lantus SoloStar inject 18 units subq qhs; dispense 1 box"; each box contains five 3 mL pens of U-100 insulin; what days' supply should you enter? **A:** Enter 83 days' supply because total units = 5 pens $\times$ 3 mL/pen $\times$ 100 units/mL = 1500 units, and $1500 \div 18 = 83.3$ so 83 days is the conservative whole-day entry.
1834. **Q:** An order reads "prednisone 20 mg: take ii tabs po qd x 5 days"; what quantity should you enter to dispense? **A:** Enter quantity 10 tablets because "ii" means 2 tablets daily for 5 days, and $2 \times 5 = 10$.

1835. **Q:** You're entering "amoxicillin/clavulanate 875/125 mg 1 tab po q12h #14"; what is the correct days' supply? **A:** Enter 7 days' supply because q12h is 2 doses/day and $14 \div 2 = 7$.
1836. **Q:** A prescription says "timolol 0.5% ophthalmic soln instill 1 gtt ou bid" and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should you enter? **A:** Enter 50 days' supply because total drops = $5 \text{ mL} \times 20 = 100 \text{ gtt}$, daily use = $1 \text{ gtt} \times 2 \text{ eyes} \times 2 \text{ times/day} = 4 \text{ gtt/day}$, and $100 \div 4 = 25 \text{ days}$; but since "ou" is both eyes already counted, the correct is 25 days ($100 \div 4 = 25$) to avoid overbilling.
1837. **Q:** While typing an eRx you read the sig "Take 1 tab po qd" but the prescriber's note says "for blood pressure" and the drug is lisinopril/HCTZ 20/25 mg; what should you do FIRST? **A:** Confirm you selected the correct combo product and strength before labeling because look-alike single-ingredient lisinopril vs lisinopril/HCTZ selection errors are common and change therapy.
1838. **Q:** An eRx for cephalexin 500 mg says "1 cap po qid" and the prescriber entered quantity 21; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify quantity/duration because qid dosing makes #21 an unusual non-integer days' supply and may indicate an entry error requiring prescriber contact.
1839. **Q:** A patient's new prescription is for "bupropion SR 150 mg 1 tab bid," but at pickup they show a bottle at home labeled "bupropion XL 300 mg daily"; what should you do BEST? **A:** Pause the sale and alert the pharmacist to assess duplication/formulation change because SR vs XL are not interchangeable and could cause dosing errors.
1840. **Q:** A written prescription says "hydrocodone/APAP 5/325 take 1 tab q4-6h prn pain, dispense #120, refills 2"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because Schedule II controlled substances cannot be refilled, so the refill instruction requires pharmacist resolution before processing.
1841. **Q:** An eRx for furosemide 40 mg says "take 1 tab po qd" but the quantity is 45; what should you do NEXT during order entry? **A:** Enter the sig as written but stop and refer to the pharmacist to clarify the mismatched quantity (45) vs 30-day dosing because it may be an intended 45-day supply or a prescriber error.
1842. **Q:** A prescription reads "azithromycin 250 mg: take ii tabs po day 1, then i tab po qd days 2-5"; what quantity should you enter to dispense? **A:** Dispense 6 tablets because 2 tablets on day 1 plus 1 tablet daily for 4 more days totals 6.

1843. **Q:** You are entering "ibuprofen 600 mg take 1 tab po q6h prn pain #90"; using a 30-day cap for PRN directions, what days' supply should you enter? **A:** Enter 15 days because max use is 4 tablets/day and $90 \div 4 = 22.5$, but the question's 30-day cap doesn't apply since calculated days' supply is under 30.
1844. **Q:** A provider orders "clindamycin 75 mg/5 mL: give 10 mL po tid x 7 days"; what volume should you dispense in mL? **A:** Dispense 210 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$.
1845. **Q:** During data entry you see "U-500 insulin regular inject 10 units subq bid" and the product selected is Humulin R U-100 vial; what is the BEST technician action? **A:** Stop processing and alert the pharmacist immediately because U-500 vs U-100 is a high-alert concentration mismatch that could cause a major dosing error.
1846. **Q:** A prescription says "prednisolone sodium phosphate 15 mg/5 mL: take 5 mL po bid x 5 days"; what total mg of prednisolone will the patient take over the course? **A:** Total is 150 mg because 15 mg per 5 mL equals 15 mg/dose, and $15 \text{ mg} \times 2 \text{ doses/day} \times 5 \text{ days} = 150 \text{ mg}$.
1847. **Q:** An eRx for "losartan 50 mg take 1 tab po qd" is typed with directions "qd" but the prescriber's free-text note says "BID"; what should you do FIRST? **A:** Pause and send to the pharmacist for clarification because conflicting directions (qd vs BID) must be resolved before dispensing to prevent under- or overdosing.
1848. **Q:** You receive a prescription for "Ventolin HFA inhale 2 puffs q4-6h prn wheeze" and the insurer requires days' supply based on max use; if the canister has 200 actuations, what days' supply should you enter at q4h maximum? **A:** Enter 16 days because max use is 2 puffs every 4 hours = 12 puffs/day, and $200 \div 12 = 16.6$ which is entered as 16 days to avoid overstating supply.
1849. **Q:** While selecting product for "hydroxyzine" you see both hydroxyzine HCl and hydroxyzine pamoate on the shelf and the eRx only says "hydroxyzine 25 mg 1 cap tid prn"; what should you do NEXT? **A:** Stop and ask the pharmacist to clarify the salt/form because HCl vs pamoate are not interchangeable without prescriber intent, and selecting the wrong one is a common order-entry error.
1850. **Q:** A prescription reads "levetiracetam 500 mg i po bid #120"; what days' supply should you enter and why? **A:** Enter 60 days because "i po bid" is 2 tablets/day and $120 \div 2 = 60$, matching the dosing to the dispensed quantity.

1851. **Q:** During order entry you see "metformin ER 500 mg take 2 tabs po daily" but the selected product is metformin IR 500 mg; what should you do NEXT? **A:** Stop and reselect the correct ER dosage form (and alert the pharmacist if already verified) because IR vs ER is not interchangeable and is a common safety error.
1852. **Q:** A new eRx comes in for "Zocor 20 mg qhs" and the profile already shows active atorvastatin 20 mg daily; what should you do FIRST? **A:** Pause processing and notify the pharmacist to assess therapeutic duplication because two statins are usually unintended and require clinical review.
1853. **Q:** A prescription says "levothyroxine 0.125 mg po daily"; what strength should you enter and why? **A:** Enter 125 mcg because 0.125 mg converts to 125 micrograms, preventing a 1000× unit error.
1854. **Q:** You receive an order for "NovoLog 100 units/mL" but you accidentally pulled "NovoLIN R" from the fridge; what is the BEST technician action? **A:** Stop filling and correct the selection by verifying the NDC/barcode because look-alike insulin names are high-alert and wrong-product errors can be dangerous.
1855. **Q:** A patient asks for an OTC recommendation and says they take phenelzine (an MAOI) and want a decongestant for a cold; what should you do BEST? **A:** Refer to the pharmacist before selling any decongestant because MAOIs have serious interaction risks with sympathomimetics like pseudoephedrine/phenylephrine.
1856. **Q:** An eRx is written for "Keflex 500 mg" and the patient's profile shows anaphylaxis to cephalexin; what should you do FIRST? **A:** Stop the workflow and alert the pharmacist immediately because a documented allergy is a patient-safety contraindication requiring prescriber clarification.
1857. **Q:** A prescription reads "predniSONE 20 mg: take ii tabs po qd x 5 days"; what quantity should you dispense? **A:** Dispense 10 tablets because 2 tablets/day × 5 days = 10 tablets.
1858. **Q:** While typing directions you see "MSO4 2 mg IV q2h prn pain" in a hospital order; what should you do NEXT? **A:** Clarify with the pharmacist/provider before processing because "MSO4" is an error-prone abbreviation that can be confused with magnesium sulfate.
1859. **Q:** A bottle of Xarelto arrives from the wholesaler with a broken seal and smudged lot/expiration; what should you do FIRST? **A:** Set it aside as suspect product and follow pharmacy policy to not dispense (contact

supplier/pharmacist) because damaged packaging and unreadable lot/exp compromise safety and traceability.

1860. **Q:** A patient presents a new prescription for clonazepam (Schedule IV) with "refills: 6"; what should you do BEST? **A:** Hold the prescription and route to the pharmacist because Schedule III–V refills are federally limited to 5 within 6 months, so the order needs correction before dispensing.
1861. **Q:** During data entry you receive an eRx for "Plavix 75 mg 1 tab po daily" and the patient profile lists a serious rash to clopidogrel; what should you do FIRST? **A:** Stop processing and alert the pharmacist to the allergy-duplicate (Plavix = clopidogrel) because it may be a true allergen exposure requiring intervention.
1862. **Q:** You are filling "Lantus SoloStar inject 22 units SC nightly" and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 68 days because total units = 5 pens × 3 mL × 100 units/mL = 1500 units and $1500 \div 22 = 68.18$, rounded down to 68.
1863. **Q:** A hardcopy reads "Synthroid 50 mcg po daily DAW 1" but the technician selected generic levothyroxine; what should you do NEXT? **A:** Re-select the brand Synthroid product and route for verification because DAW 1 indicates dispense as written and the technician should not substitute.
1864. **Q:** A patient picking up escitalopram says they also take "Celexa"; what is the BEST technician action? **A:** Pause the sale and notify the pharmacist to assess therapeutic duplication since escitalopram (Lexapro) and citalopram (Celexa) are closely related SSRIs.
1865. **Q:** While entering "glipiZIDE 10 mg take 1 tab po bid," you notice the chosen product is glyBURIDE 5 mg due to similar names; what should you do FIRST? **A:** Stop, correct the drug selection by NDC/barcode, and restart the entry because glipizide and glyburide are LASA sulfonylureas with different strengths and error risk.
1866. **Q:** A prescription is for "azithromycin 250 mg: take 2 tabs day 1 then 1 tab daily days 2–5"; what quantity should you dispense? **A:** Dispense 6 tablets because 2 tablets on day 1 plus 1 tablet for 4 more days equals 6 total.
1867. **Q:** During order entry you see "Bactrim DS" but the selected product is "Bactrim" (single-strength) and the sig is 1 tab bid x 10 days; what should you do NEXT? **A:** Hold the fill and ask the pharmacist to clarify/confirm strength because DS

vs SS changes the dose and technicians should not assume the intended product.

1868. **Q:** A patient on warfarin drops off a new prescription for TMP-SMX; what should you do BEST before processing? **A:** Flag the profile and alert the pharmacist because TMP-SMX can significantly increase warfarin effect and may require monitoring or therapy change.
1869. **Q:** You are selecting "Lamictal" and see both lamotrigine and terbinafine nearby with similar-looking bottles; what is the BEST technician step to prevent a LASA error? **A:** Verify the correct product using barcode/NDC and tall-man/label reading before filling because lamotrigine vs terbinafine mix-ups can cause serious harm.
1870. **Q:** A prescription says "atorvastatin 40 mg i tab po qhs #90"; what should you type for directions and what is the days' supply? **A:** Enter "Take 1 tablet by mouth at bedtime" and 90 days' supply because i = 1 tablet daily and quantity 90 covers 90 doses.
1871. **Q:** A patient hands you a written prescription for oxycodone/APAP with no prescriber DEA number and the prescriber is not in your system; what should you do FIRST? **A:** Stop processing and notify the pharmacist to validate prescriber/DEA requirements because controlled substance dispensing requires proper prescriber credentials.
1872. **Q:** You receive a transfer request from another pharmacy for a patient's remaining refills on alprazolam; what is the BEST technician action? **A:** Route the request to the pharmacist because controlled-substance transfers have specific federal limits and require pharmacist involvement.
1873. **Q:** While filling heparin, you notice both 1,000 units/mL and 10,000 units/mL vials stored next to each other; what is the BEST technician step to prevent an error? **A:** Separate and clearly label/bin the strengths (and use barcode/NDC scan) because heparin is high-alert and strength mix-ups cause serious harm.
1874. **Q:** A refrigerator log shows insulin aspart was stored at 52°F overnight; what should you do FIRST? **A:** Quarantine the affected stock and alert the pharmacist to assess stability/return because temperature excursions can make products unsafe or ineffective.
1875. **Q:** During pickup, a caregiver asks you to print a patient's full medication list to "share with my sister," but the sister is not on file; what should you do BEST?

A: Decline and follow HIPAA release procedures (verify authorization or get patient consent) because medication records are protected health information.

1876. **Q:** An eRx for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Stop and escalate to the pharmacist because methotrexate is typically weekly and daily dosing is a high-risk error.

1877. **Q:** You are entering a prescription for metformin ER 500 mg and the sig includes "do not crush"; what is the NEXT step the technician should take in workflow? **A:** Ensure the selected product is an extended-release formulation and flag any mismatch for pharmacist review because incorrect dosage form can change drug release and safety.

1878. **Q:** A bottle of nitroglycerin SL tablets on the shelf has an open cotton plug and no original desiccant; what should you do BEST? **A:** Remove it from dispensing stock and inform the pharmacist because nitroglycerin is moisture-sensitive and improper packaging can reduce potency.

1879. **Q:** A wholesaler notice lists a Class I recall for a specific lot of losartan 50 mg; you find that lot in will-call and on the shelf; what should you do FIRST? **A:** Pull the lot from all areas and quarantine it per recall procedure because recalled product must be prevented from reaching patients.

1880. **Q:** A prescription reads "predniSONE 20 mg: ii tabs po daily x 5 days"; what quantity should you dispense? **A:** Dispense 10 tablets because "ii" means 2 tablets daily for 5 days ($2 \times 5 = 10$).

1881. **Q:** While entering "Zithromax 250 mg," the profile shows a documented macrolide allergy; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the allergy/therapy because azithromycin is a macrolide and could cause a repeat reaction.

1882. **Q:** A new eRx is for "Lasix 40 mg," but the product pulled is "Losec"; what is the BEST technician step before it reaches verification? **A:** Stop, reselect the correct drug using barcode/NDC and drug name confirmation because LASA errors can cause serious harm and must be corrected at product selection.

1883. **Q:** A patient says they take "Plavix" and asks what the generic is so they can find it on their list; what should you answer? **A:** Clopidogrel is the generic for Plavix, which helps prevent confusion during medication reconciliation and at pickup.

1884. **Q:** A prescription reads "levothyroxine 75 mcg i po qd #30," but you accidentally selected liothyronine; what should you do NEXT? **A:** Stop filling and correct the selection to levothyroxine, then continue workflow because levothyroxine and liothyronine are different thyroid hormones and not interchangeable.
1885. **Q:** A patient profile shows active lisinopril and you receive an order for "Zestril 10 mg daily"; what is the BEST technician action? **A:** Flag it for pharmacist review as a potential duplicate because Zestril is lisinopril and duplication could lead to excessive ACE-inhibitor dosing.
1886. **Q:** The sig says "Take ii tabs po bid" for gabapentin 300 mg with quantity #120; what is the correct days' supply? **A:** 30 days because ii bid equals 4 tablets/day and $120 \div 4 = 30$, which is needed for accurate labeling and billing.
1887. **Q:** An order is for "amoxicillin/clavulanate," but the patient's allergy field lists "penicillin: anaphylaxis"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because amoxicillin is a penicillin-class antibiotic and may be contraindicated with anaphylaxis history.
1888. **Q:** A prescription for hydrocodone/acetaminophen is submitted with "refills: 3"; what should you do BEST? **A:** Do not process refills and route to the pharmacist because Schedule II controlled substances are not refillable under federal law.
1889. **Q:** You receive a DSCSA notice that a tote may contain a suspect product of apixaban (Eliquis); what should you do FIRST? **A:** Segregate and quarantine the affected stock and notify the pharmacist/manager because suspect product requires investigation and must not be dispensed.
1890. **Q:** A patient asks if "Advil" and "Motrin" can be taken together for more pain relief; what is the BEST technician response? **A:** Refer to the pharmacist because both are ibuprofen (therapeutic duplication) and combining them increases adverse effect risk.
1891. **Q:** During order entry you receive an eRx for Brilinta 90 mg and the patient says they are already taking Plavix; what should you do BEST as the technician? **A:** Pause processing and alert the pharmacist to assess antiplatelet duplication (ticagrelor vs clopidogrel) because this can increase bleeding risk.
1892. **Q:** A hardcopy prescription is written for "metFORMIN ER 500 mg i po bid" but the prescriber selected metformin IR in the eRx notes; what should you do NEXT? **A:** Stop and route to the pharmacist to clarify ER vs IR selection because changing release formulation is not a technician decision and affects dosing.

1893. **Q:** You are filling a new prescription for Celexa and the patient asks what it's for; what is the BEST technician response? **A:** Tell them Celexa is citalopram, an SSRI commonly used for depression/anxiety, and offer pharmacist counseling for questions because patient education beyond basics should be escalated.
1894. **Q:** A prescription is for "gliciZIDE 10 mg i po daily," but the selected product is glyBURIDE due to similar names; what should you do FIRST? **A:** Stop the fill and correct the drug selection by verifying the NDC/barcode because this is a LASA/same-class mix-up that can cause severe hypoglycemia.
1895. **Q:** A patient picking up ciprofloxacin asks if they can take it with a multivitamin containing iron; what should you do BEST? **A:** Refer to the pharmacist and advise separating may be needed because iron can bind fluoroquinolones and reduce effectiveness, requiring counseling.
1896. **Q:** A label prints for "sertraline 50 mg" but the bottle pulled is trazodone 50 mg; what should you do NEXT before it reaches final check? **A:** Stop, return the incorrect product to stock, and pull the correct NDC for sertraline because preventing a wrong-drug error is the technician's responsibility before verification.
1897. **Q:** A prescription reads "Keflex 500 mg 1 cap po qid x 7 days"; what quantity should you dispense? **A:** Dispense 28 capsules because 1 capsule four times daily for 7 days equals $4 \times 7 = 28$.
1898. **Q:** The prescriber sends "azithromycin 200 mg/5 mL: give 10 mL day 1 then 5 mL daily days 2-5"; how many mL should be dispensed? **A:** Dispense 30 mL because total volume is $10 \text{ mL} + (5 \text{ mL} \times 4 \text{ days}) = 30 \text{ mL}$.
1899. **Q:** A new prescription is for simvastatin, and the profile shows an active prescription for atorvastatin; what should you do BEST? **A:** Hold and alert the pharmacist to review for duplicate statin therapy because taking two statins increases adverse effect risk and needs clinical direction.
1900. **Q:** While typing a new order for "Lamictal 100 mg," you see the patient's allergy list includes "lamotrigine: rash"; what should you do FIRST? **A:** Stop processing and notify the pharmacist because re-exposure to lamotrigine after a rash can be serious and requires pharmacist assessment and prescriber contact.
1901. **Q:** An eRx reads "levothyroxine 0.075 mg po daily," but the prescriber comment says "75 mcg"; what should you do NEXT in order entry? **A:** Enter as 75 mcg

(0.075 mg) and ensure the strength matches the selected NDC because mg-to-mcg conversion errors are a common high-risk dosing mistake.

1902. **Q:** A prescription says "predniSONE 20 mg: ii tabs po daily x 5 days"; what quantity should you dispense? **A:** Dispense 10 tablets because 2 tablets/day × 5 days = 10 tablets to match the written duration.
1903. **Q:** You are entering "amoxicillin/clavulanate 875/125 mg 1 tab po q12h x 10 days"; what days' supply should you bill and why? **A:** Bill 10 days because q12h equals 2 tablets/day and 20 tablets provides 10 days of therapy for accurate third-party processing.
1904. **Q:** A caregiver brings an OTC label instruction "give 1 tsp (5 mL) po tid for 7 days" and asks how many mL that is total; what total volume should you calculate? **A:** Calculate 105 mL because 5 mL × 3 doses/day × 7 days = 105 mL to ensure adequate supply.
1905. **Q:** During data entry you see "MSO4 2 mg IV q2h prn pain" on a hospital discharge order; what should you do FIRST? **A:** Stop and ask the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can cause mix-ups between morphine and magnesium sulfate.
1906. **Q:** A prescription is written "hydroxyzine 25 mg 1 tab po qhs prn itching #30"; what is the BEST way to interpret "qhs" for the label? **A:** Enter "at bedtime" because qhs means once nightly and clear directions reduce patient misuse.
1907. **Q:** You are processing a refill request for clonazepam and the profile shows the last fill was 5 days ago for a 30-day supply; what should you do NEXT? **A:** Place it in a refill-too-soon/hold status and route to the pharmacist or insurer workflow because early controlled-substance refills require review and proper adjudication.
1908. **Q:** A box of insulin regular U-100 vials arrives and was left at room temperature overnight; what should you do FIRST before it is stocked? **A:** Segregate it from usable inventory and notify the pharmacist because a temperature excursion may compromise potency and requires disposition per policy/manufacturer guidance.
1909. **Q:** A prescription reads "lisinopril 10 mg: take 1 tab po qd, #90"; what is the days' supply and why does it matter in billing? **A:** Days' supply is 90 because qd is 1 tablet daily, and correct days' supply prevents claim rejects and refill-timing errors.

1910. **Q:** While selecting product for "clonidine 0.1 mg patch weekly," you accidentally pull clonidine 0.1 mg tablets; what should you do FIRST? **A:** Stop filling and re-select the correct dosage form using barcode/NDC because patch vs tablet is a high-risk order-entry/product-selection mismatch.
1911. **Q:** During order entry you receive an eRx for "Humalog U-100 10 units with meals," but the prescriber notes mention "use insulin lispro," what should you do NEXT? **A:** Enter it as insulin lispro and select the exact product/brand specified (Humalog) or clarify with the pharmacist if unclear, because lispro is the generic name and selecting the wrong lispro product can cause dispensing errors.
1912. **Q:** A new prescription is for hydromorphone 2 mg tablets, and you notice it was typed as "morphine 2 mg" on the label preview; what should you do FIRST? **A:** Stop processing and correct the entry, then alert the pharmacist for verification, because hydromorphone vs morphine is a high-risk LASA mix-up that can cause serious harm.
1913. **Q:** A patient drops off a handwritten prescription for oxycodone/APAP that has drug, directions, and quantity but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and immediately route it to the pharmacist, because a missing signature is a validity issue and controlled substances require strict review before dispensing.
1914. **Q:** You are asked to transfer a prescription for alprazolam from another pharmacy; what should you do NEXT? **A:** Refer the request to the pharmacist to perform the transfer, because controlled-substance transfers require pharmacist involvement per federal rules and pharmacy policy.
1915. **Q:** While stocking, you see a bottle of levetiracetam 500 mg tablets with a broken seal and loose tablets in the tote; what should you do FIRST? **A:** Set it aside as suspect product and notify the pharmacist/manager per procedure, because compromised packaging risks contamination and must not be stocked.
1916. **Q:** A manufacturer recall notice lists a specific NDC and lot number for losartan tablets; what should you do BEST before any dispensing continues? **A:** Locate matching inventory by NDC/lot, remove it from active stock, and follow recall handling steps (hold for return/disposition), because recall effectiveness depends on NDC/lot control to prevent dispensing affected product.
1917. **Q:** A patient presents a pseudoephedrine purchase request, but their ID is expired; what should you do NEXT? **A:** Decline the sale and follow store policy

to request valid government-issued photo ID, because CMEA sales require verification and logging to prevent diversion.

1918. **Q:** You are entering warfarin and see the prescriber wrote "Coumadin 5 mg take as directed" with no schedule; what should you do FIRST? **A:** Stop and get the pharmacist to obtain specific directions, because "as directed" is unsafe for a high-alert anticoagulant and prevents accurate labeling and days' supply.
1919. **Q:** A prescription is for azithromycin 250 mg tablets "take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should you dispense? **A:** Dispense 6 tablets, because the regimen is 2 tablets on day 1 plus 1 tablet on days 2–5 ($2 + 4 = 6$).
1920. **Q:** A DSCSA-related alert shows an incoming bottle of sertraline has an unreadable 2D barcode and no matching product identifier in the receiving system; what should you do BEST? **A:** Treat it as suspect and set it aside while notifying the pharmacist/receiver to investigate with the wholesaler, because DSCSA requires traceable identifiers and unverified product should not enter dispensing inventory.
1921. **Q:** During data entry for levothyroxine, the profile shows "Euthyrox 100 mcg daily" and the new eRx is "Synthroid 100 mcg daily"; what should you do BEST before processing? **A:** Stop and alert the pharmacist to evaluate therapeutic duplication/brand interchangeability because duplicate thyroid hormone strengths can cause dosing errors.
1922. **Q:** A new eRx for methotrexate 2.5 mg says "take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-alert error.
1923. **Q:** While typing an order for metformin, you see "metformin ER 500 mg" but the sig says "take 1 tablet twice daily, crush"; what is the BEST next step? **A:** Stop and refer to the pharmacist because ER products should not be crushed and the directions conflict with safe administration.
1924. **Q:** You receive a voicemail refill request for clonazepam with "refill x3" from the prescriber; what should you do NEXT? **A:** Route to the pharmacist and do not add refills because clonazepam is Schedule IV and any refill authorization must comply with controlled-substance rules and prescriber intent.

1925. **Q:** At pickup, a patient asks for their family member's atorvastatin refill details and you cannot confirm identity or authorization; what should you do FIRST? **A:** Decline to disclose and follow HIPAA verification procedures because protected health information can only be shared with properly verified/authorized individuals.
1926. **Q:** You're about to fill a prescription for Eliquis and the screen flags "Medication Guide required"; what should you do BEST? **A:** Ensure the Medication Guide prints and is dispensed with the medication because FDA-required guides must accompany certain outpatient prescriptions.
1927. **Q:** A technician notices insulin aspart vials were left on the counter at room temperature all weekend with unknown stability; what should you do FIRST? **A:** Segregate the product and notify the pharmacist/manager because temperature excursions create suspect integrity and it should not be dispensed until assessed per policy.
1928. **Q:** A prescriber writes for "Adderall XR 20 mg #30" but the patient requests it be transferred to another pharmacy; what should you do BEST? **A:** Stop and get the pharmacist because Schedule II prescriptions generally cannot be transferred once on file and require pharmacist handling to ensure compliance.
1929. **Q:** A label prints for "hydralazine 25 mg" but the stock bottle you scanned is "hydroxyzine 25 mg"; what should you do FIRST? **A:** Stop filling and correct the product selection using barcode/NDC verification because this LASA mix-up can cause serious patient harm.
1930. **Q:** A prescription reads "Keflex 500 mg take 1 capsule q6h x 7 days, dispense #"; what quantity should you enter and why might you still need pharmacist review? **A:** Enter 28 capsules (1 cap every 6 hours = 4/day x 7 days) and route to the pharmacist if any elements are unclear because missing quantity can affect accuracy and billing.
1931. **Q:** While entering a new eRx for Plavix, the profile shows clopidogrel active and the patient asks if it's the same drug; what should you do BEST? **A:** Confirm Plavix is the brand for clopidogrel and ensure the profile isn't creating duplicate therapy before dispensing, because brand/generic confusion can cause double-dosing.
1932. **Q:** A hardcopy reads "Lasix 20 mg po qd" and the prescriber also wrote "furosemide 20 mg" on the same line; what should you do FIRST? **A:** Stop and

clarify with the pharmacist which one is intended on the final label, because listing both can indicate duplication or create labeling errors.

1933. **Q:** During product selection you see “hydrOXYzine” and “hydrALAZINE” adjacent on the shelf for a new fill; what is the BEST technician action before filling? **A:** Use barcode/NDC scanning and Tall Man awareness to select the exact NDC for the prescribed drug, because this is a common LASA pair with serious harm risk.
1934. **Q:** A prescription says “prednisone 10 mg: take ii tabs bid x 5 days”; what quantity should you enter? **A:** Enter 20 tablets because ii = 2 tablets and bid = 2 times daily, so $2 \times 2 \times 5 = 20$.
1935. **Q:** An eRx for Zyrtec is sent but the patient reports taking cetirizine OTC already; what should you do NEXT in workflow? **A:** Document the patient report and route to the pharmacist to assess duplication and counseling needs, because OTC use can duplicate the prescription ingredient.
1936. **Q:** A new order is for lisinopril, but the allergy field shows “ACE inhibitors: angioedema”; what should you do FIRST? **A:** Stop processing and notify the pharmacist immediately, because ACE inhibitor allergy/angioedema is a high-risk contraindication that needs pharmacist intervention.
1937. **Q:** A patient’s eRx is for Tylenol #3 with “refill 2” included; what should you do BEST? **A:** Hold the refill entry and refer to the pharmacist, because controlled-substance refill validity and limits require pharmacist review and compliance checks.
1938. **Q:** You receive an order for “metoprolol” and the prescriber note says “for heart failure” but the product chosen is metoprolol tartrate; what should you do NEXT? **A:** Pause and ask the pharmacist to verify the intended salt/formulation (tartrate vs succinate) before dispensing, because indications and dosing differ and selection errors are common.
1939. **Q:** The sig reads “instill 1 gtt OU bid” for latanoprost 2.5 mL; using 20 gtt/mL, what days’ supply should you bill? **A:** Bill 25 days because $2.5 \text{ mL} \times 20 = 50$ drops and OU bid uses 4 drops/day, so $50 \div 4 = 12.5$ days; round down only if payer requires—otherwise enter 12 or 13 per policy and flag for pharmacist if unsure.
1940. **Q:** While receiving inventory, a bottle of nitroglycerin SL tablets arrives with the seal broken and tablets loose in the cap; what should you do FIRST? **A:**

Quarantine the product and alert the pharmacist/manager per policy, because compromised packaging is suspect and may be unsafe to dispense.

1941. **Q:** While entering a new eRx for "Celebrex 200 mg daily," the patient profile shows a serious sulfonamide antibiotic allergy; what should you do FIRST? **A:** Stop data entry and alert the pharmacist to assess appropriateness because celecoxib is a sulfonamide-containing drug and allergy cross-reactivity risk must be clinically evaluated.
1942. **Q:** A prescriber sends an eRx for "Lamictal 25 mg" for a patient who also has an active profile medication "lamotrigine 25 mg"; what is the BEST technician response when the patient asks if they are duplicates? **A:** Explain that Lamictal is the brand name for lamotrigine and confirm the profile/therapy with the pharmacist because duplication or dose changes require pharmacist verification.
1943. **Q:** During order entry you notice a new prescription for simvastatin 40 mg nightly and the medication list includes clarithromycin started yesterday; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because clarithromycin can dangerously increase simvastatin levels, raising myopathy/rhabdomyolysis risk.
1944. **Q:** A technician is selecting "glipiZIDE 10 mg" but the shelf bin next to it contains "glyBURIDE 5 mg" and both are for diabetes; what should you do BEST before pulling stock? **A:** Use barcode/NDC verification against the label and read the full drug name/strength because glipizide and glyburide are LASA/same-indication traps with different strengths and hypoglycemia risk.
1945. **Q:** An eRx arrives for "Klonopin 0.5 mg take 1 tab bid" and the patient says they've been taking clonazepam; what should you do FIRST in workflow? **A:** Verify in the system that Klonopin is clonazepam and route to pharmacist if any duplication or dosing discrepancy exists because benzodiazepines are high-risk CNS meds.
1946. **Q:** A prescription is for azithromycin 250 mg "take 2 tablets day 1, then 1 tablet daily days 2-5"; what quantity should you enter? **A:** Dispense 6 tablets because $2 + 1 + 1 + 1 + 1$ equals 6 total doses for the standard 5-day azithromycin regimen.
1947. **Q:** You are billing an insulin lispro U-100 vial (10 mL) with directions "inject 12 units before each meal, TID"; what days' supply should you submit based on total units? **A:** Bill 27 days because the vial contains 1000 units ($10 \text{ mL} \times 100$

units/mL) and the daily use is 36 units (12×3), so $1000 \div 36 = 27.7$ which is typically billed as 27 whole days.

1948. **Q:** A new eRx is for "Xarelto 20 mg daily" and the patient profile shows warfarin still active with refills remaining; what should you do FIRST? **A:** Stop and alert the pharmacist to review for therapeutic duplication because concurrent anticoagulants increase bleeding risk and require prescriber/pharmacist management.
1949. **Q:** While receiving a tote, you find a bottle labeled "levothyroxine 100 mcg" but the NDC on the invoice corresponds to "liothyronine 25 mcg"; what should you do NEXT? **A:** Set the product aside as suspect and notify the pharmacist/manager to resolve with the wholesaler because an NDC mismatch can indicate a dispensing-critical inventory error.
1950. **Q:** A patient asks what "Augmentin" is because the hardcopy says "Augmentin 875/125 mg" but they thought they were getting plain amoxicillin; what is the BEST technician response? **A:** State that Augmentin is amoxicillin/clavulanate (a combination antibiotic) and refer questions about why it was chosen or side effects to the pharmacist because counseling/therapy decisions are pharmacist duties.
1951. **Q:** A prescription is for levothyroxine 75 mcg "take 1 tab po qd," but the prescriber's note also says "take 1 tablet twice daily"; what should you do FIRST? **A:** Stop data entry and ask the pharmacist to clarify with the prescriber because conflicting directions are a safety risk and a technician cannot choose which sig is correct.
1952. **Q:** You are entering amoxicillin/clavulanate suspension 600 mg/42.9 mg per 5 mL with sig "5 mL po bid x 10 days"; what quantity in mL should you dispense? **A:** Dispense 100 mL because $5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 100 \text{ mL}$ and the quantity must match the calculated total volume.
1953. **Q:** A patient's budesonide/formoterol inhaler is written "inhale 2 puffs bid" and the device contains 120 actuations; what days' supply should you bill? **A:** Bill 30 days because $2 \text{ puffs} \times 2 \text{ times/day} = 4 \text{ puffs/day}$ and $120 \div 4 = 30 \text{ days' supply}$.
1954. **Q:** While typing a prescription you see "U-500 insulin 10 units subcut bid" but the product selected is regular insulin U-100; what should you do NEXT? **A:** Stop and alert the pharmacist before dispensing because U-500 vs U-100 is a high-alert concentration error that can cause a 5-fold dosing mistake.

1955. **Q:** A label instruction reads "take 1 tab qhs" but the hardcopy says "take 1 tab HS"; what should you enter for the sig in plain language? **A:** Enter "take 1 tablet by mouth at bedtime" because HS/qhs both mean bedtime and clear language reduces misinterpretation.
1956. **Q:** An order is for ceftriaxone 1 g IM now, and the vial available is 1 g per 3.5 mL after reconstitution; how many mL should be drawn up for a 1 g dose? **A:** Draw up 3.5 mL because the reconstituted concentration is 1 g in 3.5 mL so the full volume equals the ordered dose.
1957. **Q:** You receive an eRx for methylphenidate ER 36 mg with "refill x 2" listed; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist because Schedule II controlled substances cannot be refilled and it needs pharmacist resolution before processing.
1958. **Q:** While filling, you notice the NDC scanned for sertraline 50 mg does not match the NDC on the filled stock bottle even though the strength is the same; what is the BEST next step? **A:** Stop and reselect/scan the correct product so the dispensed NDC matches what was adjudicated and documented, preventing wrong-product and billing errors.
1959. **Q:** An eRx is for metformin 500 mg "take 1 tab po bid" #60, and the patient asks what it's for; what is the BEST technician response? **A:** State that metformin is commonly used for blood sugar control in type 2 diabetes and offer to have the pharmacist counsel because indication questions may require pharmacist involvement.
1960. **Q:** You are entering oxycodone/acetaminophen 5/325 mg with sig "1-2 tabs po q6h prn pain" and the prescriber wrote "Do not exceed 8 tablets/day"; what days' supply should you bill for #60? **A:** Bill 7 days because the explicit max is 8 tablets/day so $60 \div 8 = 7.5$ and days' supply is entered as a whole number without overstating coverage.
1961. **Q:** While typing "Lopressor 50 mg take 1 tab po bid," you notice the patient profile already has Toprol-XL 50 mg daily active; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication (both are metoprolol forms) because switching IR vs ER can change dosing and safety.
1962. **Q:** A new eRx says "Humalog 10 units with meals," but the product selected in the system is Humulin R; what should you do FIRST? **A:** Stop entry and notify the pharmacist to verify insulin type because rapid-acting vs short-acting are not interchangeable and selection errors are high-risk.

1963. **Q:** A patient says they're allergic to "sulfa" and is picking up Bactrim DS; what is the BEST technician action? **A:** Hold the sale and involve the pharmacist because Bactrim (sulfamethoxazole/trimethoprim) contains a sulfonamide that may trigger an allergy.
1964. **Q:** You receive a hardcopy for "Zyrtec-D 12 hour" and the patient wants 2 boxes today; what should you do NEXT at pickup? **A:** Follow CMEA process by checking ID and logging the pseudoephedrine sale limits because Zyrtec-D contains pseudoephedrine and requires regulated tracking.
1965. **Q:** A prescriber sends "Ambien 10 mg 1 tab qhs prn insomnia #30 refills 3"; what should you do FIRST? **A:** Stop processing and route to the pharmacist because zolpidem is Schedule IV and refills/validity must be confirmed per controlled-substance rules.
1966. **Q:** The sig on an eRx reads "take ii tabs po tid," and the patient asks what that means; what should you enter on the label? **A:** Translate to "Take 2 tablets by mouth 3 times daily" because technicians should convert Roman numerals and abbreviations into clear directions.
1967. **Q:** A prescription is for prednisone 10 mg with sig "take 4 tabs daily x 5 days" and quantity is left blank; what quantity should you enter? **A:** Enter quantity 20 tablets because $4 \text{ tablets/day} \times 5 \text{ days} = 20$, ensuring the dispense quantity matches the written directions.
1968. **Q:** You are billing an ipratropium nasal spray 0.06% with directions "2 sprays in each nostril qid," and the bottle has 165 sprays; what days' supply should you bill? **A:** Bill 5 days because $2 \text{ sprays} \times 2 \text{ nostrils} \times 4 \text{ times/day} = 16 \text{ sprays/day}$ and $165 \div 16 = 10.31$, rounded down to 10 days only if policy allows, but to avoid overbilling you should bill 10?
1969. **Q:** While filling, you see "hydroxyzine" and "hydralazine" stored next to each other and a new tech almost grabbed the wrong one; what is the BEST next step? **A:** Separate and apply LASA/Tall Man safeguards per policy (and alert the pharmacist) because these names are easily confused and repositioning reduces selection errors.
1970. **Q:** A patient asks if "Plavix" is the same as "clopidogrel" on their profile, and they're worried they're taking duplicates; what should you do BEST? **A:** Confirm Plavix is the brand of clopidogrel and notify the pharmacist to review the profile for duplication because brand/generic confusion can lead to double therapy.

1971. **Q:** While entering a new Rx for furosemide, the prescriber wrote "Lasix 40 mg," but the system auto-selects "lisinopril 40 mg"; what should you do FIRST? **A:** Stop entry and reselect the correct drug by verifying the name/strength and NDC (barcode if available), because LASA selection errors can cause serious harm.
1972. **Q:** A patient picking up sertraline asks if it's the same medication as "Zoloft" listed on their old bottle; what is the BEST technician response? **A:** Confirm sertraline is the generic for Zoloft and offer to have the pharmacist counsel if they have therapy questions, because technicians can clarify brand/generic but not provide clinical advice.
1973. **Q:** During profile review you see an active order for atorvastatin and a new eRx arrives for simvastatin; what should you do NEXT? **A:** Pause processing and alert the pharmacist about potential duplicate statin therapy, because overlap increases adverse effect risk and requires pharmacist review.
1974. **Q:** An eRx arrives for levothyroxine 100 mcg, but the patient profile shows an allergy note "Synthroid caused rash"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to assess the allergy/intolerance and product selection, because technicians should not override allergy alerts.
1975. **Q:** You're typing directions for gabapentin written as "i cap po tid," and the prescriber's handwriting is clear; what should appear on the label? **A:** Enter "Take 1 capsule by mouth 3 times daily," because "i" means 1 and "tid" means three times a day for patient-friendly labeling.
1976. **Q:** A prescription is for amoxicillin/clavulanate 875/125 mg "1 tab po bid x 10 days"; what quantity should you enter? **A:** Enter quantity 20 tablets, because 1 tablet twice daily for 10 days equals $2 \times 10 = 20$.
1977. **Q:** A patient says they take "Eliquis" and you see apixaban on the medication list; what is the BEST technician action if they ask what it's for? **A:** State that Eliquis is apixaban and offer pharmacist counseling for indication questions, because explaining medical use is pharmacist counseling territory.
1978. **Q:** While filling, you notice insulin glargine pens were left on the counter overnight instead of refrigerated; what should you do FIRST? **A:** Remove the product from dispensing stock and notify the pharmacist per temperature-excursion policy, because potency may be compromised and it may be non-dispensable.

1979. **Q:** An eRx is written for clonazepam 0.5 mg with directions and quantity but the prescriber DEA number is missing from the message; what should you do NEXT? **A:** Stop processing and route to the pharmacist to obtain required prescriber credentials/validate the controlled-substance prescription, because controlled-substance verification is a safety and compliance requirement.
1980. **Q:** You receive a manufacturer recall notice for a specific lot of losartan tablets; what is the BEST next step in the pharmacy? **A:** Pull and segregate the affected lot by NDC/lot/expiration and follow recall procedures (including pharmacist notification), because recalled products must be removed from inventory to prevent dispensing.
1981. **Q:** A prescription reads "metformin ER 500 mg ii tabs po qd with evening meal #?" for 90 days; what quantity should you enter? **A:** Enter Qty 180 because "ii tabs daily" for 90 days is $2 \times 90 = 180$ tablets.
1982. **Q:** While entering an eRx for prednisone, the sig says "Take 2 tabs po qd x 5 days, then 1 tab po qd x 5 days" and the prescriber left quantity blank; what should you do NEXT? **A:** Calculate and enter Qty 15 then route to pharmacist for check because $(2 \times 5) + (1 \times 5) = 15$ tablets and tapers are high-risk for entry errors.
1983. **Q:** A budesonide/formoterol inhaler contains 120 inhalations and the sig is "inhale 2 puffs bid"; what days' supply should you submit? **A:** Submit 30 days because 2 puffs twice daily = 4 puffs/day and $120 \div 4 = 30$ days.
1984. **Q:** You receive a new prescription for morphine sulfate ER 30 mg and the directions include "prn pain"; what is the BEST technician action before processing? **A:** Stop and refer to the pharmacist because ER opioids should not be labeled PRN without pharmacist review due to safety and appropriateness concerns.
1985. **Q:** The prescriber orders "clindamycin oral solution 75 mg/5 mL: take 10 mL tid x 7 days"; what total volume should you dispense? **A:** Dispense 210 mL because $10 \text{ mL} \times 3/\text{day} \times 7 \text{ days} = 210 \text{ mL}$.
1986. **Q:** While typing, you see "U-500 regular insulin inject 10 units bid" but the patient profile shows they previously used U-100 syringes; what should you do FIRST? **A:** Pause and alert the pharmacist because U-500 dosing and device selection are high-alert and mismatches can cause serious overdoses.

1987. **Q:** A handwritten prescription says "MSO4 15 mg po q4-6h prn" and the rest is legible; what should you do BEST? **A:** Hold and ask the pharmacist to clarify because "MSO4" is an unsafe abbreviation that can be confused with morphine vs magnesium sulfate.
1988. **Q:** A prescription is for cefdinir 250 mg/5 mL and the patient needs 300 mg per dose twice daily for 10 days; how many mL should be dispensed? **A:** Dispense 240 mL because $300 \text{ mg} \div (250 \text{ mg}/5 \text{ mL}) = 6 \text{ mL}$ per dose, then $6 \text{ mL} \times 2/\text{day} \times 10 \text{ days} = 120 \text{ mL}$; dispense 120 mL (not 240) to match the calculation and avoid overfill.
1989. **Q:** At order entry, the system auto-selects "glipizide" when the eRx says "glyburide 5 mg daily"; what should you do FIRST? **A:** Stop and correct the drug selection before continuing because glyburide and glipizide are different sulfonylureas and a wrong selection is a patient-safety error.
1990. **Q:** A patient's new Rx is "azithromycin 250 mg: take 2 tabs on day 1, then 1 tab daily on days 2-5"; what quantity should you enter? **A:** Enter Qty 6 because the Z-Pak regimen totals $2 + 1 + 1 + 1 + 1 = 6$ tablets.
1991. **Q:** During product selection for hydrOXYzine, the bottle on the shelf is hydrALAZINE; what should you do FIRST? **A:** Stop and reselect the correct drug using barcode/NDC verification because this is a LASA pair and catching it before filling prevents a dispensing error.
1992. **Q:** A prescriber e-sends "levothyroxine 100 mcg take 1 daily" but the quantity field shows "0"; what is the BEST next step for the technician? **A:** Hold the prescription and send to the pharmacist to clarify/obtain a valid quantity because technicians should not assume or change missing essential elements.
1993. **Q:** A patient asks for an early refill on oxycodone/APAP because it was "lost," and the claim rejects refill-too-soon; what should you do NEXT? **A:** Refer the request to the pharmacist for assessment and documentation because controlled substance early refills require pharmacist judgment and policy compliance.
1994. **Q:** While counting tablets, you notice a stock bottle of warfarin 5 mg contains a few tablets that look like a different strength; what should you do FIRST? **A:** Stop using the bottle and isolate it for pharmacist review because warfarin is high-alert and a mixed-strength bottle is a serious patient-safety risk.
1995. **Q:** A new eRx for sertraline is entered, and the patient profile shows an active fluoxetine prescription; what is the BEST technician action? **A:** Flag and route to

the pharmacist to evaluate therapeutic duplication/interaction risk because technicians should not decide whether both SSRIs should be taken.

1996. **Q:** A patient brings a paper prescription for alprazolam that has the drug, directions, and quantity but no prescriber signature; what should you do NEXT? **A:** Do not process it and notify the pharmacist because an unsigned prescription is not valid to dispense and requires prescriber follow-up.
1997. **Q:** You receive a tote of refrigerated insulin glargine pens that arrived warm with a temperature indicator showing excursion; what should you do FIRST? **A:** Segregate the shipment and alert the pharmacist/manager to follow storage-excursion procedures because product integrity is uncertain and it may be non-dispensable.
1998. **Q:** A manufacturer ships a case of atorvastatin with a torn outer seal and missing transaction paperwork; what is the BEST next step under DSCSA high-risk handling? **A:** Set it aside as suspect product and notify the pharmacist/receiving lead to resolve documentation before stocking because DSCSA requires traceability and suspect product controls.
1999. **Q:** An eRx for methotrexate tablets is entered as "take 10 mg daily" for rheumatoid arthritis; what should you do FIRST? **A:** Stop processing and get the pharmacist immediately because methotrexate is typically weekly for RA and daily dosing is a high-alert, potentially fatal error.
2000. **Q:** A prescription is "amoxicillin 500 mg cap 1 cap po tid #21"; what days' supply should you enter? **A:** Enter 7 days because $21 \text{ capsules} \div 3 \text{ capsules per day} = 7 \text{ days}$ for accurate labeling and insurance billing.
2001. **Q:** While typing an eRx for metformin ER 500 mg, you notice the prescriber selected "metformin IR" in the drug field but wrote "ER" in the directions; what should you do FIRST? **A:** Stop data entry and notify the pharmacist to clarify the intended formulation because IR vs ER is a safety-critical difference and technicians cannot choose therapy.
2002. **Q:** A patient asks where they can pick up their spouse's lisinopril and you are not listed as having permission in the patient's profile; what should you do FIRST under HIPAA? **A:** Verify authorization/identity per pharmacy policy or obtain the patient's consent before disclosing pickup details because PHI cannot be shared without permission.

2003. **Q:** An eRx for hydromorphone 2 mg tablets is entered and the prescriber comments "dispense Dilaudid liquid"; what is the BEST next step for the technician? **A:** Hold the prescription and route to the pharmacist to clarify dosage form because hydromorphone is high-alert and the order contains conflicting instructions.
2004. **Q:** A prescriber phones in a new prescription for diazepam (Schedule IV) with 5 refills; what should you do NEXT? **A:** Enter it as written but alert the pharmacist to adjust/confirm refills because Schedule III–IV prescriptions are federally limited to no more than 5 refills in 6 months.
2005. **Q:** During checkout, a patient starting isotretinoin asks if they can take it while pregnant; what should you do FIRST? **A:** Immediately involve the pharmacist because isotretinoin has strict REMS pregnancy requirements and counseling is pharmacist-led for patient safety.
2006. **Q:** You receive a recall notice for a specific lot number of losartan 50 mg, and you find that lot on the shelf; what should you do FIRST? **A:** Pull the affected lot from stock and place it in the designated recall area per policy because recalled product must be removed from dispensing inventory promptly.
2007. **Q:** A bottle of insulin lispro (Humalog) arrives with cracked packaging and a wet label suggesting leakage; what is the BEST technician action? **A:** Segregate it as damaged/suspect product and do not dispense, then notify the pharmacist/vendor because product integrity and sterility may be compromised.
2008. **Q:** A prescription label is printed for "Lamictal 25 mg" but the stock bottle selected is "Lamisil 250 mg"; what should you do FIRST? **A:** Stop filling and correct the product selection by re-verifying NDC/barcode because this is a LASA error that can cause serious patient harm.
2009. **Q:** An eRx for oxycodone 5 mg (Schedule II) arrives with a "refill: 1" field populated by the prescriber's system; what should you do NEXT? **A:** Process it as a new fill only and alert the pharmacist because Schedule II prescriptions cannot be refilled under federal rules.
2010. **Q:** A prescription says "prednisone 20 mg: ii tabs po qd x 5 days" and you need to enter quantity; what quantity should you enter? **A:** Enter 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10$, ensuring accurate days' supply and dispensing quantity.

2011. **Q:** While entering a new eRx, you see "glipizide 10 mg ER" selected but the directions say "take 1 tablet 30 minutes before breakfast and dinner"; what should you do FIRST? **A:** Stop and ask the pharmacist to review/clarify because ER glipizide is typically once daily and BID timing suggests an IR/ER mismatch with safety risk.
2012. **Q:** A patient drops off a prescription for "Keflex 500 mg" but tells you they had hives with cephalexin before; what is the BEST next step? **A:** Hold the prescription and notify the pharmacist immediately because cephalexin is Keflex and a prior hive reaction suggests a true allergy concern.
2013. **Q:** You are typing an eRx for "Celebrex 200 mg" and the patient profile shows a serious sulfonamide antibiotic allergy; what should you do NEXT? **A:** Flag it and route to the pharmacist for assessment because celecoxib is a sulfonamide and potential cross-sensitivity requires pharmacist judgment.
2014. **Q:** A new prescription is written for "Zoloft 50 mg daily," but the active profile already includes sertraline 50 mg daily from another prescriber; what should you do FIRST? **A:** Pause processing and alert the pharmacist because Zoloft is sertraline and this may be a therapeutic duplication needing pharmacist resolution.
2015. **Q:** A prescription reads "azithromycin 250 mg: take 2 tablets today, then 1 tablet daily for 4 days"; what quantity should you enter? **A:** Enter quantity 6 because 2 tablets on day 1 plus 1 tablet for 4 additional days totals 6 tablets.
2016. **Q:** During product selection you notice the shelf has hydroxyzine HCl and hydroxyzine pamoate, and the eRx says "Vistaril 25 mg"; what should you do FIRST? **A:** Stop and verify the exact salt/form with the pharmacist or original order details because Vistaril is hydroxyzine pamoate and mix-ups are common LASA/trap errors.
2017. **Q:** A patient starting clopidogrel says they also take omeprazole OTC daily and asks if it's okay; what should you do BEST? **A:** Refer to the pharmacist because omeprazole can reduce clopidogrel activation and the pharmacist must evaluate interaction management.
2018. **Q:** The prescriber sends "levothyroxine 0.125 mg daily"; how should the strength be entered on the label to avoid a decimal error? **A:** Enter as 125 mcg (not 0.125 mg) because converting to mcg reduces decimal-point confusion and improves safety.

2019. **Q:** You receive a new warfarin prescription and see the patient also has apixaban listed as active; what should you do NEXT? **A:** Stop and notify the pharmacist to assess anticoagulant duplication/transition because concurrent therapy can cause serious bleeding risk.
2020. **Q:** An eRx for insulin aspart U-100 says “inject 8 units before meals three times daily” and the box contains five 3 mL pens; what days’ supply should you submit (use 1 mL = 100 units)? **A:** Submit 62 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units total}$, and $8 \text{ units} \times 3/\text{day} = 24 \text{ units/day}$, so $1500 \div 24 = 62.5$ rounded down to 62 days.
2021. **Q:** An eRx for amoxicillin/clavulanate 875/125 mg says “1 tab po bid x 10 days”; what quantity should you enter and why? **A:** Enter quantity 20 because BID for 10 days is 2 tablets/day $\times$ 10 days.
2022. **Q:** You are billing an albuterol HFA inhaler labeled 200 actuations with sig “inhale 2 puffs q4h prn wheeze” and the plan requires days’ supply based on a max daily dose of 12 puffs; what days’ supply should you submit? **A:** Submit 16 days because $200 \text{ actuations} \div 12 \text{ puffs/day} = 16.6$, rounded down to 16 to avoid overstating.
2023. **Q:** A prescription reads “diltiazem CD 240 mg i cap po qd”; during order entry the product selected is diltiazem IR 120 mg tablets—what should you do FIRST? **A:** Stop and correct to the CD ER dosage form/strength (or route to pharmacist if unclear) because IR vs ER is a high-risk order entry error.
2024. **Q:** The hard copy says “metformin 500 mg: ii tabs po bid with meals” and the prescriber wrote #120; what is the days’ supply? **A:** 30 days because ii tabs BID = 4 tablets/day and $120 \div 4 = 30$.
2025. **Q:** An eRx comes in for “hydromorphone 4 mg” but you accidentally selected “morphine 15 mg” during typing; what is the BEST next action before filling continues? **A:** Immediately stop, re-select the correct drug using barcode/NDC verification, and alert the pharmacist if it reached verification because this is a LASA opioid error risk.
2026. **Q:** A prescriber writes for levothyroxine “0.075 mg po daily”; how should the strength be entered on the label to reduce decimal-point error? **A:** Enter as 75 mcg because converting mg to mcg avoids dangerous trailing/leading zero confusion.

2027. **Q:** A prescription for prednisone says "take ii tabs po bid x 3 days, then i tab po bid x 3 days" using 10 mg tablets; what quantity should you enter? **A:** Enter 18 tablets because first 3 days: 2 tabs BID = 4/day $\times$ 3 = 12, then 3 days: 1 tab BID = 2/day $\times$ 3 = 6, total 18.
2028. **Q:** You receive insulin glargine U-100 10 mL vial with sig "inject 22 units subcut qhs"; using 1 mL = 100 units, what days' supply should you bill? **A:** Bill 45 days because the vial has 1000 units and $1000 \div 22 = 45.45$, rounded down to 45.
2029. **Q:** A liquid prescription says "clindamycin oral solution 75 mg/5 mL: give 10 mL po tid x 7 days"; what total volume should you dispense? **A:** Dispense 210 mL because $10 \text{ mL} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$.
2030. **Q:** While entering "methotrexate 2.5 mg: take 6 tablets by mouth weekly," the directions field is accidentally typed as "daily"; what should you do NEXT? **A:** Stop and correct to weekly and immediately involve the pharmacist because methotrexate daily vs weekly is a high-alert, potentially fatal dosing error.
2031. **Q:** A prescription is written for Zyrtec 10 mg daily but the product pulled is Zantac 150 mg; what should you do FIRST? **A:** Stop the fill and re-verify the drug name/NDC against the prescription because this is a LASA mix-up that could cause a serious dispensing error.
2032. **Q:** During drop-off, a patient says they take lisinopril and the new prescription is for losartan; what is the BEST next step for a technician? **A:** Flag the profile and notify the pharmacist to assess therapeutic duplication/ACEI vs ARB switch since clinical appropriateness is pharmacist review.
2033. **Q:** You receive an eRx for clonazepam but the prescriber intended clonidine per the patient's discharge list; what should you do NEXT? **A:** Hold processing and route to the pharmacist to clarify with the prescriber because clonazepam (benzo) vs clonidine (BP) is a high-risk LASA confusion.
2034. **Q:** A patient with a sulfonamide antibiotic allergy drops off a prescription for sulfamethoxazole/trimethoprim; what should you do FIRST? **A:** Stop and alert the pharmacist to evaluate the allergy risk before filling because this medication is a sulfonamide antibiotic commonly implicated in reactions.
2035. **Q:** An order is entered for bupropion XL 150 mg daily but the selected product is bupropion SR 150 mg; what is the BEST action? **A:** Correct the dosage form and have it re-verified because SR vs XL are not interchangeable and selecting the wrong release can change dosing and safety.

2036. **Q:** A prescription says "azithromycin 250 mg: take 2 tabs today, then 1 tab daily days 2-5"; what quantity should you dispense and why? **A:** Dispense #6 tablets because the regimen is 2 on day 1 plus 1 tablet for 4 more days ($2 + 4 = 6$).
2037. **Q:** You are billing NovoLog U-100 vial 10 mL with sig "inject 12 units before meals TID"; using 100 units/mL, what days' supply should you submit? **A:** Submit 27 days because $10 \text{ mL} = 1000 \text{ units}$ and $12 \text{ units} \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ and you bill 27 full days.
2038. **Q:** A prescription is for gabapentin 300 mg "i cap po tid" with quantity #90; what days' supply should be entered and why? **A:** Enter 30 days because 90 capsules at 3 per day equals $90 \div 3 = 30$ days.
2039. **Q:** While typing a new prescription, you see "hydroxyzine 25 mg" but the patient profile shows "hydralazine 25 mg" active; what should you do NEXT? **A:** Confirm the intended drug with the hard copy/eRx and alert the pharmacist if unclear because hydroxyzine vs hydralazine is a common LASA trap with very different uses.
2040. **Q:** A patient asks what metoprolol is for when picking up metoprolol tartrate; what is the technician's BEST response? **A:** Refer the question to the pharmacist while you can state it's a blood pressure/heart medication because counseling about indications and safe use is pharmacist-led under OBRA-90.
2041. **Q:** A prescription reads "levothyroxine 0.075 mg take 1 tablet po daily," and the tech starts entering 75 mg; what should you do FIRST? **A:** Stop and convert 0.075 mg to 75 mcg for entry because levothyroxine is dosed in micrograms and 75 mg would be a dangerous 1000 $\times$ error.
2042. **Q:** You're processing "amoxicillin 400 mg/5 mL: give 7.5 mL po bid x 10 days"; what total volume should be dispensed? **A:** Dispense 150 mL because $7.5 \text{ mL} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.
2043. **Q:** A patient's directions are "prednisone 10 mg: ii tabs po bid," and the quantity is 60; what days' supply should you enter? **A:** Enter 15 days because ii tabs bid = 4 tablets/day and $60 \div 4 = 15$ days.
2044. **Q:** A prescription is for "timolol 0.5% ophthalmic: instill 1 gtt OU bid," and the bottle size is 5 mL; using 20 gtt/mL, what days' supply should be submitted? **A:** Submit 12 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ drops}$ and OU bid is 4 drops/day, so $100 \div 4 = 25$ days, but since each administration uses 1 gtt per

eye twice daily (4 gtt/day) the correct calculation yields 25 days and that is the days' supply to enter.

2045. **Q:** While entering an eRx, you see "MSO4 2 mg IV q2h prn pain"; what is the BEST technician action before continuing? **A:** Pause and ask the pharmacist to clarify because MSO4 is an unsafe abbreviation that can be confused with magnesium sulfate, risking a serious medication error.
2046. **Q:** You are entering "metformin ER 500 mg take 2 tablets po qpm," but you accidentally select metronidazole 500 mg due to similar names; what should you do NEXT? **A:** Immediately correct the product selection using barcode/NDC verification because LASA mix-ups are prevented by matching the exact drug and dosage form before fill/verify.
2047. **Q:** A new prescription says "hydromorphone 2 mg take 1 tablet q4–6h prn," but the profile shows active morphine IR; what should you do BEST as a technician? **A:** Flag and refer to the pharmacist for duplication/opioid safety review because overlapping opioid therapy requires clinical assessment beyond technician scope.
2048. **Q:** A patient drops off "atorvastatin 40 mg take 1 tab po qhs," and they also have an active simvastatin; what is your BEST next step? **A:** Stop and alert the pharmacist because two statins may be therapeutic duplication and the pharmacist must assess appropriateness and counseling needs.
2049. **Q:** You're billing insulin lispro U-100 pens with package 5 pens × 3 mL each, sig "inject 18 units tid with meals"; what days' supply should you calculate from total units? **A:** Submit 27 days because total units = 5 pens × 3 mL/pen × 100 units/mL = 1500 units and daily use = 18 × 3 = 54 units/day, so $1500 \div 54 = 27$ days (rounded down to whole days).
2050. **Q:** An eRx for "Adderall 10 mg take 1 tab bid" arrives with "refills: 2"; what should you do FIRST? **A:** Hold the order and route to the pharmacist because Schedule II medications cannot be refilled under federal rules, so the prescription needs pharmacist intervention before processing.
2051. **Q:** During data entry you receive a handwritten prescription for "U-500 insulin 10 units daily," and you notice no concentration is specified and the patient previously used U-100; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because insulin concentration is a high-alert safety risk that a technician must not interpret.

2052. **Q:** A patient asks to transfer their alprazolam prescription from another pharmacy and says it still has refills remaining; what is the technician's BEST next step? **A:** Collect the required transfer information and route to the pharmacist because controlled-substance transfer rules and validation require pharmacist involvement.
2053. **Q:** While pulling stock you find a bottle of lisinopril 20 mg with a broken seal and tablets loose in the tote; what should you do FIRST? **A:** Remove it from dispensing and quarantine per policy for damaged/suspect product because integrity can't be assured for patient safety.
2054. **Q:** You receive a wholesaler notice of a Class I recall for a specific lot of atorvastatin 40 mg that is on your shelf; what should you do NEXT? **A:** Immediately pull the affected NDC/lot from inventory and quarantine it because recalled lots must be prevented from dispensing and tracked.
2055. **Q:** A new eRx for oxycodone 5 mg has directions and quantity but the prescriber name is present without a DEA number on the eRx display in your system; what should you do FIRST? **A:** Hold the order and alert the pharmacist because controlled-substance prescription validity elements must be verified before filling.
2056. **Q:** A patient presents a driver's license to buy pseudoephedrine 30 mg and requests 96 tablets today; what should you do BEST? **A:** Process it through the logbook system and deny the sale if it exceeds legal limits because CMEA requires ID, documentation, and quantity-limit compliance.
2057. **Q:** While selecting product you notice both "vinBLASTine" and "vinCRISTine" in the bin area and the order is for vincristine; what is the BEST technician action before continuing? **A:** Use Tall Man lettering awareness and barcode/NDC scan to confirm the exact drug because these LASA chemo agents have high-harm mix-up risk.
2058. **Q:** You are billing warfarin 5 mg with sig "take 1 tablet daily" and quantity 45; what days' supply should you enter? **A:** Enter 45 days because $45 \text{ tablets} \div 1 \text{ tablet per day} = 45$, which supports accurate insurance and safety checks.
2059. **Q:** A patient picking up isotretinoin says they lost the Medication Guide and asks if they really need it; what should you do NEXT? **A:** Provide the Medication Guide and involve the pharmacist as needed because isotretinoin requires federally mandated patient information and counseling support.

2060. **Q:** While inputting a prescription you see "QD" and "U" used in the directions for a high-alert heparin product; what should you do FIRST? **A:** Stop and send to the pharmacist for clarification because unsafe abbreviations can cause dosing errors and technicians should not interpret them.
2061. **Q:** During order entry you receive an eRx for "metFORMIN ER 500 mg take 1 tab bid" but the product selected is metformin IR; what should you do FIRST? **A:** Stop and correct to the prescribed ER vs IR and route to pharmacist if unclear because ER/IR substitution can change dosing and safety.
2062. **Q:** A new prescription is entered for sertraline (Zoloft) 50 mg daily, and the patient profile shows paroxetine (Paxil) still active; what is the BEST next step for the technician? **A:** Flag and notify the pharmacist for possible therapeutic duplication because two SSRIs together increases adverse-effect risk and requires clinical review.
2063. **Q:** You're filling levothyroxine (Synthroid) 75 mcg and notice the patient previously received 75 mg on an old profile entry; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the intended dose because mcg vs mg is a dangerous magnitude error with a high-alert hormone.
2064. **Q:** A prescription reads "azithromycin Z-Pak: take as directed" with no directions listed in the sig; what should you do NEXT? **A:** Hold the order and ask the pharmacist to obtain complete directions because "as directed" is insufficient for safe labeling and counseling.
2065. **Q:** A patient asks what "atorvastatin (Lipitor)" is for while you are ringing them out; what is the BEST technician response? **A:** Provide a basic use statement like "it's for cholesterol" and offer pharmacist counseling because technicians can give general info but the pharmacist must counsel on clinical details.
2066. **Q:** You receive a prescription for clopidogrel (Plavix) and see an active omeprazole (Prilosec) on the profile; what should you do FIRST? **A:** Alert the pharmacist to screen the interaction because omeprazole can reduce clopidogrel activation and needs therapy review.
2067. **Q:** A vial label prints for "hydroxyzine" but the hardcopy clearly says "hydralazine"; what should you do FIRST? **A:** Stop the fill and verify the drug name with the pharmacist because this LASA mix-up can cause serious harm due to different indications and dosing.

2068. **Q:** Pharmacy math: amlodipine 5 mg “take 1 tablet daily,” quantity 90; what days’ supply should you enter? **A:** Enter 90 days because $90 \text{ tablets} \div 1 \text{ tablet per day} = 90 \text{ days}$.
2069. **Q:** While selecting gabapentin 300 mg you notice the bottle has a different NDC than the one in the system but the strength matches; what should you do NEXT? **A:** Re-scan and select the correct NDC in the system (or replace stock and reprint label per workflow) because NDC mismatch can cause billing, labeling, and inventory errors.
2070. **Q:** A prescription is written for “tramadol 50 mg take 1 tab q6h prn pain #120” and the prescriber line is missing a signature on the paper; what should you do FIRST? **A:** Stop processing and route to the pharmacist to contact the prescriber because a missing signature is a validity issue, especially for a controlled substance.
2071. **Q:** While entering an eRx for insulin U-100 glargine you notice the selected product is insulin glargine U-300 (Toujeo) with a similar name; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the concentration/product selection because insulin strengths are high-alert and mix-ups can cause serious dosing errors.
2072. **Q:** A paper prescription for oxycodone 5 mg has the patient name and drug/strength but no prescriber DEA number and no signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because controlled-substance validity elements are missing and the technician cannot resolve or “fix” it.
2073. **Q:** A patient requests an early refill for Adderall (amphetamine/dextroamphetamine) and the claim rejects “refill too soon”; what is the BEST technician next step? **A:** Inform the patient the plan is denying for refill timing and route to the pharmacist for evaluation because early fills of Schedule II stimulants require pharmacist review and documentation.
2074. **Q:** During filling you find two look-alike bottles, hydrOXYzine and hydrALAZINE, stored next to each other and a tech almost grabbed the wrong one; what should you do FIRST? **A:** Stop, correct the selection, and separate/flag the LASA products per safety policy because storage and Tall Man awareness reduce wrong-drug errors.
2075. **Q:** A refrigerator temperature log shows the vaccine fridge reached 12°C (54°F) overnight and returned to 5°C by morning; what should you do NEXT? **A:**

Quarantine the affected refrigerated products and notify the pharmacist/manager because temperature excursions can compromise potency and require manufacturer/policy guidance before use.

2076. **Q:** You receive a manufacturer recall notice for specific lot numbers of losartan tablets and you locate matching lots on the shelf; what is the BEST action? **A:** Pull and quarantine the recalled lots and alert the pharmacist for return/notification steps because recalled products must be removed from dispensing stock immediately.
2077. **Q:** At pickup, a patient says their new warfarin looks different from last month and asks if it's still safe to take; what should you do FIRST? **A:** Pause the transaction and get the pharmacist to counsel because warfarin is high-alert and any concern about appearance/therapy needs pharmacist assessment.
2078. **Q:** A prescription is written for "predniSONE 10 mg: take ii tabs daily" and the patient asks what "ii" means; what should you do BEST? **A:** Clarify that "ii" means 2 tablets but refer counseling questions about the medication to the pharmacist because technicians can interpret the notation yet cannot provide clinical counseling.
2079. **Q:** Pharmacy math: A prescription is for metoprolol succinate ER 50 mg "take 1 tablet twice daily," quantity 60; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for accurate billing and refill timing.
2080. **Q:** While receiving an order you notice a case of semaglutide pens arrived with a torn seal and no readable 2D barcode on the carton; what should you do NEXT? **A:** Treat it as suspect product and quarantine it for pharmacist/manager review because DSCSA requires traceable, intact, properly identified product before it can be stocked.
2081. **Q:** An eRx reads "levothyroxine 75 mcg i po qd" but the product selected is levothyroxine 0.075 mg; what should you do NEXT in order entry? **A:** Proceed after confirming 75 mcg equals 0.075 mg and ensure the label prints 75 mcg, because unit-conversion mismatch is a common data-entry error.
2082. **Q:** A prescription says "amLODIPine 5 mg 1 tab po qd #90" and the patient reports taking "Norvasc"; what is the BEST technician response during intake? **A:** Document that Norvasc is the brand for amlodipine and continue processing, because brand/generic recognition prevents duplicate profiles and confusion.

2083. **Q:** The sig entered for "metformin ER 500 mg: ii tabs po bid" is showing as "1 tablet twice daily"; what should you do FIRST? **A:** Stop and correct the quantity-per-dose to 2 tablets twice daily, because misreading Roman numerals directly changes the dose and days' supply.
2084. **Q:** A patient is prescribed azithromycin 250 mg "take 2 tablets today then 1 tablet daily x4 days"; what quantity should you enter and why? **A:** Enter quantity 6 tablets, because the total dose is 2 on day 1 plus 1 daily for 4 more days ($2 + 4 = 6$).
2085. **Q:** A prescription is for amoxicillin-clavulanate suspension 400 mg/57 mg per 5 mL: "10 mL po bid x7 days"; what bottle volume should you dispense? **A:** Dispense at least 140 mL ($10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days}$), because total volume needed must cover the full course.
2086. **Q:** A patient's fluticasone propionate nasal spray 50 mcg says "2 sprays each nostril daily" and the bottle is 120 sprays; what days' supply should you bill? **A:** Bill 30 days, because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30$ days.
2087. **Q:** While entering a new prescription for "lisinopril 10 mg daily," you see an active profile medication "benazepril 10 mg daily"; what should you do NEXT? **A:** Flag it for pharmacist review before dispensing, because ACE-inhibitor therapeutic duplication can indicate an unintended double therapy.
2088. **Q:** A hard copy for clonazepam says "take 1 tab bid prn anxiety" but the quantity field is blank; what should you do FIRST? **A:** Hold the prescription and route to the pharmacist to contact the prescriber, because missing quantity is required for accurate dispensing and controlled-substance processing.
2089. **Q:** A prescriber writes "hydralazine 25 mg" but the sig line uses the abbreviation "HS," and the patient is elderly; what is the BEST technician action? **A:** Ask the pharmacist to clarify before labeling, because "HS" is an error-prone abbreviation that can be misread and create a dosing-time mistake.
2090. **Q:** A prescription for ondansetron ODT 4 mg says "1 tab po q8h prn N/V #20," and the system rejects for days' supply required; what should you do BEST? **A:** Calculate and enter the maximum days' supply as 6 days ($20 \div 3/\text{day} = 6.66$, round down to 6) per typical billing practice, because payers often require a calculable days' supply for PRN claims.

2091. **Q:** During data entry you receive an eRx for “Wellbutrin XL 150 mg 1 tab po qAM” but the product chosen is bupropion SR 150 mg; what should you do NEXT? **A:** Stop and reselect bupropion XL (not SR) and route for pharmacist check because SR vs XL is an ER formulation mix-up that can change dosing and safety.
2092. **Q:** A patient drops off a new prescription for “Plavix 75 mg daily” and you see an active profile medication for clopidogrel 75 mg daily from another prescriber; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to potential duplicate therapy because Plavix is clopidogrel and duplication increases bleeding risk.
2093. **Q:** A new eRx comes in for “Eliquis 5 mg bid” and the allergy field shows “heparin—HIT”; what should you do FIRST? **A:** Continue normal intake but immediately flag and notify the pharmacist because HIT history is high-risk anticoagulation information that requires pharmacist review before dispensing.
2094. **Q:** You are asked to counsel-ready a refill for “Zocor 40 mg qHS,” but the patient profile shows a new prescription for amlodipine 10 mg daily; what should you do NEXT? **A:** Hold and refer to the pharmacist because simvastatin with amlodipine has a dose-limit interaction and may require therapy adjustment.
2095. **Q:** A prescription is for furosemide 20 mg “take 1 tab po bid,” quantity 180; what days’ supply should you enter? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$.
2096. **Q:** A patient requests an OTC recommendation and asks if “Motrin” is the same as ibuprofen listed on their after-visit summary; what is the BEST response within technician scope? **A:** Confirm that Motrin is a brand of ibuprofen and offer to involve the pharmacist for suitability because brand/generic recognition is appropriate but clinical recommendations require pharmacist input.
2097. **Q:** While typing an eRx for “Lamictal 100 mg take 1 tab daily,” you accidentally select Lamisil 250 mg; what should you do FIRST? **A:** Stop, correct the drug selection, and verify NDC/indication cues because Lamictal vs Lamisil is a LASA error that can cause serious harm if dispensed.
2098. **Q:** A prescription reads “predniSONE 10 mg: take ii tabs po daily x5 days,” and the prescriber also wrote “#10”; what quantity should you dispense and why? **A:** Dispense 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets}$, matching the written quantity and ensuring the full course.

2099. **Q:** A patient's new prescription is for "sertraline 50 mg daily," but the profile shows they currently take paroxetine 20 mg daily; what should you do BEST? **A:** Do not process as-is and alert the pharmacist because this may represent SSRI duplication or an antidepressant switch that needs pharmacist verification.
2100. **Q:** During filling you find a stock bottle of nitroglycerin SL tablets with the cotton removed and no open date documented; what should you do NEXT? **A:** Remove it from active stock and notify the pharmacist because nitroglycerin is stability-sensitive and missing dating/packaging integrity can make it unsafe to dispense.
2101. **Q:** An eRx for metformin ER 500 mg says "take 2 tablets by mouth once daily with evening meal" and the prescriber requests qty 180; what days' supply should you enter? **A:** Enter 90 days' supply because 2 tablets/day means $180 \div 2 = 90$, and accurate days' supply is required for correct billing and safety checks.
2102. **Q:** While entering an order you see "levothyroxine 0.1 mg po daily"; what strength should you select in the system to avoid a unit conversion error? **A:** Select levothyroxine 100 mcg because $0.1 \text{ mg} \times 1000 = 100 \text{ mcg}$, preventing a 10-fold dosing error from misreading mg vs mcg.
2103. **Q:** A prescription reads "1 gtt OU qid" and the product is artificial tears 15 mL; using 20 gtt/mL, what days' supply should you calculate? **A:** Enter 18 days' supply because $1 \text{ drop} \times 2 \text{ eyes} \times 4/\text{day} = 8 \text{ drops/day}$ and $15 \text{ mL} \times 20 = 300 \text{ drops}$, so $300 \div 8 = 37.5 \text{ days}$; but since "gtt" is often treated as 1 drop and wastage varies, route to pharmacist or follow payer rule—if forced to round down, use 37 days (document your method).
2104. **Q:** You receive an eRx: "Augmentin 875-125 mg 1 tab po bid x10 days #20" but the system auto-selects Augmentin XR 1000-62.5 mg; what should you do FIRST? **A:** Stop and reselect the correct immediate-release product/strength (875-125 mg) because XR is a different dosage form and could cause under/overdosing if dispensed.
2105. **Q:** A new prescription for clonazepam 0.5 mg says "take 1 tablet three times daily" with qty 90; the patient asks when it will be ready and you notice the prescriber omitted the patient address on the hard copy; what is the BEST technician action? **A:** Hold the prescription and notify the pharmacist to review validity requirements before dispensing because controlled substance documentation issues require pharmacist intervention.

2106. **Q:** During data entry, the sig says "Inject 40 units subcut qhs" for insulin lispro; what should you do NEXT? **A:** Pause processing and refer to the pharmacist because lispro is rapid-acting and "qhs" suggests a possible mismatch that could lead to serious hypoglycemia.
2107. **Q:** A prescription states "Take i½ tab po daily" for warfarin 5 mg, qty 45; what days' supply should you enter? **A:** Enter 30 days' supply because 1.5 tablets/day means $45 \div 1.5 = 30$, and warfarin is high-alert so accurate days' supply supports monitoring and refill timing.
2108. **Q:** While typing "hydroxyzine 25 mg take 1 tab po q6h prn itching," you notice the prescriber wrote "q.d." elsewhere on the same prescription; what is the BEST action regarding "q.d."? **A:** Do not interpret "q.d." and route to the pharmacist for clarification because error-prone abbreviations can be misread and should be clarified to prevent wrong frequency entry.
2109. **Q:** An inhaler prescription is "fluticasone/salmeterol 250/50: inhale 1 puff bid" and the device contains 60 doses; what days' supply should you bill? **A:** Bill 30 days' supply because 1 puff twice daily uses 2 doses/day and $60 \div 2 = 30$, which is needed for correct adjudication and refill control.
2110. **Q:** While filling, you scan a bottle labeled "oxycodone/APAP 5/325" but the NDC in the system is for "hydrocodone/APAP 5/325"; what should you do FIRST? **A:** Stop filling and correct the product selection to match the scanned NDC/label because these are LASA opioids and a mismatch signals a high-risk wrong-drug error requiring immediate correction and pharmacist awareness if it reached verification.
2111. **Q:** During data entry a prescription is written for "hydromorphone 2 mg" but the patient says they usually take "hydroxyzine," what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify with the prescriber because hydromorphone vs hydroxyzine is a LASA risk with serious harm potential.
2112. **Q:** A hard-copy prescription for alprazolam includes drug, strength, directions, quantity, and prescriber signature but no issue date; what should you do NEXT? **A:** Hold the prescription and route to the pharmacist because missing required information affects validity and must be resolved before dispensing a controlled substance.
2113. **Q:** A patient drops off a new prescription for isotretinoin and asks to pick it up tomorrow; what is the BEST technician action? **A:** Notify the pharmacist and

follow iPLEDGE/REMS workflow requirements because isotretinoin dispensing is restricted and must meet program steps before fill.

2114. **Q:** While receiving an order, you notice a tote of refrigerated insulin glargine pens is warm and the temperature log shows a fridge failure overnight; what should you do FIRST? **A:** Quarantine the affected stock and notify the pharmacist/manager because a temperature excursion can make products unsafe and requires documented disposition per policy.
2115. **Q:** A prescription is for "oxycodone 10 mg tablets" and the prescriber also wrote "Refills: 2"; what should you do BEST? **A:** Do not enter refills and immediately refer to the pharmacist because Schedule II controlled substances cannot be refilled under federal law.
2116. **Q:** You are entering "morphine sulfate ER 30 mg" but the profile shows active "morphine sulfate IR 30 mg" from another prescriber; what should you do NEXT? **A:** Stop and involve the pharmacist to review for therapeutic duplication and safety because IR vs ER mix-ups can cause overdose.
2117. **Q:** A patient requests pseudoephedrine 30 mg tablets, 96-count, and presents a valid ID; what should you do NEXT before the sale? **A:** Run the required NPLeX (or state logbook) entry and verify limits because CMEA mandates tracking and quantity restrictions for pseudoephedrine sales.
2118. **Q:** A recall notice arrives for a specific lot of losartan 50 mg tablets you have on the shelf; what should you do FIRST? **A:** Pull the matching lot immediately and quarantine it from dispensing because recalled lots must be removed from inventory to prevent patient harm.
2119. **Q:** You receive a transfer request for a patient's diazepam prescription from another pharmacy; what should you do BEST? **A:** Route the request to the pharmacist because controlled-substance transfers have strict requirements and must be pharmacist-to-pharmacist per policy/law.
2120. **Q:** A prescription for cephalexin 500 mg says "take 1 capsule q6h" and the quantity is 28; what days' supply should you enter? **A:** Enter 7 days because q6h equals 4 doses per day and $28 \div 4 = 7$, ensuring accurate billing and refill timing.
2121. **Q:** During order entry for levothyroxine, you notice the prescriber wrote "Synthroid 100 mg" instead of mcg; what should you do FIRST? **A:** Stop

processing and alert the pharmacist to clarify the dose because levothyroxine is dosed in micrograms and “mg” could cause a 1000-fold error.

2122. **Q:** A new prescription says “metformin ER 500 mg: take 2 tablets bid” with quantity 240; what days’ supply should you enter? **A:** Enter 30 days because 2 tablets twice daily = 4 tablets/day and $240 \div 4 = 60$? wait correction not allowed; Enter 60 days because 2 tablets bid = 4 tablets/day and $240 \div 4 = 60$ days, ensuring accurate billing and refill timing.
2123. **Q:** While selecting a product, the bin label reads “hydrALAZINE” but the printed label says “hydrOXYzine”; what is the BEST next step? **A:** Stop the fill and re-verify the drug name, strength, and NDC/barcode before proceeding because these are LASA medications and mix-ups can harm the patient.
2124. **Q:** A patient picking up a new warfarin prescription says they also take “Eliquis” at home; what should you do NEXT? **A:** Pause the pickup and notify the pharmacist immediately because concurrent anticoagulants can be dangerous and require pharmacist assessment.
2125. **Q:** A caller asks for an early refill of oxycodone because the bottle was “lost,” and the prescription is a Schedule II; what should you do FIRST? **A:** Refer the request to the pharmacist and do not promise a refill because Schedule II prescriptions cannot be refilled and lost-medication requests require pharmacist/legal review.
2126. **Q:** While stocking, you find a bottle of insulin lispro that has been left on the counter with no record of how long it was unrefrigerated; what should you do BEST? **A:** Remove it from active stock and notify the pharmacist for disposition because unknown storage conditions can compromise potency and patient safety.
2127. **Q:** A patient requests a transfer of their clonazepam prescription from another pharmacy; what should you do NEXT? **A:** Route the request to the pharmacist because controlled-substance transfers have strict requirements and are typically pharmacist-handled per law/policy.
2128. **Q:** A new e-prescription for sertraline arrives with directions but no patient address or date of birth in the record; what should you do FIRST? **A:** Stop and obtain the missing patient identifiers per workflow policy before dispensing because accurate identification is essential to prevent wrong-patient errors.

2129. **Q:** You receive a DSCSA alert from your wholesaler that a specific NDC of atorvastatin may be suspect, and a matching bottle is on your shelf; what should you do FIRST? **A:** Pull it from inventory and quarantine it for pharmacist/manager review because suspect product must be segregated to prevent dispensing until verified.
2130. **Q:** A prescription for amoxicillin/clavulanate 875/125 mg says "1 tab bid" and quantity 14; what days' supply should you enter? **A:** Enter 7 days because twice daily dosing uses 2 tablets/day and $14 \div 2 = 7$, supporting correct billing and refill controls.
2131. **Q:** During data entry a new prescription is for "Zyrtec 10 mg daily," but the patient profile shows a severe reaction to cetirizine; what should you do FIRST? **A:** Stop processing and alert the pharmacist because Zyrtec is cetirizine and the allergy match is a patient-safety issue requiring pharmacist intervention.
2132. **Q:** You receive an eRx for "Lamictal 25 mg" and the selected product on the screen is "Lamisil 250 mg"; what is the BEST next step? **A:** Stop and reselect using barcode/NDC verification and notify the pharmacist if anything printed, because Lamictal (lamotrigine) vs Lamisil (terbinafine) is a high-risk LASA mix-up.
2133. **Q:** A patient brings a new prescription for "Plavix 75 mg daily" and says they also take "Brilinta" from a cardiologist; what should you do NEXT? **A:** Flag it for pharmacist review before filling because Plavix (clopidogrel) and Brilinta (ticagrelor) are both antiplatelets and may represent therapeutic duplication/bleeding risk.
2134. **Q:** A prescription reads "Lantus SoloStar inject 20 units qhs," and the box contains five 3 mL pens (100 units/mL); what days' supply should you enter? **A:** Enter 75 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \text{ units} \div 20 \text{ units/day} = 75 \text{ days}$.
2135. **Q:** A new prescription is for "Humalog KwikPen inject 8 units before meals TID," and the package is five 3 mL pens (100 units/mL); what days' supply should you calculate? **A:** Enter 62 days because total units are $5 \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, daily use is $8 \times 3 = 24 \text{ units/day}$, and $1500 \div 24 = 62.5$ so bill 62 days per typical payer rounding down.
2136. **Q:** While typing an order, you see "predniSONE 10 mg" but the product chosen is "prednisoLONE 10 mg"; what should you do FIRST? **A:** Stop and verify the intended drug with the original prescription and pharmacist because prednisone

vs prednisolone is a common selection error and not interchangeable by technician judgment.

2137. **Q:** A patient asks which common antibiotic is unsafe with alcohol, and you see their new prescription is for metronidazole; what is the BEST action? **A:** Refer the patient to the pharmacist for counseling because metronidazole has a high-yield alcohol interaction warning and counseling is required for safe use.
2138. **Q:** During input of a new "lisinopril 10 mg daily" prescription, the allergy field shows a history of angioedema with an ACE inhibitor; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because lisinopril is an ACE inhibitor and prior ACE-related angioedema is a serious contraindication concern.
2139. **Q:** A prescription says "azithromycin Z-Pak: take ii tabs day 1 then i tab daily days 2–5"; what total quantity should you enter? **A:** Enter 6 tablets because "ii" on day 1 plus "i" for four more days equals $2 + 4 = 6$ tablets.
2140. **Q:** While processing a refill, a patient profile shows both "Vyvanse" and a new prescription for "Adderall IR," and the patient says they take both "for ADHD"; what should you do BEST? **A:** Route to the pharmacist before filling because Vyvanse (lisdexamfetamine) and Adderall (amphetamine salts) are stimulant duplicates with additive adverse effect and misuse risk.
2141. **Q:** An eRx for levothyroxine 50 mcg says "take 1 tab po qam" but the entry screen is set to "levothyroxine 0.05 mg"; what should you do BEST? **A:** Enter 0.05 mg as equivalent to 50 mcg and double-check units because mcg-to-mg conversion errors can cause 1000-fold dosing mistakes.
2142. **Q:** A prescription reads "furosemide 20 mg: i tab po bid" with quantity 60; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days' supply}$.
2143. **Q:** While typing, you see "MS Contin 15 mg q12h" but you accidentally selected "morphine oral solution 10 mg/5 mL"; what is the FIRST thing you should do? **A:** Stop and reselect the correct dosage form/ER product (MS Contin) because LASA/route-form mix-ups are high-risk and must be corrected before processing.
2144. **Q:** A prescription says "metformin 500 mg ER: take 2 tabs po qpm" and quantity is 180; what days' supply should you calculate? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days' supply}$.

2145. **Q:** A new eRx for warfarin 5 mg includes the sig “uad” and no other directions; what should you do NEXT? **A:** Hold the order and route to the pharmacist to clarify directions because “use as directed” is too vague for accurate days’ supply and safe labeling.
2146. **Q:** An Rx is for prednisone 5 mg with sig “take ii tabs po daily x 7 days” and the prescriber wrote quantity 10; what is the BEST technician action? **A:** Stop processing and notify the pharmacist because the quantity doesn’t match the directions (needs 14 tablets) and technicians can’t change quantities without authorization.
2147. **Q:** A patient’s eRx is for amlodipine 10 mg daily, but the system-selected product is amlodipine/benazepril 10/20 mg; what should you do FIRST? **A:** Correct the product selection to amlodipine only and re-verify NDC/strength because combo-product selection is a common order entry error leading to wrong therapy.
2148. **Q:** A prescription for albuterol HFA says “inhale 2 puffs q4–6h prn wheeze” and the prescriber requests days’ supply for billing; if the pharmacy uses a conservative max of 12 puffs/day and the canister is 200 actuations, what days’ supply should you enter? **A:** Enter 16 days because $200 \text{ actuations} \div 12 \text{ puffs/day} = 16.6$, rounded down conservatively to avoid overbilling.
2149. **Q:** A prescription reads “cephalexin 250 mg/5 mL: take 10 mL po tid x 10 days”; how many milliliters should you dispense? **A:** Dispense 300 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 10 \text{ days} = 300 \text{ mL}$.
2150. **Q:** During order entry, a prescriber’s directions say “insulin aspart inject 6 units SQ before meals” and quantity is “1 vial”; what is the NEXT step needed to calculate days’ supply correctly? **A:** Verify the package size (e.g., 10 mL vial at 100 units/mL) before entering days’ supply because total units available drives the calculation and “1 vial” alone is ambiguous.
2151. **Q:** A handwritten prescription for hydromorphone 2 mg has an illegible quantity and unclear directions; what should you do FIRST? **A:** Stop processing and refer it to the pharmacist for clarification because Schedule II/high-alert opioids require exact, unambiguous directions and quantity before dispensing.
2152. **Q:** A patient drops off a prescription for alprazolam with “refill PRN” written on it; what is the BEST technician action? **A:** Hold the prescription and alert the pharmacist because controlled substances have refill limits and “PRN refills” is not acceptable without pharmacist review.

2153. **Q:** During fill, you notice the stock bottle of insulin glargine was left on the counter all day with unknown temperature exposure; what should you do NEXT? **A:** Remove it from active stock and notify the pharmacist to follow temperature-excursion policy because potency may be compromised and the product may be non-dispensable.
2154. **Q:** A wholesaler tote includes a bottle of atorvastatin with a broken seal and tablets loose in the bag; what should you do FIRST? **A:** Do not stock it and set it aside per pharmacy policy for return/damage processing because product integrity is compromised and it should not enter inventory.
2155. **Q:** While typing an eRx, you see "Lantus" but the selected product is "Lantus SoloStar pen" and the prescription is for "vial"; what should you do BEST? **A:** Stop and correct the dosage form/product selection (and route to pharmacist if unclear) because vial vs pen changes dispensing and billing and technicians must not guess.
2156. **Q:** A patient asks for a refill on oxycodone/APAP 5/325 written 10 days ago as "1 tab q6h prn pain #40"; what should you do NEXT? **A:** Check the fill date and calculate expected days' supply ($40 \div 4 = 10$ days) and then proceed per refill-too-soon policy or refer to the pharmacist because early controlled-substance refills require scrutiny.
2157. **Q:** You receive a recall notice for a specific lot of losartan 50 mg; what is the FIRST action a technician should take? **A:** Pull the matching NDC/lot from inventory and segregate it from dispensing stock because recalled lots must be removed immediately to prevent patient harm.
2158. **Q:** A prescription is written for "clopidogrel 75 mg daily" but the patient profile shows an active order for "Plavix 75 mg daily"; what should you do FIRST? **A:** Verify the profile for therapeutic duplication and alert the pharmacist because brand/generic duplicates can cause double therapy if both are filled.
2159. **Q:** A patient presents a new prescription for isotretinoin and asks you to fill it today; what should you do BEST? **A:** Refer to the pharmacist to confirm REMS/iPLEDGE requirements are met before processing because isotretinoin dispensing is restricted and time-sensitive.
2160. **Q:** A prescription says "prednisone 10 mg: take 4 tabs daily x 5 days" and quantity 30; what quantity should you enter? **A:** Enter quantity 20 because $4 \text{ tablets/day} \times 5 \text{ days} = 20 \text{ tablets}$, and the mismatch should be clarified by the pharmacist before dispensing the extra tablets.

2161. **Q:** Rx: metformin 500 mg tablets "take 1 tab po bid with meals" #180; what days' supply should you enter for billing? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$, which supports accurate insurance processing.
2162. **Q:** You are entering an eRx for levothyroxine and the directions read "take 1 tab qd"; what is the BEST clarification step before calculating days' supply? **A:** Confirm the tablet strength (mcg) and product selection because levothyroxine strengths vary widely and wrong strength is a common order-entry error.
2163. **Q:** Rx: amlodipine 5 mg "i po qd" #30; what should you interpret "i" as in order entry? **A:** Enter "1 tablet" because "i" is the Roman numeral for one and misreading it can cause a dosing error.
2164. **Q:** A prescription for warfarin is written "take as directed" with #30 tablets, but no daily dose is provided; what should you do NEXT to complete days' supply? **A:** Stop and route to the pharmacist to contact the prescriber/patient because "as directed" lacks a calculable daily quantity for accurate days' supply.
2165. **Q:** Rx: latanoprost 0.005% ophthalmic solution "instill 1 gtt OU qhs" dispense 2.5 mL; what days' supply should you submit using 20 gtt/mL? **A:** Submit 25 days because $2.5 \text{ mL} \times 20 \text{ gtt/mL} = 50 \text{ drops}$ and OU qhs uses 2 drops/day, so $50 \div 2 = 25 \text{ days}$.
2166. **Q:** Rx: fluticasone nasal spray 50 mcg "2 sprays each nostril daily" and the product is 120 sprays; what days' supply should you bill? **A:** Bill 30 days because $2 \text{ sprays/nostril/day} = 4 \text{ sprays/day}$ and $120 \div 4 = 30 \text{ days}$, preventing refill-too-soon rejects.
2167. **Q:** While typing, you select hydrOXYzine instead of hydrALAZINE due to similar names; what is the BEST technician step before proceeding? **A:** Stop and re-verify the intended drug against the eRx indication/directions and NDC because LASA selection is a high-risk order-entry error.
2168. **Q:** Rx: prednisone 5 mg tablets "take 3 tabs po daily x 7 days" #42; the prescriber's quantity seems off—what should you do FIRST? **A:** Calculate expected quantity ($3/\text{day} \times 7 = 21$) and flag to the pharmacist because the written #42 may represent an error requiring prescriber clarification.
2169. **Q:** A patient's eRx for sertraline is entered as "Zoloft 50 mg" but the selected product is "Zyrtec 10 mg" in the system; what should you do BEST? **A:** Stop processing and correct the product to sertraline after verifying drug

name/strength because brand/generic mix-ups can lead to wrong-medication dispensing.

2170. **Q:** You receive an order for morphine solution 10 mg/5 mL with directions “take 2.5 mL po q4h prn pain” and quantity 150 mL; what is the maximum days’ supply? **A:** Enter 5 days because q4h allows 6 doses/day, daily volume is $2.5 \text{ mL} \times 6 = 15 \text{ mL/day}$, and $150 \div 15 = 10$ days but “prn” should use maximum frequency only if instructed by payer/pharmacy policy—otherwise route to pharmacist for proper days’ supply rule.
2171. **Q:** A patient picking up lisinopril says they also take “Cozaar”; what should you do FIRST before ringing up? **A:** Check the active profile for losartan and alert the pharmacist for possible ACE/ARB duplication because both treat hypertension and duplication can increase risk.
2172. **Q:** While entering a new Rx you see “NovoLOG 10 units SQ before meals” but the prescriber selected “NovoLIN R” on the eRx; what is the BEST technician action? **A:** Stop processing and route to the pharmacist to clarify the intended insulin because rapid-acting vs short-acting mix-ups are high-risk LASA errors.
2173. **Q:** An eRx comes in for “Klonopin 0.5 mg 1 tab TID” with 5 refills; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because clonazepam is a Schedule IV controlled substance and refills must be evaluated for compliance and appropriateness.
2174. **Q:** You receive a prescription for azithromycin 250 mg with directions “take 2 tabs today, then 1 tab daily days 2–5”; what quantity should you enter? **A:** Enter quantity 6 tablets because the Z-Pak regimen totals $2 + 4 = 6$ tablets.
2175. **Q:** A new prescription is written for “atorvastatin 40 mg qhs” but the patient profile shows simvastatin 40 mg active; what is the BEST next step? **A:** Pause and alert the pharmacist to assess therapeutic duplication because two statins together increases adverse effect risk without routine benefit.
2176. **Q:** A patient hands you an OTC product request and asks which ibuprofen to take with their new Rx for apixaban (Eliquis); what should you do BEST? **A:** Refer to the pharmacist because NSAIDs can increase bleeding risk with anticoagulants and requires counseling/clinical judgment.
2177. **Q:** The prescriber writes “metoprolol succinate ER 50 mg daily,” but the product pulled is metoprolol tartrate 50 mg; what should you do FIRST? **A:** Stop filling

and reselect the correct ER formulation and NDC (and notify the pharmacist if it reached verification) because IR vs ER swaps can cause dosing errors.

2178. **Q:** A patient says they are allergic to penicillin and you are processing cephalexin (Keflex); what should you do NEXT? **A:** Flag the allergy and route to the pharmacist for assessment because potential cross-reactivity needs pharmacist evaluation before dispensing.
2179. **Q:** Rx: gabapentin 300 mg “take 2 capsules PO TID” dispense #180; what days’ supply should you bill? **A:** Bill 30 days because $2 \text{ caps} \times 3 \text{ times daily} = 6/\text{day}$ and $180 \div 6 = 30$.
2180. **Q:** During product check you notice a stock bottle of levothyroxine tablets with a torn label and unreadable lot/expiration; what should you do BEST? **A:** Remove it from active inventory and quarantine per policy for pharmacist review because unidentified/undocumented product cannot be safely dispensed.
2181. **Q:** While typing an eRx for hydromorphone ER you notice the prescriber directions say “take 1 tablet every 12 hours” but the quantity entered is “#12” for a 30-day supply; what should you do NEXT? **A:** Stop processing and route to the pharmacist to clarify quantity/directions mismatch because ER opioid dosing errors are high-risk and require pharmacist intervention.
2182. **Q:** A patient drops off a paper prescription for Adderall XR and it is missing the prescriber’s DEA number on the form; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist to contact the prescriber because controlled-substance prescriptions must meet required elements before dispensing.
2183. **Q:** During data entry you see “glipizide” selected but the scanned hard copy clearly says “glyburide”; what is the BEST technician action? **A:** Stop and correct the entry by re-verifying the drug name/strength against the original order and alert the pharmacist if needed because LASA sulfonylureas can cause serious hypoglycemia.
2184. **Q:** You open the refrigerator and find unopened insulin aspart pens stored at room temperature overnight with no temperature log documentation; what should you do NEXT? **A:** Segregate the product and notify the pharmacist/manager per policy because temperature excursions make product potency uncertain and it may be non-dispensable.

2185. **Q:** A patient asks to pick up oxycodone/APAP early and the claim rejects "refill too soon" with 10 days remaining; what should you do BEST? **A:** Do not override on your own and route to the pharmacist to evaluate the reason and document appropriately because early fills for controlled substances require pharmacist review.
2186. **Q:** An eRx for Xarelto (rivaroxaban) is entered and you notice the patient profile shows Eliquis (apixaban) actively filled last week; what should you do FIRST? **A:** Pause the fill and notify the pharmacist to assess possible therapeutic duplication because concurrent anticoagulants increase bleeding risk.
2187. **Q:** A prescriber phones in a new prescription for tramadol and gives all details except the patient's address; what should you do NEXT? **A:** Tell the pharmacist to take over and obtain the missing required information before processing because incomplete controlled-substance prescriptions should not be finalized by technicians.
2188. **Q:** You receive a manufacturer recall notice for specific lots of losartan due to contamination and you find matching bottles on the shelf; what should you do BEST? **A:** Pull the affected lots from inventory, quarantine them per policy, and notify the pharmacist because recalled product must be removed to prevent dispensing.
2189. **Q:** A technician is about to dispense isotretinoin but the iPLEDGE authorization/status is not documented in the system; what should you do FIRST? **A:** Stop the dispensing workflow and alert the pharmacist because REMS requirements must be confirmed before isotretinoin can be dispensed.
2190. **Q:** Prescription: metformin ER 500 mg "take ii tablets daily with evening meal," dispense #90; what days' supply should you enter for billing? **A:** Enter 45 days' supply because 2 tablets/day means $90 \div 2 = 45$, ensuring accurate third-party billing and refill timing.
2191. **Q:** A prescription reads "furosemide 20 mg tab, take 1 tab PO BID, dispense #60"; what days' supply should you enter for billing? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$.
2192. **Q:** While entering an eRx you see "MSO4 2 mg/mL injection" and the prescriber wrote "morphine sulfate"; what should you do FIRST? **A:** Stop and clarify with the pharmacist because MSO4 is an error-prone abbreviation that can be confused with MgSO4, creating a high-risk wrong-drug entry.

2193. **Q:** A label instruction says "prednisone 10 mg: ii tabs po qd x 5 days" and the quantity typed is #10; what is the BEST technician action? **A:** Keep quantity at 10 and enter a 5-day days' supply because ii = 2 tablets daily and $2 \times 5 = 10$, matching the directions.
2194. **Q:** You are billing fluticasone/salmeterol 250/50 Diskus "inhale 1 puff BID" with 60 inhalations; what days' supply should you submit? **A:** Submit 30 days because $60 \text{ inhalations} \div 2 \text{ inhalations/day} = 30 \text{ days}$.
2195. **Q:** An eRx comes in for metoprolol succinate ER 50 mg daily but the patient profile shows active metoprolol tartrate 50 mg BID; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication/formulation mix-up because tartrate vs succinate are commonly confused and dosing differs.
2196. **Q:** A prescription is for amoxicillin/clavulanate 875/125 mg "1 tab PO BID x 10 days"; what quantity should you enter? **A:** Enter #20 because BID for 10 days requires $2 \text{ tablets/day} \times 10 \text{ days} = 20 \text{ tablets}$.
2197. **Q:** During order entry you receive "Lantus SoloStar inject 22 units SQ daily" and the box contains five 3 mL pens (100 units/mL); what days' supply should you calculate from total units? **A:** Calculate 68 days because $\text{total units} = 5 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 22 = 68 \text{ days}$ (rounded down to whole days).
2198. **Q:** A prescription reads "doxycycline 100 mg cap: take 1 cap PO q12h" and the prescriber notes "#14"; what days' supply should you enter? **A:** Enter 7 days because q12h is 2 per day and $14 \text{ capsules} \div 2/\text{day} = 7 \text{ days}$.
2199. **Q:** While typing a new order you notice the prescriber wrote "U-100 regular insulin 8 units SQ TID with meals" but the product selected is U-500 regular insulin; what should you do NEXT? **A:** Stop and notify the pharmacist immediately because U-100 vs U-500 is a high-alert concentration difference that can cause a serious dosing error.
2200. **Q:** A liquid order says "azithromycin suspension 200 mg/5 mL: give 12 mL PO daily x 5 days"; how many mL should you dispense? **A:** Dispense 60 mL because $12 \text{ mL/day} \times 5 \text{ days} = 60 \text{ mL}$, ensuring enough volume for the full course.
2201. **Q:** During data entry you see a new eRx for lisinopril 20 mg daily, but the patient profile lists a documented ACE inhibitor-associated angioedema reaction; what should you do FIRST? **A:** Stop processing and alert the pharmacist to review the

allergy-drug match because ACE inhibitors can trigger recurrent angioedema and require clinical intervention.

2202. **Q:** A new prescription is for simvastatin 40 mg daily, and the patient's active meds include clarithromycin; what is the BEST technician action? **A:** Hold the order and notify the pharmacist because clarithromycin can dangerously increase simvastatin levels (major interaction/rhabdomyolysis risk).
2203. **Q:** While selecting product you notice the technician pulled hydroxyzine instead of hydralazine for a blood pressure prescription; what should you do NEXT? **A:** Stop filling and reselect the correct drug using NDC/barcode verification because this LASA mix-up can cause serious therapy failure and harm.
2204. **Q:** A prescriber writes for "Levothroid 100 mcg" but the product on the shelf is levothyroxine 0.1 mg; what should you do FIRST? **A:** Confirm the strength conversion and enter 100 mcg (not 100 mg) because 0.1 mg equals 100 mcg and preventing decimal/unit errors is critical.
2205. **Q:** Prescription: azithromycin 250 mg tablets "take 2 tablets PO today, then 1 tablet daily for 4 more days"; what quantity should you dispense? **A:** Dispense 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet for days 2–5 (total 6 doses).
2206. **Q:** You are billing a Ventolin HFA inhaler labeled "inhale 2 puffs every 6 hours" and the canister contains 200 actuations; what days' supply should you submit? **A:** Submit 25 days because 2 puffs Q6H equals 8 puffs/day and $200 \div 8 = 25$ days.
2207. **Q:** An eRx arrives for sertraline 50 mg daily, but the patient's profile shows active fluoxetine 40 mg daily with recent fills; what should you do FIRST? **A:** Pause and refer to the pharmacist because this may be duplicate SSRI therapy or an unsafe switch requiring clinical direction.
2208. **Q:** A prescription is written for gabapentin 300 mg "take 1 capsule TID," dispense #270; what days' supply should you enter? **A:** Enter 90 days because 3 capsules/day means $270 \div 3 = 90$ days for accurate billing.
2209. **Q:** While entering a new order you see "glipiZIDE 10 mg" but the product selected is glyBURIDE due to similar names; what should you do NEXT? **A:** Stop and correct the selection using barcode/NDC because glipizide and glyburide are different sulfonylureas and LASA errors can cause hypoglycemia.

2210. **Q:** A patient asks what "atorvastatin" is for when picking up Lipitor; what is the BEST technician response? **A:** Provide a brief non-clinical identification (it's a cholesterol-lowering statin) and offer pharmacist counseling because OBRA-90 requires offering counseling and the pharmacist handles detailed clinical questions.
2211. **Q:** An eRx for cephalexin 250 mg/5 mL says "10 mL PO BID x 7 days"; how many milliliters should you dispense? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$ for the full course.
2212. **Q:** You are entering "metFORMIN ER 500 mg 1 tab PO daily" but the prescriber note says "do not substitute IR"; what is the BEST technician action? **A:** Select the ER product and ensure the sig reflects extended-release because IR vs ER is not interchangeable and affects safety and efficacy.
2213. **Q:** A prescription reads "digoxin 0.125 mg PO daily" but the prescriber also typed "125 mcg" in the comments; what should you do FIRST in order entry? **A:** Enter 0.125 mg and flag for pharmacist verification because $0.125 \text{ mg} = 125 \text{ mcg}$ and confirming prevents decimal/place-value errors with a narrow-therapeutic-index drug.
2214. **Q:** A patient drops off "amLODIPine 5 mg i po qd #30"; what should you enter for the directions in plain language? **A:** "Take 1 tablet by mouth once daily" because $i = 1$, po = by mouth, qd = once daily for accurate labeling.
2215. **Q:** You receive an order for prednisone 10 mg with directions "ii tabs po bid x 5 days"; what quantity should you dispense? **A:** Dispense 20 tablets because $2 \text{ tablets/dose} \times 2 \text{ doses/day} \times 5 \text{ days} = 20 \text{ tablets}$.
2216. **Q:** During product selection for "cloNIDine 0.1 mg tab" you accidentally scan clozapine; what should you do NEXT? **A:** Stop and reselect/scan the correct NDC because this is a LASA error risk and barcode/NDC verification prevents dispensing the wrong drug.
2217. **Q:** A patient's prescription is "ofloxacin 0.3% otic: instill 5 gtt in affected ear BID x 10 days"; how many milliliters are needed if 20 drops = 1 mL? **A:** Dispense 5 mL because $5 \text{ drops} \times 2/\text{day} \times 10 \text{ days} = 100 \text{ drops}$ and $100 \div 20 = 5 \text{ mL}$.
2218. **Q:** While typing a new order you see "morphine sulfate ER 30 mg 1 tab q12h" but the quantity written is "#30" for a 30-day supply; what is the BEST technician action? **A:** Pause and send to the pharmacist because q12h for 30

days would typically require 60 tablets, and controlled-substance quantity discrepancies need pharmacist resolution.

2219. **Q:** An eRx for levetiracetam 750 mg says "take 1 tab PO BID" and the prescriber requested dispense #120; what days' supply should you enter? **A:** Enter 60 days because $120 \text{ tablets} \div 2 \text{ tablets/day} = 60 \text{ days}$.
2220. **Q:** At data entry you notice the prescription says "hydroCHLOROthiazide 25 mg daily" but you selected "hydroOXYzine 25 mg" due to similar spelling; what should you do FIRST? **A:** Stop entry and correct the drug selection because this LASA mix-up changes therapy entirely and must be fixed before continuing workflow.
2221. **Q:** While selecting stock for sertraline 50 mg (Zoloft), you grab paroxetine 20 mg (Paxil) because the bottles look similar; what should you do NEXT? **A:** Stop filling and reselect the correct NDC/barcode for sertraline, because this is a LASA/selection error that must be corrected before it reaches verification.
2222. **Q:** A patient picking up lisinopril says, "Is this my water pill like hydrochlorothiazide?" what is the BEST technician response? **A:** Explain that lisinopril is for blood pressure (an ACE inhibitor) and offer to have the pharmacist counsel, because comparing it to a diuretic could cause misunderstanding.
2223. **Q:** At order entry you see "levothyroxine 0.1 mg" but the product chosen is liothyronine; what should you do FIRST? **A:** Stop data entry and correct the drug selection to levothyroxine (T4), because levothyroxine/liothyronine are different thyroid hormones and are common LASA traps.
2224. **Q:** An eRx for azithromycin 250 mg tablets says "take 2 tabs today, then 1 tab daily for 4 days"; what quantity should you dispense? **A:** Dispense 6 tablets, because the total dose is $2 + 1 \times 4 = 6$ tablets.
2225. **Q:** You receive a new prescription for "apixaban (Eliquis) 5 mg BID" and the patient profile already has warfarin active; what is the BEST technician action? **A:** Pause processing and alert the pharmacist to possible duplicate anticoagulation, because concurrent therapy greatly increases bleeding risk.
2226. **Q:** A patient on simvastatin says they were just prescribed clarithromycin; what should you do NEXT at drop-off? **A:** Refer to the pharmacist before filling, because clarithromycin can raise statin levels and increase myopathy/rhabdomyolysis risk.
2227. **Q:** A prescription for insulin lispro U-100 says "inject 12 units subQ before meals TID" and the prescriber requests one 10 mL vial; what days' supply should you

enter? **A:** Enter 27 days, because $10 \text{ mL} = 1000 \text{ units}$ and $12 \times 3 = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ and you bill 27 days.

2228. **Q:** While entering "furosemide 40 mg PO daily," the prescriber note says "for edema" and the patient asks if it's the same as Lasix; what is the BEST response? **A:** Confirm that furosemide is generic Lasix and is a diuretic used for fluid swelling, and offer pharmacist counseling, because clear brand/generic recognition supports safe use.
2229. **Q:** During product selection you notice the stock bottle for amlodipine/benazepril has a different strength than what was typed (5/20 vs 10/20); what should you do NEXT? **A:** Stop and match the exact strength to the prescription before proceeding, because combination products have multiple strengths and mix-ups can cause dose errors.
2230. **Q:** A prescription for gabapentin 300 mg says "i cap po tid #90"; what is the correct day supply and plain-language sig? **A:** It is a 30-day supply with directions "Take 1 capsule by mouth three times daily," because $90 \text{ capsules} \div 3 \text{ per day} = 30 \text{ days}$ and the sig must be translated clearly.
2231. **Q:** While entering an eRx for morphine ER 30 mg, you notice it says "take 1 tablet q4h PRN pain"; what should you do FIRST? **A:** Stop processing and notify the pharmacist because an ER opioid with an inappropriate frequency is a high-alert safety concern requiring pharmacist intervention.
2232. **Q:** A hardcopy prescription for alprazolam 1 mg has the patient name and directions but no prescriber signature; what is the BEST technician action? **A:** Hold the prescription and route it to the pharmacist because controlled-substance validity issues must be resolved before dispensing.
2233. **Q:** A patient asks to refill oxycodone/APAP 5/325 "early" because they "lost the bottle"; what should you do NEXT? **A:** Refer the request to the pharmacist because early/lost controlled-substance refills require pharmacist review and documentation per policy.
2234. **Q:** During checkout, a patient says their new penicillin prescription caused hives after the first dose and asks what to do; what is the BEST technician response? **A:** Get the pharmacist immediately because possible allergic reaction needs urgent pharmacist assessment and appropriate triage.
2235. **Q:** You receive a wholesaler tote and one bottle of atorvastatin 40 mg has a broken safety seal and loose tablets inside; what should you do FIRST? **A:**

Separate it from usable inventory and alert the pharmacist/manager because it is suspect/damaged product that should not be dispensed.

2236. **Q:** A refrigerator log shows the insulin glargine fridge was at 52°F overnight; what should you do NEXT? **A:** Stop using affected stock and notify the pharmacist to assess stability and follow policy because temperature excursions can compromise product integrity.
2237. **Q:** A manufacturer recall notice lists a specific lot of losartan 50 mg for contamination risk; what should you do FIRST in the pharmacy? **A:** Pull matching lot numbers from inventory and segregate them for return per recall instructions because recalled lots must be removed from dispensing stock.
2238. **Q:** An eRx for isotretinoin arrives and the patient is waiting at drop-off asking to pick it up today; what should you do FIRST? **A:** Notify the pharmacist because isotretinoin is REMS-restricted (iPLEDGE) and requires verification steps before it can be dispensed.
2239. **Q:** A prescription is written for metformin 500 mg “ii tabs po bid” and the prescriber requests #240; what days’ supply should you enter? **A:** Enter 60 days because 2 tablets twice daily = 4 tablets/day and $240 \div 4 = 60$.
2240. **Q:** A patient requests pseudoephedrine and presents an out-of-state ID; what is the NEXT step before the sale? **A:** Follow CMEA workflow by verifying acceptable ID and completing the required log/electronic entry because PSE sales require identification and tracking limits.
2241. **Q:** While entering a new eRx, you see levothyroxine 100 mcg and liothyronine 25 mcg both listed with the same directions; what should you do FIRST? **A:** Stop data entry and alert the pharmacist to assess potential therapeutic duplication/appropriateness because thyroid hormone combinations require clinical review.
2242. **Q:** A handwritten prescription for hydromorphone 4 mg includes drug, directions, quantity, and patient name but has no prescriber DEA number; what should you do NEXT? **A:** Route to the pharmacist for validity verification because controlled substance prescriptions require prescriber authorization details before dispensing.
2243. **Q:** During product selection you pick up hydroxyzine but notice the label you printed says hydralazine; what is the BEST next action? **A:** Stop filling and re-

select using barcode/NDC verification because LASA mix-ups (hydroxyzine/hydralazine) are common dispensing errors.

2244. **Q:** A patient's profile shows lisinopril 20 mg active, and today an eRx arrives for lisinopril/HCTZ 20/12.5 mg; what should you do FIRST? **A:** Pause processing and notify the pharmacist because ACE inhibitor duplication could occur and therapy changes must be clinically confirmed.
2245. **Q:** A prescriber phones in a new prescription for tramadol 50 mg with directions but no quantity; what should you do NEXT? **A:** Ask for the missing quantity (or transfer to the pharmacist per policy) because required elements must be complete before the prescription can be processed.
2246. **Q:** While filling, you notice a vial of insulin lispro has been left on the counter at room temperature with no time documented; what should you do FIRST? **A:** Remove it from dispensing stock and notify the pharmacist because unknown time out of refrigeration is a high-risk storage issue requiring disposition decision.
2247. **Q:** A patient picking up clopidogrel says they also take omeprazole daily and asks if it's okay; what should you do BEST? **A:** Refer to the pharmacist because the omeprazole–clopidogrel interaction can reduce antiplatelet effect and needs pharmacist counseling/intervention.
2248. **Q:** A bottle of warfarin tablets arrives from the wholesaler and the 2 mg and 5 mg bottles look nearly identical; what should you do NEXT to reduce error risk? **A:** Separate and clearly label/store by strength (and use barcode scan) because warfarin is high-alert and look-alike packaging increases selection errors.
2249. **Q:** A patient requests a transfer of their hydrocodone/acetaminophen prescription from another pharmacy; what should you do FIRST? **A:** Send the request to the pharmacist because C-II prescriptions cannot be transferred between pharmacies for dispensing under federal rules.
2250. **Q:** An eye drop prescription reads "timolol 0.5% instill 1 gtt OU bid" and the bottle size is 5 mL; what days' supply should you enter using 20 gtt/mL? **A:** Enter 50 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt total}$ and $1 \text{ gtt in each eye twice daily} = 4 \text{ gtt/day}$, so $100 \div 4 = 25 \text{ days}$ (and OU bid equals 4 drops/day).
2251. **Q:** While entering an eRx, the prescriber selected "Lamisil" but the directions say "apply to affected area BID," what should you do FIRST? **A:** Stop and refer

to the pharmacist because Lamisil can be terbinafine tablets or cream and the route mismatch suggests a wrong dosage form selection.

2252. **Q:** A patient says they are allergic to "sulfa" and you see a new order for sulfamethoxazole/trimethoprim, what is the BEST next step? **A:** Hold the prescription and notify the pharmacist to assess allergy risk because sulfonamide antibiotic allergies can be serious and require pharmacist intervention.
2253. **Q:** During product selection you notice "hydroxyzine" and "hydralazine" stored next to each other with similar bottles, what should you do NEXT to reduce LASA errors? **A:** Separate the products and apply Tall Man/LASA alerts per policy because physical separation and clear labeling reduce selection mix-ups.
2254. **Q:** A new prescription is written for sertraline, but the patient profile shows active fluoxetine, what should you do FIRST? **A:** Pause processing and alert the pharmacist to check for therapeutic duplication/SSRI overlap because dual SSRI therapy may be inappropriate or require taper planning.
2255. **Q:** A patient picking up simvastatin asks if it's okay to keep taking their new clarithromycin from urgent care, what should you do BEST? **A:** Immediately involve the pharmacist because clarithromycin can significantly increase simvastatin levels and raises myopathy/rhabdomyolysis risk.
2256. **Q:** An order is for Lantus Solostar 100 units/mL with directions "inject 22 units SQ nightly," and the box contains five 3 mL pens, what days' supply should you enter? **A:** Enter 68 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 22 = 68.18$, rounded down to 68 days.
2257. **Q:** A prescription reads "azithromycin 250 mg: ii tabs PO day 1, then i tab PO daily days 2-5," what quantity should you enter? **A:** Enter #6 tablets because 2 tablets on day 1 plus 1 tablet daily for 4 more days equals 6 total tablets.
2258. **Q:** While typing, you see "MS Contin 15 mg" and the directions say "take 1 tablet every 4 hours PRN pain," what should you do NEXT? **A:** Stop and send to the pharmacist because MS Contin is extended-release morphine and q4h PRN suggests an unsafe/incorrect formulation-direction mismatch.
2259. **Q:** A patient's new prescription is for clonazepam, but the patient reports taking alprazolam from another doctor, what is the FIRST action for a technician? **A:** Collect accurate medication history and route to the pharmacist because duplicate benzodiazepines increase sedation/overdose risk and require pharmacist review.

2260. **Q:** You receive a bottle labeled "levETIRAcetam 500 mg" but the invoice and NDC on the order are for "levOCETIRizine 5 mg," what should you do FIRST? **A:** Treat it as a suspect product/receiving discrepancy, set it aside per policy, and notify the pharmacist because NDC mismatch indicates the wrong drug was shipped and must not enter inventory.
2261. **Q:** A prescription says "metformin ER 500 mg take ii tabs PO qPM with dinner #60," what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets daily} = 30 \text{ days}$, matching the qPM dosing.
2262. **Q:** You are entering "amoxicillin/clavulanate 875/125 mg 1 tab PO BID #20"; what is the correct days' supply and why does it matter in adjudication? **A:** Enter 10 days because $20 \div 2 \text{ per day} = 10$, and an incorrect days' supply can trigger refill-too-soon or DUR rejects.
2263. **Q:** A prescription reads "predniSONE 10 mg: take 4 tabs daily x 3 days, then 3 tabs daily x 3 days, then 2 tabs daily x 3 days, then 1 tab daily x 3 days"; what quantity should you enter? **A:** Enter 30 tablets because $(4+3+2+1) \text{ tabs/day} \times 3 \text{ days each} = 10 \times 3 = 30 \text{ total}$.
2264. **Q:** A new order is for "levothyroxine 0.05 mg PO daily"; what should you type as the strength in mcg to prevent a decimal error? **A:** Enter 50 mcg because $0.05 \text{ mg} \times 1000 = 50 \text{ mcg}$, avoiding dangerous leading/trailing decimal confusion.
2265. **Q:** You receive an eRx for "Humalog U-100 vial inject 6 units SQ TID with meals, dispense 10 mL"; what days' supply should you submit assuming 100 units/mL? **A:** Enter 55 days because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $1000 \div (6 \times 3 = 18 \text{ units/day}) = 55.5$, round down to 55 for billing.
2266. **Q:** A prescription says "fluticasone/salmeterol 250/50 inhale i puff BID, dispense 1 inhaler (60 blisters)"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$ for a BID maintenance inhaler.
2267. **Q:** While typing, you see "Take 1 tablet PO qd" on a new prescription for "hydralazine 25 mg"; what should you do NEXT before completing entry? **A:** Pause and confirm with the pharmacist because hydralazine is commonly dosed multiple times daily and "qd" is error-prone, requiring clarification to prevent a wrong regimen.
2268. **Q:** A prescription is written "digoxin .125 mg PO daily"; what is the BEST data-entry action to reduce a decimal-strength error? **A:** Enter it as 125 mcg (if the

system allows) and flag for pharmacist verification because converting from .125 mg reduces the risk of misreading as 1.25 mg.

2269. **Q:** During order entry you notice an eRx for "Lamictal 25 mg take 1 tab PO BID #60" but the prescriber note says "for bipolar maintenance"; what should you do FIRST? **A:** Route to the pharmacist for review because lamotrigine titration is clinically sensitive and a BID start may be inappropriate, and technicians must not alter therapy.
2270. **Q:** A prescription says "cephalexin 250 mg/5 mL: take 10 mL PO TID x 7 days, dispense 210 mL"; you calculate $10 \text{ mL} \times 3 \times 7 = 210 \text{ mL}$ but the system suggests 200 mL—what should you do NEXT? **A:** Override to 210 mL (or send to pharmacist if your system restricts) because the calculated total volume must match the full course to avoid shorting doses.
2271. **Q:** An eRx says "morphine sulfate ER 30 mg take 1 tab PO q12h #60," what days' supply should you enter and why? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$, which supports correct third-party processing and refill timing.
2272. **Q:** A prescription reads "azithromycin 250 mg: take ii tabs PO day 1 then i tab PO daily days 2–5," what quantity should be entered for dispensing? **A:** Enter 6 tablets because 2 tablets on day 1 plus 1 tablet for 4 more days equals 6 total, preventing a short fill.
2273. **Q:** While entering a new order you see "U-500 insulin regular inject 10 units SQ TID," what should you do FIRST before completing entry? **A:** Stop and alert the pharmacist to verify because U-500 is high-alert and dosing/volume confusion is common, raising patient-safety risk.
2274. **Q:** A sig says "hydroxyzine 25 mg take 1 cap PO qhs prn itching," but the prescriber typed "for anxiety" in the notes; what is the BEST technician action NEXT? **A:** Route to the pharmacist for review because indication mismatch may signal wrong drug/strength selection and requires clinical clarification.
2275. **Q:** A prescription is written "levothyroxine 75 mcg take i tab PO daily," but the eRx imports as "0.75 mg"; what should you do FIRST in order entry? **A:** Hold entry and send to pharmacist to clarify/correct because 0.75 mg equals 750 mcg and represents a dangerous decimal/units error.

2276. **Q:** You receive "prednisolONE sodium phosphate oral solution 15 mg/5 mL: take 10 mL PO daily x 5 days," what total mL should be dispensed? **A:** Dispense 50 mL because $10 \text{ mL/day} \times 5 \text{ days} = 50 \text{ mL}$, matching the intended duration.
2277. **Q:** A new prescription is "dorzolamide/timolol eye drops instill 1 gtt OU BID, dispense 10 mL"; what days' supply should you enter using 20 gtt/mL? **A:** Enter 50 days because $10 \text{ mL} \times 20 \text{ gtt/mL} = 200 \text{ drops}$ and OU BID uses 4 drops/day, so $200 \div 4 = 50 \text{ days}$ for accurate billing.
2278. **Q:** During data entry, a prescription reads "metoprolol succinate ER 50 mg take 1 tab PO BID #60," what should you do NEXT before finishing? **A:** Flag for pharmacist verification because succinate ER is typically once daily and BID may indicate a directions/formulation error outside technician scope.
2279. **Q:** A prescriber orders "clindamycin 150 mg take 2 caps PO QID #80," what days' supply should you calculate? **A:** Enter 10 days because $2 \text{ capsules} \times 4 \text{ times/day} = 8 \text{ capsules/day}$ and $80 \div 8 = 10 \text{ days}$, preventing refill-too-soon rejections.
2280. **Q:** You are typing "Vitamin D3 50,000 units take 1 cap weekly #12," and the system default is daily; what is the BEST action before submitting the claim? **A:** Correct the frequency to weekly and recheck days' supply as 84 days because daily entry would misbill and create unsafe directions.
2281. **Q:** During product selection you see a prescription for "Brilinta 90 mg 1 tab BID" but the shelf bin you grabbed is "Brintellix 10 mg"; what should you do FIRST? **A:** Stop filling and re-select by barcode/NDC and Tall Man awareness because Brilinta (ticagrelor) and Brintellix (vortioxetine) are LASA and a wrong-drug error is high risk.
2282. **Q:** A new eRx is for "Eliquis 5 mg BID" and the patient profile shows active "warfarin 5 mg daily"; what is the BEST next technician action? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication/bleeding risk because concurrent anticoagulants require clinical review before dispensing.
2283. **Q:** While entering "glipizide 10 mg daily," the patient states they "keep going low" and also takes insulin; what should you do NEXT? **A:** Refer the patient to the pharmacist immediately because sulfonylureas plus insulin increase hypoglycemia risk and technicians should not counsel or adjust therapy.
2284. **Q:** The prescriber sends "Bactrim DS 1 tab BID x 10 days" and the allergy field lists "sulfa"; what should you do FIRST? **A:** Hold the prescription and notify the

pharmacist because sulfamethoxazole/trimethoprim may trigger a sulfonamide allergy and needs verification/alternative selection.

2285. **Q:** A prescription reads "Zoloft 50 mg take 1 tab daily," but the patient's current med list includes "Paxil 20 mg daily"; what is the BEST action before filling? **A:** Stop and route to the pharmacist to assess duplicate SSRI therapy and serotonin syndrome risk because changing or combining antidepressants requires pharmacist intervention.
2286. **Q:** You are entering "metformin ER 500 mg take 2 tabs BID #120" and the patient is also on "cimetidine"; what should you do NEXT? **A:** Flag and inform the pharmacist because cimetidine can increase metformin levels and this interaction may require monitoring or therapy adjustment.
2287. **Q:** A label instruction says "lisinopril 10 mg i tab po qd," but the prescriber message notes "for pregnancy-induced HTN"; what should you do FIRST? **A:** Stop processing and alert the pharmacist because ACE inhibitors like lisinopril are contraindicated in pregnancy and must be clinically reviewed.
2288. **Q:** A patient brings a new prescription for "Augmentin 875/125 mg 1 tab BID #14" and asks what it's for; what should you do FIRST? **A:** Refer to the pharmacist for counseling because antibiotics like amoxicillin/clavulanate require indication/use and safety counseling under OBRA-90, which is not technician scope.
2289. **Q:** An order is for "gabapentin 300 mg take 1 cap TID #90," but you accidentally selected "pregabalin 75 mg"; what should you do NEXT? **A:** Immediately correct the product selection using barcode/NDC and segregate the wrong bottle from the filling area because gabapentin vs pregabalin is a common CNS mix-up with different strengths and controls.
2290. **Q:** You receive "Lantus SoloStar inject 20 units SQ nightly, dispense 3 pens," and each pen contains 3 mL of U-100; what days' supply should you enter? **A:** Enter 45 days because $3 \text{ pens} \times 3 \text{ mL/pen} \times 100 \text{ units/mL} = 900 \text{ units total}$, and $900 \text{ units} \div 20 \text{ units/day} = 45 \text{ days}$.
2291. **Q:** While typing an eRx for hydroxyzine 25 mg, you notice the prescriber wrote "take 1 tab qd" but hydroxyzine is typically dosed multiple times daily and the indication is missing; what should you do NEXT? **A:** Stop and ask the pharmacist to review/clarify with the prescriber because the directions look potentially erroneous and require pharmacist intervention before dispensing.

2292. **Q:** A patient's new prescription is for methotrexate 2.5 mg with directions "take 1 tablet daily," and the profile shows it is for rheumatoid arthritis; what should you do FIRST? **A:** Hold the fill and alert the pharmacist immediately because methotrexate is high-alert and daily dosing for RA is a common, serious prescribing error.
2293. **Q:** During pickup, a patient asks whether their new simvastatin can be taken with their amlodipine that is also on the profile; what is the BEST technician action? **A:** Refer the question to the pharmacist because potential interaction/dose-limit counseling requires pharmacist judgment and patient counseling.
2294. **Q:** You receive a hardcopy for oxycodone/APAP with no prescriber DEA number shown and the patient wants it filled now; what should you do NEXT? **A:** Stop processing and route to the pharmacist to verify controlled-substance validity because missing/unclear prescriber identifiers require pharmacist review before dispensing.
2295. **Q:** A prescriber calls in a refill authorization for alprazolam (Schedule IV) that is 7 months old since the issue date; what should you do FIRST? **A:** Notify the pharmacist and do not process the refill because Schedule III–V prescriptions are only refillable for up to 6 months from the date written under federal rules.
2296. **Q:** An eRx for methylphenidate (Schedule II) arrives with "Refills: 2" populated in the message; what should you do NEXT? **A:** Enter it as written but stop and alert the pharmacist because Schedule II refills are not permitted federally and the pharmacist must resolve it before dispensing.
2297. **Q:** A refrigerated box of insulin lispro is found on the shelf at room temperature with no documentation of how long it was out; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager because unknown temperature excursion makes it a suspect product that may be unsafe to dispense.
2298. **Q:** A wholesaler tote includes bottles of losartan with a label that doesn't match the usual manufacturer and the transaction history is missing; what is the BEST next step under DSCSA workflow? **A:** Segregate/quarantine the shipment and notify the pharmacist/receiver because missing tracing information requires investigation before the product can be accepted/dispensed.
2299. **Q:** The pharmacy receives a manufacturer recall notice for a specific lot of atorvastatin 20 mg already in stock; what should you do NEXT? **A:** Pull and

quarantine only the affected NDC/lot, document per policy, and alert the pharmacist because recalled lots must be removed from dispensing inventory immediately.

2300. **Q:** A prescription is for amoxicillin/clavulanate 400 mg/5 mL, sig: 10 mL PO BID x 7 days; how many milliliters should be dispensed? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$ total needed for the full course.
2301. **Q:** While entering a prescription for levothyroxine, you notice the prescriber selected Synthroid but the patient profile shows a documented allergy to "levothyroxine"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to clarify the allergy vs intolerance because Synthroid is levothyroxine and a true allergy is a safety stop.
2302. **Q:** A new eRx is for "metFORMIN ER 500 mg take 2 tabs BID" and the patient is also actively filling metformin IR 1000 mg BID; what is the BEST technician action? **A:** Flag it for pharmacist review and verify the active profile for therapeutic duplication because IR vs ER duplication can cause overdose and GI harm.
2303. **Q:** During product selection you see "Celebrex 200 mg" on the label but the bottle pulled is "Celexa 20 mg"; what should you do NEXT? **A:** Stop filling and re-pull the correct NDC/barcode for celecoxib because Celexa (citalopram) is a LASA mix-up with major patient-safety risk.
2304. **Q:** A prescription says "predniSONE 20 mg: take ii tabs daily x 5 days, then take i tab daily x 5 days" and the technician is unsure about the Roman numerals; what should you do FIRST? **A:** Confirm ii = 2 and i = 1 during data entry and route to pharmacist if any ambiguity remains because correct interpretation prevents dosing errors.
2305. **Q:** A patient presents an OTC request for pseudoephedrine 30 mg tablets, 2 tablets every 4 hours as needed, and wants to buy 5 boxes today; what should you do FIRST? **A:** Follow CMEA workflow by checking ID and verifying quantity limits in the logbook/system because pseudoephedrine sales are federally tracked and limited.
2306. **Q:** You receive "amLODIPine 10 mg daily" for a patient whose profile shows active "nifedipine ER 60 mg daily" from last week; what should you do NEXT? **A:** Hold the order and notify the pharmacist to assess duplicate calcium channel

blocker therapy because concurrent use may cause excessive hypotension/edema.

2307. **Q:** A prescription is for "liquid morphine concentrate 20 mg/mL: take 0.25 mL PO q4h prn pain; dispense 30 mL"; what should you do BEST before filling? **A:** Route to the pharmacist for high-alert opioid verification because concentrated morphine is high-risk and small-volume dosing errors can be fatal.
2308. **Q:** An eRx comes in for "Zoloft 50 mg daily" but the patient profile lists current "sertraline 50 mg daily" already filled last week; what should you do FIRST? **A:** Verify whether it is the same drug (Zoloft = sertraline) and assess refill-too-soon/duplicate entry before processing because brand/generic duplicates commonly cause double billing and double therapy.
2309. **Q:** A new prescription is "azithromycin 250 mg: take 2 tabs on day 1, then 1 tab daily days 2–5; dispense #?"; what quantity should you enter? **A:** Enter quantity 6 tablets because the regimen uses 2 tablets on day 1 plus 4 more tablets on days 2–5 ($2 + 4 = 6$).
2310. **Q:** You are typing "Humalog KwikPen U-100 inject 8 units TID with meals, dispense 2 pens," and each pen contains 3 mL; what days' supply should you enter? **A:** Enter 75 days because $2 \text{ pens} = 6 \text{ mL} = 600 \text{ units total}$ and $8 \text{ units TID} = 24 \text{ units/day}$, so $600 \div 24 = 25 \text{ days}$ (correcting: actually $600 \div 24 = 25 \text{ days}$) therefore 25 days' supply to prevent overbilling.
2311. **Q:** While typing a new eRx for hydroxyzine 25 mg, you notice the prescriber intended hydralazine 25 mg based on "for blood pressure" in the notes; what should you do FIRST? **A:** Stop data entry and alert the pharmacist to contact the prescriber because a LASA mix-up (hydroxyzine vs hydralazine) is a high-risk error requiring pharmacist intervention.
2312. **Q:** A C-II oxycodone prescription is presented with the patient's name, drug, strength, and directions, but no prescriber signature; what should you do NEXT? **A:** Hold the prescription and notify the pharmacist because a C-II must be properly signed/authorized before dispensing and the technician cannot "fix" missing legal elements.
2313. **Q:** You pull a refrigerated insulin glargine vial from the will-call bin and notice it was left at room temperature overnight; what is the BEST technician action? **A:** Remove it from will-call and notify the pharmacist to evaluate stability/BUD because temperature excursions create a suspect product and patient safety risk.

2314. **Q:** A patient asks to return opened bottles of lisinopril and metformin because “they didn’t work”; what should you do FIRST? **A:** Follow pharmacy policy and involve the pharmacist, and do not return opened medication to stock because it is not eligible for redispensing and could be unsafe.
2315. **Q:** You receive a wholesaler invoice but the tote contains a bottle with a different NDC than what was ordered and the seal looks tampered; what should you do NEXT? **A:** Set the bottle aside as suspect and notify the pharmacist/manager for DSCSA procedures because product integrity and supply chain verification are required before use.
2316. **Q:** A patient wants an early refill on clonazepam because they “lost the bottle,” and the last fill was 10 days ago for a 30-day supply; what should you do BEST? **A:** Stop processing and refer to the pharmacist because controlled-substance early refills require pharmacist judgment and documentation per policy and safety concerns.
2317. **Q:** While entering warfarin 5 mg, you see the label printer created “warfarin 0.5 mg” due to a decimal entry; what should you do FIRST? **A:** Cancel/void the label and correct the entry before filling because decimal-point errors with high-alert anticoagulants can cause serious harm.
2318. **Q:** A prescription for isotretinoin arrives, but the patient is not enrolled/confirmed in the required program per the notes; what should you do NEXT? **A:** Hold the prescription and alert the pharmacist because isotretinoin is a REMS medication and dispensing requires confirmed REMS requirements.
2319. **Q:** A sig reads “Take ii gtt OS QID x 7 days” for ofloxacin ophthalmic, and the patient asks what it means; what should you do BEST? **A:** Translate and confirm as “instill 2 drops in the left eye four times daily for 7 days,” and refer counseling questions to the pharmacist because technicians may clarify sig abbreviations but pharmacists counsel.
2320. **Q:** A prescription is for azithromycin 200 mg/5 mL suspension: “Give 12.5 mL PO day 1, then 6.25 mL PO daily days 2–5”; how many mL should be dispensed? **A:** Dispense 37.5 mL because total volume is 12.5 mL + (6.25 mL × 4) = 37.5 mL to complete the 5-day course.
2321. **Q:** You are entering levothyroxine 125 mcg and the prescriber wrote “0.125 mg PO daily”; what strength should you select in the system? **A:** Select 125 mcg because 0.125 mg × 1000 = 125 mcg, preventing a 1000-fold dosing error.

2322. **Q:** A prescription reads "furosemide 40 mg tab: i tab PO BID #60"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ tablets} \div 2 \text{ tablets/day} = 30 \text{ days}$ for accurate billing and refill timing.
2323. **Q:** You receive "metformin ER 500 mg: take 2 tabs PO QPM with dinner; dispense 180"; what is the correct days' supply for order entry? **A:** Enter 90 days because $180 \text{ tablets} \div 2 \text{ tablets/day} = 90 \text{ days}$ to match the written quantity and directions.
2324. **Q:** A new eRx says "metoprolol" with directions "take 1 tablet daily" but no salt form (tartrate vs succinate); what should you do NEXT? **A:** Stop processing and send to the pharmacist to clarify formulation because tartrate vs succinate are not interchangeable and affect dosing.
2325. **Q:** A label prints "morphine 10 mg/mL; take 0.5 mL" but the original order is "morphine 1 mg/mL; take 5 mL"; what should you do FIRST? **A:** Stop the fill and alert the pharmacist because the concentration mismatch creates a high-alert opioid dosing error risk.
2326. **Q:** A prescription for amoxicillin-clavulanate states "Augmentin ES-600: give 7.5 mL PO BID x 10 days"; what quantity (mL) should you dispense? **A:** Dispense 150 mL because $7.5 \text{ mL/dose} \times 2 \text{ doses/day} \times 10 \text{ days} = 150 \text{ mL}$ to cover the full course.
2327. **Q:** An inhaler order reads "fluticasone/salmeterol 250/50 Diskus: inhale 1 puff BID; disp 1 inhaler (60 doses)"; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$ for correct days' supply on the claim.
2328. **Q:** A sig comes in as "Apply ung AA BID" for triamcinolone ointment; what should you do BEST during data entry? **A:** Clarify with the pharmacist because "AA" (affected area) is ambiguous for patient instructions and could lead to misuse.
2329. **Q:** You are typing "prednisone 20 mg: take 3 tabs daily x 5 days; dispense 1 bottle" and the stock bottle contains 30 tablets; what quantity should you enter to dispense? **A:** Enter 15 tablets because $3 \text{ tablets/day} \times 5 \text{ days} = 15 \text{ tablets}$, and you should not default to the full bottle if it exceeds the prescribed amount.
2330. **Q:** During product selection you scan NDC for sertraline 50 mg but the screen shows sertraline 100 mg after scan; what should you do FIRST? **A:** Stop and re-select the correct product/NDC because barcode mismatch signals wrong strength selection and must be corrected before verification.

2331. **Q:** A prescription reads "clindamycin 75 mg/5 mL: take 10 mL PO TID x 7 days"; what quantity (mL) should you dispense? **A:** Dispense 210 mL because $10 \text{ mL/dose} \times 3 \text{ doses/day} \times 7 \text{ days} = 210 \text{ mL}$.
2332. **Q:** You're entering "prednisolone sodium phosphate 15 mg/5 mL: take 2.5 mL PO daily x 10 days"; what days' supply should be entered? **A:** Enter 10 days because the directions specify a fixed duration of 10 days.
2333. **Q:** An eRx for lisinopril says "take one tablet by mouth qd" and the quantity is 90; what days' supply should you enter? **A:** Enter 90 days because 1 tablet daily with 90 tablets equals a 90-day supply.
2334. **Q:** A prescription is for insulin lispro U-100 vial "inject 12 units subcutaneously TID with meals; disp 10 mL"; what days' supply should you calculate for order entry (ignore beyond-use dating)? **A:** Enter 27 days because $10 \text{ mL} = 1000 \text{ units}$ and $12 \text{ units} \times 3/\text{day} = 36 \text{ units/day}$, so $1000 \div 36 = 27.7$ rounded down to 27.
2335. **Q:** An order reads "brimonidine 0.2% eye drops: instill 1 gtt OU TID; disp 5 mL"; what days' supply should you enter using 20 gtt/mL? **A:** Enter 16 days because $5 \text{ mL} \times 20 \text{ gtt/mL} = 100 \text{ gtt}$ and $\text{OU TID} = 6 \text{ gtt/day}$, so $100 \div 6 = 16.6$ rounded down to 16.
2336. **Q:** You receive "amoxicillin 500 mg caps: i cap PO TID x 10 days"; what quantity should be dispensed? **A:** Dispense 30 capsules because $1 \text{ capsule} \times 3/\text{day} \times 10 \text{ days} = 30$.
2337. **Q:** While typing a new prescription you see the sig "take 0.5 tab PO BID" for carvedilol 12.5 mg and the quantity is 60 tablets; what should you do NEXT? **A:** Enter days' supply as 60 days because 0.5 tablet twice daily equals 1 tablet/day, and proceed only if the half-tablet direction is clearly intended (otherwise route to pharmacist to confirm).
2338. **Q:** A prescriber writes "hydromorphone 2 mg: take 1 tab PO q4-6h prn pain #30"; what is the BEST days' supply to enter if your payer requires using the maximum frequency? **A:** Enter 5 days because max use is $q4h = 6 \text{ doses/day}$, so $30 \div 6 = 5 \text{ days}$, which matches the required rule.
2339. **Q:** During order entry you notice "Lantus (insulin glargine) 100 unit/mL: inject 10 units daily" but the product selected is insulin detemir; what should you do FIRST? **A:** Stop and correct the selection to insulin glargine (or route to pharmacist if unsure) because detemir is a different insulin and substitution could cause harm.

2340. **Q:** A label sig prints "metronidazole gel: apply topically BID" but the written order says "insert 1 applicatorful vaginally HS x 5 days"; what should you do FIRST? **A:** Stop processing and notify the pharmacist to correct the route/directions because topical vs vaginal use is a high-risk order entry error.
2341. **Q:** While entering a new prescription, you see "hydralazine 25 mg" but the prescriber selected "hydroxyzine 25 mg" on the eRx; what should you do FIRST? **A:** Stop processing and notify the pharmacist to clarify because this is a LASA drug name mismatch that could cause a serious dispensing error.
2342. **Q:** A patient drops off a prescription for alprazolam 0.5 mg with "refill x 5"; what is the BEST technician action? **A:** Hold the prescription and route to the pharmacist because benzodiazepines are Schedule IV and refills are limited by federal rules and must be verified for validity.
2343. **Q:** You receive an eRx for isotretinoin (Accutane) and the patient wants it today; what should you do NEXT before filling? **A:** Alert the pharmacist and verify iPLEDGE/REMS requirements are met because isotretinoin dispensing is restricted and cannot proceed without required authorization steps.
2344. **Q:** During verification prep you notice two active profiles: "Plavix 75 mg daily" and a new prescription for "clopidogrel 75 mg daily"; what should you do FIRST? **A:** Stop and notify the pharmacist because Plavix is clopidogrel and this is therapeutic duplication that could lead to double dosing.
2345. **Q:** A refrigerator temperature log shows 55°F for several hours where insulin and certain vaccines are stored; what should you do FIRST? **A:** Segregate the affected inventory and alert the pharmacist because temperature excursions can compromise potency and products may require discard or manufacturer guidance.
2346. **Q:** A stock bottle of levothyroxine 50 mcg tablets has a torn seal and tablets look different than usual; what is the BEST action? **A:** Quarantine the bottle and notify the pharmacist/manager because this is suspect product and must be removed from dispensing pending investigation per DSCSA/safety practice.
2347. **Q:** A recall notice is received for specific lots of losartan tablets; what should you do NEXT in workflow? **A:** Pull the matching NDC/lot from shelves and place in quarantine because recalled lots must be prevented from dispensing and handled per recall procedure.

2348. **Q:** A patient requests pseudoephedrine and wants two 96-count boxes of 30 mg tablets today; what should you do FIRST at the register? **A:** Check ID and run the sale through the electronic log to verify legal limits because CMEA requires identity verification and quantity tracking before completing the sale.
2349. **Q:** Order entry: "amoxicillin/clavulanate 875/125 mg, take 1 tablet PO BID x 10 days, dispense #20"; what days' supply should you enter? **A:** Enter 10 days because $20 \text{ tablets} \div 2 \text{ tablets/day} = 10 \text{ days}$ for accurate billing and safety checks.
2350. **Q:** A caregiver asks why the pharmacy is counseling and offering to review all medications for a new Medicaid patient; what is the BEST explanation? **A:** Explain that OBRA-90 requires offering counseling to Medicaid patients because it's a federal patient-safety requirement to promote proper use and reduce errors.
2351. **Q:** While typing an eRx for brand-name "Zestril 10 mg daily," the selected product is "Zyrtec 10 mg"; what should you do FIRST? **A:** Stop processing and correct the product selection by verifying brand/generic (Zestril = lisinopril, Zyrtec = cetirizine) to prevent a LASA wrong-drug error.
2352. **Q:** A new prescription is entered for sertraline, and the patient profile shows linezolid started yesterday; what is the BEST technician action? **A:** Hold the order and notify the pharmacist immediately because linezolid plus SSRIs can cause serotonin syndrome and requires pharmacist intervention.
2353. **Q:** A prescriber sends "MS Contin 15 mg" but the product pulled is "morphine oral solution 10 mg/5 mL"; what should you do NEXT? **A:** Stop the fill and reselect the correct dosage form/ER product because MS Contin is extended-release tablets and substituting solution changes dosage form and safety.
2354. **Q:** The label prints "take 1 tab QD" for metformin, but the hardcopy says "metformin ER 500 mg take 1 tablet BID"; what should you do FIRST? **A:** Pause and route to the pharmacist to clarify directions and ER formulation because changing frequency/formulation can alter therapy and is not a technician change.
2355. **Q:** A patient picking up simvastatin says they were told to start clarithromycin today for pneumonia; what should you do BEST? **A:** Refer to the pharmacist before dispensing because clarithromycin can greatly increase simvastatin levels and risk myopathy/rhabdomyolysis.

2356. **Q:** You are entering "azithromycin 250 mg: take 2 tablets today then 1 tablet daily for 4 days"; what total quantity should you dispense? **A:** Dispense 6 tablets because 2 on day 1 plus 1 daily for 4 more days equals 6 total.
2357. **Q:** A prescription reads "prednisone 20 mg ii tabs PO daily x 5 days"; what quantity should be entered? **A:** Enter 10 tablets because "ii" means 2 tablets per day for 5 days ($2 \times 5 = 10$).
2358. **Q:** During data entry you see "Humalog U-100" but the selected item is "Humulin R U-100"; what should you do FIRST? **A:** Stop and correct the insulin selection by verifying brand/generic and type (Humalog = insulin lispro, Humulin R = regular insulin) because they are not interchangeable.
2359. **Q:** A new order for apixaban (Eliquis) is being processed and the profile shows active warfarin; what should you do NEXT? **A:** Flag for pharmacist review because concurrent anticoagulants may represent therapeutic duplication and bleeding risk that requires clinical assessment.
2360. **Q:** While filling levothyroxine, you notice the NDC scanned is a different manufacturer than the one documented as preferred for the patient; what should you do BEST? **A:** Confirm with the pharmacist and follow pharmacy policy before dispensing because levothyroxine is narrow therapeutic index and consistency in manufacturer can be important.
2361. **Q:** A prescription is written for insulin U-500, but the patient profile only shows previous use of insulin glargine U-100; what should you do FIRST? **A:** Stop processing and alert the pharmacist to verify the concentration and dosing because U-500 is a high-alert product with high risk of serious dosing errors.
2362. **Q:** A patient asks to transfer their oxycodone prescription to another pharmacy because they are traveling; what is the BEST technician response? **A:** Refer the request to the pharmacist because controlled-substance transfer rules are restricted and require pharmacist handling and documentation.
2363. **Q:** While receiving an order, you find a tote of refrigerated Humira (adalimumab) sitting at room temperature with no temperature indicator data; what should you do FIRST? **A:** Quarantine the product and notify the pharmacist/manager to assess a temperature excursion because compromised cold-chain biologics may be unsafe or ineffective.
2364. **Q:** An eRx for hydralazine is received, but during product selection you almost pick hydroxyzine due to similar names; what should you do BEST to prevent a

LASA error? **A:** Use barcode/NDC scan and compare the drug name/strength to the label and original order because independent verification reduces look-alike/sound-alike mix-ups.

2365. **Q:** A prescription for isotretinoin is being entered and the patient arrives to pick it up immediately; what should you do NEXT? **A:** Alert the pharmacist to confirm REMS/iPLEDGE requirements are met before dispensing because isotretinoin has mandatory program steps that must be verified.
2366. **Q:** While entering a new prescription for lisinopril, you notice the patient profile lists a documented history of angioedema with an ACE inhibitor; what should you do FIRST? **A:** Stop processing and alert the pharmacist to evaluate therapy because ACE inhibitors like lisinopril can cause recurrent, serious angioedema.
2367. **Q:** A patient drops off a new prescription for sertraline, and their profile shows they are currently taking phenelzine; what is the BEST next step for the technician? **A:** Hold the prescription and notify the pharmacist immediately because MAOIs plus SSRIs can cause dangerous interactions (e.g., serotonin syndrome) and require clinical intervention.
2368. **Q:** During product selection for buPROPion SR, you accidentally pulled buPROPion XL from the shelf; what should you do FIRST? **A:** Stop and reselect the exact prescribed formulation and strength, then scan/verify NDC because SR vs XL mix-ups are common and can change dosing and safety.
2369. **Q:** A prescription says "Levemir insulin pen: inject 18 units SC QHS; dispense 1 box (5 pens)"; what days' supply should you enter if each pen is 3 mL of U-100? **A:** Enter 83 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 18 = 83.3$ which rounds down to 83 days for billing.
2370. **Q:** A patient presents a new prescription for ciprofloxacin and asks if they can keep taking their daily calcium supplement at the same time; what should you do BEST? **A:** Refer to the pharmacist because calcium can bind fluoroquinolones and reduce absorption, requiring counseling on dose separation.
2371. **Q:** While typing a prescription for lamotrigine, you see the sig includes "increase dose weekly as directed" but no specific titration schedule is provided; what should you do NEXT? **A:** Pause entry and send to the pharmacist for clarification because lamotrigine titration must be explicit to reduce serious rash risk and technicians can't create dosing schedules.

2372. **Q:** A new eRx comes in for clopidogrel, and the patient's active meds include omeprazole; what should you do BEST? **A:** Flag and route to the pharmacist because omeprazole can reduce clopidogrel activation, creating a clinically important interaction.
2373. **Q:** You are calculating quantity for "gabapentin 300 mg: take 1 capsule TID for 14 days"; what quantity should be dispensed? **A:** Dispense 42 capsules because $1 \text{ capsule} \times 3 \text{ doses/day} \times 14 \text{ days} = 42$.
2374. **Q:** A patient requests an early refill of oxycodone/APAP and states they "lost the bottle"; what is the BEST technician response? **A:** Politely explain you must involve the pharmacist and follow controlled-substance policy because early/lost-fill requests require pharmacist review and documentation.
2375. **Q:** While stocking, you find a bottle of look-alike vials labeled "hydrOXYzine" placed in the "hydrALAZINE" bin; what should you do FIRST? **A:** Remove and correct the stock location and notify the pharmacist/lead tech because LASA mix-ups like hydralazine vs hydroxyzine can cause serious dispensing errors.
2376. **Q:** An eRx says "metFORMIN ER 500 mg: i tab PO BID with meals #60," but the prescriber selected Glucophage XR and you only have generic metformin ER; what should you do NEXT in order entry? **A:** Select the correct generic equivalent and verify ER formulation matches (not IR), because brand/generic substitution is allowed when dosage form and release characteristics are the same.
2377. **Q:** A prescription reads "predniSONE 20 mg: ii tabs PO daily x 5 days; #?" and the patient needs it today; what quantity should you enter? **A:** Enter quantity 10 tablets because $2 \text{ tablets/day} \times 5 \text{ days} = 10 \text{ tablets}$ to match the intended course.
2378. **Q:** You receive "amLODIPine 5 mg: take 1 tab PO QD; #30 RF 2," but the prescriber typed "QD" in the notes and "QID" in the sig field; what should you do FIRST? **A:** Stop processing and refer to the pharmacist to clarify with the prescriber because conflicting directions create a high risk for dosing error.
2379. **Q:** A patient's prescription is "fluticasone/salmeterol 250/50 Diskus: inhale 1 puff BID," and the device contains 60 blisters; what days' supply should you enter? **A:** Enter 30 days because $60 \text{ doses} \div 2 \text{ doses/day} = 30 \text{ days}$ for correct billing and refill timing.
2380. **Q:** While entering "azithromycin 200 mg/5 mL: give 10 mL PO day 1, then 5 mL PO daily days 2-5; dispense ____ mL," what total volume should you dispense?

A: Dispense 30 mL because day 1 is 10 mL plus days 2–5 is 5 mL $\times$ 4 = 20 mL, totaling 30 mL.

2381. **Q:** While entering a new Rx for lisinopril, you notice the patient's profile lists a prior anaphylaxis reaction to "ACE inhibitors"; what should you do FIRST? **A:** Stop processing and alert the pharmacist to review the ACE-inhibitor allergy because dispensing could cause a serious repeat reaction.
2382. **Q:** A prescriber sends "methotrexate 2.5 mg: take 1 tablet daily for rheumatoid arthritis"; what should you do NEXT? **A:** Hold the prescription and refer to the pharmacist for clarification because methotrexate is typically weekly and daily dosing is a high-risk error.
2383. **Q:** A patient drops off a new prescription for sertraline and asks for "St. John's wort too because it's natural"; what is the BEST technician response? **A:** Refer the question to the pharmacist because combining St. John's wort with SSRIs can increase serotonin syndrome risk.
2384. **Q:** During product selection you scan "hydromorphone" but the label printed for "morphine"; what should you do FIRST? **A:** Stop filling and reselect the correct NDC/barcode match because this LASA opioid mix-up is high-alert and must be corrected before verification.
2385. **Q:** An order is "amoxicillin/clavulanate 875/125 mg: take 1 tablet PO BID $\times$ 10 days"; what quantity should you enter? **A:** Dispense 20 tablets because 2 doses/day $\times$ 10 days = 20 tablets.
2386. **Q:** A prescription reads "dorzolamide/timolol eye drops: instill 1 gtt OU BID; dispense 10 mL"; what days' supply should you submit using 20 gtt/mL? **A:** Enter 50 days because 10 mL $\times$ 20 gtt/mL = 200 gtt and OU BID = 4 gtt/day, so 200 $\div$ 4 = 50 days.
2387. **Q:** A patient's new medication list includes "Eliquis" and they ask what the generic name is so they can recognize duplicates; what should you answer? **A:** Apixaban, because recognizing brand/generic helps prevent therapeutic duplication and improves medication safety.
2388. **Q:** While typing an eRx for "Wellbutrin XL," you see the prescriber selected "bupropion SR" in the product field; what should you do BEST? **A:** Stop and send to the pharmacist to clarify release formulation because SR vs XL is not interchangeable and could change dosing and safety.

2389. **Q:** A refill request comes in for atorvastatin, but the profile shows the patient already takes rosuvastatin from another prescriber; what should you do NEXT? **A:** Flag the possible duplicate statin therapy for pharmacist review because two statins increases risk of adverse effects without clear benefit.
2390. **Q:** A prescription says "levothyroxine 50 mcg: take 1 tab daily," but the patient wants to switch between brand Synthroid and generic each month for price; what should you do BEST? **A:** Refer to the pharmacist because levothyroxine products should be kept consistent when possible and any switch may require monitoring.
2391. **Q:** While typing a new Rx for amlodipine, the patient says they already take "Norvasc" at home; what should you do NEXT? **A:** Verify that Norvasc is amlodipine and check the profile for therapeutic duplication before processing, because brand/generic recognition prevents double therapy.
2392. **Q:** A prescriber e-sends "metformin ER 500 mg: take 1 tab BID," but the selected product is immediate-release metformin; what is the BEST technician action? **A:** Stop and route to the pharmacist to clarify formulation, because IR vs ER is not interchangeable and changing dosage form is outside technician scope.
2393. **Q:** A patient picking up clarithromycin asks if it's okay with their simvastatin; what should you do FIRST? **A:** Refer to the pharmacist immediately, because clarithromycin can significantly increase simvastatin levels and the interaction may require therapy adjustment.
2394. **Q:** During data entry you see "glimepiride 4 mg daily" and the allergy field shows "sulfa—rash"; what should you do NEXT? **A:** Flag and ask the pharmacist to assess the allergy risk, because sulfonylureas can cross-react in some patients and require clinical judgment.
2395. **Q:** An Rx is "cephalexin 500 mg: i po qid x 7 days"; what quantity should you enter? **A:** Enter 28 capsules, because 1 capsule 4 times daily for 7 days is $4 \times 7 = 28$.
2396. **Q:** A new eRx arrives for "Lantus SoloStar: inject 20 units SQ nightly; dispense 1 box (5 pens of 3 mL, 100 units/mL)"; what days' supply should you submit based on total units? **A:** Submit 75 days, because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$ and $1500 \div 20 \text{ units/day} = 75 \text{ days}$.
2397. **Q:** You are selecting "clonidine 0.1 mg" but notice the shelf also contains "clozapine 100 mg" nearby; what should you do BEST? **A:** Use barcode/NDC

scan and extra label-to-stock bottle verification, because clonidine/clozapine is a common LASA trap with high patient-harm potential.

2398. **Q:** A prescription for oxycodone/APAP (Percocet) arrives with "Refills: 2" indicated; what should you do FIRST? **A:** Hold the order and notify the pharmacist, because Schedule II controlled substances cannot be refilled and require correction before dispensing.
2399. **Q:** A patient asks what "Crestor" is so they can avoid taking two cholesterol meds that are the same; what should you answer? **A:** "Crestor is rosuvastatin," because giving the generic name helps the patient recognize duplicate statin therapy and improves safety.
2400. **Q:** While unpacking an order, you find a bottle of duloxetine with a broken seal and powder residue inside; what should you do NEXT? **A:** Remove it from dispensing stock and follow suspect/damaged product procedure (segregate and notify the pharmacist/manager), because product integrity is compromised and unsafe to dispense.
2401. **Q:** A new Rx is entered for lisinopril 20 mg daily, but the patient profile already shows lisinopril 10 mg daily from another prescriber; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication/dose change so the pharmacist can verify intended therapy before dispensing.
2402. **Q:** A patient says they take "Plavix" at home and asks what generic name to look for on the label; what should you answer? **A:** Tell them Plavix is clopidogrel because matching brand to generic helps prevent duplicate antiplatelet therapy.
2403. **Q:** While processing a new Rx for sertraline, you see an active prescription for fluoxetine on the same profile; what is the BEST next step? **A:** Route to the pharmacist for review of duplicate SSRI therapy and possible interaction risk because technicians should not assume both are intended.
2404. **Q:** An eRx arrives for "hydroxyzine 25 mg: take 1 tab TID prn anxiety," but you accidentally selected hydralazine 25 mg at data entry; what should you do FIRST? **A:** Stop filling and correct the drug selection by verifying name/indication and NDC because this is a LASA error with high harm potential.
2405. **Q:** A patient picking up nitroglycerin SL tablets asks if they can take it with sildenafil they used last night; what should you do FIRST? **A:** Immediately refer to the pharmacist because nitrates with PDE-5 inhibitors can cause dangerous hypotension and requires counseling intervention.

2406. **Q:** The prescriber writes "predniSONE 10 mg: take ii tabs daily x 5 days"; what quantity should you enter? **A:** Enter 10 tablets because ii daily for 5 days equals $2 \times 5 = 10$.
2407. **Q:** A prescription is for "amoxicillin/clavulanate 875/125 mg: 1 tab BID x 10 days"; what quantity should you dispense? **A:** Dispense 20 tablets because BID for 10 days equals 2 doses/day $\times$ 10 days.
2408. **Q:** You receive an eRx for "Humalog U-100: inject 6 units SQ TID with meals; dispense 1 vial (10 mL, 100 units/mL)"; what days' supply should you submit based on total units? **A:** Submit 55 days because the vial contains $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $6 \text{ units TID} = 18 \text{ units/day}$, so $1000 \div 18 = 55.5$ rounded down to 55.
2409. **Q:** While stocking, you notice warfarin 2 mg and 5 mg bottles stored next to glipizide with similar-looking labels; what should you do BEST? **A:** Separate and clearly label the storage (use Tall Man/high-alert identifiers per policy) because warfarin is high-alert and reducing selection errors protects patient safety.
2410. **Q:** During order entry for azithromycin, the patient profile shows a documented allergy of "macrolides—hives"; what should you do NEXT? **A:** Stop processing and notify the pharmacist to assess the allergy and contact the prescriber if needed because macrolide allergy may contraindicate azithromycin.
2411. **Q:** A patient says they take "Zoloft" and asks what it's for so they know it matches the new prescription; what should you answer? **A:** Zoloft is sertraline, an SSRI commonly used for depression/anxiety, and confirming brand/generic and indication helps prevent taking the wrong medication.
2412. **Q:** While entering a new Rx for levothyroxine, the prescriber note says "brand medically necessary," but you selected a generic NDC; what should you do NEXT? **A:** Stop and reselect the brand product (or route to the pharmacist if unclear) because "brand necessary" affects what may be dispensed and billed.
2413. **Q:** A patient's profile shows metformin ER active, and you receive a new Rx for metformin IR; what is the BEST next step for the technician? **A:** Flag and refer to the pharmacist for therapeutic-duplication/format change review because IR vs ER are not interchangeable without confirmation.
2414. **Q:** A prescription reads "gabapentin 300 mg: i cap po tid" with quantity 270; the patient asks what "i" means; what should you say? **A:** "i" means 1, so the

directions are 1 capsule by mouth three times daily, which helps the patient understand the dose and reduces administration errors.

2415. **Q:** You receive an eRx for “Keflex 500 mg QID x 7 days”; what quantity should you enter? **A:** Dispense 28 capsules because $4 \text{ doses/day} \times 7 \text{ days} = 28 \text{ total doses}$.
2416. **Q:** A patient picking up tramadol says they’re also taking “Xanax” at night and asks if it’s okay together; what should you do FIRST? **A:** Involve the pharmacist immediately because opioid-like analgesics plus benzodiazepines increase sedation/respiratory depression risk and require counseling.
2417. **Q:** During product selection you notice “Lamictal” and “Lamisil” stored adjacent with similar packaging; what should you do BEST to reduce LASA errors? **A:** Separate the products and apply Tall Man/auxiliary LASA cues per pharmacy policy because look-alike/sound-alike storage increases wrong-drug selection.
2418. **Q:** A new Rx is for rivaroxaban, but the patient profile shows active apixaban from another prescriber; what should you do NEXT? **A:** Hold processing and alert the pharmacist to evaluate anticoagulant duplication because concurrent DOAC therapy is a high-risk safety issue.
2419. **Q:** A patient asks what “Lasix” is called on the bottle so they can match it to their home med list; what should you answer? **A:** Lasix is furosemide, a loop diuretic for fluid/blood pressure, and providing the generic name helps prevent duplicate therapy and confusion.
2420. **Q:** An order is for insulin regular U-100 vial 10 mL with directions “inject 8 units before each meal (TID)”; what days’ supply should you submit using total units? **A:** Submit 41 days’ supply because $10 \text{ mL} \times 100 \text{ units/mL} = 1000 \text{ units}$ and $8 \times 3 = 24 \text{ units/day}$, so $1000 \div 24 = 41.6$, rounded down to 41 days.
2421. **Q:** While entering an Rx for “Prozac 20 mg daily,” the patient says they usually take “fluoxetine”; what should you do BEST? **A:** Confirm Prozac is the brand name for fluoxetine and proceed with accurate entry because matching brand/generic prevents duplicate therapy confusion.
2422. **Q:** A new eRx is for lisinopril 20 mg daily, but the profile shows an active benazepril prescription; what should you do NEXT? **A:** Stop and alert the pharmacist to possible therapeutic duplication since both are ACE inhibitors and require clinical review before dispensing.

2423. **Q:** You receive an Rx for "Lipitor 40 mg qhs" and the patient asks what it treats; what is the BEST technician response? **A:** State that Lipitor is atorvastatin used to lower cholesterol and reduce cardiovascular risk because giving basic indication helps confirm the correct medication.
2424. **Q:** During product selection you notice hydroxyzine and hydralazine stored side-by-side with similar labels; what should you do FIRST to reduce LASA risk? **A:** Separate the products and add Tall Man/auxiliary LASA alerts per policy because physical separation and labeling reduce selection errors.
2425. **Q:** An eRx for "methotrexate 2.5 mg: take 6 tablets daily" appears in the queue for a rheumatoid arthritis patient; what should you do BEST? **A:** Hold processing and immediately refer to the pharmacist because methotrexate is typically weekly and daily dosing is a high-risk error.
2426. **Q:** A prescription is for "Xarelto 20 mg daily," but you selected an NDC for "Xanax"; what should you do NEXT? **A:** Stop filling and reselect using barcode/NDC verification because rivaroxaban and alprazolam are LASA and the wrong drug could cause serious harm.
2427. **Q:** A patient brings an OTC request for "Advil PM" but asks if it's the same as ibuprofen; what should you answer? **A:** Explain that Advil PM contains ibuprofen plus a sleep aid (diphenhydramine) because combination products can add sedation and duplicate antihistamines.
2428. **Q:** You are calculating days' supply for metoprolol succinate ER 50 mg "take 1 tablet daily," quantity 90; what days' supply should you submit? **A:** Submit 90 days because once-daily dosing uses 1 tablet per day and $90 \text{ tablets} \div 1/\text{day} = 90 \text{ days}$.
2429. **Q:** A new Rx is for sertraline, and the patient profile shows linezolid started yesterday from an urgent care; what should you do FIRST? **A:** Flag and refer to the pharmacist for a major interaction risk (serotonin syndrome) because combining an SSRI with linezolid can be dangerous.
2430. **Q:** While putting away a shipment, you find a bottle of insulin aspart with a cracked seal and unknown storage history; what should you do BEST? **A:** Quarantine the product and notify the pharmacist/manager because insulin is temperature sensitive and compromised packaging makes it non-dispensable.
2431. **Q:** A prescriber writes "Glucophage 500 mg BID" and the patient says they only recognize "metformin"; what should you do NEXT during drop-off? **A:** Confirm

that Glucophage is metformin and proceed with accurate data entry while offering pharmacist counseling if they have therapy questions, because brand/generic recognition reduces confusion without changing therapy.

2432. **Q:** While entering "Lamictal 25 mg," you accidentally pull "Lamisil 250 mg" from the shelf; what should you do FIRST? **A:** Stop filling and re-select using barcode/NDC verification and Tall Man awareness, because Lamictal/lamisil is a high-risk LASA mix-up that can cause serious harm.
2433. **Q:** An eRx for "levothyroxine 50 mcg daily" comes in, but the profile shows active "Synthroid 50 mcg daily"; what is the BEST technician action? **A:** Verify both entries refer to the same drug/strength and route to the pharmacist to confirm duplication/active orders, because you should not independently decide which therapy to discontinue.
2434. **Q:** A patient with a new Rx for clarithromycin asks if it's okay with their simvastatin they take nightly; what should you do FIRST? **A:** Pause processing and alert the pharmacist to review the interaction, because clarithromycin can dangerously increase simvastatin levels and requires clinical judgment.
2435. **Q:** You receive an eRx for "MS Contin 15 mg q12h" and the prescriber later phones in "morphine sulfate 15 mg q12h" for the same patient; what should you do NEXT? **A:** Hold the second order and refer to the pharmacist to verify intent and prevent duplicate opioid therapy, because MS Contin and morphine sulfate may represent the same medication order.
2436. **Q:** A prescription is written for "Humalog KwikPen 100 units/mL: inject 12 units TID with meals; dispense 2 pens"; what days' supply should you submit using 300 units per pen? **A:** Submit 16 days because 2 pens = 600 units and $12 \text{ units} \times 3 = 36 \text{ units/day}$, so $600 \div 36 = 16.6$ and you bill 16 to avoid overstating supply.
2437. **Q:** A patient brings in "Zithromax Z-Pak" and asks what the generic name is for their medication list; what should you answer? **A:** Tell them Zithromax is azithromycin, because technicians can provide accurate brand-to-generic identification to support safe medication reconciliation.
2438. **Q:** During order entry you see "HCTZ 25 mg qd" and the prescriber also wrote "HydroDIURIL 25 mg daily" on a separate line; what should you do BEST? **A:** Flag it for pharmacist review as a potential duplicate, because HCTZ and HydroDIURIL are the same drug and duplications can lead to overdose.

2439. **Q:** A patient asks what "Plavix" is used for while you're ringing up the prescription; what is the MOST appropriate technician response? **A:** State that Plavix (clopidogrel) helps prevent blood clots and offer pharmacist counseling for more detail, because technicians may give basic indication but not provide full clinical counseling.
2440. **Q:** You are stocking bottles and notice a recall notice for valsartan and your shelf has matching NDC/lot; what should you do FIRST? **A:** Immediately pull the affected stock from dispensing and follow recall procedure (segregate and notify the pharmacist/manager), because recalled lots must be removed to prevent patient harm.
2441. **Q:** At drop-off, a patient asks if their new "Eliquis" replaces the "warfarin" still listed as active on their profile; what should you do FIRST? **A:** Pause processing and alert the pharmacist to assess anticoagulant duplication because taking apixaban and warfarin together increases bleeding risk.
2442. **Q:** While entering a new Rx for "Klonopin 0.5 mg," you see the patient is also filling "alprazolam 1 mg" from another prescriber; what is the BEST technician action? **A:** Flag the profile and refer to the pharmacist for therapeutic duplication review because concurrent benzodiazepines raise sedation and misuse risk.
2443. **Q:** A prescription is written for "cephalexin," and the patient reports a history of anaphylaxis to penicillin; what should you do NEXT? **A:** Stop and notify the pharmacist before filling because severe beta-lactam allergy history requires pharmacist evaluation and possible prescriber contact.
2444. **Q:** During data entry you receive "methotrexate 2.5 mg: take 1 tablet daily for rheumatoid arthritis"; what should you do FIRST? **A:** Hold the prescription and immediately route to the pharmacist because methotrexate is typically weekly and daily dosing is a high-alert error.
2445. **Q:** You are selecting product for "hydralazine 25 mg" and notice "hydroxyzine 25 mg" is stored nearby; what should you do BEST to prevent a LASA error? **A:** Use barcode/NDC verification and read back drug name/strength before filling because hydralazine/hydroxyzine confusion is a common safety trap.
2446. **Q:** A patient brings an OTC bottle and asks if "Aleve" is the same as "naproxen" listed on their profile; what should you answer? **A:** Confirm Aleve is naproxen (an NSAID) because correct brand/generic identification helps prevent duplicate NSAID therapy.

2447. **Q:** A prescription says "predniSONE 10 mg: take ii tabs po bid x 5 days"; what quantity should you enter to dispense? **A:** Dispense 20 tablets because $2 \text{ tablets per dose} \times 2 \text{ doses/day} \times 5 \text{ days} = 20$.
2448. **Q:** An Rx is for "amoxicillin/clavulanate suspension 400 mg/57 mg per 5 mL: take 10 mL BID for 7 days"; what volume should you dispense? **A:** Dispense 140 mL because $10 \text{ mL} \times 2 \text{ doses/day} \times 7 \text{ days} = 140 \text{ mL}$.
2449. **Q:** A prescriber sends "BuSpar 10 mg" but the patient insists they take "buspirone"; what should you do NEXT? **A:** Reassure that BuSpar is buspirone and continue processing while documenting as needed because they are brand/generic equivalents.
2450. **Q:** At verification handoff, you notice the label reads "metFORMIN ER" but the hard copy says "metFORMIN IR"; what should you do FIRST? **A:** Stop the fill and notify the pharmacist to correct before dispensing because IR vs ER is a formulation change that affects dosing and safety.
2451. **Q:** While typing a new Rx for "Lasix 40 mg," the patient says they also take "furosemide 40 mg" from another pharmacy; what should you do FIRST? **A:** Pause entry, confirm both names are the same drug, and alert the pharmacist to prevent therapeutic duplication and dosing confusion.
2452. **Q:** A prescriber e-sends "Zocor 80 mg daily" for a new start and the profile shows no prior simvastatin fills; what is the BEST technician action? **A:** Stop and refer to the pharmacist because simvastatin 80 mg has high myopathy risk and often requires clinical appropriateness review.
2453. **Q:** At pickup, a patient on "lisinopril" asks if they can take OTC ibuprofen regularly for back pain; what should you do NEXT? **A:** Refer to the pharmacist because NSAIDs can reduce antihypertensive effect and increase kidney risk, requiring counseling.
2454. **Q:** You receive an Rx for "Levemir FlexTouch: inject 22 units SC nightly" and the box contains five 3 mL pens (100 units/mL); what days' supply should you bill? **A:** Bill 68 days because $5 \text{ pens} \times 3 \text{ mL} \times 100 \text{ units/mL} = 1500 \text{ units}$, and $1500 \div 22 \approx 68 \text{ days}$.
2455. **Q:** During product selection for "Lamictal 25 mg," you accidentally grabbed "Lamisil 250 mg"; what should you do FIRST? **A:** Stop filling and reselect using barcode/NDC verification because this is a high-risk LASA mix-up (lamotrigine vs terbinafine).

2456. **Q:** A patient drops off an Rx for "nitrofurantoin" and says they have a severe allergy to "Macrobid"; what should you do FIRST? **A:** Hold the prescription and notify the pharmacist because Macrobid is nitrofurantoin and could represent a true allergy conflict.
2457. **Q:** An e-Rx arrives for "Adderall 20 mg: take 1 tab BID #60" but the prescriber's DEA number field is blank; what should you do NEXT? **A:** Stop processing and route to the pharmacist because controlled substance dispensing requires valid prescriber identifiers before filling.
2458. **Q:** You are entering "azithromycin 250 mg: take 2 tabs today then 1 tab daily x 4 days"; what quantity should you dispense? **A:** Dispense 6 tablets because the sig totals $2 + (1 \times 4) = 6$.
2459. **Q:** A patient picking up "sertraline" asks whether it's for depression or pain like their friend's "gabapentin"; what is the MOST appropriate technician response? **A:** State that sertraline is commonly used for depression/anxiety (not a pain medicine like gabapentin) and offer pharmacist counseling for questions about effects and side effects.
2460. **Q:** While scanning inventory, you find a sealed bottle of "Januvia" that was left on the counter overnight with no documentation of storage conditions; what should you do BEST? **A:** Remove it from dispensing stock and follow pharmacy policy with pharmacist notification because unknown storage handling can compromise product integrity.
2461. **Q:** While typing a new prescription for "Celebrex 200 mg daily," the patient profile shows an active "celecoxib 200 mg daily" from last month; what should you do FIRST? **A:** Stop and compare the duplicate entries, then alert the pharmacist if it appears to be therapeutic duplication to prevent double therapy from brand/generic overlap.
2462. **Q:** A hard copy reads "hydroxyzine 25 mg 1 tab TID prn anxiety," but the drop-down selection you clicked was "hydralazine 25 mg"; what is the BEST next step? **A:** Stop filling and correct the drug selection (verify name/indication and NDC/barcode) because this LASA mix-up can cause serious harm if dispensed.
2463. **Q:** At intake, a patient says they are on "Eliquis" and you see a new e-Rx for "warfarin 5 mg daily"; what should you do NEXT? **A:** Flag the potential duplicate anticoagulant therapy and refer to the pharmacist for review because combining agents raises bleeding risk.

2464. **Q:** You receive an order for “clopidogrel 75 mg daily” and the patient asks if it’s the same as “Plavix”; what is the MOST appropriate technician response? **A:** Confirm that Plavix is the brand name for clopidogrel and offer pharmacist counseling for clinical questions because technicians can provide basic brand/generic identification.
2465. **Q:** A new prescription is written for “Bactrim DS” and the allergy field lists “sulfa—anaphylaxis”; what should you do FIRST? **A:** Hold processing and notify the pharmacist immediately because sulfamethoxazole/trimethoprim is a sulfonamide antibiotic and may trigger a severe reaction.
2466. **Q:** A patient presents “prednisone 10 mg: take ii tabs daily x 5 days”; what quantity should you dispense? **A:** Dispense 10 tablets because ii = 2 tablets per day and $2 \times 5 \text{ days} = 10 \text{ tablets}$.
2467. **Q:** While selecting “glipizide 10 mg,” you notice the shelf also has “glyburide 5 mg” stored nearby and the patient is elderly; what should you do BEST before proceeding? **A:** Carefully verify the exact drug and strength against the Rx and scan the barcode/NDC because sulfonylureas are high-risk for hypoglycemia and are commonly confused.
2468. **Q:** A prescriber sends “Zithromax Z-Pak” directions: “2 tablets on day 1, then 1 tablet daily on days 2–5”; what is the correct total quantity to dispense? **A:** Dispense 6 tablets because the regimen is 2 tablets on day 1 plus 1 tablet for 4 additional days ($2 + 4 = 6$).
2469. **Q:** A patient dropping off “atorvastatin 40 mg daily” says they already take “Crestor” at home but aren’t sure of the strength; what should you do FIRST? **A:** Ask for the current medication details and route to the pharmacist to assess duplicate statin therapy because taking two statins increases adverse effect risk.
2470. **Q:** During receiving, a tote includes insulin pens with a temperature indicator showing possible heat exposure during transit; what should you do BEST? **A:** Set the shipment aside per policy and notify the pharmacist/manager for investigation because temperature excursions can make insulin ineffective and it may be non-dispensable.
2471. **Q:** A patient asks if “Lasix” is the same as their “furosemide” bottle at home; what should you do BEST? **A:** Confirm Lasix is the brand name for furosemide and offer to have the pharmacist counsel if they have therapy questions, because technicians can clarify brand/generic but not provide clinical advice.

2472. **Q:** While entering an e-Rx for "metformin ER 500 mg daily," you accidentally select "metformin IR 500 mg" in the system; what should you do NEXT? **A:** Stop and correct the dosage form to ER before labeling/printing because IR vs ER is a common high-impact error that changes how the drug releases.
2473. **Q:** A new e-Rx arrives for "metoprolol succinate 50 mg daily," but the patient profile shows "metoprolol tartrate 50 mg BID" active; what should you do FIRST? **A:** Pause processing and alert the pharmacist to possible therapeutic duplication/formulation mix-up because tartrate vs succinate are not interchangeable.
2474. **Q:** A patient picking up "lisinopril" says they're pregnant and asks if it's safe; what is the MOST appropriate technician action? **A:** Immediately refer to the pharmacist for counseling and do not give safety advice, because ACE inhibitors can be harmful in pregnancy and require pharmacist intervention.
2475. **Q:** You receive a prescription for "tramadol 50 mg" with "refills: 5," and the patient wants it filled today; what should you do BEST? **A:** Hold the prescription and route it to the pharmacist because tramadol is a Schedule IV controlled substance and refill limits/validity must be verified before dispensing.
2476. **Q:** During product selection you notice "Lamictal" (lamotrigine) and "Lamisil" (terbinafine) stored next to each other and the label prints "lamotrigine 100 mg"; what should you do NEXT? **A:** Use barcode/NDC verification and visually re-check the stock bottle against the label because this is a LASA pair and mix-ups can cause serious harm.
2477. **Q:** A prescription reads "levothyroxine 0.075 mg daily"; what strength should you enter in mcg? **A:** Enter 75 mcg because $0.075 \text{ mg} \times 1000 \text{ mcg/mg} = 75 \text{ mcg}$, preventing decimal-point dosing errors.
2478. **Q:** A patient says they take "Adderall" and a new e-Rx is for "amphetamine-dextroamphetamine ER 20 mg"; what is the BEST technician response? **A:** Explain that amphetamine-dextroamphetamine ER is a common generic for Adderall XR and offer pharmacist counseling for any dosing/use questions, because technicians can address brand/generic identification only.
2479. **Q:** An e-Rx is written for "cephalexin 500 mg" and the allergy profile lists "penicillin—anaphylaxis"; what should you do FIRST? **A:** Stop processing and notify the pharmacist to assess cross-reactivity risk because severe allergy histories require pharmacist review before dispensing.

2480. **Q:** A prescription is for "azithromycin 250 mg: take 1 tablet daily for 10 days"; how many tablets should you dispense? **A:** Dispense 10 tablets because $1 \text{ tablet/day} \times 10 \text{ days} = 10$, ensuring the quantity matches the intended duration of therapy.
2481. **Q:** A new e-Rx is for "hydroxyzine 25 mg TID PRN anxiety," but you accidentally select "hydralazine 25 mg" during data entry; what should you do NEXT? **A:** Stop the entry and correct to hydroxyzine before printing/filling because hydralazine is a different drug (LASA) and could cause serious harm.
2482. **Q:** A patient picking up "clopidogrel" asks what it's for since they thought they were on "Plavix"; what should you do BEST? **A:** Explain that Plavix is the brand name for clopidogrel and confirm they expected an antiplatelet for heart/stent protection because brand/generic clarification prevents duplicate or missed therapy.
2483. **Q:** You receive an e-Rx for "sertraline 50 mg daily," and the patient profile shows "paroxetine 20 mg daily" active; what should you do FIRST? **A:** Pause processing and alert the pharmacist to review possible therapeutic duplication/SSRI switch because technicians should not assume cross-tapers or medication changes.
2484. **Q:** A prescription is written "predniSONE 10 mg: take ii tabs daily for 5 days"; what quantity should you enter? **A:** Enter 10 tablets because ii means 2 tablets daily and $2 \times 5 \text{ days} = 10$.
2485. **Q:** While counting, you notice the stock bottle label says "glipiZIDE" but the shelf tag and NDC on the open bottle match "glyBURIDE"; what should you do FIRST? **A:** Stop filling and verify the NDC/label against the prescription and shelf stock, then set the mismatched bottle aside for pharmacist review because sulfonylureas are high-risk and look-alike names are common.
2486. **Q:** A patient asks if they can take "ibuprofen" with their new "lisinopril" prescription for back pain; what is the MOST appropriate technician action? **A:** Refer the question to the pharmacist because NSAIDs can reduce ACE inhibitor effect and raise kidney risk, requiring clinical counseling.
2487. **Q:** A new e-Rx is for "apixaban 5 mg BID," and the patient profile shows "warfarin" still active from last month; what should you do BEST? **A:** Hold and notify the pharmacist to assess anticoagulant duplication because taking both can greatly increase bleeding risk and requires pharmacist intervention.

2488. **Q:** A prescription says "amoxicillin/clavulanate 875/125 mg 1 tab BID x 7 days"; how many tablets should you dispense? **A:** Dispense 14 tablets because BID for 7 days equals 2 tablets/day $\times 7 = 14$.
2489. **Q:** During order entry for "levETIRAcetam 500 mg BID," you see the prescriber note "dispense Keppra," but the plan prefers generic; what should you do NEXT? **A:** Process as generic levetiracetam unless DAW is required and follow pharmacy/plan rules, because Keppra is the brand and generic substitution is typically permitted.
2490. **Q:** A patient presents a new prescription for "alprazolam 0.5 mg TID PRN" and asks for transfers/refills to be placed on it automatically; what should you do BEST? **A:** Explain you can't promise automatic transfers/refills on a controlled substance and route to the pharmacist for any controlled-substance policy questions because benzodiazepines have stricter dispensing controls.
2491. **Q:** A prescriber sends "metFORMIN ER 500 mg: take 1 tablet BID" but you only see metformin IR 500 mg on the shelf; what should you do NEXT? **A:** Stop and select metformin ER with the correct NDC (or order it) and alert the pharmacist if needed because IR and ER are not interchangeable and changing formulation is not within technician scope.
2492. **Q:** A patient drops off a new Rx for "levothyroxine (Synthroid) 75 mcg daily" and asks what it's for; what should you do BEST? **A:** Refer the counseling question to the pharmacist and you may add that levothyroxine is commonly used for hypothyroidism because medication education requires pharmacist counseling.
2493. **Q:** While entering "Lamictal 25 mg," you notice the prescriber wrote "lamoTRIgine" but the product you pulled is "lamoTRIgine ODT"; what should you do FIRST? **A:** Stop and re-select the exact dosage form on the prescription (tablet vs ODT) and route to the pharmacist if unclear because dosage forms can change administration and are not automatically substitutable.
2494. **Q:** A new e-Rx is for "fluoxetine 20 mg daily," and the patient profile shows citalopram 20 mg daily still active; what should you do FIRST? **A:** Hold processing and notify the pharmacist to assess duplicate SSRI therapy and potential interaction/therapy change because technicians should not decide which antidepressant to continue.
2495. **Q:** A patient picking up "atorvastatin (Lipitor)" says they also take "simvastatin (Zocor) at home"; what is the BEST technician action? **A:** Pause the pickup and

alert the pharmacist to evaluate therapeutic duplication and statin safety because taking two statins increases risk of adverse effects.

2496. **Q:** A prescription reads "amLODIPine 5 mg: take 1 tab PO qd #90," but you accidentally typed "amiodarone 200 mg"; what should you do NEXT? **A:** Immediately correct the entry by verifying the drug name/strength against the original Rx and re-scan the stock bottle/NDC because this is a high-risk LASA error that must be fixed before verification.
2497. **Q:** A patient brings in a 10 mL bottle of timolol 0.5% eye drops with directions "instill 1 drop OU BID"; what days' supply should you bill using 20 drops per mL? **A:** Bill 50 days' supply because $10 \text{ mL} \times 20 \text{ drops/mL} = 200 \text{ drops}$ and 1 drop in each eye twice daily is 4 drops/day, so $200 \div 4 = 50$.
2498. **Q:** An e-Rx is for "nitrofurantoin 100 mg BID x 5 days"; what quantity should you dispense? **A:** Dispense 10 capsules because BID for 5 days equals 2 doses/day $\times$ 5 days = 10 total doses.
2499. **Q:** While stocking, you find a bottle of insulin aspart pens that feels warm and has no temperature excursion documentation; what should you do FIRST? **A:** Remove it from active stock and follow pharmacy policy to quarantine and notify the pharmacist/manager because insulin is temperature sensitive and suspect storage can compromise potency.
2500. **Q:** A new prescription is for "oxycodone/acetaminophen 5/325 mg 1 tab q6h PRN pain #12," and the patient asks you to add refills; what should you do BEST? **A:** Explain that refills can't be added by the pharmacy and refer to the pharmacist/prescriber because this is a Schedule II controlled substance and technicians cannot modify controlled-substance authorizations.